HEALTH AND FAMILY WELFARE DEPARTMENT





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CHAPTER – 1

INTRODUCTION

உற்றவன் தீர்ப்பான் மருந்துழைச் செல்வானென்று
அப்பால் நாற்கூற்றே மருந்து.

– திருக்குறள் (950)

"The patient, the physician, the medicine and the caregiver — Healthcare rests on these four interconnected pillars."

More than two thousand years ago, Poet Thiruvalluvar described healthcare not as the work of a single healer alone, but as a shared responsibility involving four essential participants. This principle remains remarkably relevant today. Modern health systems, despite their scale and complexity, continue to depend on the same truth: healthcare begins with relationships — between people, families, communities and those who provide care.

This understanding is also reflected in the World Health Organization's definition of Health as "a state of complete physical, mental and social well-being and not merely the absence of disease or infirmity." Health, therefore, cannot be viewed

through a clinical lens alone as it is closely tied to dignity, opportunity and quality of life. It shapes how individuals participate in society and contribute to the social and economic progress of communities, States and Nations.

Tamil Nadu approaches healthcare guided by the principles of Universal Health Coverage (UHC) and stands committed to ensuring equitable access to comprehensive healthcare services - Promotive, Preventive, Curative, Rehabilitative and Palliative - for every person throughout the life course, without barriers and without exclusion.

1.1. Tamil Nadu's Public Health Compact

Tamil Nadu's leadership in Public Health governance stems from decades of sustained investment in the foundations of the Health System, Infrastructure, Human Resources, Essential Medicines, Disease Surveillance and Social Protection. The State views health spending as developmental capital rather than recurring expenditure. This approach has made Tamil Nadu a National benchmark in Maternal and Child health, Emergency medical services, Organ transplantation, Drug procurement and Health financing.

This compact is anchored in five enduring principles:

- **Equity** - No citizen is left behind.
- **Accessibility** - Care is brought to the doorstep.
- **Quality** - Services meet standards maintained uniformly across the public network.
- **Affordability** - Financial hardship is never the price of seeking care.
- **Continuity -** Care follows the citizen; they are reminded and reached out to, not merely treated on demand.

These principles guide Tamil Nadu towards the achievement of Sustainable Development Goals and position the State as a model for Nation wide replication.

1.2. A State in Demographic Transition

The National Commission on Population Report has projected that Tamil Nadu's population will reach 7.75 crore in 2026. The State is among the most urbanised in India, with around 50% of

it’s people residing in urban areas. A strong industrial and economic base also attracts a substantial migrant workforce from across the country. Together, rapid urbanisation and population mobility create challenging healthcare demands, particularly in urban, peri-urban and industrial regions.

At the same time, Tamil Nadu is undergoing a profound demographic transition. With a Total Fertility Rate (TFR) of 1.3, well below the replacement level and steadily increasing life expectancy, the State is now an ageing society. According to the National Family Health Survey (NFHS-6, 2023–24), older adults constitute 16.3 percent of the population. Further, the findings from the Longitudinal Ageing Study in India (LASI) indicate a growing burden of chronic diseases, functional limitations and care dependency among the elderly.

This demographic shift requires the health system to respond to the growing demand for long-term management of Non-Communicable Diseases, Geriatric services, Rehabilitative care and sustained support for healthy ageing.

1.3. The Epidemiological Landscape

1.3.1. The Non-Communicable Disease Burden

Tamil Nadu's sustained investments in Public Health have yielded significant gains in the control of communicable diseases and improvements in population health. As a result, the State's disease burden has undergone a marked epidemiological transition. Today, Non-Communicable Diseases (NCDs) constitute the leading cause of mortality, morbidity and disability, presenting one of the most significant challenges to the health system.

According to the Global Burden of Disease Study 2023, nearly 75% of all deaths in Tamil Nadu are attributable to NCDs, with Cardiovascular diseases, Cancers, Diabetes and Chronic Respiratory Diseases accounting for the majority of premature mortality and disability-adjusted life years lost.

The National Family Health Survey (NFHS-6, 2023–24) places Tamil Nadu among the States with the highest burden of major NCD risk factors and conditions in the country.

Indicator	Men (%)	Women (%)
Hypertension prevalence	28.2	24.2
Diabetes prevalence	26.7	25.2
Overweight / Obesity	38.8	44.2

These findings are further corroborated by the ICMR–National Institute of Epidemiology Round II Survey (2023–24), which reported a prevalence of 29.4% for Hypertension and 19.5% for Diabetes among adults.

The increasing prevalence of overweight and obesity, driven by urbanization, changing dietary habits and declining levels of physical activity, is expected to further accelerate the NCD burden in the coming decade. While Tobacco use has declined over time, its prevalence remains substantial among rural men. Harmful alcohol consumption continues to contribute significantly to both NCDs and injuries and remains inadequately captured through routine surveillance systems.

Recognizing the magnitude of this challenge, Tamil Nadu has established a comprehensive

continuum of care for NCD prevention and management, encompassing population-based screening, early diagnosis, treatment, follow-up, complication management and long-term disease control.

At the centre of this response is the "*Makkalai Thedi Maruthuvam (MTM)"* programme, which has taken healthcare delivery directly to the doorstep of people. Through population-based screening, home-based follow-up, doorstep delivery of medicines for Hypertension and Diabetes, Physiotherapy Services, Palliative care including support services for patients requiring Continuous Ambulatory Peritoneal Dialysis (CAPD), the programme has strengthened continuity of care and improved access to services, particularly for vulnerable populations.

To address the complications associated with Diabetes and Hypertension, the State has implemented targeted interventions focused on early detection and timely management. The *Padham Paadhukappom Thittam* provides comprehensive screening and treatment for Diabetic foot conditions, while structured screening

protocols for renal and retinal complications facilitate the early detection of Chronic Kidney Disease and Diabetic Retinopathy.

These interventions are integrated with the State's Primary healthcare network and the Chief Minister's Comprehensive Health Insurance Scheme (CMCHIS), ensuring seamless referral pathways, financial protection for patients requiring advanced care and improved access to treatment and follow-up services across all levels of care.

Recognizing the health needs of the industrial workforce, Tamil Nadu has also launched the "*Thozhilaalargalai Thedi Maruthuvam* scheme", which delivers preventive, screening and chronic disease management services directly to workers. The initiative extends healthcare access to migrant and industrial populations, ensuring that these often underserved groups remain integrated within the State's NCD care framework.

Having established robust systems for population-based screening, early diagnosis and continuity of care, Tamil Nadu is now advancing to

the next phase of NCD control. The State's focus is shifting from measuring screening coverage and case detection towards achieving and monitoring disease control rates.

This transition reflects a strategic commitment to improving treatment outcomes, reducing complications, minimizing premature mortality and enhancing the quality of life of individuals living with chronic diseases.

1.3.2. Mental Health and Suicide Prevention

Mental health has emerged as a major Public Health priority, requiring a comprehensive and health-system-based response. According to the National Crime Records Bureau (NCRB-2024), the State recorded 19,965 suicide deaths, accounting for 11.7 per cent of the National total and representing a 2.5 per cent increase over the previous year.

However, the burden of Mental illness extends beyond suicide. The National Mental Health Survey (NMHS 2015–16) estimated that 11.8% of adults in Tamil Nadu have a Mental health condition, with a substantial treatment gap.

Recognizing these challenges, Tamil Nadu has adopted a continuum-of-care approach to mental health, integrating Promotion, Prevention, Treatment and Rehabilitation within the Public Health system through the systems described below;

- Expansion of the District Mental Health Programme to the grassroots through Health and Wellness Centres.
- De-addiction services and Emergency Care and Recovery Centres at secondary and tertiary levels.
- Tele Manas (14416) - a 24/7 crisis intervention helpline.
- The MaNaM programme for students, addresses Mental health issues in educational settings.
- Specialized tertiary care at the Institutes of Mental Health (Kilpauk and Theni) and the Tamil Nadu Institute of Mental Health and Neurosciences (TNIMHANS).

To further strengthen access and financial protection, Mental Health services are being

progressively incorporated under the Chief Minister's Comprehensive Health Insurance Scheme (CMCHIS), reinforcing the State's commitment to ensuring that Mental healthcare is an integral component of Universal Health Coverage.

1.3.3. Road Traffic Accidents and Trauma Care

Trauma and injuries, especially Road Traffic Accidents (RTAs) represent a major and largely preventable Public Health challenge in Tamil Nadu, contributing significantly to premature mortality, disability and economic loss. According to the National Crime Records Bureau (NCRB 2024), the State recorded 18,449 RTA deaths, translating to nearly 24 deaths per lakh population. As a leading cause of death among the economically productive population, Road Traffic Injuries require a comprehensive response that extends beyond emergency care to encompass prevention, timely treatment, rehabilitation and financial protection.

The Government will strengthen the inter-departmental convergence across Engineering, Education as well as Police Departments to design

and implement awareness programmes on preventive measures to be taken as well as ensuring faster response for incidents of Road Traffic Accidents.

Tamil Nadu has established a robust emergency and trauma care network under the Tamil Nadu Accident and Emergency Initiative (TAEI), covering 113 Government Hospitals and supported by 2,099 specialised ICU and Hybrid High Dependency Unit beds. Complementing this network, the State's '108' Ambulance Service maintains an average response time of 9 minutes and 35 seconds, ensuring timely access to life-saving care.

To strengthen survival outcomes and protect families from catastrophic expenditure during the most critical period following an accident, the Government has implemented the "*Innuyir Kappom – Nammai Kaakkum 48 Thittam"*, which provides cashless treatment coverage of up to Rs.2 lakh for emergency stabilisation during the first 48 hours across 773 empanelled hospitals. Together, these interventions have significantly enhanced Tamil Nadu's capacity to provide timely, accessible and equitable trauma care.

1.3.4. Cancer Care

Cancer is a major and emerging Public Health priority in Tamil Nadu and a significant contributor to premature mortality and disability. The Tamil Nadu Cancer Registry Project (TNCRP 2022) identifies cancer among the top five causes of death in the State, with Breast, Cervical, Oral, Colorectal and Stomach cancers accounting for a substantial proportion of the disease burden. Despite advances in diagnosis and treatment, a significant challenge remains, with many cancers continuing to be detected at advanced stages, limiting treatment effectiveness, increasing healthcare costs and adversely affecting survival outcomes.

Recognizing the need for a comprehensive Cancer control strategy, Tamil Nadu has adopted a continuum-of-care approach encompassing prevention, early detection, treatment, survivorship and palliative care. The State's response focuses on three strategic pillars:

- **Prevention** – HPV vaccination for adolescent girls, Tobacco cessation initiatives and Risk factor modification.

- **Early Detection and Treatment** – Strengthening population-based screening for Breast, Cervical and Oral cancers, while expanding access to diagnostics, Oncology services for Chemotherapy, Radiotherapy & Surgery, referral pathways and survivorship support.
- **Institutional Capacity Building** – Leveraging the Hub-and-Spoke Virtual Tumour Board model to extend Oncology expertise available at Tertiary care level to peripheral institutions through specialist consultations and structured mentoring.

Through these interventions, Tamil Nadu aims to improve early detection rates, reduce cancer-related mortality, strengthen equitable access to quality Oncology care and enhance survival and quality of life for individuals affected by cancer.

1.4. Medical Facilities: Reaching Every Corner

Tamil Nadu's healthcare system is organized through a three-tier network of Primary, Secondary and Tertiary care facilities that ensures equitable access to healthcare services across rural, urban, tribal and underserved regions.

The State's Primary healthcare network comprises 2,336 Primary Health Centres, including 1,408 Additional Primary Health Centres, 424 Upgraded 30-bedded Primary Health Centres, 465 Urban Primary Health Centres and 39 Urban Community Health Centres. These facilities are further supported by 9,330 Rural Health Sub-Centres and 708 Urban Health and Wellness Centres, collectively delivering a comprehensive package of Promotive, Preventive, Curative, Rehabilitative and Palliative healthcare services closer to communities.

This network is complemented by 37 District Headquarters Hospitals, 199 Taluk Hospitals and 59 Non-Taluk Hospitals providing specialist and inpatient care. At the tertiary level, Tamil Nadu has established a robust referral care and medical education ecosystem comprising 36 Government Medical Colleges. Guided by the State's vision of establishing a Government Medical College in every district, Medical Colleges are now functional in all but six districts, with concerted efforts underway to achieve universal district-level access to advanced healthcare, specialist services and medical

education. Further, 426 Hospitals-on-Wheels extend healthcare services to remote, tribal and hard-to-reach populations, reaffirming Tamil Nadu's commitment that access to quality healthcare should not be constrained by geography.

1.5. Indian Systems of Medicine: Heritage Woven into Healthcare

Tamil Nadu is the birthplace of Siddha, one of the world's oldest living medical traditions. The State has institutionalized this heritage through a network of 1,660 co-located Siddha and Indian System of Medicine (ISM) units, integrating traditional knowledge with modern Public Health delivery.

A dedicated Siddha University and the upgrade of the first Siddha Medical College at Palayamkottai to a National Centre of Excellence provide required scientific support. AYUSH practitioners are being integrated into Primary healthcare and the preventive and wellness benefits of Indian systems are also being used for chronic disease management, elderly care and lifestyle modification.

1.6. Ensuring Financial Protection as a Public Health Commitment

Reducing Out-of-Pocket Expenditure (OOPE) for health care remains a cornerstone of Tamil Nadu's commitment to Universal Health Coverage. As per the NSSO, Medicines and diagnostics constitute the largest share of healthcare expenditure borne by households, often resulting in delayed care-seeking and financial hardship. Tamil Nadu has addressed this challenge through a combination of assured access to free medicines, diagnostics and comprehensive financial risk protection.

At the centre of this strategy, the Tamil Nadu Medical Services Corporation (TNMSC) is a globally recognized model for public procurement and supply chain management. Through centralized procurement and distribution of Essential Drug List (EDL) medicines, specialty drugs, laboratory reagents, consumables and medical equipment, TNMSC leverages economies of scale to reduce procurement costs while ensuring uninterrupted availability across Public Health facilities. In addition, TNMSC supports

access to high-cost diagnostics through the operation of CT and MRI centres in Government Hospitals and the centralized procurement of laboratory reagents under the Free Diagnostic Initiative, significantly reducing the financial burden on patients and improving access to quality diagnostic services.

Complementing these efforts, the Chief Minister's Comprehensive Health Insurance Scheme (CMCHIS) serves as Tamil Nadu's flagship financial protection mechanism, shielding vulnerable families from catastrophic health expenditure. Operating under an enhanced annual income eligibility ceiling of Rs.1.20 lakh, the scheme has financed treatments worth Rs.1,329 crore for over 10.25 lakh beneficiaries during 2025–26. As the State's disease burden increasingly shifts towards Non-Communicable Diseases, Cancer, Mental Health conditions and long-term care needs, CMCHIS is being progressively expanded to support a broader continuum of care, ensuring that financial protection accompanies citizens throughout the course of illness and not merely during episodes of hospitalization.

1.7. The Road Ahead: A Health System equal to its Promise

Tamil Nadu enters 2026–27 with a health sector allocation of Rs.23,357 crore and a strong foundation of Public Health achievements. However, the State stands at a critical juncture in its epidemiological and demographic transition. While sustained investments have delivered substantial gains in Maternal and Child Health, Communicable disease control and healthcare access, the growing burden of Non-Communicable Diseases, Cancer, Mental health conditions, Injuries and Population ageing necessitates a reorientation of the health system. Future investments will focus on strengthening preventive and promotive health services, improving continuity of care and enhancing long-term health outcomes.

The next phase of health sector reforms will prioritize comprehensive Non-Communicable Disease control, with focused attention on Cardiovascular diseases, Stroke, Cancer, Diabetes and Chronic Kidney Disease. The emphasis will progressively shift from screening and case

detection towards improving disease control, reducing complications, enhancing disability-free survival and improving quality of life. Simultaneously, the State will strengthen organized cancer screening, trauma care and rehabilitation services to address the evolving disease burden.

Rapid population ageing is creating new demands for Geriatric, Long-term, Rehabilitative and Palliative care services. In response, Tamil Nadu will strengthen a continuum of healthy ageing interventions that integrates facility-based, community and home-based care. Mental health will similarly be mainstreamed within Universal Health Coverage through strengthened systems for early identification, suicide prevention, de-addiction services, treatment, psychosocial support and community-based rehabilitation.

Recognizing that health gains cannot be achieved through healthcare services alone, the State will place greater emphasis on community participation, health literacy and Citizen Engagement. Public Health programmes will increasingly promote healthy behaviours, shared

responsibility for health and stronger partnerships with communities, local governments, academic institutions and development partners.

A resilient health system requires a competent, motivated and equitably distributed workforce. To address challenges related to workforce availability, skill mix, deployment, retention and governance, Tamil Nadu is developing a Comprehensive Human Resources for Health Strategy under the National Health Mission, covering workforce production, recruitment, deployment, professional development, welfare and long-term planning.

Quality of care will remain a central pillar of health system strengthening. The State will continue to advance clinical governance, patient safety, quality assurance and service standardization across all levels of care, while promoting respectful care, accountability and patient-centred services. Strengthening quality across the continuum of care will improve public trust, increase utilization of Public Health services and further reduce out-of-pocket expenditure.

As per the principles, Tamil Nadu ensures a equitable patient-centered services with improved access through patient friendly systems promoting respectful and responsive patient experiences. The financial burden is reduced through free entitlements and seamless referral and follow-up with continuity of care.

Primary Health Centres where Tamil Nadu has set a model for the country shall continue to be the cornerstone of public health delivery. Each Health Sub Centre and PHC shall be mapped to a denominator population to set verifiable accountability standards in Public Health delivery. This precise mapping will facilitate highly targeted health interventions and optimal resource allocation, ensuring that proactive, universal care reaches every single household without exception.

Digital Health will be fully leveraged to improve effective service delivery. Telemedicine will be one of the critical components of the State's digital initiatives, to improve access in the hinterlands, outcomes and care continuum. A unified data spine shall be established across the

healthcare ecosystem. HMIS 3.0 (e-Nalam) shall act as the core platform. It shall integrate all critical health sector datasets into a centralised single source of data. This shall ease access to healthcare for the common citizen and ensure continuity of care.

The Government will keep pace with advances in technology and adopt them as they mature. Artificial Intelligence will be introduced in a measured and purposeful manner. The State shall build AI services that are secured by design. Data protection, privacy, consent and auditability shall be built in. These services shall be directed at two clear outcomes:

- **Better quality of care** — clinical decision support for treating physicians, earlier identification of risk and structured follow-up of patients along the care continuum.
- **Better decisions —** actionable insights drawn from inter-linked metrics spanning facilities, programmes, supply chains, human resources and patient feedback.

Technology shall remain a means and not an end. Every digital and AI initiative shall be measured by a single test — whether it improves the care a citizen actually receives.

Tamil Nadu's public health tradition, rooted in a two-thousand-year-old understanding of healthcare as a shared responsibility, continues to guide the State's vision for the future. Building on this legacy, Tamil Nadu remains committed to strengthening a health system that is equitable, resilient, people-centred and responsive to emerging health challenges. Through sustained investments in Public Health, Primary care, quality improvement, financial protection and health system strengthening, the State seeks to ensure that quality healthcare remains accessible to all.

CHAPTER – 2

HEALTH ADMINISTRATION

2.1 The Department of Health & Family Welfare oversees a diverse range of institutions dedicated in providing comprehensive healthcare services that meet the ever-growing demands in the areas of preventive, curative and promotive health, Medical Education, Human Resources Recruitment, Food Safety, Procurement, etc.,

A short gist of the various Directorates, Boards and Corporations functioning under the Department is listed below;

i) **Directorate of Medical Education and Research** - Responsible for Medical Education, Research and Tertiary care services.

ii) **Directorate of Medical and Rural Health Services** - Responsible for providing Secondary healthcare services and implementation of various healthcare-related Acts and Regulations.

iii) **Directorate of Public Health and Preventive Medicine -** Responsible for providing Primary Healthcare Services, disease prevention, health promotion and wide range of public health initiatives to safeguard the health of the community.

iv) **Directorate of Indian Medicine and Homoeopathy -** Responsible for promoting Medical Education and Health care under AYUSH (Ayurveda, Yoga and Naturopathy, Unani, Siddha and Homoeopathy).

v) **Directorate of Family Welfare** - Implements family planning and reproductive health-related initiatives in-order to promote Maternal and Child health.

vi) **Commissionerate of Food Safety and Drugs Control -** Focuses on ensuring food safety, observing quality determination & standards and administration of drug control regulations across the State. The Directorate of Drugs Control comes under the administrative control of the Commissionerate of Food safety and Drugs Administration.

vii) **Directorate of Tamil Nadu State Health Transport** – Vested with the responsibility for the maintenance and management of vehicles utilised by healthcare institutions under the Health and Family Welfare Department.

viii) **Medical Services Recruitment Board (MRB)** - Overall responsible for transparent and efficient recruitment of personnel for various healthcare cadres within the department.

ix) **Tamil Nadu Medical Services Corporation (TNMSC)** - Streamlines end-to-end processes behind the procurement, supply and maintenance of drugs and medical equipment for Government Medical Institutions.

x) **Tamil Nadu Medicinal Plant Farms and Herbal Medicine Corporation Limited (TAMPCOL)** – Plays a key role in the manufacture of medicines based on traditional Indian systems of medicine and distributes them for the use of health care institutions across the State.

Additionally, the Directorate of Medical and Rural Health Services (ESI) under the Labour Welfare and Skill Development Department is supported with health staff from the Health and Family Welfare Department, illustrating the collaborative efforts that are required towards reaching out comprehensive healthcare coverage and service delivery across the State.

2.2 The State's health initiatives extend beyond the realms of various Directorates to include societies, missions and programmes that aim to address specific areas of the health system and its challenges including promotion of overall well-being.

Key initiatives implemented across different sectors:

i) **National Health Mission - State Health Society -** National Health Mission (NHM) aims to provide accessible, affordable and quality Medical & Health care services to all citizens, with a primary focus on key areas such as Maternal and Child Health, immunization, communicable and non communicable disease control. NHM is the

designated Commissionerate of Maternal and Child Health Services for the State. NHM coordinates different National Programmes and ensures coordination across the health ecosystem.

ii) **Tamil Nadu Health System Project –** Tamil Nadu Health System Project (TNHSP) focusses on improving access, developing effective interventions to address the identified key health challenges, improving health outcomes through quality of service and increasing the efficiency and effectiveness of the public sector. TNHSP is designated as Commissionerate of Trauma Care for the State and is the nodal agency to implement Chief Minister's Comprehensive Health Insurance Scheme.

iii) **Tamil Nadu State AIDS Control Society** – Exclusively dedicated to the prevention and control of the spread of HIV/AIDS in the State, the Tamil Nadu State AIDS Control Society (TANSACS) implements wide range of programme for the prevention, care, support and treatment of People Living with

HIV/AIDS. The Society also oversees safe blood transfusion services across the State in coordination with the State Blood Transfusion Council.

iv) **Tamil Nadu Blindness Control Society (TNBCS)** – With focus on reducing the prevalence of preventable blindness and visual impairment, the Tamil Nadu Blindness Control Society (TNBCS) implements programmes for prevention, Promotion of eye health, treatment of eye diseases and rehabilitation services for the visually impaired.

v) **National Tuberculosis Elimination Programme (NTEP)** – NTEP works towards achieving the goal of eliminating TB in Tamil Nadu through early detection, treatment and prevention strategies, which include TB diagnosis and management, contact tracing and TB campaigns for awareness creation.

vi) **District Mental Health Programme** – Aimed at promoting Mental Health, prevention of mental illnesses and providing care and support for individuals with mental

health conditions, the National Mental Health Programme (NMHP) focuses on creating awareness on Mental Health, training of healthcare professionals and promoting access to Mental Health services in Tamil Nadu.

vii) **National Vector Borne Diseases Control Programme** – Targeting Vector borne diseases transmitted by vectors such as mosquitoes, the National Vector Borne Diseases Control Programme (NVBDCP) aims to prevent and control vector-borne diseases such as Malaria, Dengue and Chikungunya through vector control measures, disease surveillance and Medical Education.

viii) **Universal Immunization Programme** - The Universal Immunization Programme (UIP) is centred on the protection of children and adults against vaccine-preventable diseases by providing free vaccinations. Tamil Nadu's UIP focuses on achieving high vaccination coverage rates especially to prevent diseases of Public Health importance such as Polio, Measles, Rubella and Tetanus.

ix) National Leprosy Eradication Programme – a public health initiative focused on the complete control, early detection and free treatment of Leprosy across all Government health facilities of the State.

These are some of the programmes and initiatives which reflect Tamil Nadu's comprehensive approach to health care by addressing the challenges while promoting the well-being of its population through targeted interventions, partnerships and active community engagement.

2.3 The classification of health facilities in the State reflects the assorted range of healthcare institutions catering to the needs of the population as given below:

i) Government Health Institutions - These institutions are under the direct purview of the Government and includes:

1. **Health Sub Centres (HSCs)** - Catering to an average population of 5,000 in plain areas and 3,000 in hilly areas for providing Primary Healthcare services.

2. **Primary Health Centres (PHCs)** - Serving an average population of 30,000, in plain areas and 20,000 in hilly areas offers Primary Healthcare services and acts as referral points for Secondary and Tertiary care services.
3. **Urban Primary Health Centres (UPHCs)** - Located in Urban areas, including Chennai Corporation provides Primary Healthcare services.
4. **District Headquarters Hospitals, Taluk and Non-Taluk Hospitals** - Provide secondary healthcare services.
5. **Hospitals attached to Government Medical Colleges** - Provide tertiary and specialized healthcare services.

ii) **State-Special Medical Institutions** - These institutions serve specific populations, including:

1. Police Department hospitals.
2. State-owned corporations / undertakings hospitals.

3. Employees State Insurance Medical Institutions: Including ESI hospitals and dispensaries.

iii) **Medical Institutions under the Local Bodies** - Managed by Corporations and Municipalities to provide healthcare services primarily to Urban populations. It includes Urban Primary Health Centres, Urban Community Health Centres and Urban Health & Wellness Centre (UHWC).

Table 2.1: Government Health facilities in Tamil Nadu

Sl. No.	Description	Units
1.	Government Medical College Hospitals	36
2.	Hospitals attached with the Medical Colleges	67
3.	Tamil Nadu Government Multi Super Specialty Hospital	1
4.	Kalaignar Centenary Super Specialty Hospital	1
5.	Tamil Nadu Government Dental College and Hospitals	3

Sl. No.	Description	Units
6.	District Headquarters Hospitals	37
7.	Taluk Hospitals	199
8.	Non-Taluk Hospitals (NTK+WCH)	59
9.	Other Hospitals (DMS control)	28
10.	Primary Health Centres (PHCs)	1,832
11.	Health Sub Centres (HSCs)	9,330
12.	Urban Primary Health Centres (UPHCs) and Urban Community Health Centres including Greater Chennai Corporation	504
13.	Urban Health and Wellness Centres	708
14.	Employees' State Insurance (ESI) Hospitals	8
15.	ESI Dispensaries	241
16.	Indian System of Medicine – Medical Colleges	7
17.	Indian System of Medicine Hospitals and Dispensaries	1,660

The District wise details of Government Medical Health facilities in the State are as follows:

Table 2.2: District wise Health facilities

Sl. No	District	No. of Health Institutions				
		HSCs	PHCs	Government. Hospitals	Medical & Dental College Hospitals	Indian Medicine Hospitals & Dispensaries and Medical Colleges
1.	Ariyalur	125	40	3	1	24
2.	Chennai	0	159	16	5	34
3.	Chengalpattu	274	50	12	1	37
4.	Coimbatore	351	90	15	2	59
5.	Cuddalore	355	73	12	2	50
6.	Dharmapuri	256	52	6	1	39
7.	Dindigul	336	75	13	1	62
8.	Erode	312	78	10	1	61
9.	Kallakurichi	234	46	7	1	22
10.	Kancheepuram	194	29	4	-	27
11.	Kanniyakumari	272	48	10	1	63
12.	Karur	171	38	7	1	37
13.	Krishnagiri	275	63	6	1	37
14.	Madurai	316	90	12	1	51
15.	Mayiladuthurai	152	32	6	-	28
16.	Nagapattinam	112	28	6	1	26
17.	Namakkal	253	64	8	1	51
18.	The Nilgiris	194	39	9	1	34
19.	Perambalur	97	30	4	-	21

Sl. No	District	No. of Health Institutions				
		HSCs	PHCs	Government. Hospitals	Medical & Dental College Hospitals	Indian Medicine Hospitals & Dispensaries and Medical Colleges
20.	Pudukottai	255	77	16	2	50
21.	Ramanathapuram	248	60	11	1	52
22.	Ranipet	175	37	6	-	28
23.	Salem	446	108	15	1	75
24.	Sivaganga	289	54	20	1	52
25.	Thanjavur	341	78	16	1	53
26.	Theni	176	42	7	1	43
27.	Tenkasi	193	53	11	-	41
28.	Tiruchirappalli	319	86	14	1	61
29.	Tirunelveli	210	54	9	1	40
30.	Tirupathur	143	39	4	-	18
31.	Tiruppur	253	68	11	1	35
32.	Tiruvallur	342	69	13	1	54
33.	Tiruvannamalai	414	100	11	1	60
34.	Tiruvarur	197	51	9	1	42
35.	Thoothukudi	259	61	10	1	63
36.	Vellore	168	48	7	1	26
37.	Villupuram	356	67	8	1	44
38.	Virudhunagar	267	60	11	1	60
Total		**9,330**	**2,336**	**375**	**39**	**1,660**

The details of High-End Equipment in Government Health facilities in the State are given below:

Table 2.3: Specific High-end Equipment in the Districts

Sl. No.	District	MRI Scan	PETCT	CT Scan	LINAC	Cobalt therapy Machine	Brachytherapy Machine	Dialysis Machine	Digital X-ray	Cath lab
1.	Ariyalur	1	0	1	0	0	0	13	1	0
2.	Chennai	10	1	24	3	4	4	378	14	13
3.	Chengalpattu	1	0	3	0	0	0	51	1	1
4.	Coimbatore	2	1	6	1	1	1	40	3	2
5.	Cuddalore	2	0	3	0	0	0	48	1	1
6.	Dharmapuri	1	0	4	0	1	0	24	1	1
7.	Dindigul	1	0	4	0	0	0	17	1	0
8.	Erode	2	0	4	0	0	0	33	1	0
9.	Kallakurichi	1	0	2	0	0	0	14	1	0
10.	Kancheepuram	2	1	2	2	1	1	17	1	0
11.	Kanniyakumari	1	0	3	0	1	0	29	1	1
12.	Karur	1	0	2	0	0	0	16	1	0
13.	Krishnagiri	1	0	2	0	0	0	28	1	0
14.	Madurai	2	1	8	1	1	1	66	5	4
15.	Mayiladuthurai	1	0	2	0	0	0	17	1	0
16.	Nagapattinam	1	0	1	0	0	0	7	1	0
17.	Namakkal	1	0	2	0	0	0	21	1	0
18.	The Nilgiris	1	0	4	0	0	0	21	1	1

Sl. No.	District	MRI Scan	PETCT	CT Scan	LINAC	Cobalt therapy Machine	Brachytherapy Machine	Dialysis Machine	Digital X-ray	Cath lab
19.	Perambalur	1	0	1	0	0	0	12	1	0
20.	Pudukottai	1	0	3	0	1	0	31	2	1
21.	Ramanathapuram	1	0	2	0	0	0	17	1	1
22.	Ranipet	0	0	1	0	0	0	10	1	0
23.	Salem	1	1	6	1	1	1	46	3	1
24.	Sivaganga	1	0	2	0	0	0	21	2	0
25.	Thanjavur	1	1	5	1	1	1	36	4	2
26.	Theni	1	0	3	0	0	0	21	2	1
27.	Tenkasi	1	0	2	0	0	0	16	1	0
28.	Tiruchirappalli	1	0	4	0	0	0	38	5	1
29.	Tirunelveli	1	1	4	1	1	1	34	4	2
30.	Tirupathur	1	0	2	0	0	0	21	1	0
31.	Tiruppur	1	0	2	1	0	0	11	1	0
32.	Tiruvallur	1	0	5	0	0	0	50	1	1
33.	Tiruvannamalai	1	0	4	0	1	0	38	2	1
34.	Tiruvarur	1	0	3	0	0	0	37	2	1
35.	Thoothukudi	1	0	3	1	0	0	27	4	1
36.	Vellore	1	0	3	0	0	0	23	3	1
37	Villupuram	1	0	3	0	1	0	42	1	1
38	Virudhunagar	1	0	5	0	0	0	24	2	0
Total		**51**	**7**	**140**	**12**	**15**	**10**	**1,395**	**80**	**39**

2.4 The establishment of various councils through Acts of the Government of India and the Government of Tamil Nadu underlines the importance of regulating the practice of medical, nursing and paramedical professionals in Tamil Nadu. These councils as detailed below play a crucial role in ensuring the quality and standards of healthcare services by registering qualified professionals and regulating their practice.

i) **Tamil Nadu Medical Council** - Regulates the practice of Allopathic medicine and registers qualified medical practitioners in the State.

ii) **Tamil Nadu Dental Council** - Regulates the practice of Dentistry and registers qualified Dental practitioners in Tamil Nadu.

iii) **Tamil Nadu Nurses and Midwives Council** - Registers qualified nurses and midwives and regulates their practice to ensure standards of Nursing care in the State.

iv) **Tamil Nadu Pharmacy Council** - Regulates the practice of Pharmacy and registers qualified Pharmacists in Tamil Nadu.

v) **Tamil Nadu Siddha Medical Council** - Registers and regulates the practice of Siddha medicine and traditional practitioners in the State.

vi) **Board of Indian Medicine** - Regulates the practice of Ayurveda, Unani and Yoga & Naturopathy in Tamil Nadu, ensuring the standards and ethics of these traditional Indian systems of medicine.

vii) **Tamil Nadu Homoeopathy Council** - Regulates the practice of Homoeopathy and registers qualified Homoeopathic practitioners in the State.

viii) **Tamil Nadu State Allied and Health Care Professionals Council** – Registers and regulates the practice of Allied Health Care professionals like Physiotherapists, Occupational Therapists, Medical Laboratory Technicians etc., in the State.

CHAPTER - 3

NATIONAL HEALTH MISSION / STATE HEALTH SOCIETY – TAMIL NADU

3.1 Introduction

Tamil Nadu has undergone a significant epidemiological transition over the past three decades. While the State has achieved commendable reductions in communicable disease burden, the concurrent rise of Non-Communicable Diseases (NCDs) has created a dual burden that substantially inflates the overall disease burden as measured in Disability-Adjusted Life Years (DALYs). Response to this evolving Public Health landscape demands an adaptive, evidence-driven and multi-sectoral approach.

National Health Mission - Tamil Nadu (NHM-TN) has served as the cornerstone of the State's public health architecture, functioning as a strategic facilitator that enables, coordinates and strengthens the health system at every level. In alignment with the Sustainable Development Goals (SDGs), the Mission channels resources, builds

institutional capacities and forges cross-sectoral partnerships to ensure that quality health services reach every citizen, irrespective of geography, gender or socio-economic status.

NHM-TN's facilitative role spans the full continuum of care — from strengthening Health Sub-Centres, Primary Health Centres (PHCs) and Community Health Centres (CHCs) in conformity with Indian Public Health Standards (IPHS), to enabling Secondary and Tertiary institutions through targeted infrastructure investments and human resource development. The Mission implements a comprehensive portfolio of Public Health programmes encompassing Health Systems Strengthening, Reproductive, Maternal, Neonatal, Child and Adolescent Health (RMNCH+A), and the prevention and control of both Communicable and Non-Communicable Diseases.

By catalysing convergence among Government Departments, development partners, healthcare institutions and communities, NHM-TN develops integrated and sustainable health solutions. Its strategic interventions in programme

management, community participation, health financing and Digital Health continue to reinforce both Rural and Urban health systems across Tamil Nadu.

3.2 Mission

National Health Mission Tamil Nadu envisions a health system where every individual has universal access to equitable, affordable and quality healthcare services that are responsive to community needs, delivered with accountability and sustained through participatory governance.

3.3 Objectives

NHM-TN pursues the following core objectives for furtherance of its Mission grouped as domains highlighted below;

i) Maternal, Child and Adolescent Health

- Reduce Maternal Mortality Ratio (MMR) and Infant Mortality Rate (IMR) to achieve nationally and internationally benchmarked targets and reduce it further.
- Ensure universal access to comprehensive maternal, child and adolescent health services, including nutrition, immunization and skilled birth attendance.

ii) Disease Burden and Epidemiological Transition

- ❖ Reduce the overall disease burden as measured by Disability-Adjusted Life Years (DALYs) through targeted interventions across the life-cycle.

- ❖ Prevent and control Communicable Diseases while addressing the growing burden of Non-Communicable Diseases, including Cancer, Cardiovascular conditions, Diabetes and Mental health disorders.

iii) Primary Healthcare and Universal Health Coverage

- ❖ Strengthen integrated & comprehensive Primary Healthcare as the bedrock of Universal Health Coverage (UHC).

- ❖ Ensure universal access to Public Health services encompassing safe drinking water, sanitation & hygiene, nutrition and universal immunization.

iv) Population Stabilisation and Social Determinants

- ❖ Achieve population stabilisation through gender-sensitive and demographically-informed strategies addressing fertility, family planning and reproductive rights.
- ❖ Foster health-promoting behaviours and environments to reduce modifiable risk factors across the population.

v) Traditional Medicine and Health Promotion

- ❖ Revitalise local health traditions and mainstream Indian Systems of Medicine (AYUSH) within the Public Health delivery framework.

3.4 Strategies

NHM-TN strategises its goals through the following approaches with its role as a facilitator central to every intervention:

i) Governance and Partnership Facilitation

- ❖ Act as the primary liaison and coordination mechanism, aligning

priorities and resources across the Central, State and Local Self-Governments.

- ❖ Enable active involvement of Panchayati Raj Institutions and local communities in the planning, prevention, promotion and delivery of Primary Healthcare services, anchoring accountability at the grassroots.
- ❖ Empower Districts and communities with the flexibility to design and implement context-specific, locally appropriate health initiatives, fostering decentralised ownership.

ii) Health Systems Strengthening

- ❖ Facilitate expanded access to and equitable utilisation of quality health services at all levels of care, with a particular focus on underserved and vulnerable populations.
- ❖ Facilitate innovative health financing mechanisms and efficient resource stewardship to ensure the sustainability and long-term resilience of the Public Health system.

- ❖ Embed Digital Health solutions to enable real-time data capture, epidemiological monitoring and evidence-based programme management.

iii) Epidemiological Surveillance and Evidence-Based Response

- ❖ Strengthen epidemiological surveillance systems to anticipate, monitor and respond to shifting disease patterns arising from the ongoing epidemiological transition.

- ❖ Promote evidence-based planning using DALYs, disease burden data and demographic indicators to prioritise and calibrate Public Health interventions.

iv) Equity and Inter-sectoral Convergence

- ❖ Champion equity and social justice in healthcare delivery by systematically addressing gender, geographic and socio-economic disparities in health access and outcomes.

Develop and operationalise frameworks for inter-sectoral convergence, integrating promotive and preventive healthcare with nutrition, water and sanitation, education and social welfare sectors.

3.5. Implementation Framework

Though initially launched as the National Rural Health Mission (NRHM) in 2005, the initiative was later expanded to encompass Urban areas under the National Urban Health Mission (NUHM) in 2013. Subsequently, both the Rural and Urban components have been brought under the aegis of National Health Mission.

At the National level, NHM is overseen by the Ministry of Health and Family Welfare which sets the overarching policies and guidelines for its implementation across the States. At the State level, NHM-TN operates through the State Health Society (SHS) overseen by the Mission Director of NHM and supported by the State Programme Management Unit for smooth facilitation and operationalization of the Mission's activities.

The State Health Society was constituted by the amalgamation of various other health societies

created earlier for Leprosy, Tuberculosis, Blindness Control and Integrated Disease Control with the exception of the Tamil Nadu State AIDS Control Society. This consolidation of Societies under a unified single entity both at State and District level has streamlined operations and improved efficiency. The State Program Management Unit of State Health Society aids in facilitating and coordinating all the Mission's activities for optimal efficiency and impact.

a) District Health Society

The National Health Mission administers its functions at the District level through the District Health Society (DHS) chaired by the District Collector. The DHS operates under a Governing Body which holds the responsibility of planning and managing all NHM programmes within the district by overseeing their implementation at the District level through the District Programme Management Units (DPMU) and at the Block level through the Block Programme Management Units (BPMU).

b) Facility level Patient Welfare Societies

Patient Welfare Societies are facility-level registered societies which have been constituted in

all the Primary Health Centres, Government Hospitals and Medical College Hospitals for better functioning of health institutions by providing necessary patient amenities and bridging service gaps which will facilitate achievement of the patient welfare. Flexibilities are given to the Patient Welfare Societies for spending untied funds on actual requirements. The Society's objective is to foster public participation and ensure compliance with minimum facility standards, patient-care guidelines as well as Government-issued standard protocols for treatment.

c) Village Health, Sanitation and Nutrition Committee (VHSNC)

In Tamil Nadu, 15,015 Village Health, Sanitation and Nutrition Committees (VHSNCs) were constituted to empower rural communities to actively participate in decentralized health planning. These committees specifically focus on marginalised sections, enhancing community participation to systematically improve local healthcare delivery.

The chairperson of the VHSNC is the Panchayat President and the Village Health Nurse (VHN) serves as the Member Secretary. The

VHSNC acts as a subcommittee of the Gram Panchayat, comprising stakeholders such as Anganwadi workers, local school teachers, Health Inspectors, ASHAs and representatives from Self-Help Groups, aiming to promote community participation, effective communication and disease prevention. An untied fund of Rs.10,000/- is allocated to each VHSNC annually to support its activities.

The VHSNCs serve as village-level institutions for health planning and action, particularly targeting marginalised sections. This committee also provides a platform to address health service issues, raise community awareness and foster public involvement.

3.6. National Urban Health Mission (NUHM)

Tamil Nadu is one of the most urbanised States in India, with 48.45% of its population residing in urban areas. Rapid urbanisation, demographic transition, migration and changing disease patterns have increased the demand for accessible and comprehensive healthcare services, particularly among slum dwellers, migrant workers, homeless populations and other vulnerable groups.

3.6.1. Urban Health Infrastructure

a) Urban Primary Health Centres (UPHCs) & Urban Community Health Centres (UCHCs)

Urban Primary Health Centres (UPHCs) cater to an urban population of 50,000, whereas Urban Community Health Centres (UCHCs) serve a population of 2.5 lakh to 5 lakh.

At the State level, 465 Urban Primary Health Centres (UPHCs & 3 EOCs), 39 Urban Community Health Centres (UCHCs) function to make healthcare services accessible through the Corporation and Municipal local bodies.

	Urban Primary Health Centres & Emergency Obstetric Centres (EOCs)	Urban Community Health Centres
Greater Chennai Corporation	**143**	**16**
Other Corporations	**205**	**22**
Municipalities	**117**	**1**
Total	**465**	**39**

The National Sample Survey 80th Round (2025) has pointed out the effectiveness of Tamil Nadu's Urban Public Health system, with 43.3% of all outpatient consultations taking place in Government Health facilities, substantially

higher than the National average of 25.7%. Furthermore, it has pointed out that while the National average expenditure per outpatient in Government institutions is Rs.274/-, the corresponding expenditure in Tamil Nadu is only Rs.47/-, which is considerably lower in comparison.

b) Urban Health and Wellness Centres: (UHWCs)

NFHS-6 (2023-24) indicates that Tamil Nadu's ageing population has increased with 16.3% of the total population aged 60 years and above. At the same time, as non-communicable diseases have increased considerably, there has been a tangible shift in healthcare priorities from communicable diseases and Maternal and Child Health services to the management of chronic non-communicable diseases and providing for geriatric and palliative care services.

To address the ongoing epidemiological transition, Tamil Nadu has established 708 Urban Health and Wellness Centres (UHWCs) with one UHWC for every 15,000 to 20,000 population for providing 12 Comprehensive Primary healthcare services in close proximity to Urban communities,

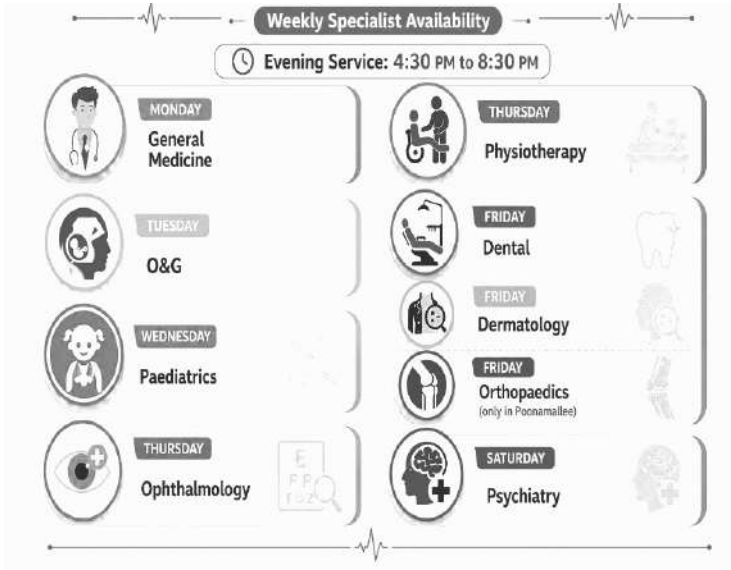
ensuring accessible primary healthcare and thereby reducing the burden on Secondary and Tertiary healthcare facilities.

All 708 UHWCs are staffed by a dedicated team of a Medical Officer, Staff Nurse, Health Inspector and Support staff. To improve accessibility for the Urban working population, UHWCs function on two shifts, from 8:00 AM to 12:00 Noon and from 4:00 PM to 8:00 PM. Since their inception, these centres have provided healthcare services to 4.54 crore Urban beneficiaries, as reported in the U-AAM portal.

c) Urban Polyclinics

Urban Polyclinics have been established to ensure continuity of care by bringing specialist services closer to the community, thereby reducing the need for patients to travel to tertiary hospitals for specialist consultations.

The establishment norm for Urban Polyclinics is to have one polyclinic for every 5-6 UPHCs, covering an approximate population of 2.5 to 3 lakh persons. Polyclinics function during evening hours (4:30 PM to 8:30 PM) to maximise accessibility for the working population.



A total of 154 Urban Polyclinics are presently operational across the State, providing specialist outpatient services across nine disciplines namely General Medicine, Paediatrics, Dentistry, Ophthalmology, Obstetrics & Gynaecology, Dermatology, Psychiatry, Physiotherapy and Orthopaedics (only at Poonamallee). Totally 5.61 lakh beneficiaries have utilised Polyclinic services during 2025-26 (HMIS 3.0).

d) City Public Health Laboratories

City Public Health Laboratories have been established under the 15th Finance Commission to

strengthen diagnostic services at Urban health facilities. CPHLs function as referral diagnostic hubs for UPHCs and UHWCs and contribute significantly in reducing out-of-pocket expenditure by providing diagnostic services free of cost to the Urban population.

A total of 35 CPHLs have been established across the State equipped with advanced diagnostic equipment including Fully Automated Analyzers which run wide range of biochemical investigations.

e) Community Engagement Activities

NFHS-6 demonstrates that social determinants continue to influence health outcomes, including maternal nutrition, child nutrition and health-seeking behaviour. Recognising that healthcare services alone cannot address these challenges, NUHM promotes community participation through observance of Urban Health & Nutrition Day facilitated by Urban Health Nurses and Mahila Arogya Samitis / Pengal Nalavazhvu Kuzhu.

A total of 3,349 Mahila Arogya Samiti (MAS) groups have been established across the

11 Municipal Corporations of Tamil Nadu which serve as community platforms for promoting health awareness and improving water, sanitation, hygiene and nutrition and for encouraging positive health-seeking behaviours among slum and vulnerable populations.

3.7. Anaivarukum Nalavazhvu Thittam / Universal Health Coverage

"Anaivarukkum Nalavazhvu Thittam" was established in 2017 as the definite roadmap for taking forward Universal Health Coverage (UHC) to every individual of the State.

The scheme guarantees that all citizens access high-quality health services ranging from promotion & prevention to treatment, rehabilitation and palliation without facing financial hardships.

Leveraging a resilient Public Healthcare infrastructure, Tamil Nadu is committed to achieving UHC by 2030, a critical milestone for Sustainable Development Goal 3 (SDG 3). In order to achieve UHC goals, the State has transformed its 4,848 Health Sub- Centres (HSC) into Health and Wellness Centres (HWC) by recruiting 4,848

Mid-Level Health Providers (MLHPs) and 2,448 Grade II Health Inspectors at HSC-HWC level.

The 12 Comprehensive Primary Health Care (CPHC) Services are provided at HSC-HWCs as given below.

1. Care in Pregnancy and Child-birth
2. Neo-natal and Infant Health Care Services
3. Childhood and Adolescent Health Care Services
4. Family Planning, Contraceptive Services and other Reproductive Health Care Services
5. Management of Communicable Diseases
6. Out-patient care for acute simple illnesses and minor ailments
7. Screening, Prevention, Control and Management of Non-Communicable Diseases
8. Basic Oral Health Care
9. Care for common Ophthalmic and ENT Problems
10. Screening and basic management of Mental Health Ailments

11. Elderly and Palliative Health Care Services
12. Basic Emergency Medical Services including Burns and Trauma.

Mid-Level Health Providers (MLHPs) drive the core operations of Health Sub-Centres – Health and Wellness Centres (HSC-HWCs) and Health Inspectors (HI Gr-II) serve as frontline Public Health guardians in the control of infectious diseases and ensuring clean village sanitation. The MLHPs execute vital clinical, managerial and community health functions in coordination with Village Health Nurses (VHNs), ASHAs, Health Inspector Gr-II and Women Health Volunteers (WHVs). By delivering an expanded range of services closer to the community, MLHPs enhance healthcare access, ensure continuity of care, regular follow-up, identify early complications, dispense medications and ensure medication compliance.

In addition to 4,848 HSC-HWCs and 708 Urban HWCs, 1,407 Primary Health Centres (PHCs), 486 Urban PHCs and 650 AYUSH HWCs have also been upgraded as HWCs to ensure standardized care across the State.

3.7.1. Community Engagement and Outreach

a. **Health Mela:** To bring specialist care to the doorstep of the community, Monthly Health Melas are conducted at all HSCs – Ayushman Arogya Mandirs (AAMs). Additionally, Specialist Medical Melas are also held at PHCs and CHCs once in every month on a Saturday.

b. **Makkal Nalavazhu Kuzhu (MNK):** Jan Arogya Samitis (JAS) i.e Makkal Nalavazhvu Kuzhu (MNK) has been constituted in all HSC – HWCs and UHWCs which strengthens social accountability and acts as a grievance redressal platform. These Committees meet on a monthly basis to facilitate community feedback and monitor service quality.

3.7.2. Digital Health: e-Sanjeevani Telemedicine

Tamil Nadu has emerged as a leader in digital health through the Telemedicine (e-Sanjeevani) platform. MLHPs at HWCs facilitate tele-consultations, connecting patients with doctors at Additional PHCs and Block PHCs. In the year 2025-26, the State has recorded 1,04,44,446 consultations at the Health Sub Centres - Health

and Wellness Centres which has significantly reduced the need for patients to travel for expert medical advice.

3.8. Maternal Health

a. Mission Single Digit: Life of every Mother Counts

The United Nations Sustainable Development Goals (SDGs) mandates a global reduction of the Maternal Mortality Ratio to less than 70 per 1,00,000 live births. As per the Sample Registration System (SRS) Bulletin 2022–24, the Maternal Mortality Ratio in Tamil Nadu stands at 25 per lakh live births, reflecting the State's robust healthcare system and continued focus on maternal wellbeing. Moving forward towards an aggressive single-digit MMR, positions Tamil Nadu's healthcare system in its stride towards achieving world-class standards.

b. Ensuring Healthy Mothers, Healthy Futures

Mothers deserve access to quality antenatal care during pregnancy, skilled assistance during childbirth, and comprehensive care and support in the postnatal period.

Tamil Nadu has consistently demonstrated a strong commitment to maternal health through the provision of dedicated antenatal care services and the implementation of strategic Public Health interventions aimed at improving maternal outcomes and reducing maternal mortality.

3.8.1. Key Measures undertaken to reduce Maternal Mortality Ratio

a. Institutionalization of State and District Task Force committees.

b. Establishment of a State-Level War Room.

c. Strengthening of State and District-Level Call Centre.

3.8.1.1. Establishment of State and District Task Force committees

The State Task Force Committee plays a pivotal role in strengthening Maternal Death Surveillance and Response (MDSR) systems through the analysis of maternal deaths and addressing key social determinants such as malnutrition and anaemia, in order to ensure equitable access to comprehensive maternal healthcare services.

The committee also focuses on strengthening healthcare infrastructure at all levels, improving the quality and accuracy of maternal health data, institutionalizing pre-birth planning and facility audits, fostering collaboration with the private sector and establishing accountability frameworks to improve overall maternal health outcomes in the State.

3.8.1.2. State-Level War Room for Maternal Surveillance

Based on the recommendations of the State Task Force Committee for the reduction of Maternal Mortality, a dedicated State War Room has been established to address persistent challenges in achieving further reductions in MMR. The State War Room functions as a centralized platform for real-time monitoring of antenatal mothers, integrated data management and rapid response mechanisms, with the overarching goal of eliminating preventable maternal deaths through system-driven innovations and strengthened surveillance.

The State War Room also monitors the implementation of a comprehensive Pre-Birth Planning Mechanism (PBPM), data harmonization, systematic data collection and follow-up mechanisms, validation of data captured in the Pregnancy and Infant Cohort Monitoring and Evaluation (PICME 3.0) portal, criteria based categorization of private healthcare institutions into Level I, II and III facilities for appropriate referral linkage, strengthening follow-up and monitoring of high-risk antenatal mothers, enhanced coordination with the "102" State and District Call Centres for systematic follow-up of antenatal mothers.

3.8.1.3. Operationalization of State and District-Level Call Centre

- A dedicated 25-seater State Call Centre has been established to provide systematic follow-up of high-risk antenatal mothers. The centre offers counselling on danger signs and symptoms during pregnancy, childbirth and the postpartum period, thereby facilitating early identification and prevention of complications and adverse maternal outcomes.

- The details of mothers requiring urgent medical attention are "red-flagged," and automated alerts are generated through the PICME 3.0 portal and communicated via SMS to the respective Village Health Nurse (VHN) or Urban Health Nurse (UHN) for prompt intervention. The concerned VHN/UHN has to mandatorily close the call within 24 hours.

- From April 2025 to March 2026, a total of 2,50,268 high-risk antenatal mothers have been counselled through the State Call Centre. A total of 4,77,804 mothers have been counselled through District call centres established across all 38 districts and 1,26,598 mothers have been reached further through call centres functioning at the Primary Health Centre (PHC) level.

3.8.2. Strengthening Institutional Deliveries through Janani Suraksha Yojana (JSY)

As per NFHS 6 (2023-24) data, the percentage of institutional deliveries in Tamil Nadu stands at 99.7% as tabulated below:

Institutional Deliveries	Tamil Nadu	India
Percentage	99.7%	90.6%

The state implements Janani Suraksha Yojana (JSY), which is a safe motherhood intervention designed to reduce maternal and neonatal morbidity and mortality by promoting institutional deliveries and ensuring access to skilled obstetric care services. Under the scheme, conditional cash or financial assistance is provided to eligible beneficiaries delivering in Government health institutions, with financial incentives of Rs.700/- for beneficiaries in rural areas and Rs.600/- for those in urban areas immediately after delivery or before discharge. This is in addition to the benefits provided under the State-run Dr. Muthulakshmi Reddy Maternity Benefit Scheme. 3,45,388 mothers have benefitted under JSY during April 2025 to March 2026.

3.8.3. Comprehensive Maternal and Neonatal Healthcare under JSSK

Janani Shishu Suraksha Karyakram (JSSK) targets the provision of comprehensive maternal and neonatal care. All pregnant women delivering in Government health facilities are entitled to

completely free and cashless childbirth services, including normal delivery and caesarean section procedures. The scheme encompasses the provision of free drugs and consumables, essential diagnostics, a nutritious diet during institutional stay and blood transfusion services wherever indicated. In addition, the programme guarantees comprehensive referral transport support, including transportation from home to the healthcare institution, inter-facility transfer in case of referral, and drop-back services to home after 48 hours of institutional stay. Through this holistic package of assured services, JSSK significantly strengthens financial risk protection, promotes institutional deliveries and contributes towards improved maternal and neonatal health outcomes. The above entitlements are also provided to sick newborns up to one year of age.

Details of Benefitted Mothers during the April 2025 to March 2026	**No. of Benefitted Mothers**
Total mothers admitted and benefitted	4,15,415
Mothers who had C-Section deliveries	1,95,211
Mothers picked up from Home and dropped at Hospital	3,65,540
Mothers dropped back to their Homes from Hospital	2,87,759

3.8.4. Comprehensive Screening and Tracking of High-Risk Pregnancies (PMSMA)

Pradhan Mantri Surakshit Matritva Abhiyan (PMSMA) scheme focuses on dedicated Antenatal Care (ANC) camps on the 9th and 24th of every month across Government healthcare facilities, including Primary Health Centres (PHCs), Taluk Hospitals and District Hospitals to strengthen comprehensive maternal surveillance and ensure early identification of high-risk pregnancies.

The initiative encompasses systematic screening and risk stratification of antenatal mothers for high-risk conditions such as Hypertensive Disorders of pregnancy, severe anaemia, Gestational Diabetes Mellitus and other obstetric complications requiring specialized care by an Obstetrician. In addition to the above, focused counselling sessions are conducted every week at PHCs covering maternal nutrition, birth preparedness, complication readiness and recognition of obstetric danger signs. The programme also facilitates timely referral linkage, longitudinal follow-up and continuum-of-care tracking for high-risk antenatal mothers through

the PICME platform, for ensuring appropriate management and achieving improved maternal and neonatal health outcomes.

5,53,479 High Risk mothers have been tracked through PICME and services provided during April 2025 – March 2026.

3.8.5. Strengthening Anaemia Prevention and Management among pregnant and lactating Women

Anaemia during pregnancy is a significant public health concern associated with adverse maternal and perinatal outcomes, including impaired intrauterine growth and low birth weight among newborns. The Anaemia-Free Tamil Nadu initiative therefore, places emphasis on improving the nutritional and hematological status of women in the reproductive age group (20–49 years), pregnant women and lactating mothers through comprehensive anaemia prevention and management strategies.

The programme strengthens maternal nutrition through systematic Iron and Folic Acid (IFA) supplementation, early detection of anaemia,

therapeutic management and timely clinical interventions aimed at reducing the burden of maternal anaemia and associated complications.

During April 2025 to March 2026:

- Through Therapeutic Iron and Folic Acid supplementation, 7,29,418 Anaemic Mothers have undergone correction of nutritional Anaemia.
- 1,68,786 mothers with moderate to severe Anaemia requiring parenteral Iron replacement were treated with administration of injection Iron Sucrose.
- 15,842 mothers with severe Anaemia and critical clinical indications have received Blood transfusion services.

3.8.6. Comprehensive Universal Screening for Gestational Diabetes Mellitus (GDM)

The programme aims for universal screening for Gestational Diabetes Mellitus (GDM), which is systematically undertaken for all pregnant women through the Oral Glucose Challenge Test (OGCT) at critical stages of pregnancy, namely between 12–16 weeks, 24–28 weeks and 32–34 weeks of

gestation. The structured screening strategy facilitates the early identification of glucose intolerance during pregnancy, enabling prompt clinical intervention, risk stratification and appropriate therapeutic management to prevent adverse maternal and foetal outcomes. Early detection and timely management of GDM play a pivotal role in reducing pregnancy-related complications, improving foetal growth and neonatal outcomes and ensuring safer motherhood. Out of 5.96 lakh mothers screened during April 2025 to March 2026, 64,413 mothers have been identified with GDM and managed appropriately.

3.8.7. "Surakshit Matritva Aashwasan (SUMAN)" – Assured Quality and Respectful Maternal & Newborn Healthcare Initiative:

Surakshit Matritva Aashwasan represents a transformative, comprehensive and convergence-based healthcare initiative aimed at strengthening the continuum of maternal and newborn care through assured, equitable and quality-driven service delivery. The programme integrates all existing maternal and newborn healthcare

interventions under a unified framework to ensure standardized, respectful, dignified and evidence-based healthcare services for every pregnant woman, mother and newborn accessing public health institutions. The core target of the scheme is to achieve Zero preventable deaths among mother & child during delivery.

Assured Service Guarantee Packages under SUMAN include;

- Comprehensive Emergency Obstetric and Newborn Care (CEmONC) service packages are operational in 130 designated CEmONC centres, ensuring round-the-clock availability of advanced Emergency Obstetric interventions, critical maternal care, operative delivery services and specialized neonatal management.
- Basic Emergency Obstetric and Newborn Care (BEmONC) packages are being implemented across 628 BEmONC centres (UG CHC – 380, UG PHC – 44, SDH–195, DH –2, WCH–7) to strengthen decentralized access to essential maternal and newborn healthcare services.

A total of 11,333 SUMAN-notified healthcare facilities are currently delivering assured, respectful, safe and quality-certified maternal and newborn healthcare services, providing guaranteed entitlement-based care packages across the State.

3.8.8. Advancing Respectful Maternity Care through Specialized Midwifery Education and Midwifery-Led Care Services

Midwifery-led care for low-risk pregnancies are globally acknowledged as an evidence-based, woman-centered approach that promotes Physiological childbirth, minimizes unnecessary caesarean sections and Obstetric interventions. It contributes significantly towards improved maternal and neonatal outcomes as a transformative step towards institutionalizing evidence-based midwifery practices and strengthening dignified, compassionate and quality maternal healthcare services.

State Midwifery Training Institute has been established at the College of Nursing, Madras Medical College to strengthen skilled midwifery services so as to enhance the quality of respectful maternity care across the State.

As part of this initiative, six Midwifery Educators are currently serving as faculty, providing structured competency-based training to Staff Nurses for a period of 18 months. While the first batch of 16 identified Staff Nurses as Nurse Practitioner Midwives (NPMs) has completed training and has been posted in the high-delivery-load centre, the second batch training for 18 Nurse Practitioner Midwives (NPMs) is underway.

To further operationalize midwifery-led care practices, a dedicated Midwifery-Led Care Unit (MLCU) has been established at Kasturba Gandhi Hospital for Women and Children, Triplicane, as a centre of excellence for capacity building and woman-centred maternity care.

3.8.9. Comprehensive Emergency Obstetric and Newborn Care (CEmONC) Services – Strengthening Advanced Maternal and Neonatal Emergency Care

At present, 130 designated CEmONC facilities are operational across the State, functioning round-the-clock to provide advanced emergency obstetric and newborn care services.

As per NFHS 6 (2023-24) data account for 63.6% of the deliveries occurring in Tamil Nadu in Govt Health facilities. These centres are comprehensively equipped with essential infrastructure, specialized human resources and critical care support systems to ensure uninterrupted emergency healthcare delivery.

In Tamil Nadu, CEmONC services have been substantially strengthened through strategic infrastructure augmentation, capacity building of healthcare personnel, specialist workforce engagement and robust referral linkage mechanisms.

3.8.9.1. The key service components available at CEmONC centres include

- Comprehensive management of complicated pregnancies and high-risk deliveries
- Availability of emergency operative obstetric services, including caesarean section procedures
- Access to safe and timely blood transfusion services
- Management of severe obstetric complications such as postpartum haemorrhage, eclampsia,

sepsis and other life-threatening maternal conditions

- Newborn resuscitation and advanced neonatal stabilization and care services
- Availability of multidisciplinary specialist support including Obstetricians, Anaesthetists, Paediatricians and skilled Staff Nurses
- Functional laboratory diagnostics and imaging services for timely clinical decision-making
- Prevention of Parent-to-Child Transmission Services (PPTCTS) for comprehensive Maternal and Neonatal HIV care.
- Emergency referral transport support through the 108 Ambulance Service network
- Assured uninterrupted power supply and critical utility support systems
- Biomedical and healthcare waste management facilities adhering to prescribed safety standards

Out of 4,15,415 deliveries in Government Institutions during 2025–2026, 89% are credited to have occurred in CEmONC centres.

3.8.10. Obstetric Hybrid High Dependency Units (HDU) – Strengthening Critical Maternal Care Services

Obstetric Hybrid High Dependency Units (HDUs) are specialized maternal critical care facilities established to provide intensive monitoring, advanced clinical surveillance and intermediate-level critical care for pregnant and postpartum women with high-risk conditions or severe obstetric complications. The Obstetric Hybrid HDUs are equipped with essential life-support systems, advanced monitoring facilities and multi-disciplinary emergency care support to ensure prompt management of life-threatening maternal complications, including severe Hypertensive disorders, Obstetric haemorrhage, Sepsis, Cardiac conditions and other critical emergencies.

A total of 5,77,597 complicated Maternal admissions have been managed through Obstetric Hybrid HDUs during April 2025 to March 2026.

3.8.11. Strengthening of Maternal and Child Health (MCH) Wings for Comprehensive Maternal and Newborn Care

To further reduce maternal and neonatal morbidity and mortality, dedicated Maternal and Child Health (MCH) Wings have been established within Comprehensive Emergency Obstetric and Newborn Care (CEmONC) centres to strengthen integrated, quality-assured, and continuum-based maternal and child healthcare services. These specialized facilities play a pivotal role in ensuring comprehensive antenatal, intrapartum, postpartum and newborn care while simultaneously facilitating access to family welfare and reproductive health services.

Under the National Health Mission, a total of 63 dedicated Maternal and Child Health (MCH) Wings have been sanctioned to strengthen specialized maternal and newborn healthcare services. During April 2025 to March 2026, 6,05,171 maternity admissions have been managed across these centres, reflecting the extensive utilization and critical contribution of MCH Wings towards comprehensive maternal and neonatal healthcare delivery in the State.

3.8.12. Capacity Strengthening through Specialist Recruitment

Approximately 15% of pregnancies are likely to develop potentially life-threatening obstetric complications requiring timely specialist intervention and advanced emergency obstetric care. To address the shortage of specialist healthcare professionals in high-delivery-load institutions, the services of Obstetricians, Pediatricians and Anesthetists are strategically engaged on contractual and pro-rata basis to strengthen comprehensive emergency obstetric and newborn care services across Government health facilities wherever applicable. This initiative significantly enhances institutional preparedness for the management of complicated pregnancies, obstetric emergencies, and critical maternal and neonatal conditions for improved maternal and newborn survival outcomes. During April 2025 to March 2026, a total of 17,066 procedures related to family welfare and birth-related events have been attended to through the hiring of specialists.

3.8.13. Caesarean Section Audit – Promoting Evidence-Based Obstetric Practices and Rational Use of Caesarean Deliveries

As per NFHS-6 (2023–24) data, C-section deliveries in the State are 46.9%, with 60.3% C-sections in private health facilities and 39.6% C-sections in public health facilities.

Caesarean Section Audit has been institutionalized across all delivery points as a strategic quality assurance initiative aimed at promoting rational obstetric practices, strengthening clinical governance and ensuring evidence-based decision-making in maternal healthcare services. Instructions have been issued to all Government and private institutions to conduct C-section audits as per the Modified Robson Classification, a globally recognized and structured analytical framework that categorizes pregnant women into ten refined clinical categories based on key obstetric characteristics. Categories 1, 2 and 5 are commonly observed when applying the Modified Robson Classification. The audit primarily focuses on reducing unnecessary and avoidable primary Caesarean sections through systematic evaluation of clinical

indications and adherence to standardized obstetric guidelines. This enables comprehensive assessment and benchmarking of Caesarean section practices across healthcare institutions.

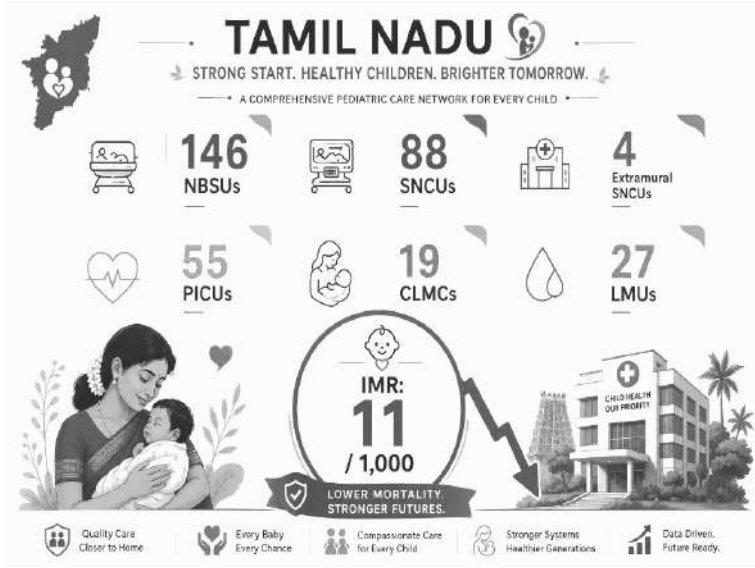
The Caesarean Section Audit serves as a critical quality improvement mechanism for enhancing Maternal health outcomes, optimizing Obstetric care practices and ensuring judicious and appropriate utilization of Caesarean delivery services.

During April 2025 to March 2026, 1,95,211 Caesarean sections have been conducted in Government healthcare institutions, of which 87% underwent systematic clinical audit as per the Modified Robson Criteria framework, with a Primary Caesarean Section Rate of 55% in Government Hospitals.

3.9. Child Health

The State's sustained and systemic investment in health infrastructure, skilled human resources and community-level interventions is reflected by recording an Infant Mortality Rate of 11 per 1,000 live births in SRS 2024 (rural: 12 and urban : 10 per 1,000 live births) and Neonatal Mortality and Under Five Mortality Rate of 8 and 14

per 1,000 live births respectively. The state continues to lead the Nation in favorable child health outcomes.



3.9.1. Facility Based Newborn Care: (April 2025 – March 2026)

3.9.1.1. Newborn Care Corners (NBCCs)

Newborn Care Corners are single-bedded units placed within every labour room and operation theatre, serving as the primary point of life-saving intervention for all neonates immediately following delivery. These units ensure

provision of warmth, resuscitation, weight recording, early initiation of breastfeeding and timely identification and referral of at-risk or sick newborns by trained staff. The State has operationalized NBCCs across 2,671 designated health facilities.

During April 2025 – March 2026, 4,01,515 neonates have received newborn care in NBCCs across the State.

3.9.1.2. Newborn Stabilization Units (NBSUs)

Newborn Stabilization Units are four-bedded intermediate care facilities that relieve tertiary hospital load while delivering evidence-based neonatal care for the community. Each unit receives and stabilizes sick newborns and low-birth-weight neonates whose condition requires monitoring and intervention. The State has established 146 NBSUs across 5 District Hospitals, 93 Sub-District Hospitals, 38 Community Health Centres and 10 Urban Community Health Centres.

During April 2025 – March 2026, 11,793 Newborns have been stabilized across all NBSUs.

3.9.1.3. Special Newborn Care Units (SNCUs)

Special Newborn Care Units are 12-bedded or larger units located in close proximity to labour rooms, staffed with dedicated and adequately trained Doctors, Staff Nurses and support personnel to provide round-the-clock comprehensive Level-II Neonatal intensive care. These units manage preterm, low-birth-weight and sick neonates through resuscitation of birth-asphyxiated newborns, management of respiratory distress, phototherapy, infection management and structured follow-up of high-risk Newborns. The State currently operates 88 SNCUs across 37 Medical College Hospitals including the attached Institutions, 31 District and 19 Sub-District Hospitals and 1 Community Health Centre.

During April 2025 – March 2026, 1,12,358 newborns have been managed and discharged through the SNCUs.

3.9.1.4. Pachilam Kuzhanthai Pathukappu Pettagam (Baby Care Kit)

It has been launched to strengthen institutional follow-up care for SNCU-discharged low-birth-weight infants by providing Iron Drops, Vitamin D3 Drops and Multivitamin Drops, ensuring

their nourishment and prevention of illnesses. The Programme was launched in February 2026. So far, 6,642 low-birth-weight infants were benefited under this initiative.

3.9.1.5. Extramural Special Newborn Care Units

The rising burden of high-risk deliveries has placed considerable demand on SNCU capacities at Medical College Hospitals, which cater to both in-born and out-born neonates. To address this gap, dedicated Extramural Units have been established at Government Medical College Hospitals in Madurai, Thanjavur, Tirunelveli and Villupuram. These units are exclusively designated for the care of sick neonates born outside tertiary facilities, thereby reducing congestion in mainstream SNCUs and improving access for out born cases.

During April 2025 – March 2026, 2,508 sick neonates have received specialized care at these extramural units.

3.9.1.6. Mother and Newborn Care Units (MNCUs)

Mother and Newborn Care Units are founded on the principle of "Zero separation" — no mother

should be separated from her newborn, including small and sick babies requiring specialized care. Unlike conventional neonatal units where mothers are visitors, MNCUs place the mother's bed alongside her baby's, making her an active participant in her newborn's recovery rather than a bystander.

7 MNCUs are functional in the State. 5,876 mothers and newborn have benefitted in the 7 MNCUs during 2025–2026.

3.9.1.7. Paediatric Intensive Care Units (PICUs)

These are specialized wards designed to deliver comprehensive healthcare services to children upto 12 years of age, addressing their specific medical needs. These units are established in District Hospitals/ Sub-District Hospitals and Medical Colleges and are equipped with essential and emergency medical devices including pulse oximeters, oxygen concentrators, multi-parameter monitors, infusion pumps, CPAP machines and ventilators to address a wide range of Paediatric conditions. To ensure effective service delivery, these units are staffed with trained Paediatricians, Nurses and support staff skilled in managing

childhood illnesses and emergencies. Currently, 55 PICUs are functional with a total bed capacity of 582 beds.

From April 2025 – March 2026, approximately 55,917 critically ill children have been managed across all PICUs in the State.

3.9.1.8. Genetic Laboratories — Centres of Excellence for Rare Diseases

Genetic Laboratories established under the Centres of Excellence (CoE) for Rare Diseases at the Institute of Child Health and Hospital for Children, Egmore, Govt Coimbatore Medical College Hospital and Government Rajaji Hospital, Madurai plays a vital role in strengthening specialized diagnostic services for children affected by rare genetic and metabolic disorders. These laboratories provide advanced diagnostic facilities including genetic testing, molecular diagnostics, biochemical investigations and screening for inherited metabolic disorders enabling early and accurate diagnosis of rare diseases.

The laboratories support clinical management, genetic counselling, prenatal diagnosis and long-term treatment planning for affected children and their families, in coordination with the genetic

departments functioning across all District Early Intervention Centres (DEICs).

During April 2025 – March 2026, 20,380 screening tests were done for children suspected for genetic disorders across the three Centres of Excellence.

3.9.1.9. Nutrition Rehabilitation Centres (NRCs)

Child anthropometric indicators based on stunting, wasting and underweight reflect long-term and short-term nutritional status in children under 5 years of age.

NFHS 6 survey	Children under 5 years who are stunted (%)	Children under 5 years who are wasted (%)	Children under 5 years who are severely wasted (%)	Children under 5 years who are underweight (%)
India	29.3	19.0	5.2	31.8
Tamil Nadu	20.7	17.4	4.9	23.2

To address this burden of malnutrition 12 Nutrition Rehabilitation Centres (NRC) have been established for the management, treatment and rehabilitation of children suffering from Severe

Acute Malnutrition (SAM) and Moderate Acute Malnutrition (MAM), particularly those with medical complications. NRCs provide facility-based care through therapeutic feeding, medical treatment, growth monitoring and nutritional rehabilitation under the supervision of trained healthcare personnel. These centres also focus on counselling mothers and caregivers regarding appropriate infant and young child feeding practices, hygiene, sanitation and home-based nutritional care to prevent recurrence of malnutrition.

During April 2025 – March 2026, 5,668 SAM and MAM children were admitted and treated in the 12 NRCs across the State.

3.9.1.10. Lactation Management

Mother's milk is an irreplaceable biological resource for sick and preterm neonates providing the most appropriate and right source of nutrition. To ensure uninterrupted access to safe, pasteurized donor mother's milk, a two-tier lactation management infrastructure has been established:

a) **Comprehensive Lactation Management Centres (CLMCs)** are co-located with SNCUs, undertake the complete chain of donor milk management such as collection, screening, pasteurization, microbiological quality testing, storage and dispensing, thereby making pasteurized mother's milk available to sick newborns who lack access to their own mother's milk. During April 2025 – March 2026, 19 CLMCs operational at Medical College Hospitals have collected 9,245 litres of mother's milk, benefiting 60,597 sick newborns.

b) **Lactation Management Units (LMUs)** provide direct lactation support to mothers, facilitate collection and storage of mother's own milk for SNCU-admitted neonates and transfer surplus donated milk to CLMCs for pasteurization. LMUs are operationalized at 27 facilities - 13 Medical College Hospitals and 14 high-load Government Hospitals and have collected 2,773 litres of human milk supporting 22,203 sick newborns.

These units also supply pasteurized donor milk to Specialized Adoption Agencies (SAAs) under the Department of Children Welfare and

Special Services to meet the nutritional needs of orphaned newborns.

3.9.2. Community Based Child Health Interventions (April 2025 - March 2026)

Delivering child health services at the doorstep of beneficiaries is the strategy for improving early childhood outcomes. A network of community health workers which includes Village Health Nurses (VHNs), Urban Health Nurses (UHNs), Accredited Social Health Activist (ASHAs) and Anganwadi Workers (AWWs), anchors a range of outreach-based interventions designed to ensure that every child, irrespective of their background, receives age-appropriate preventive and promotive care.

3.9.2.1. Home Based Newborn Care (HBNC)

7 scheduled home visits are conducted by Village Health Nurses (VHNs), Urban Health Nurses (UHNs), and ASHAs during the first 42 days of birth — on Days 1, 3, 7, 14, 21, 28 and 42. These visits focus on essential newborn care, promotion of exclusive breastfeeding, identification of danger signs, follow-up of sick and low-birth-weight newborns, counselling on postpartum care and timely referral services, with particular

emphasis on reaching families in rural and remote areas.

During the period April 2025 – March 2026, 7,03,346 newborns received HBNC services through the frontline health worker network.

3.9.2.2. Home Based Care for Young Child (HBYC)

a. HBYC through Village Health Nurses and Urban Health Nurses

Home-based support to children aged 3 to 15 months through five scheduled visits at 3rd, 6th, 9th, 12th and 15th months by Village Health Nurses (VHNs) and Urban Health Nurses (UHNs) focus on promoting early childhood development, monitoring growth milestones using the Mother and Child Protection (MCP) card, reinforcing immunization compliance and facilitating timely referral for developmental concerns.

Completion of the 6th and 9th month HBYC visits, along with MR vaccination at 9 months, is a conditionality for the final instalment of Rs.2,000/- under the Dr. Muthulakshmi Reddy Maternity Benefit Scheme, linking financial incentives directly to measurable child health outcomes.

43,76,030 HBYC visits were completed by VHNs and UHNs during April 2025 – March 2026.

b. HBYC through Anganwadi Workers

During each home visits at the 3rd, 6th, 9th, 12th and 15th months for newborns discharged from SNCUs and low-birth-weight babies, mothers and caregivers are counselled on exclusive breastfeeding upto six months, continued breast feeding with age-appropriate complementary feeding, nutrition and hygiene practices, growth and developmental monitoring, prevention and management of common childhood illnesses and timely referral of sick children to health facilities. AWWs receive a performance-linked incentive of Rs.50/- per completed visit, disbursed based on verifiable outcomes recorded in the PICME portal.

47,600 children were benefited from AWW-led HBYC visits during April 2025 – March 2026.

3.9.3. Mother's Absolute Affection (MAA) Programme

The Mother's Absolute Affection (MAA) Programme is an initiative aimed at promoting, protecting and supporting breastfeeding and optimal Infant and Young Child Feeding (IYCF)

practices. The programme focuses on improving early initiation of breastfeeding within one hour of birth, exclusive breastfeeding for the first six months of birth, continued breastfeeding up to two years and beyond and timely introduction of appropriate complementary feeding.

From April 2025 to March 2026, 7.90 lakh (99%) of all newborns were breastfed within one hour of birth.

3.9.4. Anaemia Mukt Bharat (AMB)

a). IFA Syrup

Under the programme, children between 6 months and 59 months of age are provided prophylactic Iron and Folic Acid (IFA) syrup on a bi-weekly basis to prevent and control nutritional anaemia. The programme encompasses early screening and identification of anaemia, deworming, promotion of appropriate Infant and Young Child Feeding (IYCF) practices, dietary diversification and improving awareness among caregivers regarding nutrition and hygiene. Regular growth monitoring, counselling and referral services for severely anaemic children are also undertaken through frontline health workers.

42.47 lakh under-five children received bi-weekly IFA syrup supplementation during the period of April 2025 – March 2026.

b). Junior WIFS

The programme addresses the management of anaemia among school-age children between 5 and 9 years by providing weekly Iron and Folic Acid (IFA) supplementation to improve haemoglobin levels and prevent iron deficiency anaemia. It emphasizes on nutrition education, promotion of iron-rich diets, hygiene, sanitation practices and regular health screening in schools improving the nutritional status, physical growth, cognitive development, learning capacity and overall health of children in this age group.

40.38 lakh children aged 5 to 9 years benefited from weekly IFA supplementation during the period April 2025 – March 2026.

3.9.5. Vitamin A Supplementation Programme

The Vitamin A Supplementation Programme is a targeted public health initiative aimed at providing paediatric dose Vitamin A syrup to infants and children aged 6 to 59 months, biannually ensuring wide coverage across all Districts. The

programme primarily aims to prevent childhood blindness by addressing Vitamin A deficiency, a leading cause of preventable visual impairment in children, while also significantly reducing child mortality from common infections such as measles, diarrhoea and boosting overall immune response to reduce the severity and frequency of illness in young children.

45,02,897 under five years of age received Vitamin A supplementation during the period April 2025 – March 2026.

3.9.6. National Deworming Day (NDD)

Albendazole tablets are administered biannually to children and adolescents aged 1–19 years through schools, Anganwadi Centres and healthcare institutions. Through large-scale preventive deworming and community awareness activities, the programme aims to reduce anaemia, malnutrition and worm-related morbidity while promoting better growth and overall well-being among children.

August 2025 round — 1,98,52,394 children were dewormed. February 2026 round — 1,94,81,687 children were dewormed.

3.9.7. Intensified Diarrhoea Control Fortnight (IDCF)

NFHS 6 survey	Prevalence of Diarrhoea among under 5 children (%)	Prevalence of Severe Diarrhoea among under 5 children (%)
India	7.9	0.5
Tamil Nadu	5.1	0.3

The Intensified Diarrhoea Control Fortnight (IDCF) programme aims at reducing childhood morbidity and mortality due to diarrhoeal diseases among children under-five years of age. The programme focuses on promotion of ORS and Zinc therapy, strengthening awareness on hygiene and sanitation practices, ensuring early identification and management of diarrhoea and community-based distribution of ORS packets and Zinc tablets through frontline health workers during the campaign.

During the year 2025–2026, IDCF campaign was conducted from 16th June to 31st July 2025, during which 48.81 lakh households with under-five children were reached.

3.9.8. Social Awareness and Action to Neutralise Pneumonia Successfully (SAANS)

NFHS 6 survey	Prevalence of Acute Respiratory Infections among under 5 children (%)
India	1.1
Tamil Nadu	0.6

Respiratory tract infections are the second most common illness among under 5 children. This programme addresses early identification and management of childhood Pneumonia among children under five years of age, promotion of pneumococcal vaccination, community awareness on breastfeeding, nutrition, handwashing, sanitation and danger signs and strengthening the capacity of frontline health workers.

During November 2025 – February 2026, 27,388 under-five children with Pneumonia have been identified and treated through SAANS campaign.

3.9.9. Early Childhood Development (ECD) - 102 Call Centre

The ECD 102 Call Centre is a dedicated tele-counselling and support service established to provide guidance on Early Childhood Development (ECD), child health, nutrition, growth and developmental concerns and parenting practices for children, particularly during the first 1000 days and early childhood period. The call centre facilitates timely counselling, referral services and support to parents and caregivers through trained personnel.

From April 2025 – March 2026, 1,76,818 mothers were contacted and counselled through the ECD Call Centre.

3.10. Rashtriya Bal Swasthya Karyakram (RBSK)

Rashtriya Bal Swasthya Karyakram (RBSK) is a program focused on early identification of selected health conditions among children and linking them for Medical and Surgical management, thereby providing comprehensive care for children in the community. The programme adopts a systemic and holistic approach toward the prevention, early identification and management of 30 health conditions categorized under the 4Ds namely,

- Defects at Birth
- Diseases
- Deficiencies
- Developmental Delays (including Disabilities)

The programme is implemented in Tamil Nadu through 808 RBSK teams, which includes 770 Mobile Health Teams in Rural areas, 16 teams in the Greater Chennai Corporation and 22 teams in other Urban areas.

3.10.1. Screening for Children in the age group of 0 to 18 years

3.10.1.1. Facility based Newborn screening (up to 48 hours of life)

All newborns delivered in Government health institutions are screened for birth defects by Medical Officers within 48 hours of birth. Children identified with congenital anomalies are provided with free treatment and necessary interventions following referral from the delivery point.

During April 2025 – March 2026, a total of 3,86,793 Newborn delivered in Government institutions have been screened out of whom, 684 Newborn with Congenital Anomalies have been identified and appropriately managed.

3.10.1.2. Community based newborn screening (age 0-6 weeks) for birth defects

Newborns aged between 48 hours and 6 weeks are screened at their homes by Village Health Nurses, Urban Health Nurses and ASHAs under the Home-Based Newborn Care (HBNC) programme. The frontline workers are trained to identify visible birth defects and encourage caregivers to attend to the screening at Anganwadi Centres by RBSK Mobile Health Teams.

During April 2025-March 2026, a total of 6,09,551 children in the age group of 0-6 weeks have been screened for birth defects by the Village Health Nurses/ Urban Health Nurses / ASHAs and 101 Children identified with Birth Defects have been referred for further follow up at Block PHC/RBSK/ DEIC.

3.10.1.3. Screening of Children in the age group of 6 weeks to 6 years at Anganwadi centers

Bi-annual health screening is conducted for children aged 6 weeks to 6 years at Anganwadi Centres by RBSK Mobile Health Teams. The Poshan Tracker–ICDS database, covering around 37 lakh children, is integrated with the NALAM application to facilitate tracking and monitoring.

Clinical screening for the early identification of the 4Ds is executed by the RBSK team by using a specialized 40-question digital evaluation framework built into the NALAM application.

Children identified with any of the 4Ds conditions requiring advanced clinical intervention are seamlessly referred to the District Early Intervention Centre (DEIC) for expert clinical opinion, specialized medical therapies or surgical management.

During April 2025 to March 2026, out of 37,48,444 children screened, 14,136 children have been identified with 4Ds and referred to District Early Intervention Centers for further management.

3.10.1.4. Screening of Children in the age group of 6 to 18 years at Schools

Children in the age group of 6 to 18 years enrolled across all Government and Government-aided schools undergo a comprehensive health screening once a year by the dedicated RBSK Mobile Health Teams strategically deployed @ two teams per block. The Education Management Information System (EMIS), which contains records of approximately 68 lakh students, is linked with the NALAM application to support comprehensive child health tracking.

The RBSK team conducts dedicated mobile health screening camps for school children by utilizing a specialized 40-question digital evaluation framework built into the NALAM application. During these camps, the team identifies health related issues, provides medical management and refers children who need specialized interventions/ specialist consultation/ Surgical procedures to the DEICs for further diagnosis, management and follow-up care.

During April 2025–March 2026, Mobile Health teams have screened 68,20,134 children and referred 1,18,412 children to DEIC.

3.10.2. District Early Intervention Centre (DEIC)

Children identified with the 4Ds during screening by RBSK team are referred for confirmative diagnosis to DEIC where a comprehensive specialized therapies are provided under one roof as mentioned below:-

- ❖ Medical & Nutritional Therapy
- ❖ Physiotherapy & Occupational Therapy
- ❖ Psychological, Sensory-Neural and Behavioural Therapy
- ❖ Specialized Vision, Hearing and Dental Therapy

Every DEIC operates with a dedicated, 12-member specialized clinical team to deliver coordinated and holistic intervention. A total of 39 DEICs are strategically established across the State with 32 centres located at Medical College Hospitals and 7 centres at District Headquarters Hospitals.

The centralized NALAM web portal software has been successfully extended to all 39 DEICs,

ensuring seamless, real-time logging, tracking and clinical case management of every child referred by the field teams.

Children diagnosed with 7 major congenital and chronic conditions are subjected to intensive, high-priority monitoring at both State and District levels.

Management of 7 major conditions 2025-2026					
Sl. No.	**Disease Condition**	**Children Confirmed**	**Children Medically Managed**	**Children Needed Surgery**	**Surgery Done**
1	CHD (Congenital Heart Disease)	8,411	6,705	1,706	1,624
2	RHD (Rheumatic Heart Disease)	502	480	22	22
3	Club Foot	859	716	143	141
4	Cleft Lip & Palate	670	152	518	436
5	Congenital Cataract	168	57	111	110
6	Congenital Deafness	560	410	150	126
7	NTD (Neural Tube Defect)	124	66	58	54
Total		**11,294**	**8,586**	**2,708**	**2,513**

To eliminate financial hardship for beneficiary families, all the required surgical interventions are performed free of cost under the Chief Minister's Comprehensive Health Insurance Scheme

(CMCHIS) across empanelled Government and Private empanelled hospitals. During the year 2026-27, a new DEIC will be established at Government Kasturba Gandhi Hospital for Women and Children, Chennai.

There are 32 Sensory Integration Park established near the DEIC across the State. It has textured panels and visual panels which helps the children experience touch, feel and visualize flicker of light. On an average, 40 children / day are benefitted through each of the Sensory Integration Park.

During April 2025 to March 2026, a total of 4.43 Lakh children have been registered in all 39 DEICs in the State among whom 3,13,434 children have been medically and 9,931 children surgically managed.

3.11. Adolescent Health: Rashtriya Kishor Swasthya Karyakram (RKSK)

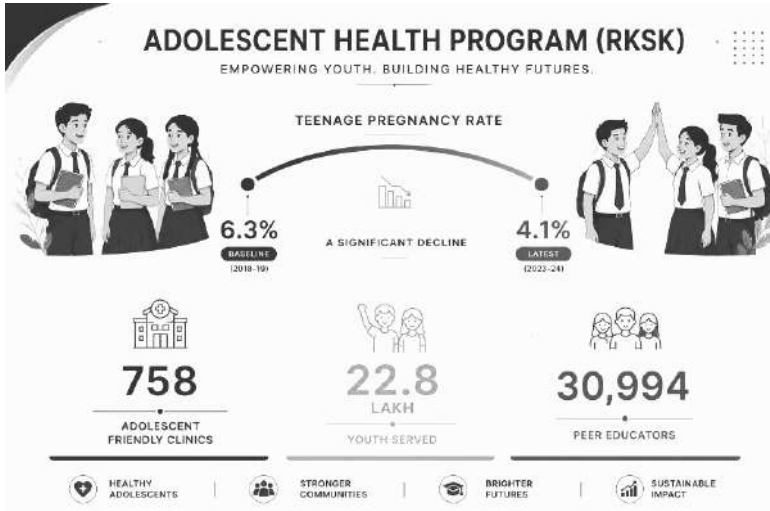
Adolescence is a crucial phase of growth and development marked by significant physical, Cognitive, emotional, psychological and social changes. Ensuring adequate nutrition, health

education, counselling and supportive guidance during this period is of utmost necessity for promoting healthy development and well-being.

To address these multidimensional health needs, the Rashtriya Kishor Swasthya Karyakram (RKSK) has been implemented for adolescents aged 10–19 years with the objective of ensuring equitable, accessible and quality adolescent health services for both in-school and out-of-school adolescents, including vulnerable and underserved groups. The programme adopts a comprehensive promotive and preventive approach through community-based interventions, adolescent-friendly health services, counselling support, behaviour change communication activities, school and community outreach programmes and strengthening of Adolescent Friendly Health Centres (AFHCs) across various levels of healthcare delivery.

3.11.1. Approaches under RKSK

The programme is implemented through two key approaches: facility-based interventions and community-based interventions.



3.11.1.1. Facility-Based Approach - Adolescent Friendly Health Centres (AFHCs)

758 Adolescent Friendly Health Centres (AFHCs) across Tamil Nadu offer comprehensive clinical and counselling services covering key adolescent health concerns such as Sexual and Reproductive Health (SRH), nutrition, substance abuse, accident and injuries, violence including gender-based violence, Non-Communicable Diseases and mental health issues. These services are provided through Block Primary Health Centres (BPHCs), District Headquarters Hospitals (DHQHs) and District Early Intervention Centres (DEICs) attached to Medical College Hospitals. The centres

are spread across with 39 clinics at District Early Intervention Centres (DEICs), 30 at District Headquarters Hospitals (DHQHs), 262 at Sub-District Hospitals (SDHs) and 427 at Community Health Centres (CHCs).

Every centre is managed by a team of trained Medical Officer & counsellor with emphasis on maintaining privacy and confidentiality. During the period from April 2025 to March 2026, 22,80,423 adolescents availed services at these centres.

3.11.1.2 Community-Based Approach

a. Peer Educator Programme

Peer Educators play an important role in adolescent health promotion by facilitating communication between adolescents and healthcare professionals and by disseminating awareness on key adolescent health issues among their peers and promoting health promotion activities in schools & communities.

The selected Peer Educators receive structured training by RBSK Medical Officers and Block Medical Officer on key adolescent health topics including nutrition, mental health, prevention of substance abuse, menstrual hygiene and reproductive health.

In addition, one student from each class from 6th to 12th standard is selected as a School Health Messenger to promote health activities at the classroom level. School Health Messengers are trained by RBSK Medical Officers and School Health and Wellness Ambassadors (teachers) to disseminate key health messages and encourage healthy practices among students.

During the academic year 2025–2026, a total of 30,994 Peer Educators were identified across 15,497 Government and Government-aided schools catering to students from 6th to 12th standard. Further, 1,08,479 students were selected as School Health and Wellness Messengers, and approximately 34,27,891 students participated in various adolescent health promotion activities conducted under the programme.

b. Teen Health Days

Teen Health Days are organized to promote awareness, health education and healthy practices among adolescents in schools and communities through coordinated efforts involving the Health Department, Education Department, local

administration, parents and community stakeholders. The day is observed once every quarter in schools by the RBSK teams to ensure continuous engagement and monitoring of adolescent health issues.

Teen Health Days focus on Adolescent Health themes such as nutrition, mental health, substance misuse, injuries and violence, non-communicable diseases, reproductive health, gender equality, healthy lifestyle practices and safe use of the internet and media. Wellness Corners are also established to orient students to promote overall wellbeing and healthy development among adolescents.

During the year 2025–2026, a total of 68,830 Teen Health Days were conducted across schools and communities.

c. Teen Health Clubs

The Teen Health Club is held once a month at Schools by frontline staff viz Village Health Nurse / Urban Health Nurse / MLHP with interactive sessions with Adolescents on topics such as mental well-being, life skills, hygiene, healthy lifestyle habits and menstrual hygiene.

During 2025–2026, a total of 1,17,233 Teen Health Club sessions were conducted across the State, covering activities such as Iron and Folic Acid (IFA) tablets distribution under Weekly Iron and Folic Acid Supplementation (WIFS), distribution of de-worming tablets and sanitary napkin, creating awareness on menstrual hygiene and mental health, orientation on sexual and reproductive health, prevention of teenage pregnancy & child marriage, family welfare services, nutrition counselling, healthy food practices, substance abuse prevention and healthy lifestyle promotion among adolescents.

d. Weekly Iron and Folic Acid Supplementation (WIFS) for Adolescents - Anemia Free Tamil Nadu

The Weekly Iron and Folic Acid Supplementation (WIFS) programme is an evidence-based intervention implemented to address the prevalence of anaemia among adolescent girls and boys through supervised weekly Iron and Folic Acid (IFA) supplementation. Under this programme, IFA tablets are provided to adolescents studying in Government and Government-aided schools and for out-of-school

adolescents through Anganwadi Centres. The tablets are administered under the supervision of teachers and Anganwadi Workers to ensure regular consumption and compliance. From April 2025 to March 2026, a total of 66,89,247 school-going adolescents and 2,27,568 out-of-school adolescents were benefited under the programme.

e. School Health and Wellness Programme

The School Health and Wellness Programme is being implemented in Government and Government-aided schools through regular school-based health education activities covering important physical, mental, emotional, nutritional and social health issues among children and adolescents.

Two teachers from every school are designated as "Health and Wellness Ambassadors" and are trained to conduct health and wellness sessions for students through interesting, joyful and interactive activities for one hour every week, ensuring active participation of school children. The programme also creates awareness on important adolescent issues such as delaying early marriage and prevention of teenage pregnancy.

A total of 3,56,300 School Health and Wellness Programme sessions have been conducted across Government and Government-aided schools during 2025–2026.

f. Awareness on Teenage Pregnancy

A total of 464 Health Sub-Centres (HSCs) across 24 blocks in 10 Districts namely Dharmapuri, Dindigul, Erode, Kallakurichi, Karur, Madurai, Namakkal, Salem, Theni and Thiruvannamalai are identified with high-prevalence for teenage pregnancy, with prevalence rates ranging from 8% to 12.8%. In view of the increased burden observed in these areas 2,320 Street Play Campaigns were implemented to disseminate key messages through culturally appropriate and community-based interactive communication methods to ensure greater public participation and outreach.

The street play programme also focuses on ill effects of child marriage, importance of menstrual hygiene, continuation of education among adolescent girls, gender sensitisation and awareness on the provisions of the Protection of Children from Sexual Offences (POCSO) Act.

NFHS-6 reported a decline in teenage pregnancy to 4.1%, compared to 6.3% in NFHS-5, reflecting improvements achieved in creating awareness in adolescent health and preventive measures taken across the state.

3.12. Training

3.12.1. Regional Training Institutes – Tamil Nadu

The role of well-trained health workers is important in delivering quality health care services. Therefore, the State gives importance to the education and training of health personnel through pre-service and in-service training programmes. Under the National Health Mission (NHM), these training activities are coordinated with different Directorates and monitored through Regional Training Institutes (RTIs) across the State to improve the skills and efficiency of healthcare workers.

3.12.2. List of Training Institute Tamil Nadu

The following Training Institutes have been assigned district-wise responsibilities for conducting training programmes:

List of Training Institute Tamil Nadu	
S. No	**Training Institute**
1.	Institute of Public Health, Poonamallee, Chennai
2.	Health and Family Welfare Training Centre, Egmore, Chennai
3.	Health and Family Welfare Training Centre, Madurai
4.	Health and Family Welfare Training Centre, Gandhigram
5.	Health Manpower Development Institute, Villupuram
6.	Health Manpower Development Institute, Salem
7.	RTIPH - Thiruvarangulam, Pudukottai

An integrated, competency-based approach under Comprehensive Primary Health Care (CPHC) refers to the delivery of holistic and people-centered healthcare services through a coordinated primary healthcare team trained in essential competencies

3.12.3. District and Block Level Training Mechanism

District and Block Level Training Mechanisms have been established through District Training Teams (DTT) and Block Training Teams (BTT) to strengthen decentralized capacity building under the health system. These teams are responsible for assessing skill gaps and the utilization of

competencies at various service delivery points. In addition, DTTs and BTTs coordinate with Regional Training Institutes (RTIs) for organizing periodic in-service training programmes.

3.12.3.1. Integrated Training of Medical Officers

Medical Officers (MOs) are trained for a period of 6 days on Comprehensive Primary Health Care services, including expanded 12 service packages such as NCDs, mental health, elderly care, palliative care, trauma and emergency care. During April 2025 – March 2026, 1,496 Medical Officers have undergone integrated training.

3.12.3.2. Integrated Staff Nurse of PHC/ UPHC/RBSK/MLHP

Staff Nurses are trained for a period of 5 days on delivery of 12 packages of services at Health and Wellness Centers, focusing on preventive, promotive and basic curative care 1,063 Staff Nurses have been trained during April 2025 – March 2026.

3.12.3.3. Integrated Refresher training to Pharmacists

The integrated training is being offered to the Pharmacists who are the Cold chain handlers at the

PHC / UPHC who are the key resources for maintaining, monitoring and managing the cold chain and responsible for safe storage of vaccine in their respective Public Health centers. 2 days training for 2,047 Pharmacists have been trained during April 2025 – March 2026.

3.12.3.4. Integrated Training of CHN / SHN / ANM / VHN / UHN/MLHP

Training is provided to frontline workers viz., CHN / SHN / ANM / VHN / UHN/MLHP through RTIs to support transformation of Sub Health Centers into Health and Wellness Centers and deliver expanded primary healthcare services. 1,644 personnel have been trained during April 2025 – March 2026.

3.12.3.5. Midwifery Training at State Midwifery Training Institute (SMTI), Chennai for Staff Nurses identified for the Midwifery Led Care Units (MLCU)

The State level Midwifery training aims to improve the quality of maternal and newborn health services, helps to reduce maternal and newborn deaths and illnesses, promote natural child birth and ensure respectful maternity care for

mothers and newborns. The second batch of 18 Staff Nurses is ongoing at the College of Nursing, Madras Medical College. In addition, a second batch of six Staff Nurses have been sent to the Fernandez Institute (NMTI), Telangana, for Midwifery Educator Training for six months.

3.12.3.6. Inter State agreement for Skill enhancement of Medical Officers from the State of Meghalaya

At the request of the Department of Health and Family Welfare, Government of Meghalaya, a Memorandum of Agreement (MoA) was signed between the Government of Meghalaya and Government of Tamil Nadu in September for the purpose of training and improving the skills of Medical professionals from Meghalaya on, Emergency Obstetric care, life-saving anaesthesia, Neonatal training and Ultrasonography.

The training is imparted in three institutions viz Kilpauk Medical College, Madras Medical College, Institute of Obstetrics and Gynaecology, Egmore, ISO, KGH, Triplicane and Institute of Child Health, Egmore. So far 91 Medical Officers have been trained under the program. The fifth batch of 6 months training with 20 Medical Officers has commenced from 2nd March 2026.

3.12.3.7. Training of Mid-Level Health Care Provider (MLHP)

Mid-Level Health Care Provider at Sub Centre-Health and Wellness Centre, provide clinical management & ambulatory care to the community and serve as an important link for ensuring continuum of care. 3,367 MLHP have been trained during April 2025 – March 2026.

3.12.3.8. Facility Based New Born Care Training to Staff Nurse

The well trained Staff Nurses with focus on skills and with participatory approach primarily provides the life-saving interventions to the babies, enable the mothers for baby care and counsel the families. 278 Staff nurses from SNCU and NBSUs have been trained during April 2025- March 2026.

3.12.3.9. Family Welfare Training

To maintain high standards in family planning services, comprehensive training sessions are regularly organized for service providers in Medical Methods of Abortion, contraceptive options, safe, effective abortion procedures, reproductive rights, legal frameworks, counseling strategies, clinical assessments, infection control and specialized skills in IUCD procedures.

- **Medical Officers Training in Medical Method of Abortion:** April 2025 to March 2026, a total of 584 Medical Officers have undergone the specialized training.

- **Training for Medical Officers and Staff Nurses in IUCD – Services:** Between April 2025 to March 2026, 221 Medical Officers and 549 Staff Nurses have received this training.

- **Training in Comprehensive Abortion Care / Medical Termination of Pregnancy:** From April 2025 to March 2026, a total of 189 Medical Officers have been trained and an additional 121 Obstetricians have received Mini Laparoscopy training during April 2025 to March 2026.

3.12.3.10. Orientation Training for Districts on National Quality Assurance Standards (NQAS)

- Under National Health Mission, Tamil Nadu, a 2-days Orientation Training on National Quality Assurance Standards (NQAS) is proposed at District Level for Medical Officers and Staff Nurses from both Rural and Urban

public health facilities. For the year 2026–2027, a total of 18 batches are proposed, including 9 batches under NHM and 9 batches under NUHM.

3.12.3.11. State Level Training of Trainers for Internal Assessors

State Level Training of Trainers (ToT) for Internal Assessors is an ongoing activity to strengthen the State's pool of trained Internal Assessors and District-level master trainers under the National Quality Assurance Standards (NQAS) programme. For the year 2026–27, a total of 8 batches are proposed.

3.12.3.12 Kayakalp Training

Kayakalp Training is also an ongoing capacity-building activity to strengthen cleanliness, hygiene, infection prevention & control practices, biomedical waste management, environmental sanitation and patient safety standards across Rural and Urban public health facilities under NHM and NUHM. For the year 2026–2027, a total of 12 batches have been proposed, including 7 batches under NHM and 5 batches under NUHM.

3.13. Tribal Health

3.13.1. Prevention and Control of Haemoglobinopathies

Haemoglobinopathies are genetic blood disorders predominantly prevalent among Tribal population. The programme aims to prevent and control Haemoglobinopathies through early detection, genetic counselling and appropriate management of blood disorders such as Sickle Cell Anaemia and Thalassemia.

Screening for Haemoglobinopathies is being carried out among adolescent students of standards 6th to 12th and unmarried school dropouts above 14 years of age in 30 tribal blocks across 14 districts. Around 2.8 lakh children have been screened till date and Genetic counselling is provided to children identified with this trait and their family members. The affected children are managed at Medical College Hospitals (MCHs) or District Head Quarters Hospitals (DHQHs).

In September 2021, the programme expanded to include universal screening of all antenatal mothers in the 30 tribal blocks, making Tamil Nadu a pioneering State in implementing this

initiative. Till March 2026, a total of 1.64 lakh antenatal mothers have been screened for Haemoglobinopathies and the birth of 39 children with Thalassemia and 13 children with Sickle Cell Anaemia has been prevented so far.

The programme has further been scaled up to community-level screening covering the Tribal population in the 0–40 years age group. Using Point-of-Care Testing (POCT) devices, 83,478 individuals have been screened in high-prevalent tribal regions including Kolli Hills (Namakkal), Yercaud (Salem), Kalrayan Hills (Kallakurichi) and Jamunamarathur (Tiruvannamalai). All screened data are being uploaded in the Government of India's Sickle Cell Portal for monitoring and followup.

3.13.2. Sickle Cell Anaemia Program through Non-Government Organizations

This initiative aims to provide comprehensive care and support services to individuals affected by Sickle Cell Anaemia among Tribal communities. The programme is being implemented in collaboration with two Tribal Non-Governmental Organizations (NGOs), namely NAWA and ASHWINI.

The programme is being implemented in the blocks of Kothagiri, Gudalur and Coonoor in The Nilgiris District, Periyanaickenpalayam and Karamadai blocks in Coimbatore District. During the period from April 2025 to March 2026, a total of 2,393 tribal beneficiaries have availed services under this scheme.

3.13.3. Integrated Centres for Haemophilia and Haemoglobinopathies (ICHH)

In Tamil Nadu, patients with Haemophilia A and B are provided treatment completely free of cost using Factor VIII and Factor IX concentrates. Patients with inhibitors are treated with Factor VII and FEIBA therapy. These services are provided on-demand in all Medical Colleges and District Hospitals across the State.

Integrated Comprehensive Haemophilia and Haemoglobinopathy (ICHH) Centres have been established in 10 Medical College Hospitals at the regional level, functioning as referral centres for specialized care. During the period from April 2025 to March 2026, a total of 507 Haemoglobinopathy patients received repeated blood transfusions and 2,351 Haemophilia patients received repeat factor transfusions through these centres.

Haemophilia prophylaxis therapy which is continued for 250 children, was initiated in 2022 across the 10 ICHH Centres to improve long-term clinical outcomes and reduce complications.

Bone Marrow Transplantation (BMT) is being provided as a curative therapy for Haemoglobinopathies. Till March 2026, a total of 10 Bone Marrow Transplants have been performed for Haemoglobinopathy patients, at Rajiv Gandhi Government General Hospital (RGGGH), Chennai and 8 Bone Marrow Transplants have been performed at the Institute of Child Health (ICH), Egmore.

3.13.4. Birth Waiting Room in Tribal PHCs

Tribal mothers in Tamil Nadu's remote hilly regions face serious challenges in accessing timely maternal healthcare due to geographical isolation. To address this critical gap, the Government of Tamil Nadu has established Birth Waiting Rooms (BWRs) within Primary Health Centres located in the foothills of tribal areas for ensuring safe deliveries for tribal antenatal mothers while reducing Maternal and Infant mortality.

The Antenatal mothers including those in high-risk category get admitted along with their

attenders and receive free nutritious meals, continuous medical monitoring and skilled obstetric care for 7 days covering pre-delivery and post-delivery periods.

A total of 21 Birth Waiting Rooms have been established in a phased manner across Tribal PHCs Statewide. The initiative has successfully benefited 2,740 tribal mothers till March 2026 reflecting the Government's unwavering commitment to ensuring equitable maternal healthcare for its most vulnerable Tribal communities.

3.13.5. Tribal Bed Grant Scheme

The objective of the Tribal Bed Grant Scheme is to ensure that the tribal patients receive necessary medical care without underlying financial hardships. It is implemented in collaboration with NGOs in The Nilgiris district, where a wide range of services, such as free diagnostics, in-patient medications, Surgeries and Dietary support during hospitalization are all provided at free of cost after hospitalization. A total of 1,680 tribal patients have benefited under this scheme during the financial year 2025-2026.

3.13.6. Tribal Mobile Medical Units

In addition to regular Mobile Medical Units, the Government of Tamil Nadu has deployed 20 existing Tribal MMUs across 14 Districts in high tribal population concentration areas, ensuring that quality healthcare reaches the most remote tribal habitations.

Each team comprising of a Medical Officer, Staff Nurse, Laboratory Technician and Driver, deliver services such as minor ailment clinics, Antenatal screening, NCD screening, diagnostic laboratory tests and supplying free medicines. Haemoglobinopathy screening is exclusively conducted among Tribal school children, enabling early detection and timely intervention.

During April 2025 – March 2026, Tribal MMU teams have conducted 10,219 Medical camps across Tribal villages and provided treatment to 5.82 lakh patients.

3.13.7. Accredited Social Health Activist (ASHA)

In Tamil Nadu, 2,650 ASHA workers (Village Health Volunteers) function under a specific work schedule as community-based health workers and deliver essential healthcare services in remote,

tribal, hilly and hard-to-reach areas, thereby ensuring easy accessibility to healthcare services. They are involved in various Public Health activities such as antenatal care, identification of high-risk pregnant women, motivating and safely accompanying pregnant women for Institutional deliveries, post-natal care for mothers and children, Home-Based Newborn Care (HBNC) and Home-Based Young Child Care (HBYC), mobilizing children for immunization sessions, maternal and child health activities, referral and follow-up of malnourished children and early detection of communicable and non-communicable diseases.

Further, ASHAs play a vital role in ensuring community participation by mobilizing beneficiaries for Village Health and Nutrition Day (VHND), Village Health, Sanitation and Nutrition Committee (VHSNC) meetings and Adolescent Friendly Health Service centres. In recognition of their dedicated field-level services and to encourage their continued efforts, performance-based incentives of Rs.5,950/- per month is being provided.

In addition, ASHAs have been enrolled under the social security schemes namely Pradhan Mantri Jeevan Jyoti Bima Yojana (PMJJBY), Pradhan Mantri

Suraksha Bima Yojana (PMSBY), and Pradhan Mantri Shram Yogi Maandhan (PM-SYM), for which the insurance premium is being borne by the National Health Mission.

During the year 2025–2026, skill upgradation training for a duration of four days was provided to 1,324 ASHAs through Regional Training Centres, thereby strengthening their capacities for delivering effective community based Healthcare services.

3.14. Mobile Medical Unit – Hospital on Wheels

Tamil Nadu operates 426 Mobile Medical Units (MMUs), for delivering quality healthcare services directly to the residents of both Rural and Urban regions.

- These units function as "Hospitals on Wheels," providing a comprehensive range of Medical services, including minor ailment clinics, antenatal check-ups, communicable and non-communicable disease screening, diagnostic laboratory tests and distribution of free medicines. Laboratory referral samples are transported to higher centers through IELS for testing. Of the 426 units, 396 MMUs serve the rural regions, 20 MMUs serve in Tribal areas

while the remaining 10 MMUs are exclusively designated for Municipal Corporation areas. From April 2025 to March 2026, the Mobile Medical Units had conducted 2.29 lakh camps across the State, benefiting a total of 2.01 crore beneficiaries.

- Under Pradhan Mantri Janjati Adivasi Nyaya Maha Abhiyan (PM-JANMAN), which is a time-bound mission to saturate all Particularly Vulnerable Tribal Group (PVTG) habitations with essential services, the Government of Tamil Nadu has taken significant strides in extending healthcare access to the most remote Tribal communities. Under this flagship initiative, 105 MMU teams comprising of both Regular and Tribal Mobile Medical Units have been exclusively deployed across 21 districts of the State, ensuring comprehensive coverage of all PVTG habitations to deliver continuous, quality medical care at the doorstep of the most geographically isolated tribal communities.

3.15. Labour Mobile Medical Units (LMMUs) - Occupational Health Services

Occupational Health Services in the State has been strengthened through deployment of

51 Mobile Health Clinics with 36 Labour Mobile Health Clinics placed under the Directorate of Public Health and Preventive Medicine and 15 Other units under the administrative control of Greater Chennai Corporation. This initiative aims to ensure early detection of health issues including assessment of Occupational health risks so as to enhance the quality of life of the labour workforce in the construction sites.

The initiative is funded by The Tamil Nadu Construction Workers Welfare Board (TNCWWB) under the Department of Labour Welfare and Skill Development. The Labour MMUs reach out the construction sites as per an Advance Tour Program and conduct Medical Camps to provide Promotive, Preventive, Screening, Treatment and Referral Services. The Camp sites are identified in consultation with the Directorate of Industrial Safety and Health and Assistant Commissioner of Labour in the districts.

In addition, the Government has issued directives to engage the existing MMUs at the block level to provide Occupational Health care services focusing on unorganized sector workers on all Saturdays.

A dedicated web based online portal is developed by NHM for monitoring the performance. In the year 2025-26, 16.86 lakh labourers have been screened and treated and 46,845 labourers referred to higher centre's for further assessment.

3.16. Multi-speciality Clinics for Transgender

Tamil Nadu has established dedicated multi-specialty clinics to provide equitable, comprehensive and respectful healthcare services to the transgender community. These clinics function once a week on an outpatient basis and cater to a wide range of healthcare needs

Services offered includes

i. Gender-affirmative hormone therapy
ii. Sex reassignment procedures
iii. Dermatological and Urological care
iv. Mental health counselling
v. Treatment for common ailments.

At present, these clinics are operational in 12 Government Medical College Hospitals across the State

1. Rajiv Gandhi Government General Hospital, Chennai

2. Government Rajaji Medical College Hospital, Madurai
3. Government Mohan Kumaramangalam Medical College Hospital, Salem
4. K.A.P.Viswanatham Government Medical College Hospital, Tiruchirappalli
5. Government Medical College Hospital, Coimbatore
6. Tirunelveli Government Medical College Hospital, Tirunelveli
7. Government Medical College Hospital, Vellore
8. Government Medical College Hospital, Villupuram
9. Government Medical College Hospital, Krishnagiri
10. Government Medical College Hospital, Dharmapuri
11. Government Medical College Hospital, Namakkal
12. Government Medical College Hospital, Thanjavur

During the period from April 2025 to March 2026, a total of 1,384 transgender persons have availed services through these clinics. The

establishment of these inclusive healthcare facilities marks a significant step towards ensuring services with dignity, equity and improved health outcomes for transgender individuals. By providing specialized and holistic care, including mental health support, the initiative aims to enhance access to quality healthcare, reduce stigma and promote social inclusion.

3.17. Non-Communicable Diseases (NCDs) – Prevention, Treatment and Control

Non-Communicable Diseases (NCDs) known for their chronicity result as a combination of Genetic, Physiological, Environmental and Behavioral Factors (WHO Fact Sheet, 2025). The spectrum of NCDs covers a wide range such as Cardiovascular diseases, Diabetes, Hypertension, Stroke, Chronic Kidney Diseases (CKDs), Cancers, Chronic Obstructive Pulmonary Diseases (COPDs), Metabolic Dysfunction-Associated Steatotic Liver Disease (MASLD), etc.,

According to WHO-2022, 74% of all global deaths and 66% deaths in India are due to NCDs.

NCDs cause significant morbidity and mortality, both in Urban and Rural populations and

across all socio-economic strata, with considerable loss in potentially productive years of life. NCDs also result in substantial Out-Of-Pocket Healthcare expenditures substantiated by NSS 80th round report 2025 for Tamil Nadu which points to an average OOPE for an In-patient at Rs.44,535/- and Rs.729/- for Outpatient.

Whilst Non-modifiable risk factors such as Age, Family history, Gender and Genetic predisposition remain unchanged, the interventions actually focus on the modifiable risk factors which are amenable for control measures. The major modifiable risk factors are Tobacco usage, Alcohol intake, Unhealthy diets, Insufficient Physical Activity, Overweight/obesity, Air Pollution and raised levels of Blood glucose, Lipids and Blood pressure.

The impact of NCDs in Tamil Nadu is driven by rapid urbanization, population ageing and evolving disease patterns. According to the Global Burden of Diseases report 2023, Ischemic heart disease remains the leading cause of death followed by Diabetes, Stroke and COPD in the State.

The NFHS-6 (2023–24) findings indicate a significant and growing burden of NCD risk factors in Tamil Nadu. Overweight and obesity continue to rise, affecting 44.2% of women and 38.8% of men aged 15–49 years, while the proportion of the population aged 60 years and above has increased to 16.3%, reflecting rapid demographic ageing.

The prevalence of elevated blood sugar levels has also increased affecting 25.2% of women and 26.7% of men aged 15 years and above, highlighting the growing Diabetes burden. Hypertension remains highly prevalent, with 24.2% of women and 28.2% of men having elevated blood pressure or receiving treatment. In addition, behavioural risk factors continue to contribute significantly to the NCD burden, with 17.7% of adults using Tobacco and 23.5% of men consuming Alcohol.

The second round of Tamil Nadu STEPS Survey 2024 report states a prevalence of 19.57% for Hypertension, 9.64% for Diabetes, and 9.86% for Both Hypertension & Diabetes (Table:3.11.2.). The control rates reported are 17.0% for Hypertension and 16.7% for Diabetes.

A comparison between Round I and Round II STEPS surveys highlights 16.9% increase among Hypertension patients and a 20.2% increase among Diabetes patients seeking care in Government facilities. The STEPS II survey findings further suggest improved awareness, better disease control, and increased utilisation of Government healthcare services for the management of Hypertension and Diabetes in Tamil Nadu in comparison with the first round of STEPS Survey conducted in 2019-20.

Table 3.3: Prevalence of Hypertension and Diabetes – India vs. Tamil Nadu

	Prevalence in %			
Disease	**India (NNMS[1])**	**India (INDIAB[2])**	**Tamil Nadu[3] (STEPS I)**	**Tamil Nadu[4] (STEPS II)**
Hypertension	28.5	35.5	34.55	29.43*
Diabetes	9.3	11.4	17.4	19.5*

***** *Including both hypertension and diabetes.*

*[**Source**: National NCD Monitoring Survey 2017-18[1], ICMR-INDIAB-17 National Cross-sectional Study published in July 2023[2], GO-TN initiated ICMR-NIE STEPS Survey Round I – 2019-2020[3], STEPS Survey Round II-2023-24[4].]*

The table given below highlights the growing burden of Non-communicable diseases (NCDs) reiterating the position of NCDs as a Public Health priority:

Table 3.4: Top 10 causes of Disability – Adjusted Life Years (DALYs) lost

Rank	India	Tamil Nadu
1.	Ischemic Heart Disease	Ischemic Heart Disease
2.	Neonatal Disorders	Diabetes
3.	Stroke	Stroke
4.	COPD	COPD
5.	Diabetes	Falls
6.	Lower Respiratory Infection	Neonatal Disorders
7.	Diarrheal Diseases	Depressive Disorders
8.	Dietary Iron Deficiency	Road Injuries
9.	Road Injuries	Self-harm
10.	Falls	Diarrheal Diseases

(**Source**: Global Burden of Diseases Report – 2023)

The State of Tamil Nadu has adopted a comprehensive approach encompassing prevention, early diagnosis, treatment, follow-up, and long-term disease management across both community and healthcare facility settings. Following the COVID-19 Pandemic, the State launched the "Makkalai Thedi Maruthuvam" (MTM) scheme to strengthen continuity of care through Doorstep screening, Drug distribution, and follow-up services for NCD management.

To support effective monitoring and service delivery, the State established the Makkalai Thedi Maruthuvam (MTM) portal, which maintains a line-listed database of all registered Hypertension, Diabetes, and Cancer patients, along with beneficiaries receiving allied services such as Physiotherapy, Palliative care, and Continuous Ambulatory Peritoneal Dialysis (CAPD) support for Chronic Kidney Disease (CKD). The portal serves as a robust digital platform for patient tracking, treatment monitoring, continuity of care, and timely follow-up.

The State's commitment to delivering integrated, patient-centric NCD services across all levels of care has strengthened health outcomes and enhanced the effectiveness of comprehensive NCD management.

3.17.1. NCD interventions in Tamil Nadu

a. Facility based services

Across the Primary, Secondary and Tertiary care facilities, a total of 2,671 NCD Clinics have been established in the State, covering 2,336 at Primary Health Centres (PHCs), 262 at Sub-District Hospitals (SDHs), 37 at District Head Quarters

Hospital (DHQH) and 36 in Medical College Hospitals (MCHs). To support service delivery, NCD Staff Nurses have been deployed at the rate of one per PHC and two per Government Hospitals and Medical College Hospitals.

The facilities provide comprehensive NCD care services, including screening for Hypertension, Diabetes and Oral Cancer for target men & women and women aged 30 and above are subjected in addition for Cervix and Breast Cancer screening. Individuals diagnosed with NCDs are provided protocol-based treatment and follow-up services for effective Disease control.

All the facilities are provided with adequate NCD drugs procured centrally through TNMSC. The services also include Annual NCD complications screening for all Hypertensives and Diabetics by screening them for Chronic Kidney Disease, Diabetic Foot, Metabolic Dysfunction–Associated Steatotic Liver Disease (MASLD) and Diabetic Retinopathy. Further, all Medical Colleges and District Headquarters Hospitals are being equipped with appropriate infrastructure to treat NCD Complications. In addition, Institutional & Community based Physiotherapy and Palliative

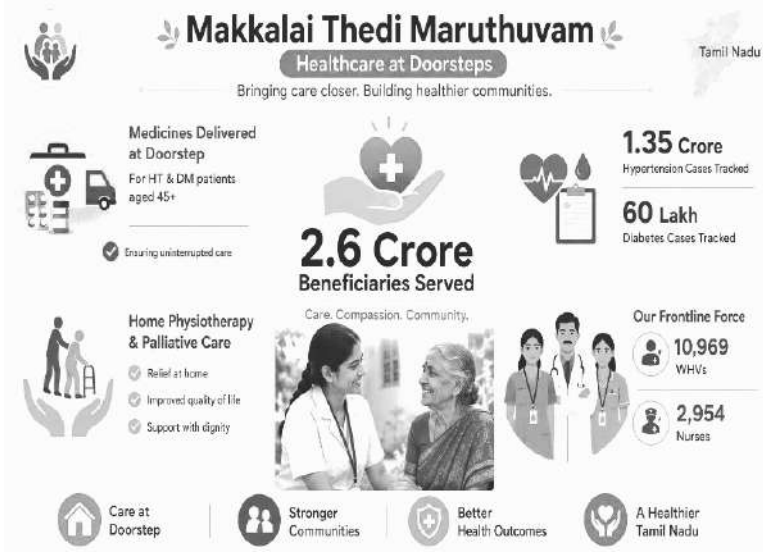
care services are provided in an integrated manner to ensure holistic management and continuity of care for patients with chronic diseases.

b. Makkalai Thedi Maruthuvam - Home based NCD services

Home-based services for Non-Communicable Diseases under "Makkalai Thedi Maruthuvam" (MTM) addresses the growing burden of Non-Communicable Diseases aimed at improving treatment compliance and disease control. The Women Health Volunteers (WHVs) of the Self-Help Group network provides preliminary screening services for Hypertension and Diabetes. They also provide doorstep delivery of HT & DM medicines to eligible beneficiaries aged 45 years and above. The STEPS II survey has complemented that 43.7% of Hypertension patients and 39.6% of Diabetes patients have availed Home-based services.

A dedicated MTM team comprising of Palliative Staff Nurse and Physiotherapist provide Home-based Palliative care and Physiotherapy services and other support services including CAPD bag distribution for End Stage Kidney Failure patients. The programme is supported by an

extensive healthcare workforce comprising 2,954 MTM Staff Nurses, 10,969 Women Health Volunteers (WHVs), 463 Palliative Care Nurses, 463 Physiotherapists, 4,848 MLHPs and 2,448 Health Inspectors who deliver frontline healthcare services across Tamil Nadu.



As of 31.07.2026, 2.65 crore beneficiaries have benefitted, out of whom, 1,38,18,165 individuals have been identified with Hypertension, 61,01,949 with Diabetes and 57,68,718 with both HT and DM as detailed below:

Table 3.5: Total No. of Beneficiaries under MTM scheme updated upto July 2026

Services	First Time Beneficiaries	Repeat Service Beneficiaries
Hypertension	1,38,18,165	2,33,76,296
Diabetes	61,01,949	1,53,87,794
Both Hypertension and Diabetes	57,68,718	1,32,81,641
Palliative Care	7,63,945	16,63,675
Physiotherapy	10,14,305	36,01,527
CAPD Bags	695	9,035
Total	**2,65,96,927**	**5,72,79,968**

The Government has initiated focussed measures under MTM to digitise the process of identifying the prevalence, tracking the care provided and hence the control rates.

c. Type 1 Diabetes Mellitus (T1DM)

Type 1 Diabetes Mellitus (T1DM) is an autoimmune condition commonly affecting children and adolescents, characterized by the complete destruction of insulin-producing beta cells of the Pancreas. Type 1 Diabetic children are at the risk of developing acute life-threatening complications such as Hypoglycemia and Diabetic Keto Acidosis. The sub-optimal metabolic control may lead to poor

growth and early onset of Vascular complications and even mortality in the long run.

As per the International Diabetes Federation Atlas, 11th Edition (2025), globally an estimated 9.5 million people are living with clinically diagnosed T1DM. As per the Global Burden of Disease (GBD) report 2023, it is estimated that 67 individuals for every 1,00,000 population in India are living with T1DM which is 95 individuals for every 1,00,000 population in case of Tamil Nadu.

A State-level T1DM Registry has been established under NHM-TN which serves as a critical tool in improving survival, ensuring continuity of care, and enhancing the quality of life for children and adolescents with T1DM in Tamil Nadu. So far, 5,664 children have been enrolled in this registry.

Regular blood glucose monitoring and adequate insulin supplementation are essential for reducing the morbidity and mortality associated with Type 1 Diabetes Mellitus. Accordingly, NHM-Tamil Nadu focuses on ensuring the uninterrupted supply of insulin, including newer insulin analogues, along with the essential consumables for all individuals enrolled in the Type 1 Diabetes Mellitus Registry.

Type 1 Diabetes Specialized Care Centres are established at 10 Medical College Hospitals under Public–Private Partnership. These centres are operational in ICH-Chennai, Salem MCH, Thanjavur MCH, Trichy MCH, Tirunelveli MCH, Dharmapuri MCH, Theni MCH, Namakkal MCH, Tiruppur MCH and Cuddalore MCH.

The Centres support children with Type 1 Diabetes by providing insulin, essential consumables and comprehensive care services, including Advanced Retinal Imaging and AI-enabled Retinopathy screening facilities.

d. Screening for NCD Complications

Long-term complications of Non-Communicable Diseases (NCDs), particularly Hypertension and Diabetes include Retinopathy, Nephropathy leading to Chronic Kidney Disease (CKD), Neuropathy and Metabolic Dysfunction–Associated Steatotic Liver Disease (MASLD). Poor disease control significantly increases the risk of developing these complications, many of which are preventable through early detection and appropriate management. However, delayed diagnosis often results in advanced disease manifestations such as Vision loss, End-stage renal

disease requiring long-term dialysis, Lower-limb amputations due to severe Diabetic Neuropathy and Liver failure resulting from long-standing MASLD.

Early screening and effective management are therefore critical to reduce morbidity, premature mortality and the associated economic burden on individuals, families and the healthcare system.

In order to systematically monitor the NCD Complication through proper identification and management, an annual NCD complication screening initiative is being planned so that, all the line-listed HT/DM patients under MTM can undergo complication screening through mobilization on the basis of pre-fixed schedule. The specialists required also will be arranged in one setting to facilitate this. NHM-TN has taken specific initiatives to identify complications at the health facility level, as detailed below:

i) Retinopathy Screening

Diabetic Retinopathy is one of the leading causes of preventable blindness among adults of working age and poses a significant Public Health

challenge. The SMART India Study (Lancet Global Health, 2022) estimates the National prevalence of Diabetic Retinopathy at 12.5% and Vision-Threatening Diabetic Retinopathy (VTDR) at 4.0% among the Diabetic population, with approximately 3 million people aged 40 years and above having VTDR in India.

As the condition often remains asymptomatic until advanced stages, early screening and timely intervention are critical to prevent vision loss and preserving quality of life among the Diabetics.

To facilitate early detection and management of Retinopathy among individuals with Hypertension and Diabetes, the State has implemented a structured screening pathway. Under this initiative, all registered Hypertension and Diabetes beneficiaries undergo an initial questionnaire-based risk assessment conducted by MTM Staff Nurses. Individuals identified as being at-risk are referred to one of the 38 designated Tele V-Care Centres, established across the State for fundus examination.

At the Tele V-Care Centres, trained Para Medical Ophthalmic Assistants (PMOAs) capture retinal images using fundus cameras, which are

subsequently transmitted for Specialist's evaluation. Individuals identified with suspected Retinal abnormalities are referred to Ophthalmologists at District Headquarters or Medical College Hospitals for clinical confirmation and treatment. Diabetic Retinopathy Evaluation Centres or Specialized Retina Treatment Centres are established within the Ophthalmology Department. This comprehensive system ensures improved outcomes among individuals with Hypertension and Diabetes. In the year 2025-26, a total of 1,08,236 individuals have undergone Retinopathy screening under the programme.

ii) Nephropathy

Chronic Kidney Disease (CKD) is caused by uncontrolled Hypertension and Diabetes. According to the GBD Study 2023, the prevalence of Chronic Kidney Disease attributable to Type 2 Diabetes Mellitus is 2.47% in Tamil Nadu compared to 1.93% nationally, highlighting the higher burden of Diabetic Nephropathy in the State. Since CKD often remains asymptomatic in its early stages, early detection through routine screening is therefore critical to enable timely intervention, prevent disease progression, reduce complications and improve long-term health outcomes.

To strengthen the early identification and management of CKD, a comprehensive screening strategy has been implemented across different levels of the healthcare system. Under this initiative, routine urine dipstick testing is conducted at HWC-HSCs and PHCs to screen individuals above 18 years for proteinuria and those detected are referred to higher centre for further evaluation.

All Diabetics and Hypertensives registered under MTM are subjected to assessment for CKD at NCD clinics by estimating e-GFR based on a set criteria. Patients requiring specialist evaluation are referred to higher-level facilities, including Sub-District Hospitals (SDHs) and District Headquarters Hospitals (DHQHs) for Ultrasound and Nephrology consultation. This systematic approach enables early detection of CKD, timely treatment, and improved patient outcomes while reducing the risk of progression to kidney failure.

In the year 2025-26, a total of 1.46 crore individuals were screened at PHCs and HSCs under the programme, of whom 43,880 have been referred for further diagnostic evaluation and management.

iii) Metabolic Dysfunction - Associated Steatotic Liver Disease (MASLD)

Metabolic Dysfunction–Associated Steatotic Liver Disease (MASLD) is increasingly associated with metabolic risk factors such as obesity, Diabetes Mellitus, Hypertension and Dyslipidemia and can progress silently to advanced liver disease if not identified early. As per the Global Burden of Disease (GBD) Study 2023, the prevalence of Metabolic Dysfunction-Associated Steatotic Liver Disease (MASLD) in India is 12.74%, while Tamil Nadu records a notably higher prevalence of 16.09%, highlighting the greater burden of this condition in the State. Since the condition is often asymptomatic in its initial stages, early screening and risk stratification are essential for timely intervention and management.

To strengthen the screening and early identification of MASLD, FIB-4 scoring is being implemented at all Primary Health Centres (PHCs) for known Hypertension and Diabetes mellitus. Based on the FIB-4 score, individuals with intermediate and high-risk scores are referred to secondary care centres, where diagnosis of MASLD and risk stratification for advanced liver fibrosis are

carried out by specialists. At the secondary care level, lipid profile and thyroid profile investigations are performed and Ultrasonography (USG) abdomen is performed to grade fatty liver disease (Grade 1, 2 and 3). Patients with Grade 2–3 fatty liver, as well as uncontrolled Diabetic and Hypertensive patients, are further referred to tertiary care centres for Specialist evaluation and further management.

This structured referral mechanism facilitates early detection, appropriate risk stratification and timely management of MASLD within the public healthcare system.

iv) Paadham Paadhukappom Thittam

In India, Diabetic foot ulcers (DFUs) affect 6.2% of individuals with diabetes (Sahu et al., 2024). 85% of DFUs results in limb amputations, indicating a significant health burden and loss in the quality of life.

The Integrated Diabetic Foot Care Programme titled "Padham Padhukaapom Thittam", covers all aspects of Podiatry such as screening, patient education, ulcer prevention, wound care, footwear, surgical offloading, rehabilitation and provision of prosthesis in case of amputation.

Diabetic Foot Screening services are made available in all 2,336 PHCs where Diabetes patients are screened for Diabetic Foot using Monofilament and Biothesiometer tests. All those suspected with foot ulcers are referred to Specially designated Diabetic Foot Clinics (DFC) for availing Surgical wound/ Ulcer debridement services across 101 DMS institutions and 36 Medical College Hospitals.

High-end Diabetic Foot Surgical units in 15 Medical College Hospitals provide Off loading surgeries and Amputations by integrating Plastic Surgery, Orthopaedics and Vascular Surgical units. 17 CMCHIS packages are exclusively assigned for High-End Surgeries under Patham Pathukapoom Thittam.

As of March 2026, 88.60 Lakh Diabetic patients have been screened in Tamil Nadu for Diabetic foot. Of those screened, 57,158 individuals identified with foot ulcers have been referred to Diabetic Foot Clinics at Secondary and Tertiary care institutions for further management. Subsequently, 32,939 Surgical procedures have been performed at Government Medical College Hospitals under the programme.

3.17.2. Cancer Care Services in Tamil Nadu

Cancer is becoming significant public health concern in Tamil Nadu, with an estimated annual incidence exceeding 1 lakh new cases according to the Tamil Nadu Cancer Registry Project (TNCRP 2021 report). The five most prevalent type of cancers in the State are Breast, Cervical, Oral, Colorectal and Stomach Cancer and the lifetime cumulative risk of developing cancer is estimated to be 1 in 11.

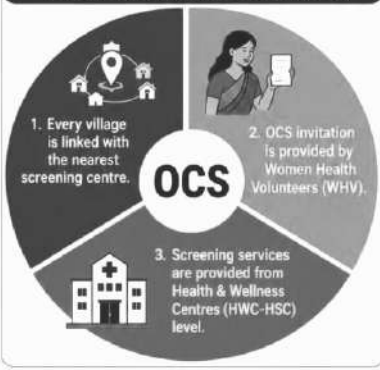
To address the growing cancer burden, while building upon the existing strengths and addressing the identified gaps in cancer care services, the Tamil Nadu Cancer Care Mission is envisaged as a comprehensive and integrated public health initiative aimed not only at sustaining the ongoing activities, but also at further strengthening the new initiatives.

3.17.2.1 Tamil Nadu Cancer Care Mission

The Mission aims to create a comprehensive cancer care pathway, with its strategies encompassing the entire continuum of cancer care including prevention, screening, early diagnosis, treatment, palliative care and survivorship services.

CANCER CARE SERVICES IN TAMIL NADU

ORGANIZED CANCER SCREENING (OCS)



CANCER SCREENING – 2025–2026

95.88 LAKH
Were invited at doorstep for screening

43.86 LAKH	43.5 LAKH	33.3 LAKH	2,446
Oral cancer screened	Breast cancer screened	Cervical cancer screened	Cancer cases identified

EQUIPMENT FOR CANCER DETECTION AND TREATMENT SERVICES

- **7** PET centres
- **11** LINAC centres
- **14** Cobalt - 60 centres
- **9** Brachytherapy centres

CANCER TREATMENT AND CARE SERVICES

- **1** State Apex Cancer Centre
- **5** Regional Cancer Centres
- **12** Medical College Hospitals

DAY CARE CHEMOTHERAPY CENTRES (DCCC)

38 DCCC centres are functioning in 38 districts.

HPV VACCINATION PROGRAMME

HPV vaccine is provided for 14-year-old girls.
Available in all Government health institutions in all districts.

Advancing comprehensive, accessible and integrated cancer care for every citizen of Tamil Nadu

I. Community-level Cancer Screening Activities

a) Organized Cancer Screening (OCS) for Oral, Breast and Cervical cancers has been expanded across all districts from the level of Health and Wellness Centres–Health Sub Centres (HWC-HSCs) to strengthen community-level early detection services. Under this initiative, OCS invitation cards are issued at the doorstep by Women Health Volunteers (WHVs) to create awareness and improve participation in cancer screening services. During 2025–2026, 95.88 lakh invitations were issued under OCS and 43.86 lakh individuals were screened for Oral cancer. In addition, 33.3 lakh women and 43.5 lakh women have undergone screening for Cervical and Breast Cancer respectively across Government health facilities leading to identification of 2,446 confirmed cancer cases.

b) Early detection of Colorectal Cancer is being piloted in Coimbatore district and is envisaged for future expansion.

c) Early detection of Lung cancers through AI-based interpretation of Chest X-rays is also planned for pilot in three institutions at Coimbatore, Madurai and Salem.

II. Institutional Oncology Diagnostic Services

Essential diagnostic facilities for confirmation and management of cancer cases have been provided across the State, including 68 Mammogram units, Colposcopy services in 216 institutions and PET in 7 centres. CT Scan, MRI, Digital X-Ray and Histopathology services are also made available in all District Headquarters Hospitals and Medical College Hospitals.

III. Cancer Management Services

Government Arignar Anna Memorial Cancer Hospital, Karapettai functions as the State Apex Centre for Cancer Care. Tamil Nadu currently has 5 Regional Cancer Centres and multispecialty Oncology units in 12 institutions. LINAC facilities are available in 11 institutions, Cobalt-60 Teletherapy units in 14 institutions and Brachytherapy services in 9 institutions.

IV. District-level Day Care Chemotherapy Centres (DCCC)

Day Care Chemotherapy Centres (DCCCs) are operational in 38 institutions to provide follow-up and maintenance Day Care Chemotherapy services closer to the medical beneficiary's residence under supervision. The decentralized availability of chemotherapy services at district level improves treatment continuity, reduces medical beneficiary's travel need and minimizes out-of-pocket expenditure, thereby enhancing accessibility and compliance to cancer care services.

V. HPV Vaccination

HPV vaccination is a novel initiative introduced for the protection of 14-year-old girls with HPV vaccine. The programme has been launched in four districts viz., Dharmapuri, Perambalur, Tiruvannamalai and Ariyalur through a school-based campaign mode. In addition, facility-based HPV vaccination services are available in all Government health institutions across the State.

VI. New initiatives

Molecular diagnostics, targeted therapy and immunotherapy services and Programme for Patient Survivorship and Navigation are other provisions planned to be strengthened under the Mission, as part of advancing comprehensive cancer care services in the State.

3.17.3. National Oral Health Programme (NOHP)

The National Oral Health Programme (NOHP) recognizes oral health as an integral component of overall health and well-being.

In order to improve accessibility and affordability of dental care, 500 dental units have been established across Primary and Secondary healthcare facilities in the State. These units are supported under the National Health Mission (NHM) and are staffed with trained Dental professionals and equipped with essential infrastructure, including Dental chairs and necessary consumables, to ensure quality service delivery.

During the period from April 2025 to March 2026, these facilities collectively recorded 45,32,008 outpatient visits and carried out 18,80,278 Dental procedures, catering to a broad spectrum of Oral Health needs.

As a measure for further strengthening dental services, a Multi-Speciality Dental Hospital has been established at Tambaram Government Hospital, which is significantly contributing to enhancing the scope of specialized Dental care.

3.17.4. Dialysis Program for Managing Chronic Kidney Disease

Chronic Kidney Disease (CKD) has emerged as a significant Public Health challenge, requiring sustained monitoring and timely intervention to prevent its progression to End-Stage Renal Disease (ESRD). ESRD, being a chronic and irreversible condition, necessitates lifelong dialysis and comprehensive medical care. Patients undergoing Haemodialysis often face multiple challenges, including limited access to treatment centres, difficulties in balancing employment or education and the financial burden associated with continuous Out-Of-Pocket Expenditure (OOPE).

In response to these challenges**,** Tamil Nadu has accorded high priority to ensure universal access to affordable Haemodialysis services, extending such services even to Sub-District Hospitals. The State ranks among the top

performers in India in terms of availability of Haemodialysis machines, significantly enhancing access across both Urban and Rural areas.



Under the Prime Minister National Dialysis Programme (PMNDP), Tamil Nadu has undertaken several initiatives to strengthen infrastructure and bridge gaps in CKD management. During the period from April 2025 to March 2026, a total of 1,395 Haemodialysis machines are operational

across 146 centres under an in-house model, including Government Medical College Hospitals, District Headquarters Hospitals, Taluk and Non-Taluk Hospitals, as well as centres through Public-Private Partnership (PPP) arrangements in the Greater Chennai Corporation and Chengalpattu.

During April 2025-March 2026, the State has conducted 9.12 lakh haemodialysis sessions, benefiting 9,471 patients.

Further, complementing facility-based services, Tamil Nadu has pioneered a community-based Continuous Ambulatory Peritoneal Dialysis (CAPD) initiative integrated with the *Makkalai Thedi Maruthuvam* programme. Under this model, CAPD consumables are delivered directly to patients' homes, enabling home-based self-dialysis. As of March 2026, 379 patients have received CAPD supplies at their doorsteps, thereby improving treatment adherence and quality of life, facilitating ambulant mode of life. The inclusion of CAPD under the Chief Minister's Comprehensive Health Insurance Scheme (CMCHIS) contributes to significant reduction in the OOPE burden for the beneficiaries.

3.17.5. National Programme for Prevention and Control of Deafness

The National Programme for Prevention and Control of Deafness (NPPCD) aims to reduce the burden of preventable hearing loss through early detection, timely treatment, and strengthening institutional capacity for ear and hearing care services. As per the Global Burden of Disease (GBD) Study 2023, the prevalence of hearing loss in India is 18.68%, while Tamil Nadu records a higher prevalence of 23.04%, indicating a greater burden of hearing loss in the State compared to the National average.



Tamil Nadu has made significant strides in implementing the programme through infrastructure development, capacity building, and expanded screening services across various levels of healthcare. Notably, Tamil Nadu is the second State in India to implement a Universal Neonatal Hearing Screening Programme. To facilitate universal screening of newborns, 64 CEmONC Centres have been equipped with Otoacoustic Emission and Brainstem Evoked Response Audiometry devices, ensuring timely detection and intervention for neonatal hearing loss.

The State has also inaugurated the state-of-the-art facility for temporal bone dissection training at the State Apex Centre for NPPCD in Rajiv Gandhi Government General Hospital. The three-day training is being conducted in batches every month since August 2025. In each batch, 4 ENT Surgeons are trained on the following aspects:

1. Temporal Bone cadaver Dissection demonstration and hands on training
2. Live Ear Surgery observation at ENT OT
3. Hands-on training on the use of OAE & BERA equipment in Audiology Department

In the year 2025-26, a total of 1,83,540 individuals were screened for deafness under the programme, of whom 5,893 have been provided with Hearing aids and 7,127 with Rehabilitation & follow-up services.

3.17.6. National Programme for Palliative Care (NPPC)

Palliative Care is an integral component of comprehensive healthcare aimed at improving the quality of life of patients suffering from chronic, life-limiting and debilitating illnesses including Cancer, advanced Non-communicable diseases, Neurological disorders, Geriatric conditions and a wide range of diseases. With the increasing burden of Non-Communicable Diseases and the growing elderly population, the need for accessible, affordable and quality Palliative care services has become increasingly relevant and significant. The National Programme for Palliative Care (NPPC) focuses on providing holistic and patient-centred care through pain relief, symptom management, psychosocial support, rehabilitative care and end-of-life services.

3.17.6.1. Palliative Care Units And Services

Palliative Care Units have been established in 34 Government Medical College Hospitals and 15 District Headquarters Hospitals to provide facility based specialised Palliative care services for patients with advanced and chronic illnesses.

The units are equipped with trained teams comprising of Medical Officers, Staff Nurses and multi purpose health workers to deliver comprehensive Palliative care services where patients receive outpatient, inpatient and follow-up supportive care services. Services include pain relief, symptom management, Nursing care, counselling, Physiotherapy support and end-of-life care. The beneficiaries of the programme include wide number of conditions such as those suffering from Cancer, Cardiac Failure, Intellectual disability, Chronic Obstructive Pulmonary Disease (COPD), Chronic Kidney Disease, Hemiplegia, Paraplegia, Alzheimer, Parkinson's, chronic disability of the elderly, etc. For children, the major disease categories which require palliative care are cancer, HIV, progressive non-malignant conditions, congenital anomalies, etc. Essential drugs including pain management medications are being ensured for un-interrupted patient care.

Additionally, at the block level, both institutional and Community Based Palliative Care Services are offered through 463 block units (all 385 Rural areas and 78 Urban areas). Across all the Districts, Home based Palliative care nursing services are provided by a trained Staff Nurse designated as 'Community Palliative Care Nurse' @ 1 per Block along with a Physiotherapist to deliver Home-based Palliative care and Physiotherapy services. This continues to remain a major component of the programme, particularly for bed-ridden, elderly and chronically ill patients. Capacity building and periodic training programmes are also conducted for healthcare professionals to strengthen service delivery and improve clinical competencies in Palliative care.

During the year April 2025 – March 2026, a total of 44,275 new patients have been registered as out-patients and 86,239 catered to as part of follow-up OP visits. Further, 22,692 patients have availed in-patient Palliative care services, 1,14,159 beneficiaries Home - based Palliative care services and 1,48,492 other Rehabilitation services.

3.18. National Programme for Health Care of Elderly (NPHCE)

Tamil Nadu is among the leading ageing States in India, with a significantly higher proportion of elderly persons compared to the national average. As per National Family Health Survey – 6 (NFHS-6), persons aged 60 years and above constitute 16.3% in Tamil Nadu against the National average of 12.9%. Based on the Mid-Year Population Estimates (MYPE-2025), Tamil Nadu has an estimated population of 7.75 crores, of which 1.26 crores are elderly individuals. The increasing elderly population necessitates strengthening of comprehensive, accessible and affordable Geriatric health care services across all levels of health care delivery. In order to address the health care needs of elderly persons, the Government of Tamil Nadu, through the National Health Mission, has implemented the National Programme for Health Care of Elderly (NPHCE) by establishing elderly-friendly health care facilities at Primary, Secondary and Tertiary levels throughout the State.

The objective of the programme is to provide accessible, affordable and high-quality long-term care services to elderly persons, strengthen

preventive, promotive, curative and rehabilitative Geriatric health care services, build capacity of Medical and Paramedical professionals in Geriatric care and ensure continuum of care through an effective referral system.

NPHCE Centres and Services

3.18.1. The National Centre for Ageing (NCA), Chennai

NCA has been established as a strategic initiative of Government of India and Government of Tamil Nadu to function as a Centre of Excellence of national importance for offering comprehensive Geriatric care. The 200 bedded Centre in the campus of King Institute of Preventive Medicine, Guindy, provides Multi-disciplinary Tertiary care services, specialized clinics, Long term and Rehabilitation services, Dementia care & Psychosocial counselling and also promotes research and human resource development.

3.18.2. Geriatric units in medical college hospitals / District hospitals

District level Geriatric care units have been established in 28 Government Medical College Hospitals and 3 District Headquarters Hospitals

with required infrastructure and human resources for providing dedicated Geriatric care services. The services include exclusively dedicated Geriatric outpatient services, inpatient care, intensive care support, Physiotherapy services, laboratory tests and specialist services through other departments such as General Medicine, Orthopaedics, Ophthalmology and ENT. Dedicated 20 bedded Geriatric wards with elderly-friendly infrastructure including anti-skid flooring, side rails and accessible sanitation facilities have also been established.

3.18.3. At Block level

Physiotherapists are positioned covering 463 block units (all 385 Rural areas and 78 Urban areas) across all the districts in the State at the rate of One Physiotherapist per unit to provide Geriatric care services both at block and community level along with Palliative care Staff nurse. Geriatric OPD for the elderly is conducted twice a week on Thursdays and Saturdays providing Primary level Geriatric care, Palliative care, Mental health services and Specialty services like Ophthalmology and ENT are some of the key geriatric services provided at the block level.

During the year April 2025-March 2026, a total of 9,87,305 Elderly persons have attended Geriatric OPD, 40,810 availed in-patient services, 2,83,977 availed Rehabilitation services and 17,00,744 Laboratory tests performed as part of diagnostics.

3.19. National Programme for Prevention and Control of Fluorosis

Fluorosis is a major Public Health problem caused by prolonged consumption of excess fluoride through contaminated drinking water, food products or industrial exposure. In Tamil Nadu, fluoride contamination of groundwater remains a concern in several districts, particularly in areas dependent solely on borewell water. Chronic exposure can lead to Dental and Skeletal fluorosis, causing deformities, disability and reduced quality of life, especially among children and vulnerable populations.

The National Programme for Prevention and Control of Fluorosis (NPPCF) has been implemented in Dharmapuri district since 2008 for surveillance, early diagnosis, management, and rehabilitation of fluorosis cases. In view of the continued burden, expansion of the programme to four additional affected districts is proposed.

Programme activities include screening in schools, ICDS centres, and endemic villages; testing of water and biological samples; training of health personnel; conduct of medical camps; micronutrient supplementation; and IEC/BCC activities on safe drinking water and fluorosis prevention. Convergence with departments concerned with rural water supply is also undertaken to promote access to safe drinking water. Severe cases are referred to tertiary care institutions for advanced management and rehabilitation support.

In the year 2025-2026, 1,512 water samples were tested for fluoride levels against the target of 1,508 samples. Medical management, including supplementation, was provided to all 1,103 diagnosed fluorosis cases, achieving 100% coverage against the target of 90%. There were no cases requiring Surgery or Rehabilitation support during the year.

The proposed expansion of NPPCF will further strengthen surveillance, early detection, community awareness, and timely management of fluorosis cases in affected districts.

3.20. Other Initiatives

3.20.1. Phasing-out Mercury Based Equipment

Mercury is amongst the most toxic naturally occurring elements, recognized globally as a major environmental contaminant and neurotoxin. Even in trace quantities, it causes serious harm to the brain, spinal cord, kidneys, eyes and liver with particularly severe consequences for pregnant women, newborns and children including developmental abnormalities and damage to Central Nervous System. The healthcare sector has historically been a significant source of mercury exposure to healthcare workers through its wide use in the making of Clinical thermometers, Sphygmomanometers, Oesophageal dilators, and Dental amalgams. Breakage, leakage or improper disposal of these devices can release Mercury vapour into healthcare settings, endangering patients, healthcare workers and the broader environment.

Recognizing these hazards, India ratified the Minamata Convention on Mercury in 2018 and is mandated to phase out Mercury-containing thermometers and Blood Pressure measuring devices.

The Ministry of Health and Family Welfare (MoHFW) along with Ministry of Environment, Forest and Climate Change (MoEFCC), World Health Organization (WHO) and United Nations Environment Programme (UNEP) is providing technical support to Tamil Nadu in the phasing out of Mercury-based equipment in the Health Sector.

In alignment with this National commitment Tamil Nadu discontinued the procurement of Mercury-based measuring devices through TNMSC from 2017 onwards. Mercury waste generated from healthcare facilities across the State is currently managed through 11 Common Treatment Facilities (CTFs), where collected mercury waste is securely stored and processed further in a safe manner.

A pilot project funded by UNDP for the collection, disposal and storage of mercury waste was initiated at the Singaperumal Koil CTF site in Tamil Nadu and a dedicated Mercury Storage Depot has been created for storing all the disposed Mercury devices. The site was also visited by the GoI team on 9th April 2026 as an explorative & learning experience. Several coordination meetings are also being held with MoHFW and other stakeholders to review progress and plan the implementation of the initiative across the State.

The standards established under the pilot project will be strengthened further with technical guidance to take it further across to other CTFs to ensure effective collection, disposal and storage of mercury waste.

A statewide gap analysis has identified approximately 10,000 mercury-based Sphygmomanometers and 1,500 mercury-based Thermometers through past procurement which are due for systematic phase-out. Going forward, the State has gone for replacement with mercury-free digital alternatives thereby safeguarding patients and healthcare workers from the toxic effects of mercury. As part of the mercury phase out initiative, districts will be instructed to ensure that all Government healthcare facilities become Mercury-free in order to become eligible for certification as Mercury-free.

3.21. Mental Health Programme

As per NCRB report (2024), Tamil Nadu recorded 19,965 suicides, accounting for 11.7% of the total suicides reported in India. In response to this significant public health concern, the State has accorded the highest priority to mental health and well-being. Mental health services are delivered

through a robust and integrated healthcare system, underpinned by established infrastructure and strengthened through targeted, evidence-based, and need-responsive initiatives to ensure comprehensive mental healthcare across the State.

The Services are provided through a network of Institutions which includes the Institutes of Mental Health (IMH), Departments of Psychiatry in Medical College Hospitals, District Mental Health Programme (DMHP) Units, De-addiction Centres and the Emergency Care and Recovery Centres (ECRCs) functioning across the state.

3.21.1. State Mental Health Authority (SMHA)

Constituted in accordance with the Mental Healthcare Act, 2017, the State Mental Health Authority oversees and regulates Mental Health services across Tamil Nadu through District Mental Health Review Boards. Till date, 955 Mental Health Establishments have been provisionally registered under the SMHA.

3.21.2. State and District-Level Service Delivery

3.21.2.1. Institute of Mental Health

While the IMH, Kilpauk, Chennai serves as the State Apex Institute for Mental Health, the

Institute of Mental Health at Theni and Regional Institute of Mental Health, Erwadi, Ramanathapuram extends specialized care to southern Districts and enhances accessibility in the Region.

3.21.2.2. Tamil Nadu Institute of Mental Health and Neurosciences (TNIMHANS)

The Tamil Nadu Institute of Mental Health and Neurosciences (TNIMHANS), Kilpauk, Chennai is a Centre of Excellence for Mental Health and a premier Government institution dedicated to offer advanced care, research and education in Mental Health and Neurosciences. The Institute provides specialized treatment, rehabilitation and academic training in Psychiatric and Neurological disorders with the objectives to strengthen accessible, evidence-based mental healthcare and Neuroscience services for beneficiaries.

3.21.2.3. District Mental Health Programme (DMHP)

DMHP is a Public Health approach to reach out Mental Health Services and is implemented in all 38 Districts for delivering both institutional and community-based Mental Health services in an integrated manner. Each unit, led by two

Psychiatrists complemented by support staff, provide Outpatient (OP) and Inpatient (IP) services at 37 District Headquarters Hospitals and one in Chennai. Outreach is conducted through Advanced Tour Programmes (ATP) at Sub-District Hospitals (SDH) and Block Primary Health Centres (BPHC). Awareness Generation is achieved through Targeted Interventions in Schools, Colleges and Prisons. 11.48 lakh beneficiaries have received Mental Health services through DMHP during the year 2025–2026.

3.21.3. Specialized Care Initiatives

3.21.3.1. Tamil Nadu State Policy for Care of Homeless Persons with Mental Illness (HPWMI) and Implementation Framework, 2024

As a pioneering & first of its kind initiative in India, this policy safeguards the rights of Homeless Persons With Mental Illness who face poor health outcomes, limited access to care and social exclusion. The framework delineates service delivery across four stages: Rescue and Acute Care, Intermediary Care, Long-term Care and Social Reintegration. The District Mental Health Committee (DMHC) under the chairpersonship of the District Collector is also designated as District

level Monitoring Committee to oversee the services provided to the HPWMI from the point of rescue to social reintegration.

3.21.3.2. Emergency Care and Recovery Centres (ECRC)

ECRCs are dedicated in-patient facilities providing comprehensive Psychiatric and Medical treatment, family re-unification and Rehabilitation services for Homeless Persons With Mental Illness (HPWMI). Currently, 34 ECRCs are operational [8 Government-run and 26 functioning with the support of Non-Governmental Organizations (NGOs)]. Since inception, 4,886 Homeless Persons With Mental Illness have been rescued and treated.

3.21.3.3. De-Addiction Services

De-addiction services are delivered through the Institute of Mental Health, Department of Psychiatry in all Government Medical Colleges and Hospitals, DMHP Units, 25 Comprehensive De-addiction and Rehabilitation Centres (CDRC), and 3 De-addiction centres established in Government Hospitals. For the year 2025-26, 1,53,013 beneficiaries have availed Out-patient services and 14,252 patients availed De-addiction services as In-patients.

3.21.4. Mental Health Promotion and Support Programmes

3.21.4.1. MaNaM (Mana nala Nallatharavu Mandram)

MaNaM is a Mental Health support programme for promoting Psychological well-being among Medical, Nursing, Dental, and Allied Health Sciences students. This programme focuses on creation of Mental Health awareness, suicide prevention, peer support for early identification of emotional distress. Trained Peer Ambassadors, Faculty facilitators, and Mental Health professionals provide support, guidance, counseling, and further care. The Government of Tamil Nadu has come out with a "Mental Health Policy for Educational Institutions" to foster a supportive, stress-free academic environment and strengthen Mental Health promotion across all educational institutions. 1,69,066 students have so far received awareness sessions through the MaNaM program.

3.21.4.2. TeleManas - Natpudan Ungalodu Mana Nala Sevai – Tele Counselling Services through 14416

Through a dedicated toll-free mental health counselling helpline (14416), Tele-mental health

services are provided 24×7 by offering Tele-counselling services by a team of Counsellors and Psychologists under the supervision of Psychiatrists. In Tamil Nadu, counselling services are augmented with psychosocial interventions, interdepartmental coordination and linkage to the DMHP at the District level. A total of 4,97,764 calls have been received since inception through March 2026. TeleManas cells also provide stress management & Counselling services to 10th, 12th and NEET aspirant students followed by referral and follow-up for high-risk cases.

3.21.4.3. Mana Nala Viyazhan – Mental Health Thursday

Mental Health Thursday is observed across Government Health Facilities for creating awareness for public and Healthcare providers on thematic topics. Public awareness activities are conducted every Thursday at PHCs to encourage health seeking for Mental health issues. The initiative aims to reduce stigma surrounding mental illness and improve Mental Health literacy and access to Psychiatric services in Government Hospitals.

3.21.4.4. Best practices

The Tamil Nadu State Policy for Homeless Persons with Mental Illness & implementation framework - 2024, Comprehensive De-addiction & Rehabilitation Centres and the MaNaM program were showcased this year as Best Practices by Government of India at the 10th National Best Practice Summit held in Chandigarh.

3.22. Communicable Disease Management - National Viral Hepatitis Control Programme (NVHCP)

The National Viral Hepatitis Control Programme (NVHCP) has it's objective to combat Viral Hepatitis with focus on achieving country-wide elimination of Hepatitis C by 2030 and ensuring significant reduction in the morbidity and mortality associated with Hepatitis B and C among the infected population, besides reducing the risk, morbidity and mortality due to Hepatitis A and E.

Under NVHCP, 42 Treatment Centres established across the State, provide screening, diagnosis and treatment services for Hepatitis B and C. Peer Supporters engaged at these

Treatment Centres at a monthly incentive of Rs.10,000/- assist and guide patients by counselling on the importance of adherence to treatment and regular follow-up. Two out of the 42 Treatment Centres, have been designated as Model Treatment Centres (RGGGH and Madurai MCH) for the management of complicated Hepatitis cases. In addition 3 Treatment Centers will be established during 2026-2027.

The services under NVHCP encompass the following;

- Hepatitis B Vaccination of Health Care Workers.
- Awareness creation on safe injection practices and other preventive measures.
- Screening, diagnosis and treatment for Hepatitis B and C.
- Screening of High-Risk Groups, Targeted groups and the General Population for Hepatitis B and C by using Rapid Card Tests at all health facilities & referral of positive cases to Treatment Centres for further evaluation and treatment.

- ❖ Universal screening of all Antenatal mothers for Hepatitis B using rapid card tests at all health facilities, with referral of HBsAg-positive mothers to higher facilities for further management. Ensuring administration of Hepatitis B Immunoglobulin (HBIG) and birth dose vaccine to newborns of Hepatitis B-positive mothers.

A total of 8.62 lakh Antenatal mothers have been screened during 2025 – 2026 for Hepatitis B Virus (HBV), and 1070 detected to be positive among the High-Risk Groups and the general population, 14,56,834 individuals have been screened for Hepatitis B and 13,04,819 for Hepatitis C. Based on viral load estimates, 2,436 new Hepatitis B patients and 2,123 new Hepatitis C have been detected and initiated on treatment.

3.23. Health System Strengthening

3.23.1. Human Resource

The NHM - Tamil Nadu has specifically achieved several milestones including the availability of qualified and skilled Human Resources for Health (HRH) who are vital for

Service Delivery and the adequate Management of the State's health system. At present, 46,616 Contract and 9,311 Regular staff which adds upto 55,927 are supported under NHM – TN and are placed across the 38 Districts of the State.

The overall management and personnel is carried out by the Programme Management Unit (PMU) at the State & District level. In Tamil Nadu, the Category - wise posts supported under NHM are listed below:-

Table 3.6: Category wise HR supported under NHM

Sl. No.	Category Wise	Sanctioned post
1.	Obstetrician	394
2.	Anesthetist	154
3.	Paediatrician	157
4.	Pathologist	14
5.	Psychiatrist	109
6.	Radiologist	18
7.	Other Specialists	18
8.	Microbiologist	80
9.	Surgeon	195
10.	Medical Officer	4,368
11.	Medical Officer-Ayush	475
12.	Dental Surgeon	564
13.	Counsellor	665

Sl. No.	Category Wise	Sanctioned post
14.	Physiotherapist	489
15.	Staff Nurse	16,148
16.	Sector Health Nurse	340
17.	Psychologist	114
18.	Social Worker	142
19.	Pharmacist	2,187
20.	Lab Technician	2,550
21.	ANM	2,315
22.	Audiologist	70
23.	MLHP	7,291
24.	Health Inspector for HWC	5,348
25.	Audiometric Assistant	31
26.	Dental Assistant	506
27.	Early intervention cum Special Educator	39
28.	Public Health Manager	23
29.	TB HV	428
30.	Dental Technician	39
31.	Radiographer	315
32.	Optometrist	44
33.	OT Assistant	91
34.	Driver	449
35.	Data Entry Operator	199
36.	Lab Attendant	479
37.	MPHW	5,380
38.	SPMU/DPMU/BPMU	3,699
Total		**55,927**

3.23.2. Fifteenth Finance Commission Grants (FC- XV) for Health

The Fifteenth Finance Commission Grants for Health (FC-XV) has allocated Rs.4,250 crore for Tamil Nadu, over a five-year period (2021-22 to 2025-26), to support critical health sector components, including civil works, diagnostics, infrastructure and service enhancement. The approved budget for each financial year is detailed below:

Table 3.7: Budget Allocation under FC-XV

Financial Year	Amount Approved (Rs. in Crore)
2021-2022	806
2022-2023	801
2023-2024	839
2024-2025	871
2025-2026	933
Total	**4,250**

The funds are primarily utilized for

- Construction of buildings in Primary and Secondary Care institutions, replacing dilapidated structures and those functioning in rented spaces.

- ❖ Strengthening diagnostics, infrastructure, and healthcare services.
- ❖ Operationalization of Health and Wellness Centres (HWCs) across the State.

As per guidelines of the State Level Committee, Rural and Urban Local Bodies, in collaboration with District Health Authorities, are responsible for executing Civil Works using standardized designs.

The number of Civil Works sanctioned under FC-XV from FY 2021-22 to FY 2025-26 is detailed below:

Table 3.8: Civil Works Sanctioned under FC-XV

Financial Year	2021-2022	2022-2023	2023-2024	2024-2025	2025-2026	Total
Health Sub Centres (HSCs)	186	130	136	144	278	874
New Buildings to PHCs	-	22	76	22	30	150
Additional Buildings to PHCs	26	-	-	30	52	108
Block Public Health Units	96	82	50	74	84	386

Financial Year	2021-2022	2022-2023	2023-2024	2024-2025	2025-2026	Total
Urban Health and Wellness Centres	593	115	-	-	-	708
City Public Health Laboratories	26	9	-	-	-	35
Additional Buildings to Urban PHCs	-	-	40	50	55	145
New Buildings to Urban PHCs	-	-	51	20	30	101
New Buildings to Urban HSCs	-	-	200	20	33	253
SDH / CHC Buildings	-	-	28	25	52	105
Total	**927**	**358**	**581**	**385**	**614**	**2,865**

Additionally, the State Level Committee has mandated that:

- Procurement & supply of equipment, reagents and consumables to be centrally managed by the Tamil Nadu Medical Services Corporation (TNMSC) Ltd, Chennai.
- Operational expenses for HWCs to be met through the existing National Health Mission – Tamil Nadu framework.

3.23.3. Prime Minister Ayushman Bharat Health Infrastructure Mission (PMABHIM)

The Prime Minister's Ayushman Bharat Health Infrastructure Mission (PMABHIM) is a strategic initiative aimed at strengthening healthcare infrastructure across Tamil Nadu. The total budget allocation for Tamil Nadu under this mission is Rs.3,001.29 crore over five years, from 2021-2022 to 2025-2026.

Implemented through the National Health Mission (NHM), PMABHIM focuses on key infrastructure enhancements including:

- Urban Health and Wellness Centres (HWCs) – Strengthening primary healthcare services.
- District Integrated Public Health Laboratories (DIPHLs) – Enhancing diagnostic capabilities across districts.
- Critical Care Hospital Blocks (CCBs) – Establishing advanced critical care facilities in each district.

Table 3.9: Sanctioned Works under PMABHIM (2021-2022 to 2025-2026)

Name of the Work	Total Units Sanctioned	Amount Sanctioned (Rs. in Crore)
DIPHL	38	47.50
100-Bedded CCB	6	243.43
50-Bedded CCB	33	783.75
Total	**77**	**1,074.68**

The sanctioned projects are at various stages of construction, with ongoing efforts to ensure timely completion. To facilitate effective project implementation, quality assurance, and adherence to timelines, a Third-Party Program Management Consultancy (PMC) has been engaged to oversee execution, ensuring optimal resource utilization and compliance with standards.

3.23.4. Integrated Essential Laboratory Services (IELS)

Integrated Essential Laboratory Services (IELS) aims to provide quality and free laboratory services across Government Health Institutions through four major components as given below:-

- Hub and Spoke Sample Transportation System

- ❖ Essential reagents, consumables and laboratory equipment
- ❖ Laboratory Information Management System (LIMS)
- ❖ External Quality Assurance System (EQAS)

Under the Hub and Spoke Sample Transportation Programme, samples are collected from spoke facilities and transported to the nearest hub laboratories for advanced testing and analysis. Through this system, samples collected at lower-level laboratories are rapidly transported to higher institutions, and the test results are delivered back to patients at the point of sample collection itself. This significantly reduces the need for repeated travel by patients and lowers out-of-pocket expenditure, while ensuring timely access to diagnostic services, particularly for people residing in remote and underserved areas.

Under the Hub and Spoke model, a total of 2,127 spoke laboratories and 752 hub laboratories have been integrated. To support sample transportation activities, 3,650 sample transport personnel have been engaged.

Under this programme, essential laboratory equipment, reagents and consumables have been supplied to strengthen laboratory infrastructure and enhance testing capacity for timely and accurate diagnosis. At present, 99 laboratory reagents and consumables are covered under the Tamil Nadu Medical Services Corporation and are supplied to all Government institutions. The supply chain management of laboratory reagents monitored through Drug Delivery Management System.

The number of laboratory tests available has increased from 29 to 63 in Primary Health Centres, from 35 to 97 in Block Primary Health Centres, from 53 to 111 in Sub-District Hospitals and from 65 to 134 in District Headquarters Hospitals. In addition, District Integrated Public Health Laboratories have been established and are functioning in all districts of Tamil Nadu as integrated centres for both clinical and public health laboratory investigations under a single roof.

Under this programme, to integrate and monitor all laboratory activities, the Laboratory Information Management System (LIMS) has been

implemented across all healthcare facilities. This system enables continuous monitoring of laboratory performance, equipment maintenance, reagent utilization and sample transportation activities, thereby enhancing operational efficiency and service delivery.

Further to ensure the quality of laboratory services, all laboratories have been linked with External Quality Assurance Systems (EQAS) in collaboration with reputed institutions such as CMC Vellore, AIIMS New Delhi and Tata Memorial Hospital, Mumbai. This framework facilitates continuous assessment, evaluation, and quality improvement of laboratory performance, thereby ensuring reliable, standardized and high-quality diagnostic services across Government health institutions. A total of 33,53,216 laboratory samples has been cumulatively tested under the programme.

3.23.5. IT Intervention

3.23.5.1. Health Management Information System (HMIS)

Tamil Nadu has been a pioneer in implementing HMIS, enabling real-time data capture across Government healthcare facilities.

HMIS is the vital digital platform that streamlines the data management in terms of data capture, analysis and reporting system. It plays a pivotal role in integrating various public health programme and hospital based patient-centric data to optimise the decision making process for overall effective service delivery.

3.23.5.2. HMIS 3.0 (e – Nalam) application

Tamil Nadu State has transited to Health Management Information System (HMIS) 3.0 developed by Centre for Development of Advanced Computing (C-DAC), which has been renamed as e-Nalam. The application is being rolled out across the State in a phased manner, covering identified districts in successive phases. Within each district, all categories of health facilities are being brought onto the platform, ranging from Health and Wellness Centres and Primary Health Centres at the community level to Sub-District, District and District Headquarters Hospitals and Medical College Hospitals, thereby ensuring that the patient record is available across every tier of the public health system.

A total of 2,834 institutions have been covered across the Districts out of 3,604 identified

health facilities. So far, 2,95,60,651 beneficiaries have been registered and Ayushman Bharat Health Account (ABHA) IDs have been created for 38,80,936 beneficiaries.

e-Nalam has been designed on the principle that the treating doctor, nurse or patient views the consolidated clinical history of the patient, comprising past consultations, diagnoses, prescriptions, laboratory results and imaging, at the point of care and irrespective of the institution at which such records were generated.

The platform places the citizen, and not the institution, at the centre of the health record. Registration is linked to the ABHA ID, enabling the record to follow the patient across facilities and Directorates and ensuring continuity of care. Laboratory results are shared directly with beneficiaries through SMS and WhatsApp, obviating a return visit to collect reports. A WhatsApp Chatbot for the public also provides access to health information and services through a familiar medium.

The lab equipment has been interfaced with e-Nalam, enabling results to flow directly from the analyser into the patient record without manual

transcription and the Lab results from hospitals are being shared with the beneficiaries through SMS and WhatsApp.

e-Nalam also captures block level Multi Super Speciality Camps conducted throughout Tamil Nadu reaching the services to 18 Lakh beneficiaries.

As part of the State initiative to ensure seamless interoperability and integrated healthcare service delivery, e-Nalam has been integrated with key health sector applications such as Chief Minister's Dashboard and Drug Distribution Management System (DDMS) for efficient drug stock monitoring and management. Integration with other critical platforms, namely PICME, CMCHIS, TAEI and 108 Emergency Ambulance Services, is currently under progress to enable comprehensive health information exchange and strengthen continuity of care across the State.

3.23.5.3. Management Information System (MIS)

MIS is a periodic consolidated report that captures essential parameters such as Institution performance, Out-Patient and In-Patient services, Staff details, Lab Services, Antenatal / Postnatal and Delivery Reports, Surgeries carried out etc.,

3.23.6. National Quality Assurance Standards Programme

The National Quality Assurance Standards (NQAS) Programme was introduced in 2013 as part of the National Health Mission's commitment to improve the quality of healthcare services in public health facilities across India. The programme was designed to address the quality gaps in service delivery, patient safety, Infection Control and overall healthcare management at both Primary and Secondary care levels.

During the Year 2025-2026, Tamil Nadu has achieved a significant milestone: 1,073 Government Health facilities—including 6 Sub-District Hospitals, 3 Community Health Centres, 161 Primary Health Centres, 62 Urban Primary Health Centres and 841 Health and Wellness Centres (Health Sub-Centres) have received National NQAS certification. This brings the total number of certified health care facilities to 1,943.

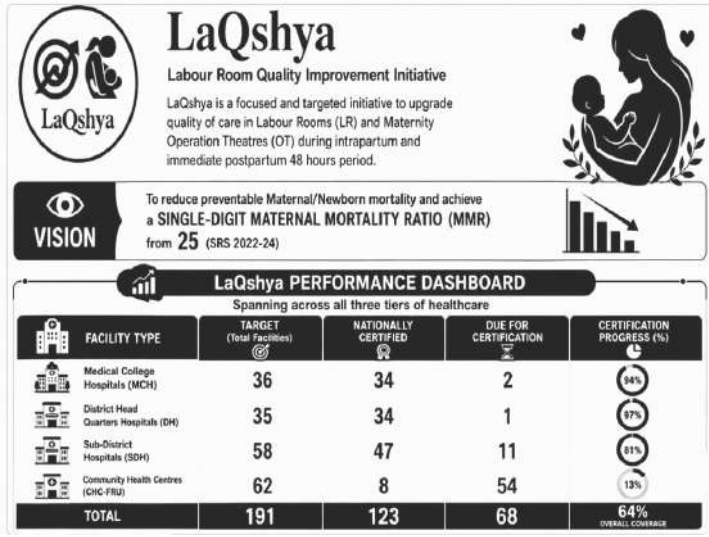
Year	2018 -2019	2019 -2020	2021 -2022	2022 -2023	2023 -2024	2024 -2025	2025-2026	Total
No. of Health Care Facilities	22	47	71	214	309	207	1,073	1,943

[No certification were taken up in 2020-2021 (Covid year)]

3.23.6.1. LaQshya: Labour Room Quality Improvement Initiative

LaQshya is a Labour Room Quality Improvement Initiative with a focused and targeted approach to upgrade quality of care in Labour Rooms (LR) and Maternity Operation Theatres (OT) during intrapartum and immediate postpartum 48 hours period. The vision is to reduce preventable Maternal/Newborn mortality to achieve a single-digit Maternal Mortality Ratio (MMR) from 25(SRS 2022-24). The key pillars are adherence to strict clinical protocols, equipment availability, timely stabilization and referral, enhance the satisfaction of beneficiaries visiting the health facilities and provide Respectful Maternity Care (RMC), benefiting every pregnant woman and newborn delivering in government health facilities.

Effective from 1st April 2026, LaQshya is now fully integrated into the National Quality Assurance Standards (NQAS) framework for all DHs, SDHs and CHC-FRUs to embed maternal health into a sustainable quality ecosystem, excluding the Medical College Hospitals. The program spans across all three tiers of healthcare, as detailed below:



The financial incentives are released directly to institutions upon obtaining National Certification and after every surveillance assessment, to maintain quality standards and infrastructure, as given below.



3.23.6.2. MusQan

MusQan aims at ensuring provision of quality, child-friendly services across 80 facilities - Government Medical Colleges, District Head Quarters Hospitals and selected Sub-District Hospitals and Community Health Centres. So far, 23 facilities have achieved National Certification under MusQan (3 Medical Colleges, 16 District Hospitals and 4 Sub-District Hospitals) and received incentives for their achievement.

3.23.6.3. Kayakalp

Kayakalp Award Programme aims to inculcate a culture of improving cleanliness in Government health facilities for positive health outcomes with ongoing assessments and peer review of performance related to hygiene, cleanliness and sanitation. During the year 2025-2026, 6,561 Government health facilities have been awarded under Kayakalp Programme and received incentives.

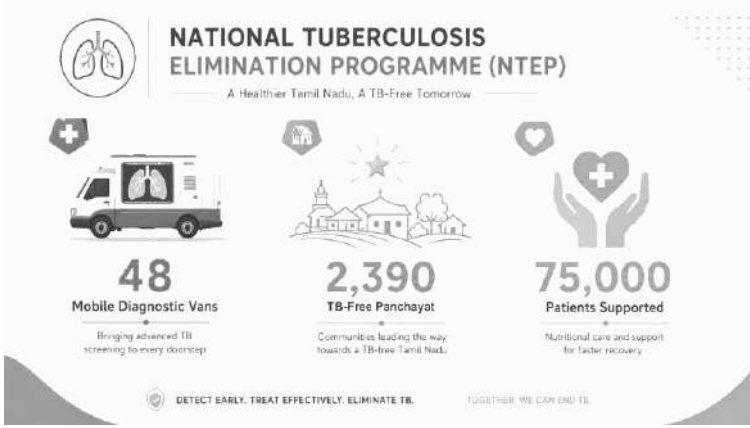
Table 3.10: Year wise number of Kayakalp Awarded Facilities

Year	2021-2022	2022-2023	2023-2024	2024-2025	2025-2026	Total
No. of Health Care Facilities	1,767	2,616	4,758	6,165	6,561	21,867

3.24. National Tuberculosis Elimination Programme (NTEP)

The National TB Elimination Programme in the State is committed to achieve a TB-free Tamil Nadu by the attainment of the following objectives:

1. Early diagnosis of Drug susceptibility and Drug Resistance Tuberculosis in patients and prompt treatment with quality drugs and treatment services.
2. Prevention strategies include contact tracing in high-risk / vulnerable populations.
3. Multi-sectoral response for addressing social determinants.



Based on the Tamil Nadu TB Prevalence Survey (TNTPS), TB Prevalence is calculated as 210 per lakh population (2021-2022) and TB Case Notification rate is 125 in Tamil Nadu (TB India Report 2024).

Key achievements

3.24.1. Expansion of diagnostic infrastructure

Culture & Drug Sensitivity (C&DST) Labs	CBNAAT Machines	Truenaat Machines	Hand held X-rays Machines	Mobile Diagnostic units
5	127	339	40	48

Over the last 3 years, three new culture and drug susceptibility testing laboratories have been established in the State and have received lab accreditations, increasing the total number of such labs to 5 (Tiruchirappalli, Coimbatore, GH-Tambaram, IRL, Chennai & IRL, Madurai), which has allowed decentralization of drug sensitivity testing and quicker turnaround times for availability of test results. As per the new diagnosing algorithm by the Central TB Division, X-ray is considered as mandatory screening tool for TB detection and the handheld X-rays are being

deployed to all districts. The State has adopted 100% upfront NAAT for sputum testing in 2026.

3.24.2. Presumptive TB Examination Rate (PTBER)

A presumptive TB examination is indicated when a person presents with suggestive symptoms, has risk factors for TB or has been exposed to someone with active TB. By identifying TB early through screening, treatment can be promptly started to reduce transmission and prevent complications. The state has adopted 100% Molecular Testing using CBNAAT and TrueNat for TB diagnosis.

Year	2019	2021	2022	2023	2024	2025
PTBER (Per one lakh population)	1,349	960	1,953	2,963	4,076	3,043

3.24.3. Active Case Finding (MDUs)

The functioning of 48 Mobile Diagnostic Units (equipped with digital X-ray units) has greatly facilitated active case finding for TB in the community. Tamil Nadu is the first State to implement a model of Mobile Diagnostic Units on such a large scale. In the year 2025, about

6,26,288 beneficiaries were screened with chest X-rays through the MDUs and 2,339 cases of TB were diagnosed.

3.24.4. Implementation of 3HP TB Prevention Therapy

Tamil Nadu was the first State in the country to procure 37,814 patient courses of 3HP (Isoniazid + Rifapentine) fixed dose combination tablets at the State level for the expansion of TB preventive therapy to more risk groups (such as household contacts over 5 years of age and special risk groups). In the year 2025, about 23,467 household contacts have been provided TB Prevention Therapy.

3.24.5. TB Free Panchayats

The number of TB Free Panchayats in Tamil Nadu has increased from 443 in 2024 to 1,711 in 2025 and further to 2,390 in 2026, showing sustained progress in community-led TB elimination. This sharp increase reflects strengthened local health systems, active Panchayat engagement and effective grassroots interventions towards achieving TB elimination goals.

3.24.6. Nikshay Poshan Yojana (NPY)

To improve nutritional status of TB patients, a financial incentive of Rs.500/- per month is increased to Rs.1000/- per month, throughout the treatment period to all notified TB patients through Direct Benefit Transfer (DBT) under Nikshay Poshan Yojana Scheme. 75,000 TB Patients have received incentive for Nutritional Support through NPY.

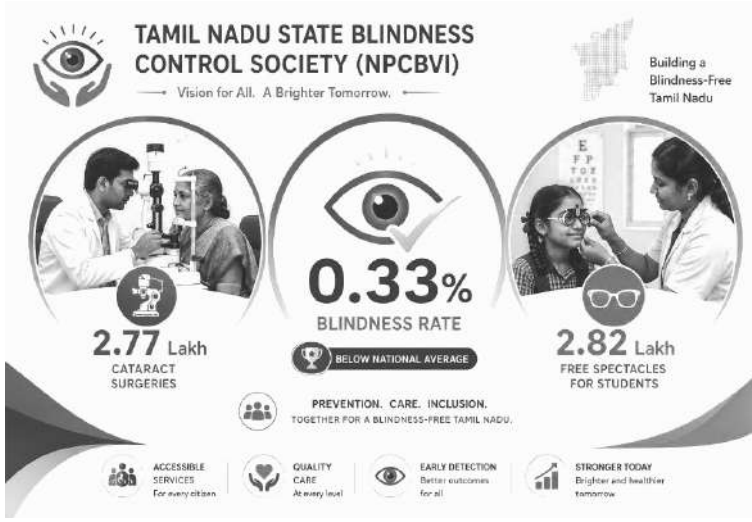
3.24.7. Nikshay Mitra

This initiative provides additional support to TB patients undergoing treatment, offering diagnostic, nutritional and vocational assistance in selected health facilities, blocks, urban wards, districts and the State. It encourages elected representatives, corporates and NGOs to contribute, aiding patients on their path to recovery. 21,527 TB patients were benefitted through this initiative in the year 2025. The Differentiated TB Care was showcased during 2026 as Best Practice by Government of India at the 10th National Best Practices summit held in Chandigarh.

3.25. Tamil Nadu State Blindness Control Society

Tamil Nadu State Blindness Control Society was established in 1996 and later brought under

the administrative control of National Health Mission, Tamil Nadu in 2006. The State Blindness Control Society headed by Mission Director, National Health Mission, Tamil Nadu and 38 District Blindness Control Societies under their respective District Collector / Chairman form a vertical programme under Non-Communicable Diseases (NCD) - National Heath Mission, Tamil Nadu with the objective to eliminate avoidable blindness in our State.



As per the "Rapid Assessment of Avoidable Blindness Survey" done by All India Institute of Medical Sciences (AIIMS) in 2018, the prevalence

of blindness in overall population for Tamil Nadu is 0.33% compared to the prevalence of 0.38% for India. The other causes are operable cataract and its complication (74%), Diabetic Retinopathy (5%), Glaucoma (12%) and other causes like Trauma & injuries, Corneal Opacity, Congenital Cataract, "Retinopathy of Prematurity", etc (9%).

In Tamil Nadu, 36 Government Medical College hospitals, 37 Government District Head Quarters Hospitals, 29 Sub District Hospitals, 38 Government Tele V Care Centres, 92 NGO Eye hospitals on MoU with DBCS provide cashless eye care services to poor patients. Additional financial assistance is provided under National Health Mission, Tamil Nadu for cataract surgeries, and Chief Minister's Comprehensive Health Insurance scheme covers other specialty services. There are 410 in-service eye surgeons, 402 PMOAs and 38 contractual optometrists providing eye care services. 253 eye surgeons have received training under NPCB VI for phacoemulsification, corneal transplantation, retina laser treatment, strabismus management and provide quality advanced specialty eye care services. Tamil Nadu is one among the best performing State under NPCBVI Programme.

3.25.1. Cataract IOL Surgery

All Government eye hospitals and NGO eye hospitals have performed 2,77,973 surgeries and a Grant in Aid of Rs.45.65 crore was provided by National Health Mission, Tamil Nadu.

Table 3.11: The eye care services provided in our State during the year 2025-26 are provided below:

Hospital	Target (Catops)	Performance (Catops)	Grant Released (Rs. in crore)
Government Hospitals	1,25,000	99,461	9.95
NGO Eye Hospitals	1,75,000	1,78,512	35.70

3.25.2. Eye Ball Collection

All eye banks, both Government and NGO have collected 10,973 eye balls as eye donation and a Grant in Aid of Rs.109.73 lakh was released to their eye banks by National Health Mission, Tamil Nadu. The utilization percentage for keratoplasty is about 52% and the remaining eye balls un fit for keratoplasty surgery are used for medical research & training purpose.

Activity	Target (Eye Balls)	Performance (Eye Balls)	Grant released (Rs.in Lakh)
Eye Ball Collection	11,000	10,973	109.73

3.25.3. Free Spectacles for School Children

Children studying in Government and Government Aided school's between 6th to 12th standard. are screened for eye problems including refractive errors. This year 2,82,961 free spectacles are provided to needy children; Rs.625.00 lakh is provided to Tamil Nadu Medical Services Corporation Limited for procurement and supply of spectacles by National Health Mission, Tamil Nadu.

3.25.4. Free Spectacles for Old Persons

Free spectacles are provided to poor persons above 45 years to correct their near vision defect. This year 2,242 spectacles was issued free to the needy beneficiaries from the Government eye hospitals; Rs.130.00 lakh is provided to TNMSC for procurement and supply of spectacles by National Health Mission, Tamil Nadu.

3.25.5. CMCHIS Ophthalmology Speciality services

Specialty eye care services other than Cataract surgery are provided cashless to poor

people under CMCHIS scheme like keratoplasty, Trabeculectomy, Vitreo Retinal Surgery, Laser treatment, etc. The Government Hospitals have successfully completed 11,888 procedures and received Rs.740.70 lakh as reimbursement during the year 2025-26. Which is 50% of the amount earned by private hospitals.

3.25.6. Tele V Care Centers

The 38 "Tele V Care Centers" have conducted eye screening for 87,373 beneficiaries and identified 5,998 operable cataract beneficiaries. Out of them 3,257 Cataract surgery was performed at Government eye hospitals.

3.25.7. Mobile Ophthalmic unit

The Government Hospitals have conducted 1,738 eye camps and have performed 21,252 cataract surgeries. Last year, additionally two mobile ophthalmic units was provided to Krishnagiri and Tiruvarur districts and Rs.60.00 lakh has provided to Tamil Nadu Medical Services Corporation Limited for procurement by National Health Mission, Tamil Nadu.

3.25.8. Ophthalmic Equipment for Government Hospitals

16 Ophthalmic Equipment was provided to Head Quarters Hospital Rasipuram and Government hospital Dharapuram of Namakkal and Tiruppur Districts respectively. Rs.60.00 lakh was provided to Tamil Nadu Medical Services Corporation Limited for procurement and supply by National Health Mission, Tamil Nadu.

3.25.9. Screening for Diabetic Retinopathy

Annual Fundus Photography for diabetic patients is done at all Government hospitals regularly. This year a pilot project to screen those Diabetic patients with or without high Blood Pressure enrolled in "Makkalai Thedi Maruthuvam" scheme is being implemented in Harur, Aathoor, Parangipettai and Thirumanthurai blocks. Necessary follow-up eye care services will be provided for them to prevent blindness.

3.25.10. Training for Eye Surgeons and Optometrists

25 Eye surgeons have undergone in-service training under NPCBVI in various ophthalmic Specialities at Regional Institute of Ophthalmology

(RIO), AIIMS and other major NGO eye hospitals. Optometrist was given hands on training in "Tele ophthal services"; for this programme Rs. 2.00 lakh was provided by NHM, Tamil Nadu.

3.25.11. Information, Education and Communication activity

IEC Activities are undertaken by all District Blindness Control Societies on specific occasions, like "Eye Donation Fortnight celebration", "World sight day", "Glaucoma Awareness week" etc.; Rs.20.00 lakh is provided by NHM, Tamil Nadu to all District Blindness Control Societies for completing this activity.

During the year 2026-2027 the following activities under National Programme for Control of Blindness and Visual Impairment programme have been proposed in our State:-

Activity	Target (in numbers)	Budget (Rs. in Lakh)
Cataract Surgery by Government Hospitals	1,25,000	1250.00
Cataract Surgery by NGO Eye Hospitals	2,00,000	4000.00
Eye Ball Collection	11,000	110.00

Free Spectacles to School Children	3,10,000	629.30
Free Spectacles to Old Persons for Near Vision	1,52,000	129.96
Ophthalmic Equipment to 80 Government Eye Hospitals	34 Ophthalmic Equipment / 207 Nos.	1325.00
New Eye Operation Theatre & Eye Ward	Medical College Hospital, Chengalpattu and Head Quarters Hospitals @ Tenkasi & Mannargudi	300.00
IEC Activities	38 DBCS	20.00
PMOA Training	83 PMOAs	2.00

All the above activities will help us create Cataract Blindness Backlog Free districts and reduce the burden of avoidable blindness in our state.

"When one person donates his eyes, two blind persons can get vision"

3.26. National Leprosy Eradication Programme - Tamil Nadu

Leprosy is a chronic infectious disease, primarily affecting the skin and peripheral nerves.

National Leprosy Eradication Program is aligned with the Sustainable Development Goals (SDGs) and the WHO Global Leprosy Strategy, aiming to achieve zero transmission, zero disability and zero discrimination by the year 2030.

3.26.1 Goals

- Interruption of Leprosy transmission with Zero Child case
- Reduce Grade 2 disability cases < one case / Million Population
- Zero disability among new Child cases
- Zero Stigma & Discrimination against Persons affected by Leprosy

3.26.2 Activities in NLEP

a) Active Case Detection activities

Active case detection is done through various activities such as Slum Survey, Saturday Survey, Contact Survey, School Survey by Field Staff, Leprosy Case Detection Campaign (LCDC), Vulnerable Population Survey, etc. Through these activities a total of 2,112 cases have been identified during April 2025 to March 2026.

b) Passive Case Detection

A total of 1,011 cases were reported during April 2025 to March 2026 through passive case finding & IEC activities from Primary Health Centers / Urban Primary Health Centers, Government Hospitals, Government Medical College Hospitals, Private Medical College Hospitals, Private Practitioners and Private Hospitals and Non-Government Organizations.

c) Sparsh Leprosy Awareness Campaign (SLAC)

The campaign was conducted during 30.01.2026 to 13.02.2026 across all the PHCs resulting in detection of 53 new cases. All identified cases are initiated on Multi-Drug Therapy (MDT).

Table 3.12: NLEP State Profile (2025 - 2026)

Indicator	2025 - 26
Screened Population	7,75,01,793
New Cases Detected	3123
New Case Detection Rate / Lakh pop.	3.94
Prevalence Rate / 10,000 pop.	0.34
New Child Cases Detected	160
Child Proportion	5.12%
New Grade II Deformity Cases	112
Deformity Grade II case Proportion	3.59%
Deformity Rate / million pop.	1.41

d) Post Exposure Prophylaxis (PEP)

Single-dose Rifampicin was administered to 67741 contacts including family, neighborhood and social contacts.

e) Disability Prevention and Medical Rehabilitation (DPMR)

The programme ensures comprehensive care for affected persons to prevent disability progression and improve quality of life through various activities given below

- Distribution of self-care kits and MCR footwear
- Reconstructive surgeries for eligible cases
- Management of lepra reactions and neuritis
- Financial assistance through disability maintenance grants

Table 3.13: Services Provided to Leprosy Patients (2025–26)

Sl. No.	Particulars of Services	2025 – 26
1	Total No. of Self-care kits distributed	26,532
2	Total amount utilized for Self-care kit, Aid and Appliances	Rs.7,95,960/-
3	Total No. of MCR footwear given	17,202
4	Total amount utilized for procurement of MCR footwear	Rs. 68,80,800/-
5	Total No. of Reconstructive Surgeries (RCS) performed	131
6	Total amount utilized for RCS incentive	Rs.15,72,000/-
7	No. of patients treated for Lepra Reaction and Neuritis	338
8	No. of persons receiving Rs. 2000/- per month under Disability Maintenance Grant	11,398

3.27. Conclusion

The National Health Mission has, over the past two decades, anchored the investments made in public health in this State and the achievements recorded in the preceding paragraphs stand testament to the role it has played. The

achievements are asserted further through the findings of the 80th Round of the National Sample Survey on Household Social Consumption: Health, released in April 2026, offer independent affirmation of the course the State has pursued.

The average out-of-pocket medical expenditure per hospitalisation in Government health facilities in Tamil Nadu was recorded at ₹1,357, the lowest among the major States of the country and less than one-fifth of the all-India average of ₹6,631, while 49 per cent of all out-patient episodes in the State were treated in Government institutions as against a national average of 30.9 per cent — the highest such proportion recorded for any major State.

These two findings describe a public system that people choose to use that asks for very little when they do. They are not incidental outcomes. They are the cumulative result of sustained investment in free drugs and diagnostics, cash-free maternal and newborn entitlements, emergency transport, universal screening of children and adolescents, and a community-level outreach cadre, each of which removes from the household a cost that would otherwise fall upon it at the very

moment of illness. Financial protection, in the experience of this State, is secured less through reimbursement after the event than through provision at the moment of need; and it is the National Health Mission that finances that provision.

The Mission is therefore not an adjunct to the State's health system but the financing, human resource and information architecture upon which its principal outcomes rest. The Government is equally conscious that this achievement is contingent upon the continued adequacy of Mission resources, and that any contraction in support would be transferred, directly and immediately, to the household in the form of higher out-of-pocket expenditure, falling first upon those least able to bear it.

The Government will accordingly continue to strengthen the Mission's institutional and financial base, so that every citizen of Tamil Nadu shall receive, in a Government institution, care that is comprehensive in scope, assured in quality and cashless at the point of delivery.

CHAPTER - 4

MEDICAL EDUCATION AND RESEARCH

4.1. Vision

The Government of Tamil Nadu remains firmly committed to strengthening medical education, expanding equitable access to healthcare services, modernizing tertiary care institutions, advancing research and innovation, and improving governance through technology-enabled administration.

The policy vision of the Government is to reduce both morbidity and mortality, and to strengthen the healthcare system as well to ensure that excellence in medical education and advanced healthcare services remains socially inclusive, regionally balanced, and affordable to all sections of the population of Tamil Nadu.

4.1.1. Creation of the Directorate

The Directorate of Medical Education was established in the year 1966 following its bifurcation from the Directorate of Medical and Rural Health Services, thereby enabling it to

function as an independent body dedicated exclusively to the advancement of medical education and tertiary care.

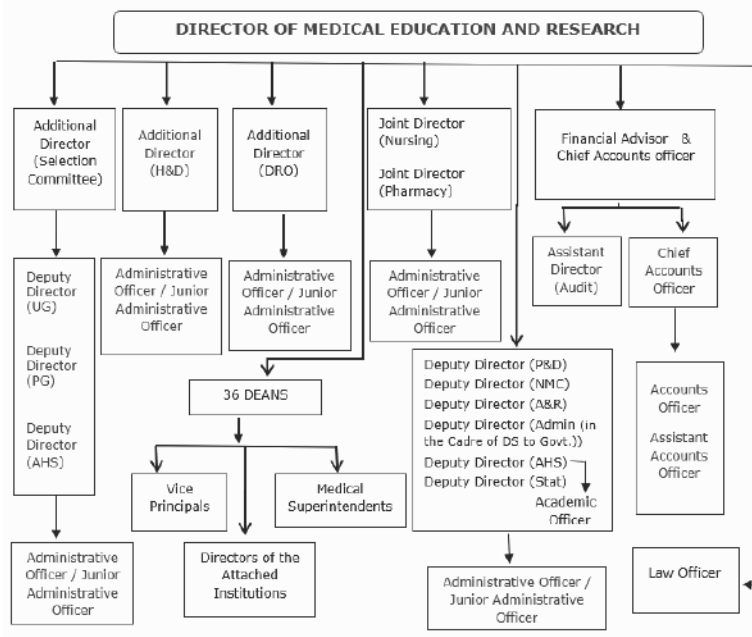
Subsequently, the Directorate was redesignated as the Directorate of Medical Education and Research in recognition of the growing importance of research, innovation and evidence-based medical practice in modern healthcare delivery.

At present, the Directorate is committed to delivering high-quality clinical services while simultaneously promoting excellence in medical education, professional training and pioneering research initiatives aimed at meeting the evolving demands of contemporary medicine.

4.1.2. Administrative Structure of the Directorate

The Directorate is headed by the Director of Medical Education and Research and is supported by an administrative team comprising 2 Additional Directors in the rank of Dean, 1 Additional Director in the rank of District Revenue Officer, 2 Joint Directors for Nursing and Pharmacy, 1 Deputy Director in the rank of Deputy Secretary, 8 Deputy Directors, 1 Academic Officer for Allied

Health Sciences (AHS), 1 Financial Advisor and Chief Accounts Officer (FA&CAO), along with 280 personnel serving in various categories.



The 36 Government Medical Colleges and their attached hospitals are administered by Deans, supported by Medical Superintendents, Vice-Principals, Directors and Superintendents as applicable. This administrative structure ensures effective academic supervision, efficient hospital management and institutional accountability.

4.2. Clinical Services in Tertiary Care Hospitals

The Government considers tertiary care institutions as one of the most important instruments for reducing Out-Of-Pocket Expenditure (OOPE) for the public, functioning in close coordination with the Chief Minister's Comprehensive Health Insurance Scheme (CMCHIS).

The Out-of-Pocket Expenditure for inpatient care in our State is Rs.1,364/- in Government institutions as compared to Rs.74,168/- in private institutions. Similarly, the OOPE for outpatient services is Rs.51/- in Government institutions as against approximately Rs.1,100/- to Rs.1,700/- in private institutions.

[**Sources**: National Health Accounts 2022–23 (MoHFW / NHSRC, released 27 May 2026); NSS Report No.596 (MoSPI, April 2026); WHO / UN SDG]

Tamil Nadu records the lowest Out-of-Pocket Expenditure (OOPE) in Government healthcare institutions while simultaneously recording one of the highest private healthcare expenditures among the states of India.

The Government of Tamil Nadu, will continue to strengthen advanced tertiary and quaternary healthcare services in order to improve public access to specialized treatment facilities across all regions of the State.

Current Performance Indicators (January 2026 – May 2026)

Sl.No.	Details	2026
1.	Number of Beds	68,122
2.	OP Strength (per day)	1,20,755
3.	IP Strength (per day)	41,390
4.	Cath Lab Procedures	19,101
5.	Major Surgeries	2,39,196
6.	Dialysis Cycles	2,35,681
7.	Neurosurgeries	9,461
8.	Orthopaedic Surgeries	1,08,051
9.	Eye Surgeries	79,554
10.	ENT Surgeries	76,050
11.	Urological Surgeries	30,083
12.	Laboratory Tests	6.08 Crore
13.	MRI Scans	1,08,549
14.	CT Scans	8,01,599

4.3 Mortality to Mobility: Using Cause-of-Death Data to Drive Health System Reforms

Table 4.1 Age-wise Mortality Distribution

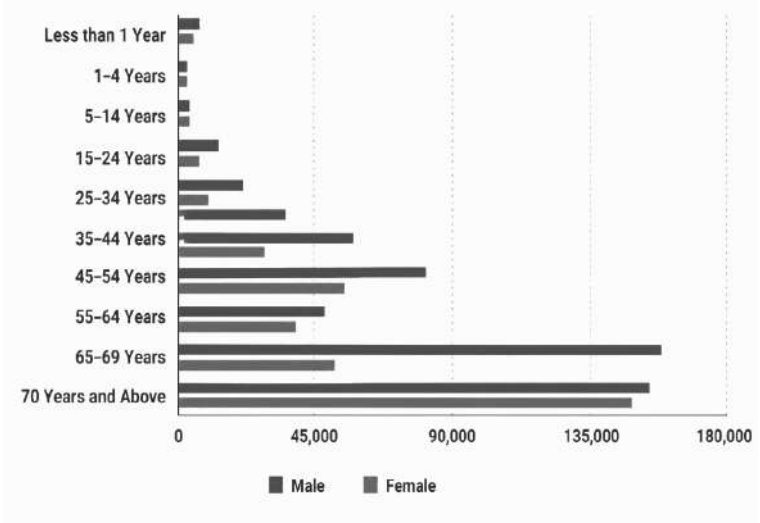
Age of Deceased	Male	Female	Persons / Year
Less than 1 Year	5,392	3,887	9,279
1 – 4 Year	951	762	1,713
5 – 14 Year	2,040	1,510	3,550
15 – 24 Year	9,169	4,460	13,629
25 – 34 Year	17,134	6,177	23,311
35 – 44 Year	31,584	11,407	42,991
45 – 54 Year	54,740	26,270	81,010
55 – 64 Year	81,836	51,366	1,33,202
65 – 69 Year	43,908	34,970	78,878
70 Years and Above	1,55,092	1,50,550	3,05,642
Total	**4,01,846**	**2,91,359**	**6,93,205**

[**Source**:Civil Registration System, Tamil Nadu,Global Burden of Disease Study 2023 (IHME), cardiovascular diseases, cancers, chronic respiratory diseases, diabetes and road traffic injuries together account for the majority of deaths in this distribution, and most of the premature deaths below 70 years are preventable through early detection and timely treatment]

According to Institute for Health Metrics and Evaluation (IHME), Global Burden of Diseases (GBD) 2025, disorders such as cardiovascular diseases, non-communicable diseases including

cancers, road traffic injuries and self-harm represent the major causes of death among individuals in the 15–49 years age group in India.

Age-wise Mortality Distribution



Data indicates an increasing death rate attributable to cardiovascular diseases, particularly among the middle aged and elderly population. Age-specific and gender-specific mortality patterns further reveal significant public health trends. Infectious diseases continue to remain an important cause of mortality among children, whereas non-communicable diseases, particularly cardiovascular diseases, have emerged as

increasingly dominant causes of mortality in older age groups and are now rising steadily even among younger populations.

*[**Source**: MCCD 2023 & Tamil Nadu Journal of Public Health and Medical Research, January – March 2025, Vol. 5]*

4.3.1. Decoding Mortality: Evidence-Based Strategies for Reducing Preventable Deaths

The Government of Tamil Nadu is continuously improving both access to healthcare services and the quality of treatment for the above conditions. The Government is committed to strengthening existing Non-Communicable Disease (NCD) and cancer screening services while simultaneously establishing an efficient referral network connecting primary healthcare facilities with tertiary-level cancer treatment centres.

4.3.2. Free Master Health Check-Up for All in Medical College Hospitals

Tamil Nadu has a high prevalence of non-communicable diseases, with hypertension affecting more than 30% of the population and diabetes affecting more than 10% of the population.

Further, cardiovascular diseases account for nearly 36% of all deaths. The cardiovascular disease death rate by sex is estimated at 349 per 100,000 among men and 265 per 100,000 among women. In addition, ischemic heart disease contributes significantly, accounting for 22.1% all-cause mortality and 25.7 per 100 person-years according to National registry data.

[**Source**: ICMR INDIAB-2023, NFHS-6 (2023-2024), WHO India Report, National Heart Failure Registry]

Periodic master health check-ups are essential for the early detection of asymptomatic diseases such as diabetes mellitus, hypertension, cardiovascular disease, anaemia and refractive errors. Annual comprehensive screening facilitates early diagnosis, enables timely treatment, prevents serious complications including heart failure and renal failure and significantly improves overall quality of life.

Accordingly, free basic Master Health Check-ups shall be provided through Government Medical College Hospitals. Senior citizens specific comprehensive screening will also be made available in all Medical College Hospitals.

In addition, a specialized Master Health Check-Up programme exclusively designed for adolescent girls will be operationalized under the Departments of General Medicine and Obstetrics & Gynaecology to identify and treat potentially serious conditions such as anemia. This intervention will contribute significantly towards further reduction of Infant Mortality Rate (IMR) and Maternal Mortality Rate (MMR).

4.3.3 Cancer Care Facilities under DME & R

Tamil Nadu has successfully won the battle against major infectious epidemics. However, Non-Communicable Diseases (NCDs), including cancer, cardiovascular diseases, renal disorders, metabolic diseases and road traffic accidents have now emerged as major public health challenges.

4.3.3.1 Rising Cancer Incidence in Tamil Nadu

The incidence of cancer in Tamil Nadu has shown a steady upward trend. Compared to the year 2019, the number of new cancer cases in 2023 has increased significantly among both men and women. The estimated number of new cancer cases in Tamil Nadu for the year 2025 is 1,00,097.

[**Source**: TNCRP-2023]

Table 4.2 Incidence of Cancer

Category	Year 2019	Year 2023	Increase
Men	35,826 new cases	37,931 new cases	1.06 times
Women	42,852 new cases	49,225 new cases	1.15 times
Total	78,678 new cases	87,156 new cases	1.11 times

[Source: TNCRP 2023]

The Government is committed to establishing Tamil Nadu as a national model for comprehensive, equitable, technology-enabled and patient-centered cancer care by creating an integrated continuum of prevention, screening, early diagnosis, treatment, rehabilitation, palliative care and survivorship services through a Hub-and-Spoke Model, linking community-level screening with advanced tertiary cancer centres.

4.3.3.2 Upscaling Cancer Treatment Facilities for Equitable and Standardized Access

At the State Apex Cancer Centre Arignar Anna Memorial Cancer Hospital, advanced cancer diagnostic and treatment facilities are being operationalized effectively to improve cancer care services.

Tamil Nadu presently has 5 Regional Cancer Centres at Coimbatore Medical College Hospital, Government Royapettah Hospital, Thanjavur Medical College Hospital, Government Rajaji Hospital Madurai, and Tirunelveli Medical College Hospital, ensuring equitable regional access to specialized cancer care.

Each Regional Cancer Centre provides Medical Oncology, Surgical Oncology, Radiation Oncology, Nuclear Medicine, Molecular Diagnostics, Palliative Care, Rehabilitation Services, Cancer Registry Unit and Tele-Oncology Centre services.

High-end radiotherapy and surgical oncology equipment are being provided across Government Medical Colleges to strengthen cancer treatment services.

4.3.3.3 Cancer Screening Initiatives

a. Strengthening Existing Screening Services

The Government aims to expand and standardize screening for:

- Oral Cancer
- Breast Cancer
- Cervical Cancer

Screening for Breast, Cervical and Oral cancers is being conducted regularly in all Medical College Hospitals under Organized Cancer Screening Programmes (OCS).

b. Strengthening the Hub-and-Spoke Model

To further strengthen Organized Cancer Screening services, the Government is implementing:-

- ❖ Line-listing of eligible population
- ❖ Digital screening registry
- ❖ Call-recall system
- ❖ Tracking of screen-positive individuals
- ❖ Monthly district review

c. Universal Cancer Screening for In-Patients

The Government aims to implement cancer screening for all admitted in-patients across Tamil Nadu.

4.3.3.4 Specialized Screening Modalities

a) Mammograms

Mammography enables early diagnosis of breast cancer and detection of pre-cancerous

lesions, facilitating early treatment and complete cure. Breast cancer remains one of the most common cancers affecting women, but it is also among the most curable cancers when detected early.

Mammography has been incorporated into the Master Health Check-Up programme for women. In addition, it is performed as a standalone screening investigation across 41 hospitals (51 machines).

A total of 26,053 mammograms have been performed during the current year.

b) VIA/VILI Screening

Screening for cervical cancer is routinely conducted on an outpatient basis for women in reproductive age groups across all Government Medical College Hospitals. Early detection and treatment significantly improves outcome.

c) Oral Cancer Screening

Oral cancer screening enables early detection of pre-cancerous lesions and identifies potentially malignant oral disorders such as leukoplakia, erythroplakia and oral submucous fibrosis before malignant transformation occurs. Most oral cancers

are preventable if identified during the pre-cancer stage. Screening services are being provided in all Medical College Hospitals.

d) Lung Cancer Screening Pilot Programme

Tamil Nadu has a high burden of tobacco use which causes lung and oral cancers. Therefore, the Government proposes pilot implementation of Low-Dose CT (LDCT) Lung Cancer Screening for:

- Age group 50–74 years
- Heavy smokers and ex-smokers
- High-risk occupational exposure groups

Potential pilot districts include Chennai, Coimbatore, Salem, Tiruppur and Cuddalore.

e) PET Scans

PET scan facilities, which help diagnose cancer and assess disease spread, have been installed under Public Private Partnership (PPP) mode in 7 Medical College Hospitals, including:

- Rajiv Gandhi Government General Hospital, Chennai
- Tirunelveli Medical College Hospital
- Thanjavur Medical College Hospital

- ❖ Government Mohan Kumaramangalam Medical College Hospital, Salem,
- ❖ Government Rajaji Hospital, Madurai
- ❖ Arignar Anna Memorial Cancer Hospital
- ❖ Coimbatore Medical College Hospital

A total of 15,846 PET scan investigations have been performed during the current year. Additional PET scan facilities are proposed at:

1. Kalaignar Centenary Super Speciality Hospital, Guindy.
2. Mahatma Gandhi Memorial Government Hospital, Tiruchirappalli
3. Government Medical College Hospital, Villupuram
4. Government Medical College Hospital, Tiruvallur

f) Linear Accelerators (LINACs)

Linear Accelerators (LINACs), which provide highly precise radiation therapy to cancer tissue while minimizing damage to surrounding normal tissues, have been installed in 11 Government Medical College Hospitals.

The Government will continue phased expansion of LINAC facilities to all Government Medical College Hospitals to ensure complete regional coverage across Tamil Nadu.

Sl. No.	Name of institution	LINAC (2025-2026)
1.	Rajiv Gandhi Government General Hospital, Chennai	1
2.	Thanjavur Medical College Hospital	1
3.	Government Rajaji Hospital, Madurai	1
4.	Government Coimbatore Medical College Hospital	1
5.	Tirunelveli Medical College Hospital	1
6.	Government Mohan Kumaramangalam MCH, Salem	1
7.	Thoothukudi Government Medical College Hospital	1
8.	Tamilnadu Government Multi Super Speciality Hospital, Chennai	1
9.	Government Medical College Hospital, Tiruppur (installation under process)	1
10.	Government Royapettah Hospital, Chennai	1
11	A.A.M. Cancer Hospital, Karapettai, Kanchipuram	2
Total		**12**

g) Cancer Chemotherapy Decentralization

Cancer chemotherapy services are being decentralized so that treatment becomes accessible in all Medical College Hospitals through a "Virtual Tumor Board Model".

Cancer specialists from Hub Institutions provide expert guidance to spoke institutions, enabling specialist and general doctors to administer standardized treatment based on case-by-case virtual review.

At present, 11 Hub Institutions with institutional virtual tumor boards cover the entire State through mapped spoke institutions.

S. No.	Hub with Institutional Tumour Board	Spoke institutions linked to Hub through Virtual Tumour Board (VTB)
1.	Government Arignar Anna Memorial Cancer Hospital (GAAMCH), Kancheepuram	Chengalpattu, Tiruvannamalai, Vellore, Ranipet, Tirupathur
2.	Rajiv Gandhi Government General Hospital (RGGGH), Chennai	IOG, Cuddalore
3.	Stanley Medical College & Hospital, Chennai	Tiruvallur, Periyar Nagar Peripheral Hospital

S. No.	Hub with Institutional Tumour Board	Spoke institutions linked to Hub through Virtual Tumour Board (VTB)
4.	Tamil Nadu Government Multi Super Speciality Hospital, Omandurar	Villupuram, Kallakurichi
5.	Regional Cancer Care (RCC), Royapettah	Dharmapuri, Salem, Krishnagiri, Nagapattinam, Anna Nagar Peripheral Hospital
6.	Regional Cancer Care (RCC), Madurai	Ramanathapuram, Dindigul, Sivagangai, Theni
7.	Regional Cancer Care (RCC), Thanjavur	Pudukottai, Tiruchirappalli, Ariyalur, Tiruvarur
8.	Regional Cancer Care (RCC), Tirunelveli	Kanniyakumari
9	Regional Cancer Care (RCC), Coimbatore	Erode, Namakkal, Tiruppur, The Nilgiris, Karur, ESI Hospital Coimbatore
10.	Regional Cancer Care (RCC), Thoothukudi Medical College Hospital	Virudhunagar
11.	Institute of Child Health (ICH) (For Pediatric Malignancies)	All Pediatric Department in the MCH across TamilNadu

Every patient diagnosed with cancer receives standardized, quality-assured treatment.

This model significantly reduces Out-of-Pocket Expenditure (OOPE) by minimizing travel costs while ensuring specialist consultation closer to the patient's place of residence.

4.3.4 Cardiac Care

Cardiovascular diseases account for nearly 36% of all deaths, with mortality rates estimated at 349 per 100,000 among men and 265 per 100,000 among women. Ischemic heart disease contributes 22.1% all-cause mortality with 25.7 per 100 person-years.

In South India, 87% of sudden cardiac deaths are due to myocardial infarction, of which 69% are acute presentations. In cardiovascular emergencies such as heart attack and stroke, every minute is critical.

[**Source**: NFHS-6, WHO India Report, National Heart Failure Registry].

With nearly one in every three deaths in Tamil Nadu attributable to cardiovascular disease, and with 87% of sudden cardiac deaths linked to myocardial infarction, Cath Labs are critical for emergency angioplasty, stroke care, and minimally invasive interventions. These facilities substantially reduce mortality by enabling restoration of cardiac blood flow during the crucial "Golden Hour".

4.3.4.1 Cath Labs

Tamil Nadu currently has 39 Cath Lab units across 26 Medical College Hospitals, supported by a workforce of 86 cardiothoracic surgeons and 151 cardiologists.

A total of 19,101 cardiac interventional procedures have been performed till date free of cost for eligible patients under the Chief Minister's Comprehensive Health Insurance Scheme (CMCHIS).

The Government policy is to establish Cath Lab facilities in all Government Medical Colleges, with adequate manpower in a phased manner, so as to achieve a situation where no patient is required to travel more than 60 minutes for undergoing a Cath Lab procedure.

4.3.4.2 Strengthening of Cardiology Departments

One of the major initiatives of the Government is to ensure adequate cardiologists availability for functioning of Cath Labs in Medical College Hospitals including Tiruvarur, Kanniyakumari, Dharmapuri, Theni, Cuddalore, Ramanathapuram, The Nilgiris, Tiruvallur, Tiruppur, Kallakurichi, Dindigul, Krishnagiri and Erode.

4.3.5 Renal Care

According to Indian Council of Medical Research (ICMR) study, nearly 3.2% of the Indian population has impaired kidney function (eGFR <60). Chronic Kidney Disease (CKD) prevalence in Tamil Nadu is estimated to range between 8.4% and 8.7% among adults, based on the Madras Medical College STEP Survey 2022–23.

Further, according to MCCD 2023, 2,950 female deaths and 8,542 male deaths were attributed to renal failure, making CKD the 5th leading cause of death in Tamil Nadu.

[**Source**: Madras Medical College STEP Survey 2022–23 & MCCD 2023 Chapter]

To strengthen the Siruneeragam Kakkum Seermigu Maruthuva Thittam, the Government is focusing on early screening for kidney disease through blood creatinine testing and urine dipstick screening at the primary care level. Doorstep screening for diabetes mellitus, hypertension and kidney disease, along with portable dialysis machines for home-based care through Makkalai Thedi Maruthuvam (MTM) services, are being actively promoted.

As part of its public renal care policy, the Government proposes to provide adequate dialysis infrastructure in all Government Medical College Hospitals.

The Government is taking major steps to strengthen renal healthcare delivery. At present, Government Medical College Hospitals have 85 Nephrologists and 95 Urologists, with phased expansion of specialist manpower planned across Government institutions.

4.3.5.1 Dialysis Services

A total of 893 dialysis machines are currently available across Government Medical College Hospitals.

For the year 2025-2026, 5,55,675 Dialysis cycles have been provided for Chronic Kidney Disease patients, averaging approximately 1,522 dialysis cycles per day.

Through expansion of nephrology services, implementation of the Hub-and-Spoke model, provision of free dialysis services, and early disease detection programmes, Tamil Nadu is adopting a comprehensive long-term strategy aimed at reducing kidney-related mortality.

4.3.6 Mental Health Initiatives

Mental health has emerged as a major public health concern requiring urgent policy intervention.

During the year 2024, Tamil Nadu reported 19,965 suicides, representing the highest number recorded in South India. Among these, 2,821 suicides involved housewives, which is the highest recorded figure in India. In addition, studies indicate that 51.2% of students in higher education experience depression-related symptoms.

[**Source**: NCRB 2024 & PHQ-9 Study 2024]

4.3.6.1 District Mental Health Programme (DMHP)

The District Mental Health Programme (DMHP) functions as the backbone of decentralized mental healthcare delivery in Tamil Nadu. The programme has been implemented across all 38 districts, where psychiatry outpatient services are regularly offered.

District Mental Health Programme (DMHP) has successfully decentralized psychiatric services, thereby bringing mental healthcare services within reach of the general population.

4.3.6.2 Institute of Mental Health, Chennai

Institute of Mental Health Chennai, admits approximately 5,000 medical beneficiaries annually, delivering specialized psychiatric care while also facilitating reunification of wandering psychiatric patients with families both within India and abroad.

4.3.6.3 De-Addiction Centres

A total of 3 De-Addiction Centres have been established in Government Hospital for management of substance abuse disorders, along with vocational rehabilitation services.

4.3.6.4 Comprehensive De-Addiction Rehabilitation Centres (CDRC) – 25

The Government proposes expansion of Comprehensive De-Addiction Rehabilitation Centre (CDRC) services to all districts in Tamil Nadu, ensuring comprehensive treatment for persons suffering from substance use disorders.

4.3.6.5 Key Government Policy Initiatives for Strengthening Mental Healthcare

The Government proposes the following interventions:

- ❖ Establishment of Emergency Mobile Psychiatric Teams equipped with vehicles for conducting home visits and transporting psychiatric patients to Government Hospitals. Teams shall include support workers, staff nurses and social workers.

- ❖ Attempted Suicide Intervention Programme, wherein attempted self harm survivors shall be systematically followed up through telephonic counselling and home visits.

- ❖ Strengthening of School Mental Health Programmes to address rising mental health concerns among students caused by academic stress, poverty, substance abuse, internet addiction, media addiction, cellphone dependency and other emerging social pressures.

4.3.6.6 MaNaM – Mananala Nallaadharavu Mandram

As part of the MaNaM Thittam, ManaNala Nallaatharavu Mandram (Students Mind Health Support Forum) has been inaugurated in all Medical colleges across Tamil Nadu and is being progressively expanded to all colleges and schools across the State.

The primary objective of this initiative is promotion of student mental health and psychological well-being through awareness generation activities and structured training programmes for both teachers and students.

Members of the Mandram function as peer-support groups, offering psychological first aid to students experiencing mental distress and guiding them toward professional mental healthcare services. These services are linked to 'Natpudan Ungalodu Mananala Sevai' - Tele-Consultation Services and District Mental Health Programme Clinics.

A specialized training module titled "Students Mental Health Promotion and Wellbeing" has been developed for teachers.

At present, 1,69,066 students have benefited through stress management sensitization programmes, while approximately 3,957 activities, including art, music, dance and related therapeutic interventions, have been conducted.

4.3.7. Fertility Improvement Initiatives

Tamil Nadu currently has a Total Fertility Rate (TFR) of 1.3 children per woman (SRS 2022–24), which is significantly below replacement level fertility.

Among births recorded in the three years preceding the survey, 1% were birth order four or higher, compared to 2% in NFHS-4.

[**Source**: NFHS Survey 5 by MOH&FW]

Although Tamil Nadu has achieved fertility rates comparable to developed countries, the issue of childless couples remains a significant social and medical concern.

The Out-of-Pocket Expenditure (OOPE) for infertility treatment in private hospitals remains beyond the financial reach of poor and marginalized populations.

4.3.7.1 Centre for Assisted Reproduction (CAR) – Government Infertility Services

The Government remains committed to providing infertility assessment and treatment facilities free of cost to all eligible couples in Tamil Nadu.

At present, an IVF Centre / Infertility Clinic is functioning at Rajiv Gandhi Government General Hospital.

To ensure affordability and wider accessibility, the Government plans to expand infertility services to all Government Medical College Hospitals in a phased manner.

4.3.8. Robotic Surgery

The need for robotic surgery in Tamil Nadu arises from the State's large population base, increasing burden of complex diseases, particularly cancer and the significant advantages robotic-assisted surgery offers over conventional surgical methods.

Robotic surgery provides:

- Up to 50% reduction in blood loss
- Up to 50% shorter hospital stay
- Faster recovery
- Reduced postoperative pain
- Lower infection risk

Nearly 90% of cancers can potentially be managed through robotic surgery. The technology enables highly precise lymph node dissection without damaging vital nerves, which is particularly critical in prostate, urological, gastrointestinal and oncological surgeries.

Tamil Nadu Government Multi Super Speciality Hospital, Omandurar, Chennai has installed the advanced Robotic Surgical System, enabling complex surgeries to be performed with superior precision using robotic-assisted technology.

A total of 530 robotic surgeries have been successfully completed using this advanced equipment.

The centre also functions as an advanced training facility for surgeons from Government Medical Colleges in the region.

4.3.9. High-End Neuro Microscopes

High-end neuro microscopes equipped with advanced digital hybrid visualization technology are essential for enabling precision neurosurgery, oncological surgery and plastic surgery, while significantly improving patient safety and surgical outcomes across Government healthcare institutions.

Key advantages include enhanced safety and surgical precision through 3D robotic visualization, colour-coded visualization ensuring complete tumour removal, reduced risk of neural and soft tissue injury, reduced operative time and lower infection rates, motorized zoom and focus improving workflow efficiency, ergonomic design enabling long-duration complex procedures.

A total of 3,707 neurosurgical procedures have been performed during the current year using 19 Neuro Microscopes installed across 11 Government Medical College Hospitals.

4.3.10. Organ Transplantation

Tamil Nadu has consistently remained at the forefront of organ transplantation in India. Government Medical College Hospitals have been providing transplantation services with consistently good outcomes.

At present, Tamil Nadu has 13 Government Hospitals functioning as Transplant Organ Retrieval Centres (TORC) and licensed transplant hospitals and 35 Government Hospitals functioning as Non-Transplant Organ Retrieval Centres (NTORC).

From the inception of this programme up to 30.04.2026, a total of 14,212 organ donations have taken place.

Organ Transplantation Statistics (September 2008 to April 2026)

Category	**Overall (05.09.2008 – April 2026)**
Donor	2,433
Major Organs	
Organ	Overall
Heart	1,015
Lungs	1,047
Liver	2,153
Kidneys	4,292

Pancreas	49
Small Bowel	36
Stomach	4
Hands	9
Total	**8,605**
Minor organs & tissues	
Tissue	Overall
Heart Valve	1,097
Corneas	3,592
Skin	379
Blood Vessels	2
Bone	508
Spine Bone & Disc Tissue	26
Abdominal Flap	3
Total	**5,607**
Grand Total	**14,212**

Tamil Nadu has launched a dedicated website and mobile application for organ registration and allocation - the first such initiative in India**.**

This platform has enabled transparent and efficient allocation of organs to eligible patients through a technology-driven system that ensures accountability and fairness in transplantation services.

4.3.11. Regional Renal Transplant Teams

A renal (kidney) transplant is a surgical procedure in which a healthy kidney from a living or deceased donor is transplanted into a patient suffering from End-Stage Renal Disease (ESRD) or kidney failure. Renal transplantation remains the preferred treatment option over lifelong dialysis, as it offers significantly improved quality of life and longer life expectancy.

The Government proposes the establishment of a structured Hub-and-Spoke model for Regional Renal Transplant Teams. Under this framework, designated Hub Institutions shall maintain multidisciplinary renal transplantation teams consisting of specialists in Nephrology, Vascular Surgery, Urology, Anaesthesiology and other allied specialties.

The Hub Institution shall be responsible for supporting, supervising and coordinating transplant services for affiliated spoke institutions.

Table 4.3 Proposed Hub and Spoke Institutions

Sl. No.	Hub Institution	Spoke Institutions
1.	Thanjavur Medical College Hospital	1. Government Tiruvarur Medical College Hospital 2. Government Kallakurichi Medical College Hospital, Kallakurichi 3. Ariyalur Medical College Hospital, Ariyalur 4. Karur Medical College Hospital, Karur
2.	Coimbatore Medical College Hospital	1. Government Mohan Kumaramangalam Medical College Hospital, Salem 2. Government Dharmapuri Medical College Hospital 3. Erode Medical College Hospital, Erode 4. Govt Medical College Hospital, The Nilgiris
3.	Madurai Medical College	1. Theni Government Medical College Hospital 2. Virudhunagar Government Medical College Hospital Virudhunagar 3. Ramanathapuram Government Medical College Hospital 4. Dindigul Government Medical College Hospital, Dindigul

4.3.12. Tamil Nadu Geriatric Care

Tamil Nadu's elderly population is projected to reach 18.2% by 2031, necessitating significant strengthening of geriatric healthcare services. The Government's policy focus includes active ageing, dementia care, elder abuse prevention, hospital-based care, doorstep healthcare delivery, non-communicable disease screening for senior citizens, and pension-linked healthcare support systems.

[**Source**: TN Budget 2024–25, Social Welfare Policy Note, NPHCE Guidelines]

The Government proposes to predict risks such as elderly persons suddenly becoming weak and falling, deterioration of health conditions, and injuries; and through this, to improve preventive medical treatment protocols. Further, the Government also proposes to conduct home visits to collect data and utilize it for enhancing medical treatment. In addition, the Government proposes establishment of IoT-enabled Smart Pillboxes integrated with MTM/NCD infrastructure, enabling monitoring of high-risk drug adherence and generation of automated alerts in cases of missed medication.

4.3.12.1. Clinical Workflow Changes

The Government proposes the following systemic reforms for strengthening geriatric healthcare delivery

4.3.12.2. Dedicated Geriatric Care Pathways

Sunday and early-morning Priority Special Clinics will be established across Government Medical Colleges to reduce waiting time for elderly patients and minimize caregiver wage loss.

4.3.12.3. Community and Mental Health Integration

Physical health monitoring will be integrated with local activity hubs promoting peer mentoring, light exercise, health literacy and mental health support, thereby reducing isolation and depression among the elderly.

The Government further proposes to strengthen dementia care services, improve availability of trained geriatricians, and establish rural old-age homes as part of its long-term elderly care strategy.

4.3.12.4. National Centre for Ageing, Guindy

National Centre for Ageing, Guindy is a 200 bedded specialized geriatric care centre

established with financial assistance from the Government of India at a cost of Rs.151.17 crore.

It is a comprehensive, fully equipped centre dedicated exclusively to healthcare services for the elderly population. It is the second such geriatric care centre in India after All India Institute of Medical Sciences, New Delhi

Table 4.4 Hospital Performance Statistics (2025–2026)

Sl. No.	Category	Statistics (2025–2026)
1.	Out-Patients	2,58,220
2.	Inpatient Admissions	36,043
3.	Major Surgeries	249
4.	Minor Surgeries	1,769
5.	Laboratory Investigations	4,43,456
6.	CT Scan	7,460
7.	MRI	2,223

4.3.12.5. Regional Geriatric Centres (RGC)

Given Tamil Nadu's 16.3% of population being above 60 years as per NFHS-6, availability of robust geriatric healthcare infrastructure is critical.

National Programme for Health Care of the Elderly (NPHCE) vision is to ensure peace of mind for persons aged above 60 years through guaranteed access to quality healthcare services.

DME & R will focus on developing high-quality tertiary care services delivering comprehensive healthcare exclusively for older persons under one roof, supplemented by community-based care systems.

Key objectives:

- Preventive, curative and rehabilitative geriatric care
- Training health professionals in geriatrics and supportive care
- Rehabilitation training systems
- Promotion of research in geriatrics and gerontology
- Establishment of dedicated geriatric research databases
- Address healthcare needs of elderly patients suffering predominantly from chronic disabling non-communicable diseases

- ❖ Reduce long-term healthcare expenditure by preventing complications
- ❖ Improve quality of life by reducing disability burden
- ❖ Develop trained geriatric manpower across the State
- ❖ Strengthen research in geriatrics and gerontology

The State will develop a strategic response to Tamil Nadu's demographic transition and will aim to establish sustainable infrastructure for comprehensive geriatric care, professional training and research across all geographic regions of the State.

4.3.13. Quality of Care

4.3.13.1. National Accreditation Board for Hospitals (NABH)

National Accreditation Board for Hospitals & Healthcare Providers (NABH) establishes the highest benchmarks for patient safety, clinical quality and transparent hospital operations. NABH accreditation ensures regulatory compliance while

providing a structured framework for systematizing hospital operations, standardizing medical procedures, and continuously monitoring clinical outcomes.

In line with the Government's policy of improving quality of care in public hospitals, NABH accreditation processes have been completed at:

- Tiruvallur Government Medical College Hospital
- Omandurar Government Medical College Hospital

Further, the following institutions have already obtained Entry-Level NABH Certification:

- Tiruvannamalai Government Medical College Hospital
- Theni Government Medical College Hospital
- Kanniyakumari Government Medical College Hospital, Nagercoil

During the current year, the Government has made provisions for recertification of these hospitals and for expansion of NABH accreditation processes to additional Government Medical College Hospitals.

4.3.13.2. Outsourcing of Housekeeping and Security Services

In order to improve cleanliness, sanitation, and overall support services across Government Medical College Hospitals, housekeeping and security services have been outsourced.

For the financial year 2026–27, the total manpower deployed across 125 institutions stands at 21,683 personnel, including housekeeping staff, security personnel and supervisory staff.

This initiative is intended to ensure higher standards of hospital hygiene, improved infection control, better patient experience and efficient institutional support services.

4.4. Establishment of Multi Super Speciality Hospital

According to IHME – Global Burden of Disease (GBD) 2023, non-communicable diseases such as cardiovascular diseases, renal diseases, cancers and road traffic injuries, self-harm continue to be among the major causes of mortality in the 15–49 years age group in India.

[**Source**: MCCD 2023; GBD 2023 (IHME)]

Multi Super Speciality Blocks in tertiary care hospitals are critical for strengthening advanced healthcare infrastructure and providing comprehensive super-speciality services under one roof, particularly in underserved regions of the State.

Government Medical Colleges located in Madurai, Tirunelveli, Coimbatore, Salem and Tiruchirappalli serve large rural populations and are witnessing increasing healthcare demands arising from non-communicable diseases, trauma cases, and complex tertiary care conditions.

The establishment of Multi Super Speciality Hospital / Regional Cancer Care Blocks in these institutions will significantly improve access to advanced specialties including Cardiology, Neurology, Nephrology, Oncology and Critical Care Medicine.

The initiative will reduce dependence on metropolitan tertiary care centres, minimize patient travel burden, reduce treatment expenditure and improve timely access to specialized healthcare services.

In addition, these institutions will strengthen emergency response capacity and significantly

improve medical education by exposing students and trainees to advanced infrastructure and specialized clinical practices.

The Government proposes phased implementation to ensure efficient and systematic infrastructure development.

4.5. Medical Education System

Tamil Nadu continues to maintain one of the strongest public medical education systems in India, supported by sustained Government investment in undergraduate, postgraduate and super-speciality education, research infrastructure, and equitable student access policies.

The Government remains committed to further strengthening medical education infrastructure to meet future healthcare workforce requirements while preserving principles of social justice and equitable access.

4.5.1. Leading Medical Education System in the Country

Tamil Nadu continues to maintain one of the leading Government medical education systems in India.

At present, Government medical institutions in Tamil Nadu offer:

Table 4.5 Government Sector Seats

Course	Number of Seats
MBBS	5,200
MD/MS	2,358
DM/MCh	422

In addition to Government institutions, private institutions, State Private Universities and Deemed Universities contribute an additional 7,800 MBBS seats.

This extensive educational capacity continues to position Tamil Nadu as one of the largest producers of medical professionals in the country.

4.5.2. Reservation of Postgraduate (MD/MS) and Super-Speciality (DM/MCh) Seats for In-Service Candidates

The Government will continue reservation for in-service candidates in postgraduate and super-speciality courses within the existing policy framework.

This policy ensures strengthening of public healthcare institutions by retaining specialist manpower within Government service and creating opportunities for career progression among doctors already serving in Government institutions.

The Government remains committed to sustaining this policy as part of its long-term public healthcare workforce strengthening strategy.

4.5.3. Increase of Postgraduate Seats in Existing Medical Colleges

Applications have been submitted for creation of 653 additional postgraduate seats across 32 Government Medical Colleges from the academic year 2026–2027.

This increase is intended to address future healthcare workforce demands while strengthening specialist training capacity across the State.

4.5.4. Financial Assistance to Students Admitted Under 7.5% Government School Quota

Students admitted under the 7.5% Government School Reservation Quota will continue to receive comprehensive Government support covering tuition fees, hostel expenses, books and educational Support.

The Government of Tamil Nadu, will continue this intervention as a core social justice initiative aimed at improving access to medical education for students from historically disadvantaged educational backgrounds.

The Government remains committed to ensuring that financial constraints do not become barriers to professional education for Government school students.

4.5.5. Dental, Nursing, Pharmacy and Allied Health Sciences

Tamil Nadu has developed a strong parallel healthcare education ecosystem through substantial investments in Dental Sciences, Nursing Education, Pharmacy Education and Allied Health Sciences training programmes.

Dental Education

Table 4.6 Dental Education Seats

Course	Seats
Bachelor of Dental Surgery (BDS)	2,200
Master of Dental Surgery (MDS)	432

The Government continues to invest in infrastructure development and student support systems in Dental Education.

Nursing Education

Table 4.7 Nursing Education Seats

Course	Seats
Diploma in General Nursing and Midwifery (DGNM)	7,535
B.Sc Nursing	12,207
M.Sc Nursing	1,647
Post Basic Diploma Nursing	480
Post Basic B.Sc Nursing	1,133

Nursing education capacity includes Government institutions as well as self-financing institutions offering DGNM Programmes, B.Sc and M.Sc Nursing courses, Post Basic Nursing Programmes, Specialty Diploma Courses

Pharmacy Education

Table 4.8 Pharmacy Education Seats

Course	Seats
Diploma in Pharmacy	6,780
Bachelor of Pharmacy	10,290
Master of Pharmacy	2,307

4.5.6. Allied Health Sciences

In Allied Health Sciences, 11,694 seats are currently available in Government Medical Colleges.

The Government plans to expand Allied Health Sciences education through new specialized courses including:

- Clinical Psychology
- Medical Laboratory Technology
- Radiography and Imaging Technology
- Dialysis Technology
- Accident and Emergency Care Technology
- Critical Care Technology

This expansion is aimed at building a multidisciplinary healthcare workforce capable of supporting modern tertiary healthcare delivery systems.

4.5.7. Medical Research

The Directorate has developed a strong research ecosystem designed to promote scientific inquiry, evidence-based medicine, and academic excellence across Government Medical Colleges.

The current research infrastructure includes:

Table 4.9 Medical Research

Research Infrastructure	Number
Multidisciplinary Research Units	8
Virology Research and Diagnostic Laboratories	9
Model Rural Health Research Unit	1

Current research performance indicators for the year include:

Table 4.10 Research Performance Indicators.

Parameter	Current Status
Ongoing Faculty Research Projects	4,601
Published Research Articles	1,904
Indexed Publications	1,712
Research Grants received	Rs.60.86 Crore

All Government Medical Colleges under the Directorate possess necessary infrastructure required for conducting evidence-based clinical research and academic studies.

4.5.8. Policy to Rescind National Eligibility cum Entrance Test (NEET)

The Government, views that the National Eligibility cum Entrance Test (NEET) has severely disadvantaged students from rural areas, Government schools, Tamil-medium backgrounds and socio-economically disadvantaged families, while fostering a commercialised coaching economy that favours the more affluent.

Public confidence in NEET as a fair and secure measure of merit has, moreover, been progressively eroded. The recent leak and consequent re-conduct of the examination having further shaken the trust of students and parents in its integrity.

The Government believes that opposition to NEET is fundamentally linked to expanding equitable access to medical education and preserving opportunities for socially disadvantaged students.

The Government's Policy approach combines:

- Legal intervention
- Legislative advocacy
- Institutional support measures for disadvantaged students

The Government continues to pursue policy measures aimed at protecting equitable access to medical education while safeguarding opportunities for students from underprivileged and Government school backgrounds.

4.6. E-Governance

The Directorate has been actively involved in implementing digital governance systems across all major administrative and service delivery channels. The four major areas of digital transformation currently under implementation include:

- Administration through e-Office
- Digital Hospital Information Management through e-Nalam (HMIS)
- Medico-Legal Examination and Post-Mortem Reporting (MedLEaPR)
- Development of the official Directorate website

These initiatives are intended to improve efficiency, transparency, accountability, and institutional coordination across all institutions functioning under the control of the Directorate of Medical Education and Research.

4.6.1. Digital Governance in Administration (e-Office)

The e-Office system was implemented in the Directorate of Medical Education and Research and in all institutions functioning under its administrative control on 27.10.2025.

Administrative workflow across the Directorate is now being performed through the e-Office platform via the WAW Portal, developed by National Informatics Centre and implemented by Tamil Nadu e-Governance Agency

The system has been designed to replace traditional paper-based administrative processes with a fully electronic workflow-based system aimed at improving governance, increasing transparency, and establishing paperless offices.

The Directorate is now communicating with other Government Departments and Directorates entirely through e-Office mode.

4.6.2. Digital Health Care – e-Nalam

e-Nalam is a web-based monitoring system that enables real-time access to and analysis of patient beneficiary data.

It is being implemented by the National Health Mission – Tamil Nadu under the Health and Family Welfare Department, Government of Tamil Nadu.

The system has now evolved into an advanced version incorporating additional modules including:

- ❖ Laboratory Reports Management
- ❖ Pharmacy Management Systems

The Government has decided to implement e-Nalam across the State in a phased manner.

At present, e-Nalam has already been implemented in 15 locations, while implementation in the remaining institutions is planned for completion by August 2026.

4.6.3. MedLEaPR

MedLEaPR means Medico Legal Examination and Post-mortem Reporting. The MedLEaPR software was developed by NIC Haryana. As per Hon'ble High Court judgement order, a web based system for issuing Postmortem Certificate has been implemented at the Institutions under the administrative control of the Director of Medical

Education and Research in coordination with NIC Tamil Nadu from 03.10.2025 onwards as a standalone platform facilitates entering Medical Legal cases.

And then the scope was extended to Directorate of Medical & Rural Health Services in February 2026. Later it was integrated with CCTNS (Crime and Criminal Tracking Network System) on 26.03.2026.

Cases entered till date: 1,01,988; 46,200 post-mortem reports and 55,788 medico-legal cases; 4,658 doctors on-boarded.

Total No. of Institutions: 36 Medical Colleges (DME & R)

Totally 6 forms are available in the portal:

1. Post Mortem Module
2. MLR cum wound certificate
3. Fitness Report of Remand Prisoner/ Screening of Prisoner Form
4. Sexual Offence Form – Survivor
5. Sexual Offence Form – Accused
6. Age Estimation

All the forms are available in the live environment.

CCTNS integration has been implemented from 26.03.2026.

4.6.4. Benefits

- **Enhanced Accuracy & Transparency:** Replaces narratives with structured, template-driven inputs and allows digital/pictorial representations of injuries
- **Real-Time Data Sharing:** Integrated directly with CCTNS (Crime and Criminal Tracking Network and Systems), securely streaming reports to Investigating Officers (IOs)

4.7. Technology Initiatives under DMER

The Directorate is implementing several technology-driven initiatives aimed at strengthening academic administration, student management, training systems, organ transplantation services, and institutional transparency.

4.7.1. State Medical Education – Learning Management System (SME-LMS)

The State Medical Education Learning Management System (SME-LMS) portal will function as a unified digital academic platform for all Government medical institutions in Tamil Nadu.

The portal will host:

- ❖ Academic resources
- ❖ Faculty profiles
- ❖ Student profiles
- ❖ Course content
- ❖ Institutional updates
- ❖ Educational analytics for quality improvement

As a leading Government medical organization, the Directorate prioritizes continuous improvement in medical education and patient care. The SME-LMS portal is intended to support medical professionals, researchers, and students by providing centralized academic resources and educational support systems.

4.7.2. Foreign Medical Graduates – Compulsory Rotating Medical Internship (CRMI) Portal

As per the notification issued by the National Medical Commission (NMC) 384 seats (7.5% of the annual student intake of each college) were permitted for Foreign Medical Graduates (FMGs) to undergo Compulsory Rotatory Medical Internship (CRMI) in the 36 Government Medical Colleges

functioning under the control of Directorate of Medical Education in Tamil Nadu.

Subsequently, based on States request, the NMC extended the permission for FMG internship by increasing 750 additional internship vacancies in District Headquarters Hospitals and 370 additional vacancies in non-teaching hospitals

Tamil Nadu is one of the first State in our country to allow FMG internship in District Headquarters hospitals and this was implemented in the portal from July 2026 for internship allotment.

Hence at present in Government hospital a total of 1,134 seats (384 Government Medical College Hospitals, 750 District Head Quarters Hospital), non teaching private hospitals 370 seats, 1,215 private Medical College Hospitals, 711 State Private University and 11 seats in ESI Medical College Hospitals are available for FMG students to do Internship. This additional internship seats increased the availability of the FMG seats from 2,321 to 3,441.

4.7.3. District Residency Programme Portal

The District Residency Programme has been standardized and implemented across Government, Private Institutions and Deemed Universities throughout Tamil Nadu.

To improve posting management and ensure continuity of postgraduate training, a dedicated online portal has been developed.

Current Status – 2026

A total of 4,463 postgraduate students have been allotted postings through the portal, including:

- Government Medical Colleges – 2,264 students
- Private Institutions and Deemed Universities – 2,199 students

Postings have been distributed across 141 District and Sub-District Hospitals in 38 Districts.

4.7.4. Family Adoption and Community Mentoring Programme – "Community First" Portal

The Family Adoption and Community Mentoring Programme has been designed to help

MBBS students better understand community health, rural social determinants of health and the practical foundations of primary healthcare delivery.

Through the Community First Portal, the programme enables longitudinal digital data collection and analysis involving approximately 35,000 students and 400 faculty members.

Participation includes Government, private, and deemed medical institutions across the State.

This initiative strengthens community-oriented medical education while improving understanding of public health realities at the grassroots level.

4.7.5. Transplant Authorization Committee Portal

An online Transplant Authorization Committee Portal is proposed for:

- ❖ Processing transplant applications
- ❖ Document upload and verification
- ❖ Decision-making related to organ transplantation

A centralized database is also proposed for maintaining transplant-related records including:

- ❖ Donor information
- ❖ Recipient information
- ❖ Medical records
- ❖ Clinical protocols

This initiative is intended to improve transparency, efficiency and accountability in organ transplantation services.

4.7.6. Essentiality Certificate Portal

Government institutions, private managements, educational trusts and agencies proposing to start new courses or increase intake capacity in Government, self-financing, State private university or deemed university institutions are required to obtain prior State Government approval before applying to statutory councils.

To improve transparency and efficiency in processing such applications, an online Essentiality Certificate Portal is being developed.

The portal will streamline:

- ❖ Application submission
- ❖ Assessment procedures
- ❖ Institutional verification
- ❖ Decision-making processes

4.8. Integrated Health Care Professional Training Centres (IHCPTC)

Integrated Health Care Professional Training Centres constitute an important component of capacity-building initiatives under TAN-QuEST, implemented under the Tamil Nadu Health System Reforms Program.

These centres are intended to provide training for:

- ❖ Doctors
- ❖ Nurses
- ❖ Allied and Health Care Staff
- ❖ Field Staff

The programme covers institutions functioning under:

- ❖ Directorate of Medical Education and Research

- ❖ Directorate of Medical and Rural Health Services
- ❖ Directorate of Public Health and Preventive Medicine

Currently, such centres are functioning in:

- ❖ Tiruchirappalli, Salem, Tirunelveli, Chengalpattu, Thoothukudi, Coimbatore, Thanjavur & Vellore

Additional centres are proposed in:

- ❖ Omandurar Medical College
- ❖ Tiruvallur
- ❖ Kallakurichi
- ❖ Kanniyakumari
- ❖ Pudukottai
- ❖ Madurai
- ❖ The Nilgiris
- ❖ Tiruppur

4.9. Tamil Nadu Compendium of Case Reports (TCCR)

The Tamil Nadu Compendium of Case Reports (TCCR) is a medical e-journal dedicated to publishing high-quality peer-reviewed case reports across various fields of medical sciences.

Selection of case reports is based on:

- ❖ Novelty of findings
- ❖ Ethical standards
- ❖ Interdisciplinary relevance
- ❖ Uniqueness of observations
- ❖ Accessibility of supporting data
- ❖ Quality of scientific presentation

The journal serves as an academic platform for disseminating clinically significant observations and interpretations with wide clinical relevance.

TCCR supports clinicians by establishing a high-quality publication platform for sharing unusual, rare, or scientifically significant clinical presentations.

Reports published through TCCR enhance medical understanding of rare diseases, unusual presentations and evolving management strategies, thereby contributing to clinical learning and scientific advancement.

4.10. Tamil Nadu Quality Enhanced Structured Training (TAN-QuEST)

TAN-QuEST is a capacity-building and quality enhancement training programme functioning under the Tamil Nadu Health System Reform Project.

Following successful pilot implementation at Tirunelveli Medical College, the programme is being expanded to:

- Government Mohan Kumaramangalam Medical College, Salem
- Chengalpattu Medical College
- K.A.P. Viswanatham Medical College, Tiruchirappalli

The long-term objective is expand this training across healthcare personnel working under all Health Department Directorates.

The programme aims to institutionalize structured professional training with emphasis on:

- Effective communication
- Patient-centered care
- Quality healthcare delivery

- ❖ Professional competency development
- ❖ Strengthening humane and high-quality healthcare systems across Tamil Nadu

The programme represents an important institutional initiative for strengthening service quality standards and improving overall patient experience across public healthcare institutions in the State.

CHAPTER - 5

MEDICAL AND RURAL HEALTH SERVICES

5.1. Introduction

The Directorate of Medical and Rural Health Services (DM & RHS) serves as the crucial middle tier of Tamil Nadu's pioneering three-tier healthcare delivery model. While primary care is successfully anchored by the Directorate of Public Health and Preventive Medicine (DPH & PM) and tertiary care by the Directorate of Medical Education and Research (DME & R), the DM & RHS governs the comprehensive network of secondary care healthcare institutions across all districts.

The mandate of this Directorate is to deliver high-quality, accessible and free Medical and surgical care along with specialized diagnostic services to rural, semi-urban and industrial workforces, filtering unnecessary loads on tertiary medical institutions and keeping out of pocket medical expenditures to a bare minimum.

5.2. Vision

To strengthen secondary care hospitals by improving access, quality, efficiency, patient safety

and continuity of care, while reducing the burden on tertiary care hospitals and enhancing public trust in secondary care hospitals.

5.3. Objectives

1. Improve quality of clinical services.
2. Strengthen emergency and trauma care.
3. Enhance patient safety and infection control.
4. Improve availability of human resources and equipment.
5. Increase efficiency through digital health systems.

5.4. Key Areas for Improvement

5.4.1. Infrastructure Strengthening

- Upgradation of hospital buildings.
- Expansion of ICU, HDU, operation theatres, and emergency departments.
- To improve water supply, sanitation, waste management, and power backup.
- To ensure accessibility for elderly persons and people with disabilities.

5.4.2. Human Resource Development

- Medical officers, Specialists, nurses and allied health professionals to be appointed as per IPHS norms.
- To implement continuous medical education and skill development programme.
- To strengthen staff retention through incentives and career progression opportunities.
- To promote multidisciplinary team-based care.

5.4.3. Quality Assurance and Accreditation

- To implement standard treatment guidelines and clinical protocols.
- To strengthen quality assurance committees.
- To encourage certification under quality standards such as National Quality Assurance Standards (NQAS)
- To conduct regular clinical audits and mortality reviews.

5.4.4. Patient Safety and Infection Prevention

- To strengthen hospital infection control practices.
- To improve hand hygiene compliance and antimicrobial stewardship.
- To establish adverse event reporting systems.
- To ensure safe medication management practices.

5.4.5. Emergency and Trauma Care

- To ensure 24×7 emergency services.
- To strengthen trauma stabilization units.
- To ensure availability of essential emergency drugs and equipments.
- To improve referral linkages with tertiary hospitals.

5.4.6. Diagnostic and Laboratory Services

- To upgrade laboratory and imaging facilities.
- To ensure timely reporting of investigations.
- To strengthen quality control systems.
- To expand tele-radiology and tele-consultation services.

5.4.7. Digital Health and Health Information Systems

- To strengthen hospital management information systems.
- To use digital dashboards for monitoring key performance indicators.
- To promote telemedicine services for specialist consultations.

5.4.8. Maternal and Child Health Services

- To strengthen 24/7 comprehensive emergency obstetric and newborn care.
- To improve high-risk pregnancy management
- To enhance newborn stabilization and referral systems.
- To promote family-centered care.

5.4.9. Non-Communicable Disease (NCD) Care

- To strengthen screening and management of hypertension, diabetes, cancer and cardiovascular diseases.
- To improve rehabilitation services.

5.4.10. Patient-Centered Care

- To reduce waiting times.
- To strengthen grievance redressal mechanisms.
- To improve communication between healthcare providers and patients.
- To conduct patient satisfaction surveys regularly.

5.4.11. Medical Equipment Management

- To ensure annual maintenance contracts.
- To strengthen inventory management systems.
- To minimize equipment downtime.

5.4.12. Monitoring and Evaluation

Key indicators

- Bed occupancy rate.
- Average length of stay.
- Surgical site infection rate.
- Maternal and neonatal mortality rates.
- Emergency response time.
- Patient satisfaction score.
- Referral and readmission rates.

Expected Outcomes

- Improved quality of healthcare services.
- Reduced preventable morbidity and mortality.
- Increased patient satisfaction.
- Better utilization of secondary care facilities.
- Reduced burden on tertiary hospitals.
- Progress towards universal health coverage.

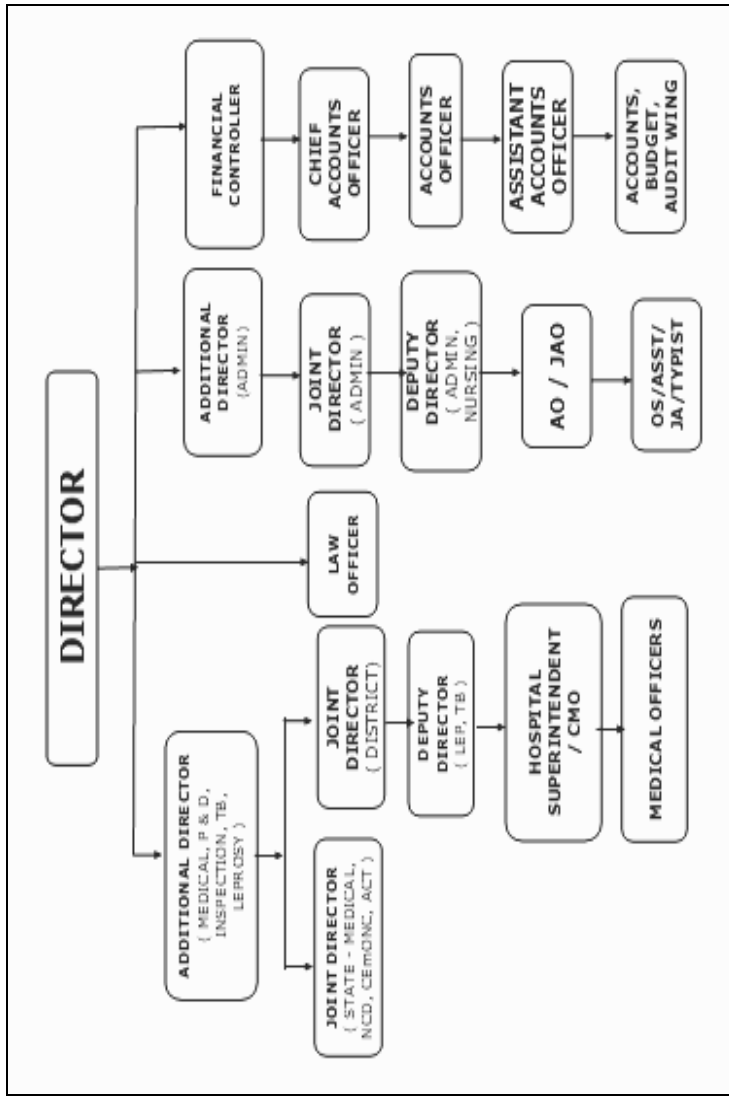
5.5. Institutional Framework & Infrastructure

The Director of Medical and Rural Health Services is in-charge of secondary care services in Tamil Nadu. The Directorate renders its services through the following grid of Hospitals.

Table 5.1: Types of Hospital

Sl.No.	Types of Hospital	Number
1.	District Headquarters Hospitals	37
2.	Taluk Hospitals	199
3.	Non-Taluk Hospitals	59
4.	Women and Children Hospitals	7
5.	Dispensaries	11
6.	Tuberculosis Hospitals	2
7.	Leprosy Hospitals	7
8.	Rehabilitation Institution cum Mental Hospital	1
Total		**323**

Administrative Structure of the Directorate



5.5.1. Comprehensive Emergency Obstetric and Newborn Care (CEmONC)

To meet the Sustainable Development Goals (SDGs) ahead of national timelines, 24/7 dedicated CEmONC centers operate across 94 Secondary care hospitals (35 District Headquarters Hospital, 58 Sub District Hospital and 1 Women and Children Hospital) under the control of this Directorate.

- **Clinical Staffing:** Round-the-clock deployments of obstetricians, paediatricians, and anaesthetists supported by continuous blood storage centres.
- **Near-Miss Audits:** The Directorate utilizes a rigid framework of institutional and community-level maternal death and "near-miss" audits to alter clinical protocols dynamically based on severe maternal survival cases.

Table 5.2: Statistics

Sl. No.	Details	2025 -2026
1.	Total Maternity Admissions	2,63,668
2.	Total Complicated maternity admissions	1,94,751

Sl. No.	Details	2025 -2026
3.	Total Deliveries	1,35,643
4.	Number of Live Birth	1,33,910
5.	Total Caesarean Deliveries	74,879
6.	Night Caesarean (6 PM to 6 AM)	18,908
7.	Total Number of neonatal admission	47,025
8.	Total Number of scan done for Obstetrics & Gynaecology cases	5,38,176
9.	Number of units of Blood Bags collected	1,05,789
10.	Blood transfusion done for Maternity cases	47,497

5.5.2. Special Newborn Care Units (SNCU) & Newborn Stabilization Unit (NBSU)

50 Secondary care hospitals (31 District Headquarters Hospital and 19 Sub District Hospitals) are armed with an advanced SNCU and 98 Secondary care hospitals (5 District Headquarters Hospitals and 93 Sub District Hospitals) are equipped with Newborn Stabilization Units (NBSU). These units are equipped with computerized infant radiant warmers, phototherapy grids and specialized neonatal nursing lines to manage low birth weight, neonatal

sepsis, and respiratory distress by minimizing the risks of referring out vulnerable newborns to tertiary care institutions.

Sl. No.	Category	Number of Children Admitted (2025-2026)
1	SNCU	31,868
2.	NBSU	9,535

5.5.3. Tamil Nadu Accident and Emergency Initiative (TAEI)

The Tamil Nadu Accident and Emergency Initiative (TAEI) has been established in 72 Secondary care hospitals (31 District Headquarters Hospitals and 41 Sub District Hospitals) to strengthen emergency and trauma care services across secondary care hospitals. The initiative focuses on providing timely, accessible and quality emergency medical care for accident victims and other critically ill patients. This initiative focuses on enhancing response to road traffic accidents, cardiac emergencies, stroke, poisoning and other medical emergencies.

5.5.4. Dialysis Services

The Directorate has significantly expanded dialysis services to 72 Secondary care hospitals (31

District Headquarters Hospitals and 41 Sub District Hospitals) for patients suffering from Chronic Kidney Disease and End-Stage Renal Disease, particularly in rural and marginalized areas. This helps in provision of free dialysis services to eligible beneficiaries and reducing out-of-pocket expenditure. Further, Public-private partnership (PPP) model has been implemented in selected facilities to enhance service availability.

Table 5.3: Performance of Dialysis Services

Sl. No.	Particulars	For the year 2025-2026
1.	Number of Dialysis Machines	495
2.	Total Dialysis Cases	33,355
3.	Number of Dialysis Cycles done	3,04,148
4.	Average Number of Dialysis Cycle / Machine / Day	2

Other Services provided and programmes offered in Government Hospitals

The following Medical Services and programmes are rendered by the District Headquarters Hospitals / Taluk / Non-Taluk Hospitals:-

- Out - Patient and In-Patient Services
- Antenatal and Postnatal care
- Medicine, Surgery, Obstetrics and Gynaecology, Ophthalmology, E.N.T, Venereology, Orthopaedics, Anaesthesiology, Child Health, Dental, Psychiatry, Leprosy, Tuberculosis, Diabetology, Cardiology, Ambulance Services, Laboratory Services
- Accident and Emergency Services
- Intensive Care Unit (ICU)
- Cath Lab Services (Kancheepuram DHQH)
- Family Welfare
- Non-Communicable Diseases (NCD)
- Makkalai Thedi Maruthuvam (MTM)
- National TB Elimination Programme (NTEP)
- Blindness Control Programme
- Deafness Control Programme
- District Mental Health Programme
- Poison Treatment Centre
- De-Addiction Centre
- Day Care Chemotherapy

- Radiology
- Pain and Palliative Care Unit
- Geriatric Care
- Leprosy Control Programme
- Emergency Care and Recovery Centre
- Chief Minister's Comprehensive Health Insurance Scheme
- Innuyir Kappom Thittam
- Occupational Health Services

5.5.5. DNB Courses in Secondary Care Hospitals

To bridge the gap in specialist availability at the grassroots level without the heavy infrastructure demands of starting new medical colleges, the State leverages the National Board of Examinations in Medical Sciences (NBEMS) framework to run DNB Broad Specialty and Post-MBBS Diploma courses directly inside secondary care hospitals.

The objective is to Decentralize Post-Graduate Training, Upgrading Secondary Healthcare Infrastructure, and securing a Specialist Pipeline.

Table 5.4: Specialty wise Accredited seats for DNB courses 2026

Sl. No.	Specialties	Post MBBS	Post Diploma	2 years Diploma	Total Accredited seats
1.	General Medicine	4	-	-	4
2.	General Surgery	3	-	-	3
3.	OBG	6	5	19	30
4.	Paediatrics	2	2	13	17
5.	Orthopaedics	2	2	-	4
6.	Anaesthesia	-	-	9	9
7.	Radio Diagnosis	-	-	2	2
Total		**17**	**9**	**43**	**69**

5.5.6. MedLEaPR

The Directorate handles massive volume of Medico-Legal Cases (MLCs) and Post-Mortem Examinations (PMEs). In order to eliminate the systemic delays, physical overhead, and critical judicial challenges associated with handwritten reports, the Government of Tamil Nadu is scaling the implementation of MedLEaPR

The objective is to Standardize Documentation, Inter-Departmental Interoperability, Judicial Efficiency and Accountability & Data Safety. In order to

implement the strategy in secondary care hospitals the MLR - Injury Report, Post-Mortem Report, Specialized MLR Modules and Digital Signing (eSign) are the targets which require core digital format transformation.

MedLEaPR have been brought into functioning since February 2026. Till date, 302 institutions functioning under the control of this Directorate have been enrolled in MedLEaPR.

Table 5.5: Performance under MedLEaPR

S. No.	Particulars	Total Numbers
1.	The total number of MLC cases entered in the portal	8,490
2.	The total number of PM cases entered in the portal	4,151
3.	Doctors registered	3,214

5.5.7. National Quality Assurance Standards (NQAS)

The implementation of National Quality Assurance Standards (NQAS), which is institutionalized by the Ministry of Health and Family Welfare (MoHFW), has made the secondary care facilities in the State to systematically

transform to patient-centric, highly accountable, and safety-compliant institutions. Tamil Nadu consistently ranks among the top-performing states nationally in NQAS certifications.

The objective is to Institutionalize Clinical Governance, Reduction of Out-of-Pocket Expenditure, Standardization of Secondary Care Safety and Fiscal Capitalization. Till date, 102 Secondary care hospitals (32 District Headquarters Hospital and 70 Sub District Hospitals) have been certified under National Quality Assurance Standards (NQAS)

5.5.8. Health Management Information System (HMIS)

To enhance universal health access, programmatic transparency and clinical excellence, the Government of Tamil Nadu has institutionalized a comprehensive, web-based Health Management Information System (HMIS). This digital architecture which is being implemented in secondary care hospitals unifies and links patients, clinicians and health administrators functioning under this Directorate.

The objective of the policy is to Abolish Manual Registries, Continuity of Patient Care, Evidence-Based Health Administration and National Digital Integration.

Table 5.6: HMIS 3.0 - Implementation Phases

Sl. No.	Phase I	Phase II	Phase III	Phase IV	Phase V
	from 13.10.2025	from 09.03.2026	from 05.06.2026	-	-
1	Namakkal	Nagapattinam	Dindigul	Coimbatore	Pudukottai
2	Krishnagiri	Tiruppur	Mayiladuthurai	Karur	The Nilgiris
3	Kallakurichi	Ariyalur	Thirupathur	Erode	Theni
4	Virudhunagar	Perambalur	Kanniyakumari	Vellore	Tiruvarur
5	Ramanatha -puram	Ranipet	Chengalpattu	Madurai	Sivagangai
6	Tirunelveli	Kancheepuram	-	Dharmapuri	Cuddalore
7	Salem	Tiruvallur	-	Thoothukudi	Thanjavur
8	Tenkasi	Tiruvannamalai	-	Villupuram	Tiruchirappalli

5.5.9. Critical Care Blocks

The primary objective of establishing Critical Care Blocks at the secondary care level is to ensure assured, affordable, and quality critical care services closer to the community. Intensive and emergency care services have been provided primarily in private tertiary centres and Government Medical College Hospitals. This initiative aims to run self-sufficient, trauma and pandemic-ready wings in District Hospitals.

In this regard, one 100 bedded Critical Care block (GHQH Cuddalore) and Twenty 50 bedded Critical Care blocks (3 District Headquarters Hospital and 17 Sub District Hospitals) have been sanctioned at a cost of Rs.515.05 crore for hospitals functioning under the control of this Directorate.

5.5.10. District Integrated Public Health Laboratories

District Integrated Public Health Laboratories comprehensively upgrade the diagnostic, surveillance and epidemic-tracking muscle.

The policy intent behind establishing a District Integrated Public Health Laboratories in 37 secondary care hospitals (24 District Headquarters Hospital and 13 Sub District Hospitals), at a cost of Rs.46.25 crore, functioning under the control of this Directorate is to create a single, unified, diagnostic hub that merges clinical diagnostics with public health surveillance.

Epidemic tracking operated in completely isolated labs, have caused critical delays during outbreaks. District Integrated Public Health Laboratories eliminate these delays by establishing an all-in-one network under one roof.

5.5.11. Infrastructure Strengthening

Strengthening secondary care institutions is not merely an infrastructure upgrade; it is a strategic shift from centralized reactive care to decentralized proactive health security.

The establishment of a District Headquarters Hospital in all districts is a vital step towards Universal Health Coverage. In order to move close to the objective of this policy 19 District Headquarters Hospitals and 4 on par with Government hospitals has been established at a cost of Rs.1018.65 crore.

5.6. Key Statutory & Regulatory Functions

Beyond direct public hospital management, the Directorate functions as the regulatory enforcement body safeguarding patient ethics and minimum clinical standards across the private health sector.

5.6.1. The Tamil Nadu Clinical Establishments (Regulation) Act 1997

The Directorate mandates the strict registration of all clinical entities—spanning standalone single-doctor clinics, private hospitals,

multi-specialty corporate hospitals and laboratories. The district-level Joint Directors of Health Services (JDHS) conduct inspections to verify minimum floor areas, baseline lifesaving apparatus, qualified nursing registers and ethical biomedical waste pathways.

48,619 certificates have been issued so far under the Tamil Nadu Clinical Establishments (Regulation) Amendment Act, 2018.

5.6.2. Assisted Reproductive Technology (Regulation) Act, 2021 & Surrogacy (Regulation) Act, 2021

Enforcement of these two central acts is entirely managed through this Directorate to systematically eliminate commercial exploitation:

Registration Controls: Separate mandatory state licenses are issued for Level 1 Clinics, Level 2 Clinics (IVF/ICSI setups) and specialized ART Banks.

Altruistic Monitoring: The decentralized District Medical Boards under the JDHS review all individual applications to issue legal Certificates of Medical

Essentiality/Eligibility-ensuring surrogacy remains purely altruistic and backed by a mandatory 36-month comprehensive health insurance for the surrogate mother.

Penal Action: Severe action against Unregistered fertility hubs or centres practicing illegal sex selection, equipment seize and immediate cancellation of license, inviting criminal prosecution.

License: So far, 430 Level I centres, 428 Level II centres, 136 ART banks and 107 Surrogacy Hospital licenses have been issued.

5.6.3. Pre-Conception & Pre-Natal Diagnostic Techniques (PCPNDT) Act, 1994

To uphold a healthy gender ratio, the Directorate exercises strict surveillance over scanning facilities. Every ultrasound equipment across the State must register with this Directorate. District enforcement teams deploy routine decoy operations and surprise site inspections to detect and penalize illegal foetal sex determination. 9,579 Scan centres (1,793 – Government Hospitals: 7,786 – Private Hospitals) have been registered under this Act.

In order to conduct decoy operation, routine as well as surprise inspection and for carrying out various administrative activities related to acts governed by this Directorate in State as well as District level 40 New vehicles has been procured at a cost of Rs.2.90 crore.

Table 5.7: Financial Outlay

Sl. No.	Particulars	Budget Estimate (in Rs.)
1.	Periodical Maintenance	30,87,00,000
2.	Major Works	39,11,00,000
3.	Grants for Current Expenditure	94,39,000
4.	Medicines	132,00,00,000
5.	Machinery and Equipment Maintenance	15,00,00,000
6.	Contract Payment	59,43,93,000

Table 5.8: Performance and Statistical Data

Sl. No.	Data	Numbers
1.	Average OP/day	2,27,417
2.	Average IP/day	21,432
3.	Major Surgeries/month	22,033
4.	Minor Surgeries/month	1,54,987

Sl. No.	Data	Numbers
5.	MRI Scan/month	1,129
6.	CT Scan/month	34,170
7.	USG/month	77,371
8.	Echo/month	9,882

5.7. Conclusion

The Directorate of Medical and Rural Health Services remains committed in providing 24/7 comprehensive, equitable and high-quality healthcare services to the people of Tamil Nadu. Further, the Directorate will continue to strengthen healthcare infrastructure, improve service delivery, expand specialist care and ensure better health outcomes for all citizens.

MEDICAL AND RURAL HEALTH SERVICES
(Employees' State Insurance Scheme)

5.8. Introduction

Employee State Insurance Scheme is an integrated Social Security Scheme which provides Health Care, Sickness benefit, Maternity benefit, Disablement benefit, Unemployment allowance etc., to the Insured Persons and their eligible family members. The ESI Act was passed in 1948. ESI Schemes administrated by the Employees State Insurance Corporation and implemented in 38 districts of Tamil Nadu from 23rd January 1955.

Tamil Nadu Employee State Insurance Scheme is providing medical care to 44.77 Lakh Insured Persons, 1.79 Crore of Dependents (i.e. their spouse, children and parents).

5.8.1. Vision

To provide effective and excellent Primary, Secondary and Tertiary medical care to the Insured Persons and their family members.

5.9. Enrollment in ESI scheme

1) The ESI Scheme is eligible to all non seasonal workers working in shops & factories and other

establishments in organised sectors wherein 10 or more persons are employed.

2) The Employees getting monthly salary up to Rs.21,000/- are eligible to avail the ESI scheme.

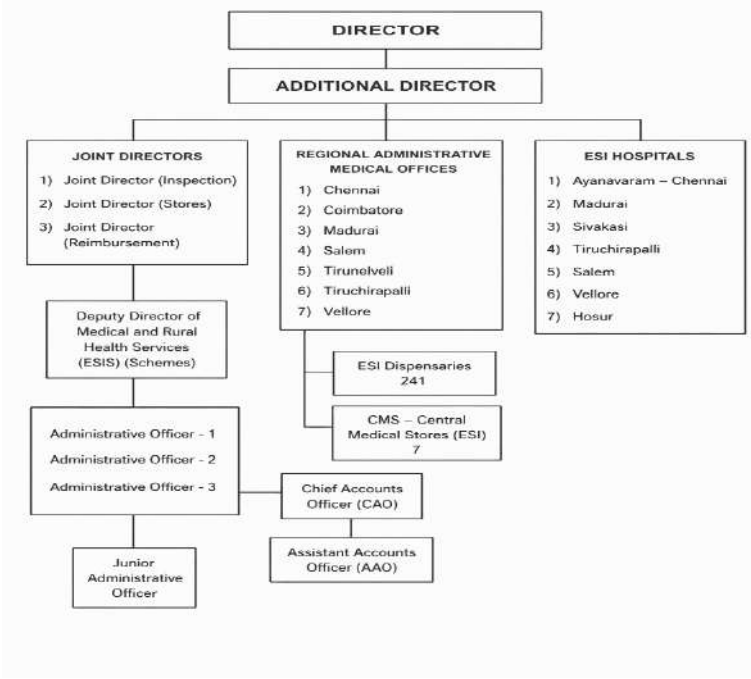
3) The Employees earning upto Rs.176/- per day are exempted from the contribution amount.

4) This scheme is applicable to the employees who are differently abled due to Autism, Cerebral palsy, Mental retardation and Multiple disabilities and drawing monthly salary upto Rs.25,000/-.

5.10. Scheme Funding

A monthly contribution at a fixed percentage of 0.75% and 3.25% of wages paid by the employees and employers respectively as their contribution to avail the benefits of the ESI schemes (w.e.f. 01.07.2019). The ESI Corporation and the State Government are sharing the expenditure in the ratio of 7:1. The ESI Corporation contributes 7/8 share of the medical expenditure incurred for the implementation of this ESI Scheme and Tamil Nadu State Government contributes 1/8 share.

5.11. Organisational Structure



5.12. AYUSH units under ESI scheme

A combined treatment of Ayurveda, Yoga, Unani, Siddha and Homoeopathy (AYUSH) is provided to the insured persons, in addition to allopathic treatment facilities in the following ESI Hospitals and ESI Dispensaries.

Sl. No.	AYUSH Units	ESI Hospitals	ESI Dispensaries
1.	Ayurveda	8	13
2.	Yoga	8	0
3.	Unani	3	3
4.	Siddha	8	43
5.	Homoeopathy	3	3

5.13. Services Offered in ESIS Hospitals (2025-2026)

Maternal and Child Health	OP	IP
Obstetrics & Gynaecology	56,867	5,839
Paediatrics	26,461	2,169

Trauma Care	OP	IP
Orthopaedics	1,06,499	3,664
Anaesthesia	34,160	1,939

Surgical Care	OP	IP
General Surgery	65,633	5,394
ENT	31,496	1,675
Ophthalmology	38,119	1,094

Medical Care	OP	IP
General Medicine	1,57,729	11,186
Dermatology	37,458	884
Psychiatric Medicine	7,743	426

Super Speciality	OP	IP
Urology	2,029	-
Surgical Oncology	228	-
Medical Oncology	1,289	252

Others	OP	IP
AYUSH	1,51,842	-
Dental	25,191	-

5.14. Package deal "Tie-up Hospitals" and Insured Persons Reimbursement

There are 66 speciality and 84 super speciality private tie-up hospitals for providing cashless treatment under package deal CGHS (Central Government Health Scheme Rate) to the Insured persons and their family members. In case of emergencies medical expenditures incurred by Insured Persons in Private Hospitals other than Tie - up Hospitals will be reimbursed based on the Central Government Health Scheme rates.

5.15. Supply of medicines

Medicine supply through Central Rate Contract (CRC) and all life saving drugs for Oncology, Haematology and Nephrology are also supplied immediately as per the recommendation of the doctors.

5.16. Performance

Data	2025-2026	
OP/day	ESI Hospitals	2,587
	ESI Dispensaries	19,317
IP/day	521	
Major Surgeries/ month	411	
Minor Surgeries/ month	3,482	

5.17. Construction Of New 100 Bedded ESI Hospitals

The Employees' State Insurance Scheme Corporation has proposed to construct a new 100 Bedded ESI Hospitals in Tamil Nadu at Dindigul, Kanniyakumari, Erode, Chengalpattu and Ranipet. Sriperumbudur and Tuticorin 100 bedded ESI hospitals are under construction by ESI Corporation.

CHAPTER - 6

PUBLIC HEALTH AND PREVENTIVE MEDICINE

6.1. Introduction

The Directorate of Public Health and Preventive Medicine (DPH&PM), Government of Tamil Nadu, is one of the oldest public health establishments in India and plays a pivotal role in providing preventive, promotive and curative healthcare services across the State. It is responsible for disease prevention and control, public health surveillance, health promotion, emergency response and the implementation of National Health Programmes and State-specific public health initiatives.

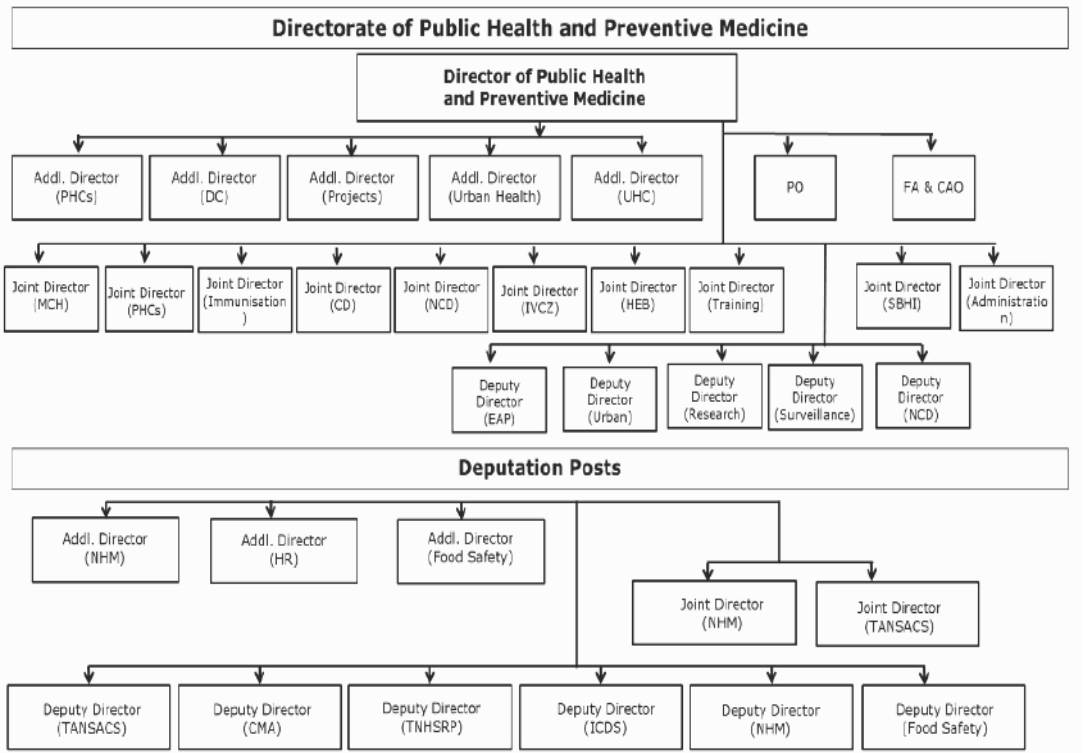
This Directorate also administers the Civil Registration System (CRS) for the registration of births and deaths, generating vital statistics that support planning, monitoring, and evidence-based policy formulation. Through its extensive healthcare infrastructure and trained workforce,

DPH&PM contributes significantly to advance Universal Health Coverage (UHC) and improving population health outcomes in Tamil Nadu.

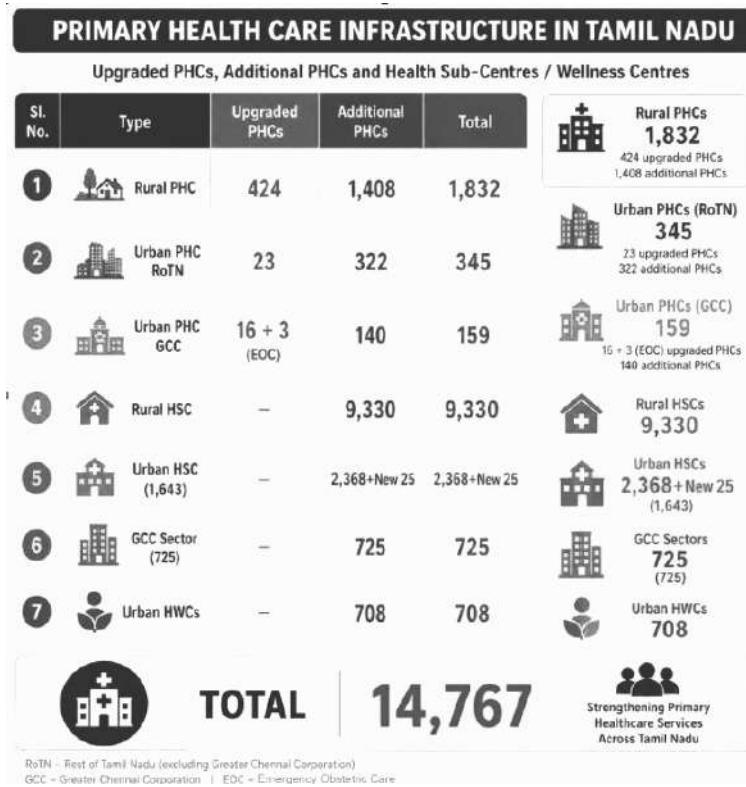
Tamil Nadu has consistently demonstrated strong performance across key health indicators, as reflected in the National Family Health Survey NFHS-6, and subsequent administrative estimates. The State is undergoing a demographic transition characterised by declining fertility and an increasing elderly population, resulting in an ageing population structure. Maternal and child health indicators, including the Infant Mortality Rate (IMR) and the Maternal Mortality Ratio (MMR), remain among the best in India, reflecting the strength and effectiveness of the State's primary healthcare system.

6.2. Administrative Structure of DPH&PM

The Directorate of Public Health and Preventive Medicine functions through a well-defined administrative hierarchy.


Directorate of Public Health and Preventive Medicine
Director of Public Health and Preventive Medicine
Addl. Director (PHCs)
Addl. Director (DC)
Addl. Director (Projects)
Addl. Director (Urban Health)
Addl. Director (UHC)
PO
FA & CAO
Joint Director (MCH)
Joint Director (PHCs)
Joint Director (Immunisation)
Joint Director (CD)
Joint Director (NCD)
Joint Director (IVCZ)
Joint Director (HEB)
Joint Director (Training)
Joint Director (SBHI)
Joint Director (Administration)
Deputy Director (EAP)
Deputy Director (Urban)
Deputy Director (Research)
Deputy Director (Surveillance)
Deputy Director (NCD)
Deputation Posts
Addl. Director (NHM)
Addl. Director (HR)
Addl. Director (Food Safety)
Joint Director (NHM)
Joint Director (TANSACS)
Deputy Director (TANSACS)
Deputy Director (CMA)
Deputy Director (TNHSRP)
Deputy Director (ICDS)
Deputy Director (NHM)
Deputy Director (Food Safety)

Public Health Infrastructure



6.3. Primary Health Care System in Tamil Nadu

Primary Health Centres (PHCs) form the backbone of the public healthcare delivery system and serve as the first point of contact for the provision of essential healthcare services.

PHCs provide a comprehensive range of services, including outpatient care, maternal and child health services, immunization, family welfare services, management of communicable and non-communicable diseases, basic laboratory services, drug dispensing, nutrition counselling, school and adolescent health services and referral support. They also play a vital role in disease surveillance, outbreak detection, health promotion and community health education.

The Directorate of Public Health and Preventive Medicine plays a pivotal role in strengthening primary healthcare services across Tamil Nadu through an extensive and well-established healthcare delivery network. A total of 2,336 Primary Health Centres, 9,330 Rural Health Sub-Centres, 2,393 Urban Health Sectors and 708 Urban Health and Wellness Centres function across the rural and urban areas of the State, ensuring accessible, affordable, equitable, and quality primary healthcare services for the population.

The Directorate has significantly strengthened primary healthcare through the expansion of infrastructure, deployment of human resources, and integration of services across rural

and urban healthcare systems. Additional service delivery platforms, including Mobile Medical Units, Dental Units, and Urban Polyclinics, further enhance healthcare accessibility, particularly for underserved and remote populations.

Collectively, these interventions have contributed to improved service coverage, early disease detection, better maternal and child health outcomes and accelerated progress towards achieving Universal Health Coverage (UHC) in Tamil Nadu.

6.3.1. Rural Primary Health Care Services

Primary healthcare services for the residents of 12,518 Village Panchayats and 472 Town Panchayats in Tamil Nadu are delivered through an extensive network of Rural Primary Health Centres (PHCs). These centres are established in accordance with the norms prescribed by the Government of India, at the rate of one PHC for every 30,000 population in plain areas and one PHC for every 20,000 population in hilly and difficult-to-access areas. While establishing PHCs, due consideration is given to accessibility, availability of healthcare services and maintaining a minimum distance of 8 kilometres from the nearest existing

health institution to ensure the equitable distribution of healthcare facilities.

The rural primary healthcare system is supported by a multidisciplinary team comprising Medical Officers, Staff Nurses, Pharmacists, Laboratory Technicians, Health Inspectors, Village Health Nurses and Multi purpose Hospital workers.

Of 1,832 PHCs functioning across the State, 1,820 operate in Government-owned buildings, 11 function in rent-free buildings and 1 function in a rented building.

A total of 424 PHCs have been upgraded as Upgraded Primary Health Centres (UGPHCs) with 30-bedded facilities to provide round-the-clock inpatient and emergency care, thereby strengthening access to rural healthcare and reducing the referral burden.

The Government of Tamil Nadu will further strengthen primary health care services in rural areas by expanding PHC coverage, enhancing infrastructure, augmenting human resources and leveraging digital health technologies.

6.3.2. Urban Primary Health Care Services

Tamil Nadu is one of the most urbanised States in India, with around 50 percent of its population residing in urban areas, including 25 Municipal Corporations and 150 Municipalities.

Urban primary healthcare services are provided under the National Urban Health Mission (NUHM) through 504 Urban Primary Health Centres (UPHCs & UCHCs), established as per the norm of one UPHC for every 50,000 population, with a special focus on vulnerable and underserved communities.

These centres provide 12 comprehensive primary healthcare services, namely maternal healthcare, child healthcare, adolescent healthcare, mental healthcare, management of communicable and non-communicable diseases, elderly care, palliative care, ophthalmic care, ENT services and oral healthcare services.

In addition, 39 Urban Community Health Centres (UCHCs), established for every 2.5 lakh population, along with 154 Urban Polyclinics, provide specialist outpatient services and strengthen referral care in urban areas.

6.3.3. Urban Health and Wellness Centres (UHWCs)

Urban Health and Wellness Centres (UHWCs) provide comprehensive primary healthcare services closer to communities by delivering preventive, promotive, curative, rehabilitative and palliative care.

To improve healthcare accessibility, particularly for the urban workforce, 708 Urban Health and Wellness Centres (UHWCs) which provide extended evening outpatient services, are functioning across the State.

These centres improve access to essential healthcare services, reduce dependence on higher-level healthcare facilities and strengthen the continuity of care in urban settings.

6.3.4. Health Sub-Centres (HSCs)

Health Sub-Centres (HSCs) represent the grassroots level of the public health system and serve as the primary interface between the community and the healthcare system.

They are responsible for household-level outreach, immunization, antenatal and postnatal

care, disease surveillance, health education, follow-up care.

Tamil Nadu has an extensive network of 9,330 Rural Health Sub-Centres and 2,393 Urban Health Sectors**,** including those functioning under the Greater Chennai Corporation. Strengthening infrastructure remains a key priority, with 6,806 Rural Health Sub-Centres and 311 Urban Health Sub-Centres operating from Government-owned buildings.

Efforts are underway to replace 1,624 dilapidated buildings and provide permanent infrastructure for Rural Health Sub-Centres currently functioning in rented or rent-free premises. Simultaneously, steps are being taken to establish permanent infrastructure for all Urban Health Sub-Centres.

6.3.5. Urban Polyclinic Services

Urban Polyclinics are an integral component of Tamil Nadu's urban primary healthcare system and have been established to provide specialist healthcare services closer to the community. With the rising burden of non-communicable diseases such as hypertension, diabetes and cardiovascular

diseases, coupled with the significant out-of-pocket expenditure on specialist care reported in national surveys, Urban Polyclinics improve access to affordable specialist healthcare services within the public health system.

A total of 154 Urban Polyclinics have been established within Urban Primary Health Centres, including those functioning under the Greater Chennai Corporation. These centres provide specialist outpatient services during extended evening hours, enabling the working population to access speciality medical services without loss of working hours.

Urban Polyclinics have recorded substantial utilisation, with approximately 5.61 lakh beneficiaries availing services during 2025–26 and 88,893 beneficiaries during April to May 2026–27, reflecting the speciality care provided at primary healthcare level.

By strengthening continuity of care, facilitating the early diagnosis and management of chronic diseases, reducing pressure on higher-level healthcare facilities, and minimising referrals, Urban Polyclinics have significantly enhanced the efficiency of the urban primary healthcare system.

The Government of Tamil Nadu will further strengthen urban health care by expanding Urban Primary Health Centres and polyclinics, enhancing outreach services for slums and migrant populations, including workplace health programmes and prioritizing comprehensive mental health and geriatric care to meet the changing health needs of the urban population.

6.4. Healthcare Reach

During the year 2025–2026, PHCs in Tamil Nadu recorded a total of 16.73 crore outpatient consultations and 41.45 lakh inpatient admissions, reflecting the strong reliance of communities on the public primary healthcare system.

During April–May 2026, PHCs registered 2.50 crore outpatient consultations and 6.30 lakh inpatient admissions, demonstrating sustained demand for and continued utilisation of primary healthcare services.

On an average, each PHC catered to approximately 196 outpatient visits per day and provided inpatient care to around 148 patients per

month, highlighting the substantial workload managed at the primary healthcare level.

The high utilisation of public health facilities reflects the effectiveness of Tamil Nadu's sustained investments in strengthening primary healthcare infrastructure, human resources, diagnostic services, the availability of essential medicines and round-the-clock service delivery. Primary Health Centres (PHCs) continue to play a pivotal role in improving access to quality healthcare services across the State.

6.5. Maternal and Child Health Care Services

As per Sample Registration System 2022-2024, Tamil Nadu has achieved a Maternal Mortality Ratio (MMR) of 25 per 1,00,000 live births, reflecting the sustained strengthening of maternal health services across all levels of care. This achievement has been supported by universal registration and continuous monitoring of antenatal mothers, timely provision of antenatal care services, assurance of institutional deliveries, and the systematic identification and referral of high-risk pregnancies to higher-level healthcare facilities. Community-level follow-up by Village Health Nurses, together with strengthened

emergency obstetric care services, has further contributed to the reduction in maternal mortality.

As per Sample Registration System 2024, the State has also achieved Infant Mortality Rate (IMR) of 11 per 1,000 live births, reflecting improved newborn survival outcomes. This decline is attributed to strengthened newborn care services, near-universal immunization coverage, increased institutional deliveries and enhanced early detection and management of neonatal complications, supported by effective referral systems and a strong continuum of care.

These achievements reflect the continued strengthening of Tamil Nadu's public health system, particularly at the primary healthcare level. Institutional delivery coverage, robust maternal and child healthcare services, and sustained public health interventions have collectively contributed to improved maternal and child health outcomes across the State.

6.6. MMR and IMR Trend as per SRS

The Government of Tamil Nadu envisions achieving further reductions in the Maternal Mortality Ratio (MMR) and Infant Mortality Rate

(IMR) through comprehensive nutritional interventions, with a particular emphasis on combating anaemia among women and children.

6.6.1. Dr. Muthulakshmi Reddy Maternity Benefit Scheme (MRMBS)

Home deliveries increase the risk of maternal and newborn morbidity and mortality due to delayed recognition of danger signs and complications, as well as inadequate access to emergency obstetric and neonatal care. In contrast, institutional deliveries provide skilled birth attendance and timely access to emergency services, enabling effective management of complications while facilitating early postnatal care, newborn immunization and linkage to essential health and nutrition services, thereby improving maternal and child health outcomes.

As per NFHS-6, 99.7% of deliveries that occurred in the preceding five years were institutional deliveries. While institutional deliveries play a crucial role in improving maternal and child health outcomes, hospitalization for childbirth can impose a significant financial burden on families due to wage loss resulting from absence of work and out-of-pocket healthcare expenses. To

mitigate these challenges, improve maternal health, and promote institutional deliveries, the Government of Tamil Nadu has introduced several maternity benefit schemes like Dr.Muthulakshmi Reddy Maternity Benefit Scheme (MRMBS).

The Dr. Muthulakshmi Reddy Maternity Benefit Scheme (MRMBS), implemented on 15.09.2006, is a flagship maternal welfare programme of the Government of Tamil Nadu aimed at improving maternal and child health outcomes through financial assistance, nutritional support and the promotion of safe motherhood practices.

Under the co-branded MRMBS–PMMVY 2.0 framework, eligible beneficiaries receive financial assistance of up to Rs.18,000/- in phased installments linked to antenatal care, institutional delivery and postnatal childcare milestones. The benefit structure varies according to parity and beneficiary category.

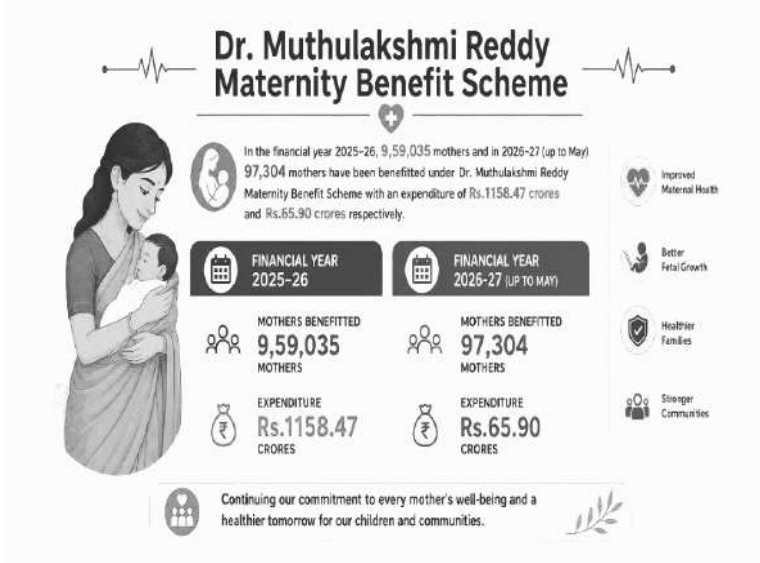
The scheme covers women belonging to vulnerable groups, including Scheduled Castes (SCs)**,** Scheduled Tribes (STs)**,** Below Poverty Line (BPL) households, persons with disabilities, PM-JAY beneficiaries, VB-GRAM workers, PM-Kisan

beneficiaries, e-Shram card holders, National Food Security Act (NFSA) ration card holders, Accredited Social Health Activists (ASHAs), Anganwadi Workers and Helpers and other eligible low-income categories.

Table 6.1: Dr. MRMBS-PMMVY Scheme Installment details

Installment	Term of Installment	Total Amount (In Rs.)	First Pregnancy (For both Male and Female Child)		2nd Female Child		2nd Male Child	Higher Order Birth mothers & Migrant mothers from other state
			MRMBS (IFHRMS Treasury)	PMMVY 2.0 (DBT from SNA)	MRMBS (IFHRMS Treasury)	PMMVY 2.0 (DBT from SNA)		
Nutrition Kit-1	3rd month of pregnancy	2,000	2,000	0	2,000	0	2,000	2,000
1st Installment	4th month of pregnancy	6,000	3,000	3,000	6,000	0	6,000	0
Nutrition Kit-2	6th month of pregnancy	2,000	2,000	0	2,000	0	2,000	2,000
2nd Installment	After 4th month of delivery	6,000	4,000	2,000	0	6,000	6,000	4,000
3rd Installment	After 9th month of delivery	2,000	2,000	0	2,000	0	2,000	0
Total		**18,000**	**13,000**	**5,000**	**12,000**	**6,000**	**18,000**	**8,000**

During the year 2025–26, 9.59 lakh beneficiaries were given financial assistance, with an expenditure of Rs.1,158.47 crore. During the year 2026–27 (April and May)**,** 97,304 beneficiaries were given financial assistance, with an expenditure of Rs.65.90 crore.



6.6.2 Thaimaaman Thanga Mothiram Thittam

To further strengthen maternal and newborn welfare and to reinforce public confidence in Government health institutions, the Government of Tamil Nadu has launched the Thaimaaman Thanga Mothiram Thittam. Rooted in the cultural tradition

of *"Thaimaaman Seer"*, the scheme provides a one-gram gold ring to every newborn delivered in Government health facilities, symbolising the State's commitment to celebrating motherhood and welcoming every child with dignity and care. The initiative seeks to transform childbirth in public institutions into an occasion of pride while promoting equitable access to quality maternal and newborn services.



The scheme covers all newborns delivered in Government institutions, irrespective of birth order. By combining cultural values with public health objectives, the scheme complements existing maternal welfare initiatives and further

strengthens Tamil Nadu's efforts towards improving maternal and neonatal outcomes while enhancing community trust in the public healthcare system.

6.6.3 Maternal Nutrition Kit (MNK)

In alignment with Sustainable Development Goal (SDG) Target 2.2, which aims to end all forms of malnutrition by 2030, the Government of Tamil Nadu is committed to improving maternal nutrition and reducing anaemia among pregnant women. Maternal anaemia is a major contributor to adverse pregnancy outcomes, including preterm birth, low birth weight, and poor neonatal health.



Adequate Iron and Folic Acid (IFA) supplementation during pregnancy is essential for the prevention and treatment of maternal anaemia. As per NFHS-6 (2023–24), only 64.5% of antenatal mothers who delivered during the five years preceding the survey consumed Iron and Folic Acid tablets for 180 days, highlighting the need to strengthen adherence to IFA supplementation.

To improve maternal nutrition and reduce anaemia, two Maternal Nutrition Kits (MNKs) are provided to eligible pregnant women through Primary Health Centres.

Each kit, procured through the Tamil Nadu Medical Services Corporation (TNMSC), contains nutritional supplements and essential items, including Mother's Health Mix, Iron supplements, Albendazole tablet, dry dates, Aavin ghee and hygiene support materials.

During the year 2025–26, 12.60 lakh Maternal Nutrition Kits were distributed. During the year 2026–27 (up to May), 1.19 lakh kits were distributed, contributing to improved maternal nutrition and foetal development.

The Government of Tamil Nadu will continue to strengthen maternal nutrition by promoting

dietary diversity, ensuring adequate micronutrient supplementation and enhancing behaviour change communication to improve maternal and child health outcomes.

6.6.4 Mother and Baby Care Kit

Postnatal care is crucial for ensuring the health and well-being of both the mother and the newborn during the critical period following childbirth. Timely postnatal care promotes exclusive breastfeeding, supports essential newborn care practices and contributes to reducing maternal and neonatal morbidity and mortality.



To strengthen postnatal care and promote newborn health, Mother and Baby Care Kits containing essential items such as baby clothing, bedding, mosquito nets, hygiene products and newborn care materials are provided to mothers delivering in Government health facilities. This initiative promotes institutional deliveries, exclusive breastfeeding, infection prevention and improved newborn care practices.

During the year 2025–26**,** 4.12 lakh Mother and Baby Care Kits were distributed. During the year 2026–27 (April and May)**,** 63,725 kits were distributed.

The Government of Tamil Nadu will continue to strengthen postnatal care services through timely postnatal visits and home-based care for mothers and newborns. It will further promote exclusive breastfeeding, essential newborn care practices and awareness of neonatal and maternal danger signs to facilitate the early referral of complications and improve maternal and neonatal health outcomes.

6.7. Menstrual Hygiene Programme (MHP)

Menstrual hygiene is a significant public health issue affecting adolescent girls and women due to limited access to sanitary napkins, inadequate awareness and socio-cultural stigma. Poor menstrual hygiene contributes to reproductive tract infections, school absenteeism, reduced participation in daily activities and adverse reproductive health outcomes. Evidence from the National Family Health Survey (NFHS) and other public health studies highlights the importance of menstrual health education and access to sanitary napkins in improving dignity, health and well-being.

The Menstrual Hygiene Programme (MHP), implemented in Tamil Nadu since 2011, aims to promote safe menstrual hygiene practices, improve reproductive health awareness and ensure equitable access to sanitary napkins for adolescent girls and women.

Under the programme, free sanitary napkins are provided to adolescent girls aged 10–19 years who have attained puberty, school and college students, out-of-school girls, postnatal mothers,

women prisoners and women admitted to Government health facilities.

The programme is implemented by the Directorate of Public Health and Preventive Medicine in coordination with the Directorate of School Education and the Integrated Child Development Services (ICDS). Distribution is carried out through Schools, Colleges, Anganwadi Centres and Government health facilities, along with awareness activities on menstrual hygiene management and environmentally safe disposal practices.

During the year 2025–26, the programme benefited an average of 59.20 lakh beneficiaries per month received free sanitary napkins.

The Government of Tamil Nadu will continue to strengthen menstrual hygiene education through integration with the Rashtriya Bal Swasthya Karyakram (RBSK) and the Rashtriya Kishor Swasthya Karyakram (RKSK)**,** while enhancing school-based interventions, improving community awareness and promoting environmentally safe disposal practices to further improve women's health, dignity and well-being.

6.8. Palli Sirar Kannoli Kappom Thittam

Refractive errors are a significant cause of visual impairment among school-aged children in Tamil Nadu, adversely affecting educational performance, class room participation and overall well-being. Many cases remain undiagnosed due to limited awareness, highlighting the need for systematic school-based vision screening and early correction.

Palli Sirar Kannoli Kappom Thittam is a flagship initiative of the Government of Tamil Nadu aimed at the early detection and correction of refractive errors among students studying in Classes VI to XII in Government and Government-aided schools.

The programme provides school-based vision screening, comprehensive ophthalmic examination for suspected cases, referral services for advanced ophthalmic care and free distribution of corrective spectacles through schools.

The programme is jointly implemented by the Directorate of Public Health and Preventive Medicine and the School Education Department,

with support from Para Medical Ophthalmic Assistants and trained nodal teachers.

During the year 2025–26, a total of 41,76,093 students were screened across the State. Based on clinical assessment, spectacles were provided for 2,82,961 students received free corrective spectacles, contributing significantly to the early correction of refractive errors among school children.

The Government of Tamil Nadu will continue to strengthen the programme by promoting digital vision screening tools, establishing spectacle banks, enhancing awareness on appropriate screen-time practices and increasing parental participation to improve early detection and treatment adherence.

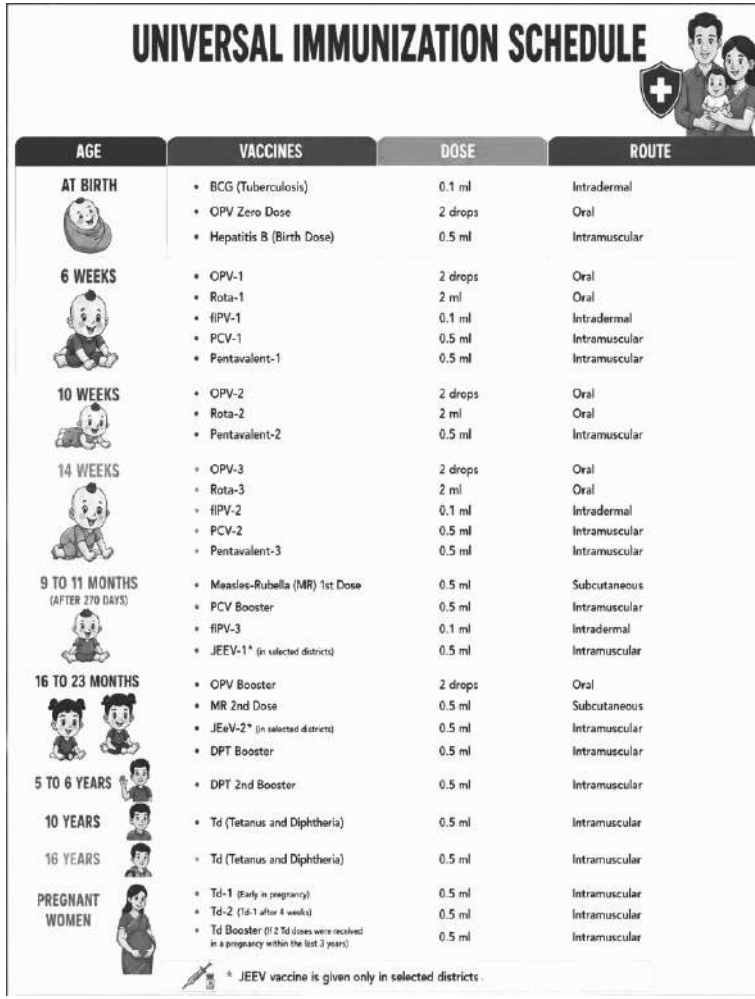
6.9. Immunization

6.9.1. Universal Immunization Programme (UIP)

According to the latest United Nations (UN) 2024 estimates, approximately 4.9 million children under five years of age continue to die each year worldwide, with a significant proportion of these deaths attributable to vaccine-preventable

infectious diseases that can be averted through routine immunization.

Routine Immunization Schedule



UNIVERSAL IMMUNIZATION SCHEDULE

AGE	VACCINES	DOSE	ROUTE
AT BIRTH	• BCG (Tuberculosis)	0.1 ml	Intradermal
	• OPV Zero Dose	2 drops	Oral
	• Hepatitis B (Birth Dose)	0.5 ml	Intramuscular
6 WEEKS	• OPV-1	2 drops	Oral
	• Rota-1	2 ml	Oral
	• fIPV-1	0.1 ml	Intradermal
	• PCV-1	0.5 ml	Intramuscular
	• Pentavalent-1	0.5 ml	Intramuscular
10 WEEKS	• OPV-2	2 drops	Oral
	• Rota-2	2 ml	Oral
	• Pentavalent-2	0.5 ml	Intramuscular
14 WEEKS	• OPV-3	2 drops	Oral
	• Rota-3	2 ml	Oral
	• fIPV-2	0.1 ml	Intradermal
	• PCV-2	0.5 ml	Intramuscular
	• Pentavalent-3	0.5 ml	Intramuscular
9 TO 11 MONTHS (AFTER 270 DAYS)	• Measles-Rubella (MR) 1st Dose	0.5 ml	Subcutaneous
	• PCV Booster	0.5 ml	Intramuscular
	• fIPV-3	0.1 ml	Intradermal
	• JEEV-1* (in selected districts)	0.5 ml	Intramuscular
16 TO 23 MONTHS	• OPV Booster	2 drops	Oral
	• MR 2nd Dose	0.5 ml	Subcutaneous
	• JEeV-2* (in selected districts)	0.5 ml	Intramuscular
	• DPT Booster	0.5 ml	Intramuscular
5 TO 6 YEARS	• DPT 2nd Booster	0.5 ml	Intramuscular
10 YEARS	• Td (Tetanus and Diphtheria)	0.5 ml	Intramuscular
16 YEARS	• Td (Tetanus and Diphtheria)	0.5 ml	Intramuscular
PREGNANT WOMEN	• Td-1 (Early in pregnancy)	0.5 ml	Intramuscular
	• Td-2 (Td-1 after 4 weeks)	0.5 ml	Intramuscular
	• Td Booster (If 2 Td doses were received in a pregnancy within the last 3 years)	0.5 ml	Intramuscular

* JEEV vaccine is given only in selected districts.

According to NFHS-6 (2023–24), 90% of children aged 12–23 months are fully vaccinated (based on vaccination cards or mothers' recall). Sustaining high immunization coverage is essential to prevent the resurgence of vaccine-preventable diseases.

The Universal Immunization Programme (UIP) is one of Tamil Nadu's most successful public health programmes, providing free vaccination services to protect pregnant women, infants, children and adolescents against 12 vaccine-preventable diseases through the administration of 11 vaccines. The Vaccine-Preventable Diseases (VPDs) covered under the programme include Tuberculosis, Diphtheria, Pertussis, Hepatitis B, Haemophilus influenzae type b (Hib), Tetanus, Poliomyelitis, Measles, Rubella, Rotavirus disease, Pneumococcal disease and Japanese Encephalitis (in selected endemic districts).

Tamil Nadu consistently maintains immunization coverage above 99%, with approximately 8.82 lakh pregnant women and 8.06 lakh infants and children vaccinated annually as per HMIS reports.

6.9.2. Routine Immunization

Routine immunization services are delivered through an extensive network of health facilities and outreach sessions across the State. This includes age-appropriate vaccination of pregnant women, newborns, infants, children and adolescents in accordance with the guidelines of the Government of India.

6.9.3. Electronic Vaccine Intelligence Network (eVIN)

Maintaining vaccine potency requires strict adherence to the recommended storage temperatures throughout the vaccine supply chain. To strengthen vaccine logistics and ensure uninterrupted vaccine availability, Tamil Nadu implemented the Electronic Vaccine Intelligence Network (eVIN) in October 2019.

The eVIN platform enables real-time monitoring of vaccine stocks and storage temperatures, ensuring uninterrupted vaccine availability and maintenance of vaccine potency across all cold chain points in the State.

6.9.4. U-WIN Digital Immunization Platform

Incomplete vaccination, missed doses, migration of beneficiaries, poor record

maintenance and difficulties in tracking unvaccinated children remain major challenges to achieving universal immunization coverage. To address these challenges, Tamil Nadu successfully implemented the U-WIN Digital Immunization Platform during 2023–24.

The platform digitally tracks vaccination services for pregnant women and children up to 16 years of age under the Universal Immunization Programme. It facilitates beneficiary registration, vaccination tracking, QR-based digital certificates, SMS reminders and nationwide portability of immunization records.

Currently, 6,453 health facilities have been onboarded onto the U-WIN platform, strengthening programme monitoring and reducing the number of missed and zero-dose beneficiaries.

The Government of Tamil Nadu envisions an integrated Immunization Ecosystem in which eVIN, U-WIN, HMIS, ABDM, birth registration systems, and disease surveillance platforms are seamlessly connected. This integration will enable real-time demand forecasting, identification of missed beneficiaries and delivery of personalised immunization services.

6.9.5. Vaccine-Preventable Disease (VPD) Surveillance

Although immunization has significantly reduced the burden of vaccine-preventable diseases, these diseases continue to remain a public health concern due to the risk of outbreaks and disease resurgence. Therefore, a robust Vaccine-Preventable Disease (VPD) Surveillance System is essential to continuously monitor disease occurrence, facilitate the early detection of outbreaks, evaluate programme performance and support disease control, elimination and eradication initiatives.

Tamil Nadu maintains a comprehensive Vaccine-Preventable Disease Surveillance System with technical support from the World Health Organization (WHO). Surveillance is undertaken for Poliomyelitis, Measles, Rubella, Diphtheria, Pertussis, and Neonatal Tetanus to facilitate early outbreak detection, monitor programme performance and support disease elimination efforts.

6.9.6. Adverse Events Following Immunization (AEFI) Surveillance

Adverse Events Following Immunization (AEFI) Surveillance is a critical component of the

immunization programme that ensures vaccine safety and maintains public confidence in immunization services.

The World Health Organization (WHO) defines an Adverse Event Following Immunization (AEFI) as *"any untoward medical occurrence which follows immunization and which does not necessarily have a causal relationship with the usage of a vaccine."*

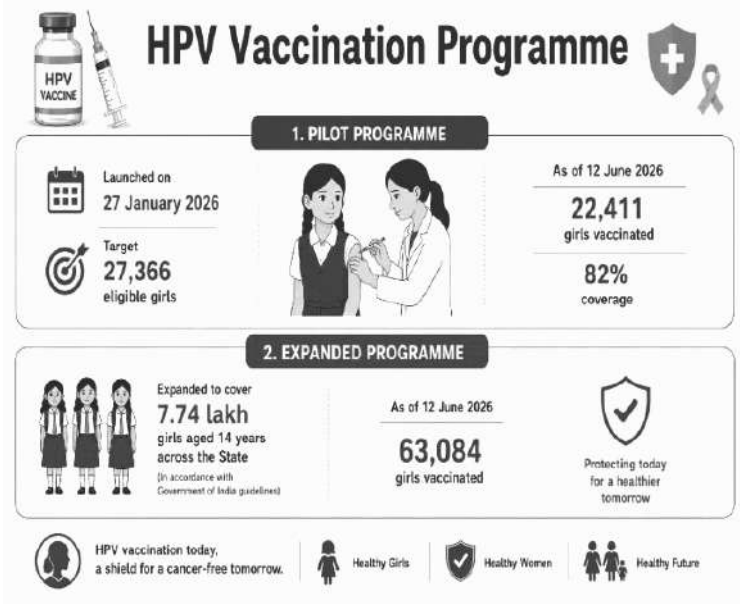
Serious and severe AEFI cases are reported and monitored through the Government of India's Safe-Vac Portal**,** enabling timely investigation, causality assessment and appropriate corrective action.

The Government of Tamil Nadu will continue to strengthen laboratory-supported surveillance, enhance real-time digital reporting, expand training on outbreak response and vaccine safety, integrate advanced data analytics and improve public communication.

6.9.7. Human Papillomavirus (HPV) Vaccination Programme

Despite being largely preventable, cervical cancer caused by persistent Human Papillomavirus

(HPV) infection continues to impose a substantial burden of morbidity, mortality and premature loss of healthy life among women in Tamil Nadu, underscoring the need for strengthened vaccination and screening strategies.



According to the latest estimates of the ICMR–National Cancer Registry Programme (NCRP), the burden of cervical cancer remains substantial. With the projected increase in cervical cancer incidence, there is an urgent need to intensify preventive strategies through HPV vaccination.

To prevent cervical cancer and reduce the long-term burden of HPV-related diseases, the Government of Tamil Nadu launched the Human Papilloma virus (HPV) Vaccination Programme for 14-year-old girls studying in Government and Government-aided schools.

The programme was launched on 27.01.2026 in four pilot districts - Dharmapuri, Ariyalur, Tiruvannamalai, and Perambalur -targeting 27,366 eligible girls. As of 12th June 2026**,** 22,411 girls had been vaccinated, achieving 82% coverage**.**

Subsequently, the programme was expanded in accordance with the Government of India guidelines to cover 7.74 lakh girls aged 14 years across the State. As of 12th June 2026, 63,084 girls have been vaccinated under the expanded programme.

The Government of Tamil Nadu will continue the phased expansion of the HPV Vaccination Programme by strengthening vaccine safety monitoring, enhancing community awareness, and reaching eligible girls who have not yet been vaccinated, thereby contributing to the long-term prevention of cervical cancer in the State.

6.10. Makkalai Thedi Maruthuvam (MTM)

6.10.1. A Globally Recognised Model for Community-Based Prevention and Control of Non-Communicable Diseases

Non-Communicable Diseases (NCDs) are among the leading causes of morbidity, disability, and premature mortality. Their silent progression often delays diagnosis, resulting in complications, reduced productivity, financial hardship and increased pressure on the health system. According to NFHS-6 (2023–24)**,** hypertension affects 28.2% of men and 24.2% of women, while diabetes affects 26.7% of men and 25.2% of women in Tamil Nadu, highlighting the need for a robust community-based approach to NCD prevention and management.

Recognising the limitations of conventional facility-based care in ensuring regular follow-up and continuity of treatment for chronic diseases, the Government of Tamil Nadu launched the flagship Makkalai Thedi Maruthuvam (MTM) programme on 5th August 2021. The programme has transformed healthcare delivery by bringing healthcare services directly to the doorsteps of beneficiaries and providing an integrated

continuum of care encompassing prevention, screening, treatment support, rehabilitation, palliative care, complication surveillance, and referral services.

Over the years, MTM has evolved into an umbrella platform for comprehensive NCD management and expanded beyond routine hypertension and diabetes care to include the screening and management of Chronic Kidney Disease (CKD), diabetic complications, Chronic Obstructive Pulmonary Disease (COPD)**,** Metabolic Dysfunction-Associated Steatotic Liver Disease (MASLD), community-based organised cancer screening and healthy lifestyle promotion initiatives.

The Government of Tamil Nadu is committed to reducing premature mortality due to NCDs through equitable access to preventive, promotive, curative, rehabilitative, and palliative healthcare services.

Makkalai Thedi Maruthuvam serves as the flagship platform for achieving universal access to quality healthcare through the early detection of diseases and complications, continuity of care, timely referrals, prevention of disability and

premature mortality, promotion of healthy lifestyles, and the effective use of digital technologies to improve health outcomes.

The programme focuses on providing comprehensive home-based healthcare services, improving access for vulnerable populations, and strengthening the continuum of care for NCDs.

The State's NCD strategy under MTM is built upon five interconnected pillars:

- Bringing Healthcare to the Doorstep through community-based service delivery.
- Early Detection of Diseases and Complications through systematic screening and surveillance.
- Ensuring Continuity of Care through referral, follow-up, and rehabilitation services.
- Promoting Healthy Lifestyles through community engagement and health promotion.
- Addressing Emerging Public Health Challenges through evidence-based interventions.

The programme is supported by an extensive network of frontline healthcare workers and health institutions.

Table 6.2: Human Resources

Category	Number
Women Health Volunteers	10,969
MTM Staff Nurses	2,174
Physiotherapists	463
Palliative Care Nurses	463

This integrated network of Health Sub-Centres, Primary Health Centres, Taluk and Non-Taluk Hospitals, Government Hospitals and Medical College Hospitals facilitates seamless healthcare service delivery from the community level to tertiary healthcare institutions.

MTM provides screening and management of hypertension and diabetes, doorstep delivery of medicines and treatment support, home-based physiotherapy services, palliative care services, continuous Ambulatory Peritoneal Dialysis (CAPD) support services, cancer care support, referral and follow-up services.

Since its inception up to March 2026, 2.6 crore beneficiaries have availed services under the programme. During the year 2026–27 (April and May), an additional 2.99 lakh beneficiaries were benefitted.

The programme has demonstrated measurable improvements in disease control, with hypertension control increasing from 7.3% to 17.0% and diabetes control improving from 10.8% to 16.7%, respectively from the STEPS-1 survey to STEPS-2 Survey conducted during 2019–20 and 2023–24.

The programme has significantly improved healthcare accessibility for elderly persons, individuals living with chronic diseases, and other vulnerable populations requiring long-term healthcare support, thereby strengthening continuity of care and improving quality of life.

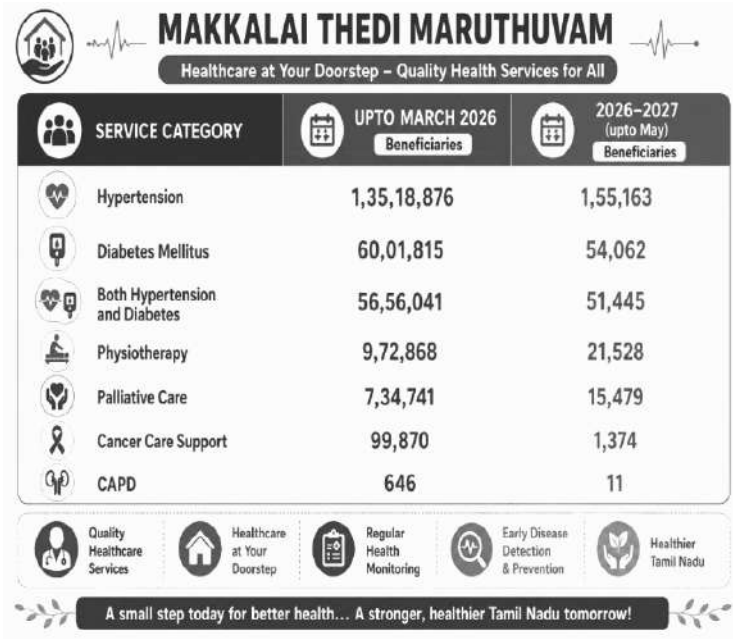
Tamil Nadu continues to strengthen digital systems for continuity of care and long-term monitoring of chronic diseases.

Key initiatives include

- ❖ Telemedicine and specialist consultation support

- ❖ Digital monitoring and referral tracking systems
- ❖ Integrated complication surveillance systems
- ❖ Beneficiary outcome monitoring systems.

Table 6.3: MTM Beneficiary Details



Makkalai Thedi Maruthuvam has transformed healthcare delivery in Tamil Nadu by shifting the focus from episodic facility-based care to proactive,

community-centred and continuum-based healthcare. The programme has strengthened access to healthcare, enabled the early detection and timely management of chronic diseases and their complications and emerged as a globally recognised model for integrated community-based NCD care. The Government of Tamil Nadu remains committed to further strengthening this flagship initiative to reduce premature mortality due to NCDs, prevent disability and improve the quality of life of the people of Tamil Nadu.

6.10.2. Urine Dipstick Testing for Early Detection of Chronic Kidney Disease

Chronic Kidney Disease (CKD) is a progressive condition that often remains asymptomatic until the advanced stages and is commonly associated with diabetes and hypertension. Delayed diagnosis can lead to kidney failure requiring dialysis or transplantation, resulting in substantial morbidity, mortality and financial burden on affected households. According to the NSSO 80th Round, kidney failure accounted for 1.7% of all hospitalised cases, underscoring the need for early detection and timely intervention.

Recognising the growing burden of CKD, the Government of Tamil Nadu introduced Urine Dipstick Testing for Early Detection of Chronic Kidney Disease in July 2023. Under this initiative, all individuals aged 30 years and above are screened at primary healthcare facilities as part of routine NCD services to facilitate early identification and timely referral.

As of 31st March 2026, a total of 1.46 crore individuals were screened, of which 43,880 individuals tested positive and were referred for further evaluation and treatment. During the year 2026–27 (April and May), an additional 6.96 lakh individuals were screened, with 1,438 individuals identified with abnormal findings. Of these, 1,190 beneficiaries were referred for further evaluation, and 1,331 beneficiaries underwent estimated Glomerular Filtration Rate (eGFR) assessment.

Among the beneficiaries who underwent evaluation during 2026–27 (April and May), 760 individuals were diagnosed with Stage 1 CKD, 267 with Stage 2**,** 223 with Stage 3, 67 with Stage 4 and 14 with Stage 5 CKD and are undergoing treatment.

To ensure continuity of care for advanced kidney disease, 20 beneficiaries were initiated on haemodialysis, while 30 beneficiaries were initiated on Continuous Ambulatory Peritoneal Dialysis (CAPD).

This initiative has strengthened early detection, timely referral and continuity of care, thereby preventing progression to end-stage renal disease and reducing the long-term burden of chronic kidney disease.

6.10.3. Screening for Diabetic Complications

Uncontrolled diabetes can lead to complications affecting the eyes, kidneys, nerves, and cardiovascular system, many of which remain asymptomatic until the advanced stages. Early detection and timely intervention are essential to prevent disability, reduce complications and premature death, also improve quality of life. Accordingly, Tamil Nadu has integrated comprehensive screening for diabetic complications into routine NCD services under **'**Makkalai Thedi Maruthuvam'.

a. Retinopathy Screening

Diabetic retinopathy is a major cause of preventable blindness among persons with

diabetes. To facilitate early diagnosis and timely management, retinopathy screening is carried out through 38 Tele-Vision Care Centres with integrated referral services.

As of 31 March 2026**,** 1.08 lakh individuals were screened, and 2,131 retinal abnormalities were identified. During the year 2026–27 (April and May), an additional 12,410 individuals were screened and 154 retinal abnormalities were detected.

Among the beneficiaries identified with retinal abnormalities during 2026–27 (April and May), 17 were advised laser therapy and 40 were advised intravitreal injections, facilitating timely intervention and preventing avoidable visual impairment.

b. Nephropathy Screening

Screening for diabetic nephropathy has strengthened the early identification and management of renal complications. As of 31st March 2026, 2,278 Stage 4 CKD patients and 1,030 Stage 5 CKD patients were identified and linked to appropriate treatment and follow-up.

c. Diabetic Foot Care and Prevention of Disability

Diabetic neuropathy and foot complications are major causes of disability and lower-limb amputations. To address this, diabetic foot screening and follow-up services have been integrated into routine NCD care.

Up to May 2026**,** 3.58 lakh beneficiaries were screened, of whom 3,058 beneficiaries were referred for further management and 2,871 beneficiaries were placed under treatment and follow-up.

Diabetic Foot Screening Clinics established in all Primary Health Centres have also provided follow-up care to 57,158 individuals at risk of neuropathy-related complications, thereby contributing to the prevention of disability and reduction in amputations.

These interventions have strengthened the early detection and management of diabetic complications, contributing to improved quality of life and reduced disability among persons living with diabetes.

The Government of Tamil Nadu will continue to strengthen annual screening and early detection

of diabetic complications, alongside a State-wide Diabetic Foot Care Programme to prevent amputations. By integrating these services across Primary Health Centres, the State aims to reduce delayed diagnosis, improve survival and safeguard the quality of life of around 80 lakh people living with diabetes.

6.10.4. Community-Based Organised Cancer Screening Programme

Cancer is one of the leading causes of morbidity and mortality and contributes substantially to the burden of non-communicable diseases. According to the NSSO 80th Round, cancer accounts for 4.0% of all hospitalised cases and imposes considerable financial hardship on affected households. In addition, ICMR–National Cancer Registry Programme (NCRP) estimates project the cancer burden in India to increase from 1.39 million cases in 2020 to 1.57 million cases by 2025, underscoring the importance of early detection and timely treatment.

Recognising the growing burden of cancer, the Government of Tamil Nadu has implemented the Community-Based Organised Cancer Screening

Programme across all 38 districts for the early detection of oral, breast, and cervical cancers. The programme facilitates timely diagnosis, referral and initiation of treatment, thereby improving survival and quality of life.

During 2025–2026, 95.88 lakh invitations were issued under OCS and 43.86 lakh individuals were screened for Oral cancer. In addition, 33.3 lakh women and 43.5 lakh women have undergone screening for Cervical and Breast Cancer respectively across Government health facilities leading to identification of 2,446 confirmed cancer cases.

These interventions have strengthened the early detection and management of common cancers, contributing to improved survival and enhanced quality of life among affected individuals.

The Government of Tamil Nadu has mandated notification of all confirmed cancer cases and is strengthening cancer control through community and opportunistic screening, tobacco cessation, health education and comprehensive treatment services. Early detection and prevention of breast and cervical cancers are promoted

through breast self-examination awareness, HPV vaccination and Pap smear screening.

6.10.5. Idhayam Kaapom Thittam

Cardiovascular diseases (CVDs) are a major cause of premature mortality, disability, and increasing healthcare expenditure. According to the NSSO 80th Round, cardiac diseases accounted for 10.3 hospitalisations per 1,000 population in rural areas and 11.9 hospitalisations per 1,000 population in urban areas, reflecting the growing burden of cardiovascular diseases and the need for timely emergency cardiac care.

To strengthen the early diagnosis and emergency management of acute cardiac events, the Government of Tamil Nadu has implemented Idhayam Kaapom Thittam at Health Sub-Centres and Primary Health Centres. The programme facilitates the prompt identification and referral of individuals presenting with acute chest pain and provides immediate administration of cardiac loading-dose medications, including Aspirin 300 mg, Clopidogrel 300 mg and Atorvastatin 80 mg, thereby reducing mortality and long-term disability associated with acute cardiac events.

Since its inception up to March 2026, 23,627 beneficiaries have availed services under the programme. During the year 2026–27 (April and May), 1,061 individuals presenting with acute chest pain were reported, of whom 1,047 beneficiaries received cardiac loading-dose medications and were referred for higher-level care. Subsequently, 1,041 beneficiaries were diagnosed and placed under follow-up.

These interventions have strengthened timely emergency cardiac care and contributed to improved outcomes among patients with acute cardiovascular events.

The Government of Tamil Nadu will continue to strengthen the early detection of cardiovascular risk factors at the community level, expand emergency response capacity at Primary Health Centres, and integrate digital health technologies to facilitate seamless specialist consultation and referral. Continued emphasis is being placed on strengthening the capacity of frontline healthcare workers, ensuring the uninterrupted availability of essential cardiac medicines and reinforcing referral linkages with higher-level healthcare institutions.

6.10.6. Chronic Obstructive Pulmonary Disease (COPD) Screening Programme:

Chronic Obstructive Pulmonary Disease (COPD) is a major public health challenge driven by tobacco use, indoor air pollution and an ageing population. According to NFHS-6 (2023–24), tobacco use in Tamil Nadu was reported among 17.7% of men and 4.8% of women, while the State's growing elderly population further contributes to the burden of chronic respiratory diseases. COPD is a significant cause of morbidity, mortality and Disability-Adjusted Life Years (DALYs), underscoring the need for early detection and timely management.

Strengthening screening, raising public awareness, and ensuring the prompt initiation of treatment are essential for improving survival, reducing financial hardship and enhancing the quality of life of affected individuals.

The Government of Tamil Nadu introduced the Chronic Obstructive Pulmonary Disease (COPD) Screening Programme during 2025–26 to facilitate the early identification and management of chronic respiratory diseases. The programme incorporates

symptom-based screening, spirometry assessment and linkage with Tobacco Cessation Centres.

As of 31 March 2026, 9.58 lakh individuals were screened for COPD, of whom 16,956 individuals were referred for further evaluation and management. In addition, 1,713 tobacco users were linked to Tobacco Cessation Centres for cessation support.

During the year 2026–27 (April and May), 1.82 lakh individuals were screened for COPD, of whom 2,032 beneficiaries were referred for further evaluation and management. In addition, 100 tobacco users were linked to Tobacco Cessation Centres for cessation support.

These interventions have strengthened the early detection and management of chronic respiratory diseases while promoting tobacco cessation, thereby contributing to improved respiratory health and quality of life.

The Government of Tamil Nadu will continue to strengthen community-based COPD screening, enhance respiratory care services, expand tobacco cessation and pulmonary rehabilitation services

and promote measures to reduce exposure to air pollution and other respiratory risk factors.

6.10.7. Metabolic Dysfunction-Associated Steatotic Liver Disease (MASLD)

Metabolic Dysfunction-Associated Steatotic Liver Disease (MASLD), previously known as Non-Alcoholic Fatty Liver Disease (NAFLD), is emerging as a major public health concern due to the increasing prevalence of Obesity, Diabetes and Sedentary lifestyles. If left undiagnosed, MASLD can progress to cirrhosis and hepatocellular carcinoma and is also associated with an increased risk of cardiovascular diseases and diabetes.

In line with the Operational Guidelines issued by the Ministry of Health and Family Welfare, Government of India, under the National Programme for Prevention and Control of Non-Communicable Diseases (NP-NCD), Tamil Nadu is carrying out MASLD Screening to facilitate the early diagnosis and management of liver disease.

As of 31st March 2026, 4,79,866 individuals were screened, and 4,294 individuals were referred for further evaluation and treatment. During the year 2026–27 (April and May), 98,890 individuals

were screened, of whom 1,080 individuals were identified as suspected cases and referred for further management. Subsequently, 77 beneficiaries were placed under treatment and follow-up.

The Government of Tamil Nadu will continue to strengthen MASLD prevention and control by promoting healthy lifestyles, strengthening specialist referral pathways and developing disease registries. These measures will help prevent progression to cirrhosis, liver cancer and cardiovascular diseases, thereby reducing long-term Disability-Adjusted Life Years (DALYs) and healthcare costs.

6.10.8. Workplace-Based NCD Intervention Programme

Recognising that a large proportion of the working population may be missed through routine community-based screening, the Government of Tamil Nadu launched the Workplace-Based NCD Intervention Programme in 2024 to provide screening for Hypertension, Diabetes and Lifestyle-related risk factors at workplaces, including industrial establishments employing migrant workers. The programme aims to promote the

early detection of non-communicable diseases, facilitate timely intervention, and improve workforce health and productivity.

As of 31st March 2026, approximately 5.17 lakh employees across 499 industries had been screened and identified 24,088 individuals with hypertension, 17,374 individuals with diabetes mellitus and 10,098 individuals with both hypertension and diabetes.

These interventions have strengthened the early detection of non-communicable diseases among the working population and contributed to improving employee health and productivity.

The Government of Tamil Nadu will continue to strengthen the Workplace-Based NCD Intervention Programme by expanding regular workplace screening to cover more employees, strengthening referral linkages, integrating digital health records for effective follow-up and promoting healthy lifestyle practices.

6.10.9. Health Walk (Nadappom Nalam Peruvom)

According to NFHS-6 (2023–24), 44.2% of women and 38.8% of men aged 15–49 years in Tamil Nadu have a Body Mass Index (BMI) greater

than 25 kg/m², indicating a high prevalence of overweight and obesity. Elevated BMI is associated with an increased risk of non-communicable diseases, including cardiovascular diseases, diabetes, hypertension and other obesity-related health conditions. Regular physical activity plays a vital role in the prevention and control of non-communicable diseases by improving cardiovascular health, maintaining a healthy body weight and promoting overall well-being.

To encourage healthy lifestyles and increase physical activity among the population, the Government of Tamil Nadu launched the Nadappom Nalam Peruvom initiative and established dedicated Health Walk pathways across all districts of the State.

As of 31st March 2026, 1.41 lakh individuals participated in organised walking activities and 1.14 lakh individuals underwent health screening through associated health camps. During the year 2026–27 (April and May), 11,616 individuals participated in organised walking activities, while 9,509 individuals availed health screening services.

These initiatives have promoted behavioural change and the adoption of healthy lifestyles, thereby contributing to the prevention and control of non-communicable diseases.

The initiative will be further strengthened by integrating health education, nutritional counselling and stress management into Health Walk activities, making the programme more comprehensive and effective in promoting healthy lifestyles and reducing the burden of non-communicable diseases.

6.10.10. Dental and Oral Health Care Services

Oral diseases are a major public health concern, as they adversely affect nutrition, speech and quality of life. Untreated dental conditions may progress to infections, tooth loss and systemic complications. Early detection and timely treatment are essential to prevent complications and reduce the long-term healthcare burden.

To strengthen oral healthcare delivery, the Government of Tamil Nadu has established 395 Dental Units in Primary Health Centres across the State. These units are staffed by Dental Surgeons and Dental Assistants and are equipped

with essential dental infrastructure to provide quality oral healthcare services.

The Dental Units provide comprehensive oral healthcare services, including dental screening, treatment of common dental conditions, oral health education, oral cancer screening and referral services for specialised care.

Dental care services have demonstrated strong utilisation. During the year 2025–26, a total of 28.38 lakh beneficiaries accessed dental services, while 4.08 lakh beneficiaries availed services during the year 2026–27 (April and May), reflecting sustained demand for oral healthcare services.

The establishment of Dental Units has improved access to affordable oral healthcare, enhanced awareness of oral hygiene practices, enabled the early detection of dental diseases and reduced the burden of untreated oral conditions in the community.

The Government of Tamil Nadu will continue to strengthen oral healthcare services by reinforcing the National Oral Health Programme, expanding school-based oral health activities,

strengthening community dental services, integrating oral cancer screening with tobacco control programmes, linking dental services with Mobile Medical Units, promoting digital dental records and enhancing public awareness through targeted oral health promotion campaigns.

6.10.11. National Tobacco Control Programme (NTCP)

Tobacco use imposes a substantial health and economic burden by causing cancers, cardiovascular diseases, Chronic Obstructive Pulmonary Disease (COPD) and adverse pregnancy outcomes, while also exposing non-smokers to the harmful effects of second-hand smoke.

The National Tobacco Control Programme (NTCP) has been implemented across all districts of Tamil Nadu since 2007 to reduce tobacco consumption, prevent the initiation of tobacco use, and protect the population from tobacco-related diseases. According to NFHS-6 (2023–24), tobacco use remains a public health concern, with 17.7% of men and 4.8% of women aged 15 years and above reporting the use of tobacco in any form.

The programme is implemented under the Cigarettes and Other Tobacco Products Act (COTPA), 2003, which prohibits smoking in public places, restricts tobacco advertisements, prohibits the sale of tobacco products to minors and within 100 yards of educational institutions and mandates statutory health warnings on tobacco products.

This Programme adopts a comprehensive strategy encompassing law enforcement, public awareness campaigns, behaviour change communication, school and college-based interventions, community mobilisation and tobacco cessation services. These interventions have strengthened tobacco control measures, reduced exposure to tobacco and promoted healthier, tobacco-free environments across the State.

As of 31st March 2026, Tamil Nadu has declared 49,397 schools**,** 2,248 colleges and 3,920 villages as tobacco-free.

The Government of Tamil Nadu will continue to strengthen tobacco control by expanding tobacco cessation services, integrating oral cancer screening with tobacco control activities, enhancing community-level awareness, intensifying youth-

focused prevention initiatives and strengthening enforcement of the provisions of the COTPA, 2003.

6.11. Preventive Health Camps

Emphasizing that prevention is better than cure, the Government of Tamil Nadu has initiated Preventive Health Camps to facilitate the early detection and timely management of diseases. Early diagnosis before the development of complications helps preserve productive life years and improve population health outcomes.

Preventive Health Camps are conducted to promote the early detection and timely management of communicable and Non-Communicable Diseases, thereby reducing Complications, Disability and Healthcare costs. The initiative focuses on improving access to preventive healthcare services, particularly for underserved and vulnerable populations.

The camps provide comprehensive healthcare services, including health screening, specialist consultations, diagnostic investigations, health education, treatment initiation and referral to higher-level healthcare facilities for advanced management.

Implementation is supported by multidisciplinary teams comprising Medical Officers, Specialists, Staff Nurses, Pharmacists, Laboratory Technicians and Public Health personnel, with service delivery facilitated through Mobile Medical Units, laboratory support and well-established referral linkages.

During the year 2025–26, a total of 1,256 Preventive Health Camps were conducted, benefiting approximately 18.53 lakh individuals across the State.

The programme has strengthened the early detection of priority health conditions, improved health-seeking behaviour, enhanced access to preventive healthcare services and contributed to the State's progress towards Universal Health Coverage.

The Government of Tamil Nadu will continue to strengthen the Preventive Health Camps programme by enhancing service quality, strengthening referral and follow-up mechanisms, and expanding access to preventive healthcare services, thereby promoting early diagnosis and improving population health outcomes.

6.12. Mobile Medical Units (MMUs)

Tamil Nadu's healthcare system faces the challenge of delivering equitable healthcare services to tribal, hard-to-reach areas and urban underserved, vulnerable and marginalized populations. To improve access to essential healthcare services in these areas, reduce the need for long-distance travel to health facilities and minimise out-of-pocket expenditure, the Government of Tamil Nadu has established the Hospital on Wheels programme through Mobile Medical Units (MMUs).

To ensure equitable access to healthcare for remote and underserved populations, Tamil Nadu operates 477 Mobile Medical Units (MMUs), including Tribal Mobile Medical Units (TMMUs) and Labour Mobile Medical Units (LMMUs). These units provide health screening, essential laboratory services, medicines and referral support closer to their communities.

Each Mobile Medical Unit is staffed by a Medical Officer, Staff Nurse, Laboratory Technician and Driver, ensuring the delivery of comprehensive primary healthcare services at the community level.

The Mobile Medical Units provide outpatient consultations, basic laboratory investigations, maternal and child healthcare services, screening for communicable and non-communicable diseases, distribution of essential medicines, health education and referral services.

During the year 2025–26, a total of 2.29 lakh medical camps were conducted, benefiting approximately 2.01 crore beneficiaries. During the year 2026–27 (April–May), 26,385 outreach camps were conducted, benefiting approximately 20 lakh beneficiaries, reflecting sustained outreach and continued utilisation of Mobile Medical Unit services.

The Government of Tamil Nadu will continue to strengthen Mobile Medical Unit services by expanding geographical coverage, strengthening diagnostic and laboratory services, integrating digital health systems, ensuring the uninterrupted supply of essential medicines and enhancing community participation, thereby reinforcing MMUs as a key pillar of accessible and equitable healthcare delivery across the State.

6.13. National Iodine Deficiency Disorders Control Programme (NIDDCP)

Iodine is an essential micronutrient required for thyroid hormone production, normal growth, brain development, and neurological function. Iodine Deficiency Disorders (IDDs), including goitre, impaired cognitive development, learning disabilities and adverse pregnancy outcomes, remain a preventable public health concern. Universal consumption of adequately iodized salt is the most effective strategy for the prevention and control of IDDs.

The National Iodine Deficiency Disorders Control Programme (NIDDCP) aims to ensure adequate iodine intake across the population and prevent iodine deficiency-related health and developmental consequences. According to NFHS-6, 92.8% of households in Tamil Nadu consume iodized salt, indicating the necessity for sustained efforts to achieve universal coverage.

The key activities of the programme are as follows:

- Ensuring the use of iodized salt by all households.

- Monitoring the iodine content in salt.
- Conducting goitre surveys among school children.
- Assessing urinary iodine levels.
- Quality control and monitoring.
- Public awareness and behaviour change interventions.

During the year 2025–26, a total of 8,091 salt samples were tested, of which 85% met the prescribed iodine standard.

During the year 2026–27 (April and May), 1,457 salt samples were tested, with 92% containing iodine levels above the recommended standard of 15 ppm, indicating improved compliance with iodization standards and enhanced product quality.

The Government of Tamil Nadu will continue to strengthen the National Iodine Deficiency Disorders Control Programme by promoting the universal use of adequately iodized salt, strengthening real-time monitoring of non-compliant salt brands, conducting periodic school-based goitre surveys, establishing sentinel

sites for urinary iodine surveillance, strengthening collaboration with Food Safety authorities to prevent the distribution of substandard salt and enhancing public awareness on the importance of consuming adequately iodized salt.

6.14. Communicable Diseases Control

Communicable diseases continue to pose a significant public health challenge due to their impact on morbidity, mortality and economic productivity. Diseases such as tuberculosis, Leprosy, HIV-AIDS, dengue, malaria, influenza, diarrhoeal diseases, COVID-19 and other vaccine-preventable diseases continue to affect vulnerable populations and require sustained surveillance and comprehensive public health interventions to prevent transmission and outbreaks. Factors such as poor sanitation, unsafe drinking water, overcrowding and limited access to healthcare further increase the risk of disease transmission and outbreaks.

The programme aims to prevent, control and reduce the burden of communicable diseases through early detection, timely treatment, effective

surveillance, outbreak prevention and a coordinated public health response.

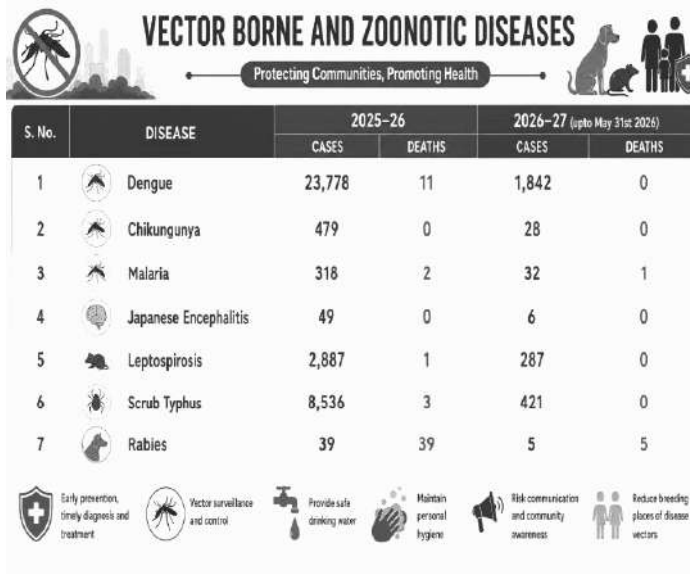
Major programme activities include disease surveillance and outbreak detection, laboratory diagnosis and confirmation, rapid response to public health emergencies, vector control measures, immunization support activities, risk communication and health education and monitoring and containment of epidemic-prone diseases.

The programme is implemented through the Integrated Disease Surveillance Programme (IDSP) using the Integrated Health Information Platform (IHIP). Surveillance activities are supported by a network of 38 District Integrated Public Health Laboratories and one State Public Health Laboratory, along with trained epidemiologists, microbiologists, data managers and field-level public health personnel.

The programme benefits the entire population of Tamil Nadu by strengthening disease surveillance, reducing disease transmission, preventing outbreaks and ensuring timely public

health action during disease outbreaks and other public health emergencies.

Table 6.4: Vector borne and zoonotic diseases



S. No.	DISEASE	2025–26		2026–27 (upto May 31st 2026)	
		CASES	DEATHS	CASES	DEATHS
1	Dengue	23,778	11	1,842	0
2	Chikungunya	479	0	28	0
3	Malaria	318	2	32	1
4	Japanese Encephalitis	49	0	6	0
5	Leptospirosis	2,887	1	287	0
6	Scrub Typhus	8,536	3	421	0
7	Rabies	39	39	5	5

The Government of Tamil Nadu will continue to strengthen communicable disease prevention and control through intensified vector control measures, including anti-larval activities and fogging, ensuring safe drinking water through chlorination and regular monitoring of residual chlorine levels, periodic cleaning of Overhead Tanks (OHTs), strengthening disease surveillance, enhancing Information, Education and

Communication (IEC) activities and providing essential medicines and laboratory services through medical camps. These measures will further strengthen outbreak preparedness, improve early detection and response and reduce the burden of communicable diseases across the State.

6.15. Notifiable Diseases under the Tamil Nadu Public Health Act, 1939

Certain communicable diseases have been designated as notifiable diseases owing to their potential for rapid transmission and significant public health impact. The Tamil Nadu Public Health Act, 1939 mandates the notification of specified communicable diseases to public health authorities to enable timely surveillance, epidemiological investigation and implementation of appropriate control measures. The programme aims to ensure the timely reporting of notified diseases, strengthen disease surveillance and outbreak detection, facilitate rapid investigation and implementation of control measures and support evidence-based public health action and epidemic preparedness.

Table 6.5: List of Notifiable Diseases

S.No.	Name of the Disease
1.	Cerebrospinal Fever
2.	Chickenpox
3.	Cholera
4.	Diphtheria
5.	Leprosy
6.	Measles
7.	Plague
8.	Rabies
9.	Scarlet Fever
10.	Smallpox*
11.	Typhus
12.	Yellow Fever
13.	Enteric Fever
14.	Tuberculosis
15.	Whooping Cough
16.	Infectious Hepatitis (Hep. A, B, C, D & E)
17.	Epidemic Influenza (Including Swine Flu (AH1N1), Avian Influenza, Influenza Like Illness)
18.	Viral Encephalitis

S.No.	Name of the Disease
19.	Haemorrhagic Fever (Including Dengue, KFD etc.)
20.	Malaria
21.	Poliomyelitis
22.	Tetanus
23.	HIV/AIDS
24.	Chikungunya
25.	Leptospirosis
26.	Cancer
27.	COVID-19
28.	Mucormycosis
29.	Snake Bite

Although smallpox was eradicated globally in 1980, it continues to remain a notifiable disease under this Act.

6.16. Universal Health Coverage (UHC)

6.16.1. Universal Health Coverage through Health and Wellness Centres

Tamil Nadu has been a pioneer in advancing Universal Health Coverage (UHC) by ensuring access to comprehensive primary healthcare

services without financial hardship for all. Since 2017, the State has expanded Health and Wellness Centres (HWCs) across rural and urban areas, comprising

- 4,848 Health Sub-Centres - Health & Wealth Centre
- 1,407 Additional Primary Health Centres
- 486 Urban Primary Health Centres
- 708 Urban Health and Wellness Centres
- 650 AYUSH HWCs

These facilities provide comprehensive primary healthcare services closer to communities, reducing out-of-pocket expenditure and strengthening access to preventive, promotive, curative, rehabilitative and palliative healthcare services.

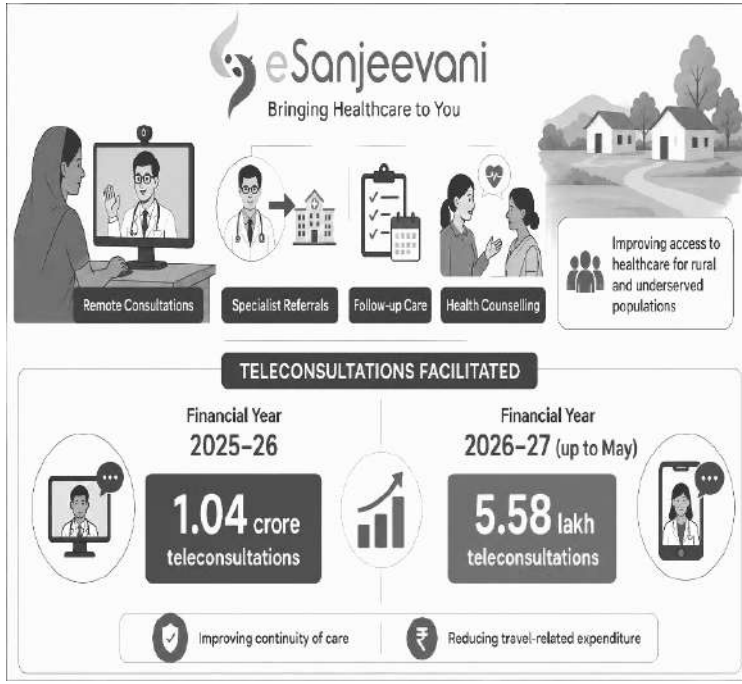
During the year 2025–26, HWC–Health Sub-Centres recorded 5.55 crore outpatient visits, while 75 lakh outpatient visits were recorded during the year 2026–27 (April and May).

The Government of Tamil Nadu will continue to strengthen Universal Health Coverage through

digital innovations, expansion of service delivery at Health and Wellness Centres, and improved access to comprehensive primary healthcare services.

6.16.2. e-Sanjeevani Telemedicine Services

The e-Sanjeevani telemedicine platform enhances access to healthcare by providing remote consultations, specialist referrals, follow-up care, and health counselling, particularly for rural and underserved populations.



During the year 2025–26, the platform facilitated 1.04 crore teleconsultations, while 5.58 lakh teleconsultations were provided during the year 2026–27 (April and May), improving continuity of care and reducing travel-related expenditure.

6.16.3. Community Participation through Makkal Nalavazhvu Kuzhu (MNK)

Community participation is strengthened through Jan Arogya Samitis (JAS), known in Tamil Nadu as Makkal Nalavazhvu Kuzhu (MNK). Functioning at Health and Wellness Centres, Health Sub-Centres, Primary Health Centres, Urban Primary Health Centres and Urban Health and Wellness Centres,

- these committees promote community engagement
- local health planning
- social accountability
- community monitoring of healthcare services
- Establishing a link between the community and the healthcare system

6.16.4. Wellness Activities

Health promotion and disease prevention are strengthened through regular wellness activities conducted at Health and Wellness Centres.



These include

- ❖ yoga sessions
- ❖ physical activity programmes
- ❖ walkathons
- ❖ cyclathons

- ❖ health education on nutrition
- ❖ mental well-being
- ❖ tobacco and alcohol cessation
- ❖ healthy lifestyle

During the year 2025–26, 15.62 lakh wellness sessions were conducted, benefiting 4.14 crore participants. During the year 2026–27 (April and May), 2.66 lakh wellness sessions were conducted, benefiting 64.02 lakh individuals.

6.16.5. Health Melas

To improve access to healthcare services, Health Melas are being conducted every Saturday across the State.

The following services are provided through these Health Melas:

- ❖ Disease screening services
- ❖ Laboratory diagnostic facilities
- ❖ Specialist medical consultations
- ❖ Referrals for higher-level treatment
- ❖ Follow-up care monitoring
- ❖ Health awareness and education activities

During the year 2025–26, 96,649 Health Melas were conducted, benefiting approximately 168 lakh individuals. During the year 2026–27 (April and May), 14,651 Health Melas were conducted, benefiting 23.73 lakh individuals.

6.17. Training and Capacity Building of Public Health Workforce

A skilled public health workforce is fundamental to effective healthcare delivery, disease prevention, disease surveillance and the achievement of Universal Health Coverage. The Directorate of Public Health and Preventive Medicine places significant emphasis on continuous capacity building through pre-service and in-service training programmes for public health personnel.

Pre-service and in-service training programmes are conducted for Health Officers, Medical Officers, Staff Nurses and other paramedical personnel through eight Regional Training Institutes (RTIs) and training programmes organised under the National Health Mission.

The Directorate also regulates and administers training programmes for frontline public health personnel, including Auxiliary Nurse Midwives (ANMs) and Health Inspectors (HIs), ensuring uniform standards in public health education, training and professional practice.

The Directorate oversees 86 ANM Training Schools (11 Government and 75 Private) and 56 Health Inspector Training Institutions. It is responsible for curriculum implementation, accreditation, conducting examinations and issuing certificates, quality assurance and competency-based skill development.

Training programmes focus on key public health domains, including maternal and child health, immunization, sanitation, disease surveillance, communicable disease control and other emerging public health priorities.

These initiatives strengthen the availability of a competent and skilled frontline public health workforce, thereby improving the quality of primary healthcare services and supporting better public health outcomes across Tamil Nadu.

6.18. Civil App – Digital Infrastructure Monitoring System

The Civil App is a digital infrastructure monitoring platform developed by the Directorate of Public Health and Preventive Medicine in collaboration with the Tamil Nadu e-Governance Agency (TNeGA). The platform has been developed to strengthen transparency, accountability and efficiency in the implementation of healthcare infrastructure projects across the State.

The platform provides the following features through web-based dashboards and a mobile application:

- Real-time monitoring of construction activities.
- Stage-wise recording and tracking of work progress.
- Immediate identification of delays in project implementation.
- Expedited corrective actions to ensure timely completion of works.

Currently, 3,462 healthcare infrastructure projects, covering the construction, renovation and modernisation of healthcare facilities are being monitored through the Civil App.

The platform has significantly strengthened project tracking, supervision, and evidence-based decision-making, resulting in improved project implementation, timely completion of infrastructure works and enhanced governance. These improvements contribute to the availability of quality healthcare infrastructure and improved healthcare service delivery across Tamil Nadu.

6.19. Civil Registration System (CRS) and Medical Certification of Cause of Death (MCCD)

6.19.1 Civil Registration System

The Civil Registration System (CRS) ensures the universal, continuous and compulsory registration of births, deaths and stillbirths. It provides legal identity, facilitates access to government services and generates reliable birth and mortality statistics for planning, policy formulation and public health decision-making.

The Registration of Births and Deaths Act, 1969, as amended by the Registration of Births and Deaths (Amendment) Act, 2023, together with the Tamil Nadu Registration of Births and Deaths Rules, 2025 (effective from 31st July 2025), is currently being implemented in Tamil Nadu. The Government of Tamil Nadu has modernised the registration system through digital platforms, improved transparency and enhanced citizen access to services.

The Director of Public Health and Preventive Medicine is the Chief Registrar of Births and Deaths in Tamil Nadu. The District Revenue Officer functions as the District Registrar and the District Health Officer as the Additional District Registrar.

Birth certificates are issued free of cost for all children whose births are registered within 21 days of delivery. Further, Birth events occurring in Government Health institutions are registered immediately and birth certificates are issued before discharge following delivery.

Similarly, death and stillbirth events shall be registered within 21 days from the date of occurrence. Events reported after 21 days may be registered under the delayed registration provisions of Section 13 of the Act, subject to the prescribed fee and applicable procedures.

The Government has also extended the timeline for inclusion of names in birth certificates for registrations exceeding 15 years up to 26th September 2026.

Table 6.6: Details of Births and Deaths in Tamil Nadu

Year	Births (in lakh)	Deaths (in lakh)
2020	9.39	6.91
2021	9.12	8.74
2022	9.36	6.96
2023	9.02	6.93
2024	8.47	6.95
2025	8.02	6.89

6.19.2. Medical Certification of Cause of Death (MCCD) and e-Mortality Software

Medical Certification of Cause of Death (MCCD), provided under Sections 10(2) and 10(3) of the Registration of Births and Deaths Act, 1969, is recorded in Form 4 for institutional deaths and Form 4A for non-institutional deaths. It is the primary source of cause-specific mortality data for public health planning and evidence-based policy formulation. To improve the quality and timeliness of mortality data, the web-based e-Mortality (e-Mor) module has been implemented.

Tamil Nadu has achieved 100% death registration since 2012. However, in 2024, medically certified causes of death were available for only 41% of registered deaths, highlighting the need to strengthen mortality surveillance.

To address this gap, the District Civil Registration System (CRS) Approach was piloted in Karur and Krishnagiri districts. The initiative integrates the WHO-recommended Verbal Autopsy methodology with digital technology to ascertain causes of community deaths lacking medical certification.

The pilot increased cause-of-death coverage from 41% to 98% in the pilot districts and reduced ill-defined causes of death from 46% to 13%, demonstrating substantial improvement in the completeness and quality of mortality data.

Based on these outcomes, the District CRS Approach, including Verbal Autopsy, is proposed to be expanded across Tamil Nadu in a phased manner by leveraging existing district-level infrastructure and strengthening technical capacity to generate high-quality mortality data for evidence-based planning, policy formulation and programme implementation.

CHAPTER - 7

FAMILY WELFARE

7.1. Introduction

The programmes implemented over the years have brought success taking the State to a new demographic frontier. As the paradigm shifted, three decades of below-replacement fertility has resulted in a cohort of women entering the reproductive ages itself contracting - the annual registered births have declined steadily. When viewed in combination with the share of the population aged 60 years and above being about 16 per cent at present and projected to approach one in five by 2036, if the fertility age and replacement rate is left unattended, this trajectory leads to an ageing society, a shrinking workforce and, in time, absolute population decline.

The Government's primary challenge is therefore building sustainability of the State's demographic balance while protecting every gain made in maternal and child health. Tamil Nadu, which led the nation through the demographic transition, will now lead it through the post-transition era.

7.1.1. Vision

The Government accordingly reorients the Family Welfare Programme from population control to Family Well-being. The vision of the Government is that every family in Tamil Nadu attains the family size it desires wherein every pregnancy is wanted, every birth is safe and every child born is enabled to thrive.

In this vision, the couple unable to conceive has as strong a claim upon the public health system as the couple seeking to space or to limit births. Tamil Nadu will be the first State to integrate the full continuum of infertility care into its Family Welfare Programme — preconception counselling and prevention at Health and Wellness Centres, standardised diagnostic work-up, ovulation induction and Intra-Uterine Insemination at District and Headquarters Hospitals, Medical colleges and Assisted Reproductive Technology centres, including In Vitro Fertilisation, in selected Government Medical Colleges, registered under the Assisted Reproductive Technology (Regulation) Act, 2021.

The Government will ensure that this reorientation carries no element of compulsion and

the full basket of contraceptive choice, spacing services and safe abortion care will continue undiminished and no incentive or disincentive shall ever be linked to the number of children.

Further, even as aggregate fertility declines, the composition of the pregnant segment of population and births will be continuously monitored. Teenage pregnancies, though declining, persist in identified pockets and carry elevated risks of anaemia, low birth weight and obstetric complications, besides curtailing the education and life chances of the adolescent girl. Every such pregnancy will be line-listed and tracked as high-risk through PICME, and prevented at source through convergence with the School Education and Social Welfare Departments, adolescent health services and firm enforcement of the Prohibition of Child Marriage Act, 2006.

In equal measure, high order births - births of order three and above, again a diminishing share of total births, contribute disproportionately to maternal deaths, as the State's maternal death audits confirm, on account of short birth intervals, anaemia and the risks of multiparity. The high

order births will be flagged as high-risk in PICME and institutional delivery under specialist care followed by postpartum family planning counselling would be provided in a focussed manner. The Maternal Mortality Ratio of the State, at 25 (SRS 2022-24), shall be driven steadily lower: the same system that helps couples conceive shall ensure that no adolescent becomes a mother, and that no mother is lost to a preventable high-risk birth.

7.2. Objectives

The objectives of the reoriented Family Welfare Programme are:

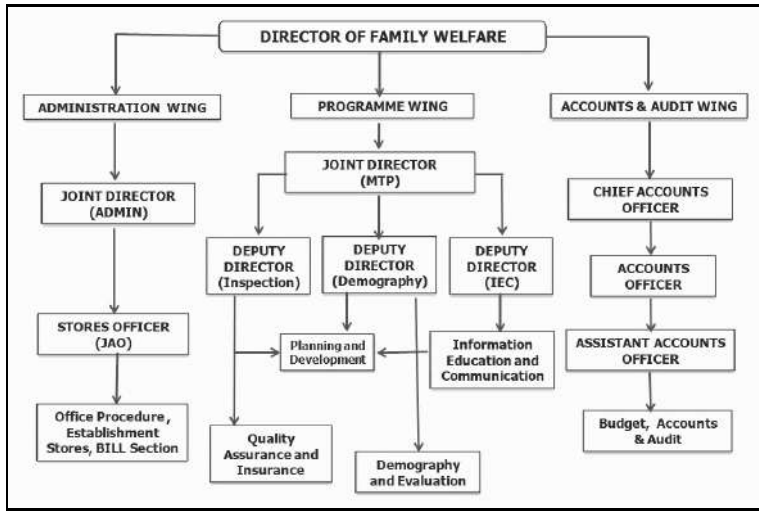
- ❖ to arrest the drift towards population decline by enabling every couple to achieve its desired family size — through facilitation, never through compulsion;
- ❖ to establish comprehensive infertility care as a public service across a tiered continuum from the Health and Wellness Centre to the Government Medical College ART centre, regulated under the Assisted Reproductive Technology (Regulation) Act, 2021;

- ❖ to preserve undiminished the full basket of contraceptive choice and safe abortion services;
- ❖ to eliminate teenage pregnancy and child marriage through inter-departmental convergence and strengthened adolescent health services;
- ❖ to reduce the contribution of high order and high-risk births to maternal mortality through universal PICME-based tracking and one hundred per cent specialist-attended institutional delivery of high-risk mothers;
- ❖ to consolidate the Dr. Muthulakshmi Reddy Maternity Benefit Scheme, the Thaimaaman Thanga Mothiram Thittam, nutrition support and childcare services into a single architecture of support for the young family

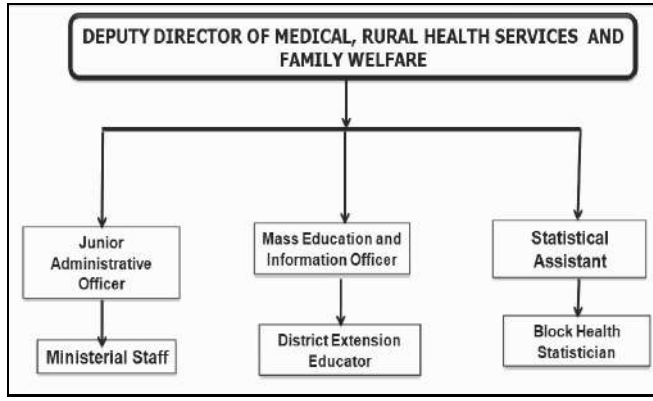
A total number of 2,621 posts have been sanctioned under various units in the State for the implementation of family welfare programmes.

7.3. Family Welfare - Administrative Structure

i). Directorate of Family Welfare – Chennai



ii). District Family Welfare Bureau



7.4. Functions of Directorate of Family Welfare

The Directorate of Family Welfare is the Nodal Directorate coordinating with Directorates of Medical Education and Research, Public Health and Preventive Medicine, Medical and Rural Health Services, Medical and Rural Health Services (ESI), Local Bodies, Non-Government Organisations and Private Sector for evaluation and monitoring the activities of the family welfare programme for successful implementation and to achieve the objectives through:

- Ensuring the standards of family welfare sterilizations quality through various committees as per the Guidelines of Government of India and Hon'ble Supreme court Directives.
- IEC awareness through Mass Media, Mid-media for Behavioral Change Communication (BCC)
- Sensitization of available Family Welfare services, newer contraceptives and Comprehensive Abortion Care Services to the stakeholders.

- ❖ Ensuring Home Delivery of contraceptives (HDC) and promotion of newer contraceptives

- ❖ Updating of Family Welfare reports in the National Web Portal HMIS - Integrated Health Information Portal (IHIP).

- ❖ Family Welfare commodity Management through Family Planning-Logistics Management Information System (DVDMS -FPLMIS).

- ❖ Imparting trainings to service providers for Family Planning temporary, limiting methods and Comprehensive abortion care (CAC).

- ❖ Approval of private facilities for Medical Termination of Pregnancy and Sterilization services through District level Committee.

- ❖ Monitoring and evaluating the impact of Family Welfare Programme through survey / studies and submission of reports to the Government.

7.5. Programmes and Services

I. Family Welfare Programme

The following temporary and limiting methods of Family Welfare services are provided in all public health facilities.

a. Temporary spacing methods

<table>
<tr><th>Spacing Methods</th><th>Type</th><th>Objective</th><th>Beneficiaries reached</th></tr>
<tr><td rowspan="2">Intra Uterine Contraceptive Device (IUCD)</td><td>Copper 'T' 380A (10 Years)</td><td rowspan="5">i) Healthy Timing and spacing of pregnancy for wellbeing of mother and child.

ii) Population stabilization.</td><td rowspan="5">Eligible women seeking temporary spacing methods</td></tr>
<tr><td>Copper 'T' 375 (5 Years)</td></tr>
<tr><td rowspan="2">Oral Contraceptive pills</td><td>Daily Pills – Mala N</td></tr>
<tr><td>Weekly pills - Chhaya (Centchroman Pills)</td></tr>
<tr><td colspan="2">Injectable contraceptive – Antara</td></tr>
<tr><td colspan="2">Male Condom – Nirodh</td><td rowspan="2">To avoid unintended pregnancies</td><td rowspan="2">Women in the reproductive age group</td></tr>
<tr><td colspan="2">Emergency Contraception - EC pills</td></tr>
</table>

b. Limiting Family Planning Methods

Male Sterilization	Conventional Vasectomy	To acquire desired family size on voluntary basis	Couples seeking permanent method
	No Scalpel Vasectomy (NSV)		
Female Sterilization	Mini Lap (PS/TAT), LSCS with Sterilization & Laparoscopic Sterilization (LS)		

c. Family Welfare - Newer Contraceptives

As the Expansion of Basket of Choices on Contraceptive methods, Newer Contraceptives Subdermal Single Rod Implant and Injection Antara (Subcutaneous) have been introduced in Selected Districts of the State as follows:

1. Subdermal Single Rod Contraceptive Implant in three Districts -
   a. Chennai
      i). Institute of Obstetrics and Gynaecology Government Hospital for women and children, Egmore.
      ii). Government Kasthuribai Gandhi hospital for women and children.

b. Dharmapuri

i). Government medical college & hospital, Dharmapuri.

ii). Government Head Quarter's hospital, Pennagaram.

c. Madurai

iii). Government Rajaji Medical college and hospital, Madurai

2. Subcutaneous Injectable Antara Contraceptive in Six Districts - Erode, Ramanathapuram, Coimbatore, Salem, Sivaganga and Virudhunagar.

The newer contraceptive services implemented in the above districts as a pilot project. For providing these newer contraceptive services, trainings and awareness activities have been conducted.

II. Comprehensive Abortion Care Programme (CAC)

Programmes	Type	Objective	Beneficiaries
Comprehensive Abortion Care Services	Medical method of Abortion (MMA)	a. Ensure safe, legal and accessible abortion services. b. To reduce maternal morbidity and mortality due to unsafe abortion.	Women requiring safe abortion services.

	Surgical method of Abortion (MVA / EVA / Others)	c. To provide comprehensive reproductive health services including counselling, abortion care, post abortion care and contraception.	

7.5.1. MTP ACT and Amendments

The Medical Termination of Pregnancy (MTP) Act was enacted in 1971 and amended in 2021. It provides the framework for provision of safe and legal abortion service (MTP services) in the State and Country.

- The Act allows termination of pregnancy by a Registered Medical Practitioner (RMP)
  - up to 20 weeks gestation for a broad range of indications
  - up to 24 weeks for special categories of women and
  - anytime during the pregnancy for substantial foetal abnormalities on approval by Medical Board.
- The MTP Act offers protection to a practitioner if she/he adheres to and fulfills all the requirements under the MTP Act.

- ❖ Up to 20 weeks of gestation one RMP opinion is required
- ❖ For 20 to 24 weeks of gestation opinion of two RMP is required.
- ❖ Beyond 24 weeks, opinion of Permanent Medical Board (PMB) is required.
- ❖ Name and other particulars of a woman whose pregnancy has been terminated shall not be revealed except to a person authorised by any law for the time being in force.
- ❖ The condition on failure of contraception has been extended to any woman or her partner.
- ❖ Confidentiality to be strictly maintained.

7.6. Permanent Medical Board in Government Medical College Hospitals for MTP

Permanent Medical Boards are constituted to access and approve / deny the pregnancy termination for substantial foetal abnormalities beyond 24 weeks of gestation. It provides multi-disciplinary expert evaluation and timely recommendations in complex pregnancy cases.

Composition of medical board

(a) A gynaecologist, (b) A paediatrician, (c) A Radiologist or sonologist (d) Such other number of members as may be notified in the official gazette by the State Government or Union territory as the case may be.

Table 7.1: Family Welfare Performance 2025-2026

Male Sterilization	Female Sterilization	Condom User	Oral Pills	IUCD
426	2,15,825	62,865	11,145	3,57,479

Antara	Chhaya	EC Pills	Implant	MTP
15197	11676	35129	1907	117639

III. Information, Education and Communication programme (IEC)

Activities		Objective	Beneficiaries reached
World Population Day	Every year 11th July in the State, District and Block level.	i). To promote informed family planning practices. ii). To increase awareness about population stabilizations and sustainable developments. iii). To encourage universal access to contraception and Comprehensive Abortion Care services.	i). Adolescents and young people ii). Men and Women seeking family planning services iii). Society

Activities		Objective	Beneficiaries reached
		iv). To promote gender equality and women's empowerment in reproductive decision making.	
Vasectomy Fortnight	Every year 21st November to 4th December in District and Block level.	i). To create awareness among men about NSV as a safe simple and effective method of permanent contraception ii). To promote greater male participation and shared responsibility in family planning. iii). To dispel myths, misconception and fears related to vasectomy	i). Eligible couples who have completed their desired family size. ii). Married men seeking permanent Contraception

7.7 Sensitizing Age at Marriage and Reduction of Teenage Pregnancies

Awareness meetings regarding age at Marriage and Prevention of teenage pregnancies were conducted along with the coordination of the other departments (Social welfare, child protection, Integrated Child Development Scheme (ICDS), education etc)., for adolescent girls in 400 (385-Rural + 15-Urban) Blocks during 2025-2026.

Table 7.2: Family Welfare Scheme for Sterilization and IUCD acceptors

Sl. No	Schemes	Objectives	Coverage	Outlay	Beneficiaries reached
A	Sterilization Services	Permanent contraception	Men and women opting for permanent methods	Compensatory for loss of wages Male Acceptor - Rs.1100 Female Acceptor - Rs.600	Sterilization acceptors
B	IUCD Programme	Long-acting reversible contraception	Post Partum & Post abortal mothers	PPIUCD & PAIUCD acceptor - Rs.300	Women accepting PPIUCD & PAIUCD

d. Family Welfare Indemnity Scheme (FPIS)

The Government of India introduced the Family Planning Indemnity Scheme with effect from 1st April, 2013. Under the scheme the quantum of compensation being paid to the families of the acceptors of sterilization in the event of death / failure / complication. The following are the insurance benefits for the family welfare sterilization acceptors and service providers

e. Committees and Boards under Directorate of Family Welfare

The following Committees are in existence to ensure the quality of Family Welfare and MTP services provided in the State

- ❖ State Level Quality Assurance Committee: Developing the quality assurance policy and guidelines for the State.
- ❖ State Family Planning Indemnity Sub-committee: Redress dispose and disperse claims.
- ❖ District Level Quality Assurance Committee: Dissemination of quality assurance policy and guidelines
- ❖ District Family Planning Indemnity Sub-committee: Process claims received from the clients and submit to State.
- ❖ District Level Committee: Approval of Private Nursing Homes for performing MTP services.

Table 7.3: Insurance Benefits under Family Planning Indemnity Scheme

Sec	Coverage	GOI Share	State Share	Total
SECTION I (A-D) For Beneficiaries				
IA	Death following sterilization in hospital or within Seven days from the date of discharge from the hospital	Rs.2.00 lakh	Rs.2.00 lakh	Rs.4.00 lakh
IB	Death following sterilization within eight to thirty days from the date of discharge from the hospital	Rs.50,000	Rs.50,000	Rs.1,00,000

IC	Failure of sterilization leading / not-leading to child birth	Rs.30,000	Rs.30,000	Rs.60,000
ID	Cost of treatment up to sixty days arising out of complication from the date of discharge	Actual cost not exceeding Rs.25,000	Actual cost not exceeding Rs.25,000	Actual cost not exceeding Rs.50,000
SECTION II: Empanelled Doctors under Public and Accredited Private/NGO Sector and Health Facilities under Public and Accredited Private/NGO Sector				
II	Indemnity insurance per doctor per facility but not more than four cases per doctor in a year.	Up to Rs.2.00lakh per case of litigation		Up to Rs.2.00 lakh per case of litigation

7.8. Sustainable Development Goals (SDG)

Sustainable Development Goals (SDGs) are directly linked to family welfare, particularly in Goal 3 (Good Health and Well-being) and Goal 5 (Gender Equality), which focus on reducing maternal mortality, providing universal access to sexual/reproductive health services, and empowerment of women.

Access to family planning is a key cross-sectoral investment impacting all 17 goals, including poverty reduction, education, and economic growth.

SDG 3: **Good Health and Well Being:** Focuses on reducing maternal mortality, neonatal / child

mortality, improving reproductive health services including Family Welfare services.

Indicator 3.7: To increase the modern Contraceptive Prevalence Rate to 70% by 2030.

SDG 5: Gender Equality: Focuses on ensuring universal access to sexual and reproductive health and reproductive rights.

Indicator 5.6: Aims to ensure universal access to sexual and reproductive health and rights. To Reduce Unmet Need to "Nil" by 2030.

Table 7.4: Indicators of Family Welfare Programme

S.No.	Indicators	India	Tamil Nadu
1.	Crude Birth Rate	18.3	11.6
2.	Crude Death Rate	6.4	6.8
3.	Total Fertility Rate	1.9	1.3
4.	Birth by Birth Interval (%)		
	< 24 Months	20.7	19.5
	Between 24-36 Months	25.8	27.3
	>36+Months	53.5	53.3
[Source : SRS – Sample Registration System-2024]			
5.	Unmet Need (%)	8.5	6.4
6.	Modern Contraceptive Prevalence Rate (mCPR%)	52.7	65.1
[Source: NFHS – National Family Health Survey VI – 2023-24]			

a. Total Fertility Rate

Total Fertility Rate (TFR) is the average number of children a woman would be expected to have during her lifetime if she experienced the current age-specific fertility rates throughout her reproductive years (typically ages 15–49).

b. Unmet Need in Family Planning

Women who want to avoid or delay pregnancy but are not using any contraceptive method are considered to have an unmet need for family planning.

Promoting temporary family planning methods involves educating individuals on spacing pregnancies using methods such as injectable contraceptives (Antara / DMPA), Chhaya pills, Implant and IUDs (Copper T) that allow for the return of fertility. Effective promotion includes counseling on the benefits of spacing, removing myths and increasing access to these methods through government programs.

7.9. Key Activities (Newer initiatives)

- Self-Care Kits containing condoms, Emergency Contraceptive Pills and Pregnancy test kits, installed in 9,213 Health Sub–Centre, Urban and Rural Health and

wellness Centres to improve privacy, safety and continuity of reproductive health services, especially for women and couples who may face barriers in accessing health facilities.

- Sugam Model is a multi-purpose display tool to create awareness on various Family Welfare methods. 537 SUGAM Models installed in CEmONC Centres, Block Primary Health Centres and Urban Community Health Centres in family planning OPD.

7.10. Conclusion

The Family Welfare Programme of Tamil Nadu thus stands at an inflexion that few programmes anywhere have been called upon to navigate: conceived to moderate fertility, it must now safeguard it. The couple longing for a child will find the public health system at their side, from the counselling room of the Health and Wellness Centre to the ART laboratory of the Government Medical College; the couple wishing to wait, to space or to stop will find every method available and their choice fully respected; the adolescent girl will be kept in school and away from the labour room; every high-risk and high order birth will be tracked to a safe, specialist-attended delivery; and the

young family will be surrounded by a single architecture of support in maternity benefit, nutrition and childcare. Through all of this runs one unbroken principle: the State facilitates, and never compels.

Demographic balance is not a statistical nicety; it is the foundation upon which the social security, the workforce and the prosperity of Tamil Nadu in the second half of this century will rest. The Government commits itself to this reorientation with the same resolve with which an earlier generation of health workers carried the small-family norm to every village — confident that Tamil Nadu, having shown the nation how the demographic transition is completed, will now show how a society passes through it with its mothers safe, its children wanted and its families whole. In this, the Government is guided by the oldest wisdom of this land, which counts among all things gained none greater than the blessing of worthy children:

பெறுமவற்றுள் யாமறிவது இல்லை அறிவறிந்த மக்கட்பேறு அல்ல பிற.

– திருக்குறள் 61

"Of all the blessings we know, none is greater than the gift of children endowed with wisdom."

CHAPTER 8

INDIAN MEDICINE AND HOMOEOPATHY

"உடம்பார் அழியில் உயிரார் அழிவர்
திடம்பட மெய்ஞ்ஞானம் சேரவும் மாட்டார்
உடம்பை வளர்க்கும் உபாயம் அறிந்தே
உடம்பை வளர்த்தேன் உயிர் வளர்த்தேனே"
– திருமூலரின் திருமந்திரம்

(If the body perishes, life too is lost. Without a sound body, true wisdom is hard to attain. Knowing the means to preserve the body, I nurtured the body — and thereby nurtured the soul too – Thirumoolar in Thirumanthiram*)*

8.1. Introduction

The Directorate of Indian Medicine and Homoeopathy integrates all five AYUSH systems - Ayurveda, Yoga & Naturopathy, Unani, Siddha and Homoeopathy - into Tamil Nadu's public health framework. The Department operates dedicated units and hospitals, runs Government Medical Colleges and controls drug quality to deliver comprehensive, cost-effective and holistic healthcare to the people of Tamil Nadu.

Tamil Nadu is the only State in India where all five AYUSH systems fall under a dedicated and exclusive Department of Indian Medicine and Homeopathy with a unique distinction of availability of Government Medical Colleges under all five systems. These systems offer preventive, promotive, curative, rehabilitative and rejuvenative care that is both globally recognised and deeply rooted in local tradition.

8.2. Indian Systems of Medicine and Homoeopathy (AYUSH)

8.2.1. Siddha

Siddha is one of the world's oldest traditional medical systems, originating in Tamil civilisation thousands of years ago. It continues to be widely practised in Tamil-speaking regions worldwide. The system is rooted in the classical framework theory of Mukkutram — Vali (Air), Azhal (Fire) and Iyam (Water) — and the Aimbootham (five elements). Siddha medicine encompasses both preventive and curative approaches and promotes physical and mental well-being through plant-based, herbal, mineral and animal-based formulations.

8.2.2. Ayurveda

Ayurveda is based on the Tridosha and Pancha Maha Bhuta theories. It emphasises health maintenance through harmony of body, mind, senses and spirit. Ayurvedic treatments - including Panchakarma and para-surgical procedures such as Ksharakarma, Leech therapy, etc. - are effective in managing chronic and non-communicable diseases. Herbo-mineral formulations support long-term health promotion and disease prevention.

8.2.3. Unani

Unani medicine (Tibb-e-Unani) adopts a holistic, nature-based approach tailored to each patient's constitutional type (Mizaj). It is particularly effective for chronic and lifestyle-related conditions including skin disorders, joint diseases, digestive issues, respiratory ailments and stress-related problems, using herbal formulations, dietary regulations and regimental procedures.

8.2.4. Homoeopathy

Homoeopathy was founded by Dr. Samuel Hahnemann in Germany in 1790 on the principle of

"Similia Similibus Curentur" (like cures like). It treats the individual holistically — mind, body and soul. The medicines are palatable, easy to administer and suitable for all age groups, including children and the elderly, with minimal side effects and limited dietary restrictions.

8.2.5. Yoga & Naturopathy

Yoga & Naturopathy provides chemical drug-free therapeutic interventions including Yoga Therapy, Massage, Hydrotherapy, Mud Therapy, Diet Therapy, Herbal Therapy, Acupuncture and Acupressure. This system facilitates disease prevention, rejuvenation and management of non-communicable and lifestyle disorders with minimal or no adverse effects.

8.3. Vision

The Government of Tamil Nadu aims to strengthen AYUSH services as an integral component of public healthcare with greater emphasis on preventive, promotive and wellness-oriented approaches.

The Department seeks to improve the health and well-being of the population through increased awareness of healthy lifestyle practices, balanced nutrition, physical activity, mental wellness and traditional healthcare approaches. Special attention is being given to lifestyle disorders, healthy ageing, rehabilitation and community-based wellness initiatives.

The Department will continue to strengthen Siddha, Ayurveda, Yoga & Naturopathy, Unani and Homoeopathy services through existing healthcare institutions, educational establishments, research activities and public health programmes.

Efforts will also focus on promoting evidence-informed practices, improving service delivery, enhancing access to quality healthcare and preserving the rich traditional medical heritage of Tamil Nadu for future generations.

The overall vision is to provide accessible, affordable and people-centred healthcare services that contribute to improved quality of life and healthier communities across the State.

Administrative Setup

Level	Office / Position
Head of Departmet	Director of Indian Medicine and Homoeopathy
Senior Officers	Joint Director; Joint Director (Yoga & Naturopathy) (Ex-Officio); Financial Advisor & Chief Accounts Officer (Finance Department)
Research Head	Chief Scientific Officer / Director, R&D Wing for Indian Systems of Medicine
Academic Heads	Principals of Government Siddha, Ayurveda, Unani, Yoga & Naturopathy and Homoeopathy Medical Colleges
Regulatory bodies	State Licensing Authority (Indian Medicine); Government Analyst, State Drugs Testing Laboratory (Indian Medicine)
Hospital	Superintendent, Arignar Anna Government Hospital of Indian Medicine, Chennai
District Level	District Siddha Medical Officers (22 Offices)
Field Units	Government AYUSH Dispensaries, ISM wings in Government Hospitals / Primary Health Centres

8.4. Academic Courses in Indian Medicine and Homoeopathy

Seven Government Medical Colleges function under the Directorate of Indian Medicine and Homoeopathy, offering undergraduate education across Siddha, Ayurveda, Unani, Homoeopathy and Yoga & Naturopathy. Undergraduate intake per institution ranges from 50 to 100 seats. Postgraduate education is provided in Siddha, Unani, Yoga & Naturopathy Streams and seats are allocated based on specialisation.

In addition, 48 self-financing colleges across the State provide a combined capacity of 4,280 Undergraduate and 282 Postgraduate seats, significantly augmenting the academic output of AYUSH professionals in Tamil Nadu.

8.5. AYUSH Paramedical Diploma Courses

Two paramedical diploma courses - Diploma in Integrated Pharmacy (D.I.P.) and Diploma in Nursing Therapy (D.N.T.) - have been offered since 2008 as self-supporting programmes at two institutions: Arignar Anna Government Hospital of

Indian Medicine, Chennai and Government Siddha Medical College, Palayamkottai. Each institution provides 50 seats in each course. These trained cadres support AYUSH clinical procedures and in-house herbal pharmacy operations.

8.6. Human Resources

The sanctioned strength of teaching and clinical personnel is presented in the tables below.

Table 8.1 Teaching Personnel in Government AYUSH Medical Institutions

System	P	R	L	L.Gr.II	Total
Siddha	27	21	25	40	113
Ayurveda	3	16	0	18	37
Homoeopathy	13	15	17	8	53
Unani	7	11	10	10	38
Yoga & Naturopathy	28	22	44	50	144
Total	**78**	**85**	**96**	**126**	**385**

P - Professor

R - Reader

L - Lecturer

L.Gr.II - Lecturer Grade II

Table 8.2: Clinical Side Personnel

System	District Siddha Medical Officer	Medical Officer	Assistant Medical Officer	Total
Siddha	22	14	863	899
Ayurveda	–	10	79	89
Homoeopathy	–	–	61	61
Unani	–	3	38	41
Yoga & Naturopathy	–	4	157	161
Total	**22**	**31**	**1,198**	**1,251**

Table 8.3: Paramedical Staff (Regular) in AYUSH Institutions

Category	Regular
Nursing	117
Pharmacist	870
Therapeutic Asst.	247
Para-Nursing & Group D	944
Total	**2,178**

Table 8.4: Contractual Staff in AYUSH Institutions

Category	NRHM	NAM	Total
AYUSH Medical Officer / Consultant	475	296	771
Pharmacist / Dispenser	424	29	453
Therapeutic Assistant	71	156	227
Other Staff	475	326	801
Total	**1,445**	**807**	**2,252**

8.7. District Offices

The Directorate functions through 22 District Offices covering all 38 Revenue Districts of the State. Each office is headed by a District Siddha Medical Officer, who also serves as Drug Inspector under Section 33G of the Drugs and Cosmetics Act, 1940. The officers oversee 1,653 AYUSH units across all tiers of the public health system and enforce drug quality standards.

Table 8.5: Distribution of AYUSH Centres across the State

System	Total
Siddha	1,091
Yoga & Naturopathy	281
Homoeopathy	111
Ayurveda	106
Unani	68
Integrated AYUSH Hospital	03
Grand Total	**1,660**

8.8. Outpatient Beneficiaries in AYUSH (2025–26)

The average daily outpatient attendance across Government AYUSH units during 2025–26 is as follows:

Table 8.6: Outpatient Statistics

System	Average Daily Outpatients
Siddha	74,398
Yoga & Naturopathy	11,461
Homoeopathy	7,108
Ayurveda	6,089
Unani	3,181

8.9. Drug Budget for AYUSH

Of the 1,653 AYUSH wings, 985 are funded through the State Budget and 668 through the National AYUSH Mission (NAM) under a 60:40 (Central: State) funding arrangement, with support also from the National Health Mission (NHM) as given in the table below:

Table 8.7: AYUSH Drug Budget (Rs. in crore)

Year	2023–2024	2024–2025	2025–2026
State Budget	27.50	28.87	28.87
NAM	27.91	51.54	25.46
NHM	0.72	1.19	–
Total	**56.13**	**81.60**	**54.33**

Medicines are primarily procured from TAMPCOL and from in-house pharmacies at Arignar Anna Government Hospital of Indian Medicine

(AAGHIM), Chennai, and Government Siddha Medical College & Hospital, Palayamkottai.

8.10. State Drug Licensing Authority for Indian Medicine

The State Drug Licensing Authority (Indian Medicine), Tamil Nadu, has been operational since 29 November 2007. It issues manufacturing licences for Siddha, Ayurveda and Unani drugs under the Drugs and Cosmetics Act, 1940 and Rules, 1945, and is the designated licensing authority for Drug Testing Laboratories in the State. 15 Drug testing laboratory licences have been issued to date.

The Director of Indian Medicine and Homoeopathy is the Controlling Authority responsible for regulatory oversight and enforcement against non-compliant manufacturers. District Siddha Medical Officers are notified as authorised Gazetted Officers under Section 8 of the Drugs and Magic Remedies (Objectionable Advertisements) Act, 1954, within their respective jurisdictions.

As on 31 March 2025, a total of 626 drug manufacturer licences have been issued. During 2025–26, 97 new licences have been granted.

8.11. State Drugs Testing Laboratory (Indian Medicine and Homoeopathy)

Established in Chennai in 2003, the State Drugs Testing Laboratory (SDTL) ensures quality control of Ayurveda, Siddha, Unani and Homoeopathy drugs under Chapter IV-A of the Drugs and Cosmetics Act, 1940. The laboratory analyses statutory samples referred by Drug Inspectors/District Siddha Medical Officers and operates specialised divisions for Siddha, Ayurveda, Unani, Homoeopathy, Chemistry, Pharmacognosy and Microbiology.

Advanced instruments including HPTLC, HPLC, Gas Chromatography and Atomic Absorption Spectroscopy are deployed for standardisation, quality control, marker estimation, steroid detection and heavy metal analysis. During 2025–2026, a total of 2,692 samples were tested, of which 2,664 were of Standard Quality and 28 were Not of Standard Quality. Samples from Andaman and Nicobar Islands (UT) are also tested under a Memorandum of Understanding (MoU).

In order to achieve NABL accreditation of this laboratory, central grant Rs.231.57 lakh has been sanctioned under the AYUSH Oushadhi Gunvatta Evam Uttpadan Samvardhan Yojana (AOGUSY) for new infrastructure, machinery, equipment and manpower. The infrastructure strengthening is under progress.

8.12. Tamil Nadu Medicinal Plant Farms and Herbal Medicine Corporation Limited (TAMPCOL)

Established on 27 September 1983, TAMPCOL is the nodal agency for procurement and supply of Indian Systems of Medicine and Homoeopathic medicines, instruments, machinery and equipment to all institutions under the Directorate of Indian Medicine and Homoeopathy. Its authorised and paid-up share capital is Rs. 300 lakh.

During 2025–26, TAMPCOL manufactured 188 varieties of medicines across Siddha, Ayurveda, Unani, Yoga & Naturopathy and Veterinary categories. A pay ward was inaugurated at the International Institute of Yoga & Naturopathy Medical Sciences, Chengalpattu on 6 February 2026. Manufacture of cosmetic products

and soaps commenced at the Alathur unit, supported by machinery procurement worth Rs. 61.62 lakh.

Table 8.8: Sales Performance of TAMPCOL

Financial Year	Net Sales (Rs. Lakh)	Net Profit (Rs. Lakh)
2023–2024	5,792.75	318.18
2024–2025	5,784.25	316.96
2025–2026 (Unaudited)	10,421.00	922.80

8.13. Arignar Anna Government Hospital of Indian Medicine (AAGHIM), Chennai

Established in 1970 at Arumbakkam, Chennai, AAGHIM is a 310-bedded hospital providing outpatient and in-patient services under Siddha, Varma, Ayurveda and Unani systems, with Homoeopathy only on outpatient basis. It is the only AYUSH hospital campus in the country offering all five AYUSH systems under one roof. Diagnostic facilities include Laboratory Services, Ultrasonography and X-ray. In-patients are provided with AYUSH-based regulated diets to improve therapeutic outcomes.

8.13.1. Siddha

Regular General OP and daily Special OP clinics address Skin Diseases, Diabetes, Bronchial Asthma, AIDS, Cancer, Hypertension, Spinal Ailments and Infertility. Nilavembu Kudineer and Kabasura Kudineer are freshly prepared and distributed daily. In-patient management covers Hemiplegia, Arthritis, Spondylosis and Gynaecological Disorders through internal medications and external therapies. The Cancer unit is being upgraded to an independent department.

8.13.2. Varmam

The Varmam Department, with 35 sanctioned beds, specialises in Pain due to Trauma and Lifestyle Musculoskeletal Disorders such as Cervical Spondylosis, Lumbar Spondylosis and Sciatica. It provides both in-patient and outpatient care. A Siddha Rehabilitation Day Care Centre has recently been established within this department.

8.13.3. Ayurveda

The 55-bedded Ayurveda in-patient ward manages Chronic Neurological and Orthopaedic Ailments including Osteoarthritis, Paralysis,

Cervical and Lumbar Spondylosis through Panchakarma and Kriyakarma procedures, including Shirodhara, Vamanam, Virechanam and Abhyangam.

8.13.4. Unani

The Unani Department has a 100-bedded in-patient facility functioning in association with the Government Unani Medical College. Therapeutic procedures include Dalak (massage), Hammam (therapeutic bathing), Hijama (cupping), Fasad (venesection) and Irsal Alaq (leech therapy).

8.13.5. Yoga and Naturopathy

Nine Yoga & Naturopathy Lifestyle Clinics function in association with AAGHIM, providing Yoga Therapy, Hydrotherapy, Mud Therapy, Diet Therapy, Herbal Therapy, Acupuncture, Acupressure and Electrotherapy. These are used for the prevention and management of Diabetes, Hypertension, Obesity, Musculoskeletal Disorders, Respiratory Ailments, Cancer Rehabilitation and Mental Health conditions.

8.13.6. Homoeopathy

The Homoeopathy outpatient wing treats chronic conditions including Psoriasis, Eczema,

Bronchial Asthma, Renal and Gall Bladder Calculi, PCOS and Fibroid Uterus, with a focus on non-surgical management.

8.14. State AYUSH Society – Tamil Nadu

Constituted in 2015, the State AYUSH Society, Tamil Nadu, implements the National AYUSH Mission (NAM), a Centrally Sponsored Scheme funded on a 60:40 (Central: State) basis. During 2025–26, Rs.54.12 crore as mother sanction was released by the Government of India through the SNA–SPARSH digital platform. The cumulative grant since inception stands at Rs.530.57 crore.

Table 8.9: Year-wise Grants-in-Aid under National AYUSH Mission (Rs. in Crore)

Year	Central Share	State Share	Total
2015–2016	0.88	0.58	1.46
2016–2017	19.81	13.20	33.01
2017–2018	27.89	18.59	46.48
2018–2019	23.26	15.51	38.77
2019–2020	19.94	13.29	33.23
2020–2021	6.17	4.11	10.28
2021–2022	23.48	15.66	39.14
2022–2023	24.29	16.19	40.48

2023–2024	66.36	44.24	110.60
2024–2025	73.80	49.20	123.00
2025–2026	32.47	21.65	54.12
Total	**318.35**	**212.22**	**530.57**

8.14.1. Key programmes implemented under NAM

- ❖ Magapperu Sanjeevi Scheme – Maternal and child healthcare through Siddha and Yoga & Naturopathy interventions; antenatal and postnatal support to promote safe and natural deliveries.
- ❖ School Health Programme (Ilam Thalir Thittam) – Preventive healthcare and wellness for adolescent students in Government schools.
- ❖ AYUSH Tribal Mobile Medical Units – Quality AYUSH healthcare services in remote and tribal areas.
- ❖ AYUSH Geriatric Healthcare Services – Specialised care for the elderly.
- ❖ Cancer OPDs – Integrative AYUSH therapies for Medical Oncology Beneficiaries.
- ❖ Musculoskeletal Disorder (MSD) Care Programme – Chronic joint and pain management.

Special Schemes

8.14.2. Magapperu Sanjeevi Scheme

This scheme distributes Siddha Medicine Kits to pregnant women at Government healthcare centres for the benefit of Maternal and Child Health (MCH). The objectives are to improve the health of pregnant and lactating mothers, promote normal deliveries and increase the incidence of normal birth-weight babies. During the year 2025-26, a total of 40,296 Siddha Medicine Kits have been distributed under this scheme.

8.14.3. School Health Programme (Ilam Thalir Thittam)

Siddha doctors at Primary Health Centres distribute five types of Siddha medicines to adolescent students in Government High and Higher Secondary Schools across all 38 Districts. During the year 2025-26, a total of 1,86,453 students have benefited under this scheme.

8.14.4. Musculoskeletal Disorder Care Programme

Seventy AYUSH Musculoskeletal Disorder units (Siddha – 50, Yoga & Naturopathy – 6, Ayurveda – 6, Unani – 4, Homoeopathy – 4) have

been established across Tamil Nadu. These provide accessible and effective AYUSH care for individuals with chronic musculoskeletal conditions, incorporating health education, counselling and therapeutic interventions by trained AYUSH Medical Officers. During the year 2025-26, a total of 4,00,207 patients have been benefited.

8.14.5. Tribal AYUSH Mobile Units

The Tribal AYUSH Mobile Units Scheme delivers quality, cost-free AYUSH healthcare to tribal populations in remote and hard-to-reach areas. Services include preventive, promotive, curative and rehabilitative care alongside health education and referral support. During the year 2025-26, a total of 1,81,914 beneficiaries have been covered under this scheme.

8.15. Tamil Nadu State Medicinal Plants Board (TNSMPB)

The Tamil Nadu State Medicinal Plants Board was constituted and registered on 26th October 2009 (Reg. No. 286/2009) with 11 members and a seven-member Executive Committee. The Board promotes the cultivation and conservation of medicinal plants, supports the establishment of herbal gardens in schools and institutions, and

creates awareness among farmers and NGOs. Special emphasis is placed on *Ashwagandha* and *Shatavari* cultivation. Financial support for operations is provided by the National Medicinal Plants Board (NMPB), New Delhi. Over the last five years, 14 projects at a total approved cost of Rs.1.8 crore have been sanctioned by NMPB and successfully implemented.

8.16 Pharmacy

8.16.1. Arignar Anna Government Hospital of Indian Medicine, Chennai

The in-house Pharmacy prepares Siddha, Ayurveda and Unani formulations for outpatients, in-patients and ten attached wings. During 2025–26, 161 Siddha, 182 Ayurveda and 88 Unani medicines were prepared. An in-house Quality Control Laboratory ensures product standards. The Pharmacy is undergoing renovation at a cost of Rs. 78.90 lakh under the Tamil Nadu Health Systems Project.

8.16.2. Government Siddha Medical College Pharmacy, Palayamkottai

This institutional Pharmacy has been operational since 1966. It manufactures over

100 Siddha formulations in 23 dosage forms under a valid manufacturing licence with Good Manufacturing Practice (GMP) certification, in compliance with the Drugs and Cosmetics Act, 1940 and Rules, 1945. An in-house Quality Control Laboratory ensures quality of medicines at all times.

8.17. Research and Development Wing for Indian Systems of Medicine, Chennai

The R&D Wing supports research and academic activities for faculty and students across all AYUSH systems. It houses four principal laboratories — Analytical Chemistry, Microbiology, Pharmacology & Toxicology and an Animal House — equipped with modern instrumentation. The wing undertakes pre-clinical and observational studies in collaboration with the Central Council for Research in Siddha and the National Institute of Epidemiology, Chennai.

Eight pre-clinical acute toxicity studies on Siddha formulations and six pharmacological studies on Unani medicines have been completed. 160 Post-graduate students have undergone Research induction programmes (NAATRANGAAL

and SWIMER). A Clinical Research Steering Committee and Institutional Ethics Committee are in place. Memoranda of Understanding have been signed with Captain Srinivasa Murthy, Central Ayurveda Research Institute, Chennai, and Anna University for collaborative research.

8.18. AYUSH Textbook Publication Wing and "Nalam" Monthly Magazine

A revolving fund of Rs. 2 crore has been allocated for printing AYUSH medical textbooks and publishing the Nalam monthly magazine. Since 2016, 30 titles of Siddha textbooks have been reprinted, 4 revised editions published and 3 new books released. These are available for sale at the Dr.Ambedkar Central Library, Chennai, and the Government Siddha Medical College Library, Palayamkottai.

The Nalam monthly magazine, published since 2016, promotes AYUSH healthcare awareness among the public. With over 5,000 subscribers, it is distributed across Tamil Nadu through the postal network.

8.19. Integration of AYUSH with Modern Medicine

The Government of Tamil Nadu promotes a patient-centred approach by integrating AYUSH systems with modern healthcare services in appropriate settings. AYUSH interventions support lifestyle modification, wellness promotion, rehabilitation, healthy ageing and supportive care, thereby contributing to improved quality of life.

Yoga and Naturopathy Lifestyle Clinics functioning in 34 Government Medical College Hospitals support the management of non-communicable diseases and 8,80,502 individuals have benefited across the State during the year 2025-26.

Integrated AYUSH services are also being provided in premier institutions such as Arignar Anna Government Cancer Hospital and Research Institute, Kancheepuram, Institute of Obstetrics and Gynaecology, Chennai, and Institute of Mental Health, Kilpauk, Chennai, benefiting patients through supportive and wellness-oriented interventions.

The Department will continue to strengthen such collaborative approaches to enhance holistic and people-centred healthcare services.

8.20. Conclusion

The Government of Tamil Nadu continues to accord high priority to the development of Indian Medicine and Homoeopathy. Through sustained investments in healthcare infrastructure, education, human resources, quality assurance, research and public health programmes, the Department is expanding access to affordable, holistic and people-centred healthcare services across the State.

The Department remains committed to strengthening Siddha, Ayurveda, Yoga & Naturopathy, Unani and Homoeopathy systems in Tamil Nadu by promoting preventive and promotive healthcare, preserving the rich heritage and advancing evidence-informed medical practices for the benefit of the people.

CHAPTER 9

TAMIL NADU HEALTH SYSTEMS PROJECT

9.1 CHIEF MINISTER'S COMPREHENSIVE HEALTH INSURANCE SCHEME (CMCHIS) 2026-2027

9.1.1. Introduction

The Chief Minister's Comprehensive Health Insurance Scheme (CMCHIS) serves as a cornerstone financial protection mechanism for the Government of Tamil Nadu. It is strategically engineered to reduce out-of-pocket medical expenditure, safeguard vulnerable households from catastrophic medical debts, and expand public access to high-quality secondary and tertiary medical treatments across both public and empanelled private healthcare providers.

Amidst a shifting public health landscape marked by an escalating burden of Non-Communicable diseases (NCDs), increased cancer prevalence, a rapidly aging demographic, and high long-term treatment costs, this scheme functions as a critical intervention tool to ensure Universal Health Coverage (UHC) and guarantee equitable financial protection across the State.

9.1.2. Milestone of the scheme

Tamil Nadu is a pioneer in public healthcare, being one of the first Indian states to launch a massive, state-funded health insurance programme for Below Poverty Line (BPL) and low-income families. By creating a scalable model that balances public resources with private healthcare infrastructure, the State laid the groundwork for Universal Health Coverage in India.

- 23.07.2009: Initially rolled out as the Tamil Nadu Government flagship programme 'Kalaignar's kaapedu thittam' with sum insured value as Rs.1 Lakh per year per family.
- 11.01.2012: Upgraded comprehensively into the Chief Minister's Comprehensive Health Insurance Scheme (CMCHIS). The sum insured is Rs.1 Lakh per year and Rs.1.5 Lakh per year per family for a few specialised procedures.
- 2017 onwards, the sum insured is Rs.1 Lakh per year and for few specialised procedures it is increased to Rs.2 Lakh per year per family.

- 23.09.2018: Integrated with the Government of India's Ayushman Bharat Pradhan Mantri Jan Arogya Yojana (AB-PMJAY), operating seamlessly as CMCHIS-PMJAY. The sum insured has increased to Rs.5 Lakh per year per family.
- As of 2026, out of 1.45 crore families, 86.50 lakh families are identified as exclusive beneficiaries under the PMJAY framework. The 60% of the premium is borne by the Government of India and remaining 40% is borne by the State Government.
- The premium towards the remaining 58.5 Lakh families are paid completely through State funds.

9.1.3. Eligibility Criteria, Coverage, & Financial Allocation

- Income Threshold: Residents of Tamil Nadu with an annual family income below Rs. 1,20,000/- are eligible for inclusion.
- Assurance Cap: Enrolled beneficiary families receive a robust insurance cover of Rs. 5.00 lakh per family, per year.

- Total Beneficiary Base: Currently, 1.45 crore families are actively enrolled in the scheme throughout Tamil Nadu. (Inclusive of PMJAY beneficiaries)

- Current Policy Year Budget (FY 2026-2027):
  - Annual Premium Rate: Rs.849/- per family.
  - Total Covered Families: 1,45,51,697 families.
  - Total Budget allocation: Rs.1,235 Crore.

9.1.4. Inclusion of Special Categories

To advance the state's vision of equitable health equity, special categories are enrolled into CMCHIS without strict socio-economic filtering, as on 31.03.2026:

Sl. No.	Special Categories	Beneficiary Count
1.	Differently abled	10,03,996
2.	Unorganized workers including construction sector	17,03,000
3.	Cleanliness workers welfare board members	29,787
4.	Orphaned Children	8,573
5.	Transgender	5,343

Sl. No.	Special Categories	Beneficiary Count
6.	Inmates from Mentally ill homes	2,620
7.	Inmates residing in old age homes	1,252
8.	Sri Lankan Tamil refugees	8,437
9.	Non-orphans from child care institutions	557
10.	Families of Accredited and Periodical Journalists	622
11.	Inmates from Institute of Mental Health, Kilpauk	111
Total		**27,64,298**

9.1.5. Achievements

The scheme has achieved unprecedented scale and success in healthcare delivery from its inception on 23.07.2009 to 31.03.2026:

- Procedure Scope: Covers 2,053 medical and surgical procedures (including 8 high-end procedures, 11 follow-up procedures, and 52 comprehensive diagnostic protocols).
- Network Strength: A robust network of 2,226 empanelled hospitals, striking a balance between 981 Government Hospitals and 1,245 Private Healthcare facilities.

- Between 23.07.2009 to 31.03.2026, more than 1.6 crore people received medical treatment. The total approved cost for this treatment was Rs.15,840 crore. Out of these, government hospitals treated over 56 lakh people, costing Rs.6,354 crore (40%). This specific funding goes directly towards improving infrastructure and strengthening equipment at the Government Hospital concerned.

- These earnings by the Government hospitals are utilised for infrastructure development in the form of installation of medical equipment thereby ensuring uninterrupted supply of service delivery.

9.1.6. High-End surgeries (11.01.2012 to 31.03.2026)

A dedicated carve-out for high-end life-saving tertiary interventions has successfully saved 18,876 lives with an aggregate expenditure of Rs.1,618.10 Crore from Corpus fund:

Sl. No.	Procedure	No. of Beneficiaries	Amount Approved (Rs. in Crore)
1.	Cochlear Implant	6,397	445.47
2.	Renal Transplant	6,848	270.66
3.	Liver Transplant	2,462	536.95
4.	Bone Marrow	2,783	294.07
5.	Heart Transplant	251	42.38
6.	Heart & Lung Transplant	38	9.00
7.	Double Lung Transplant	51	10.82
8.	Auditory Brain Stem Implant	46	8.75
Total		**18,876**	**1,618.10**

9.1.7. Evidence-Based Current Scenario and Insights

CMCHIS delivery adaptations prioritize public data indicators compiled via CRS, SRS, NFHS-6, NSSO and GBD:

- Preventable Mortality: Civil Registration System (CRS) data indicates 3.70 lakh annual deaths occur under the age of 70 in Tamil Nadu. Nearly 55% are potentially preventable and 2 lakh deaths are avoidable

through quality primary care. Notably, 63% of these premature deaths occur among working-age males.

- The Cost Contrast: NSSO surveys reveal that Tamil Nadu's public hospital system reports the lowest average Out-of-Pocket Medical Expenditure (OOPME) for inpatient care nationwide. Conversely, private sector care in the state remains high at an average of Rs.74,168 per hospitalization, vastly exceeding the national private average of Rs.50,508.

- Coverage Trajectory: Data from the National Family Health Survey (NFHS-6) alerts policy planners to a slight contraction in insurance coverage, with urban coverage dipping from 61.1% (NFHS-5) to 56.7%, and rural coverage shifting from 71.4% (NFHS-5) to 64.7%.

- In our state, cardiovascular diseases are the leading cause of death, accounting for 50% of medically certified deaths MCCD 2023, followed by COPD at 9.3%, injuries at 7.0%, cancer at 3.6%, diabetes mellitus and kidney

diseases at 3.4%, and mental disorders at 0.3%. It also points out that although evidence suggests around 60,000 cancer deaths may occur annually in the state, MCCD reports only about 25,000 deaths, indicating that nearly 35,000 deaths may not be captured in the official registration system.

(Source: *Globocan 2022 & NCRP 2020)*

9.1.8. Way Forward

As the current 5 years tender cycle concludes in Jan-2027, the Government proposes a comprehensive suite of progressive reforms for the subsequent operational phase:

A. Decentralized Network Expansion and Awareness

- Launching micro-targeted mass communication campaigns to ramp up grassroots insurance awareness.
- Empanelling a larger grid of hospitals at the Taluk level to significantly eliminate geographical barriers to care.
- Executing procedure package rate revisions to align the insurance benefit structure with modern disease and morbidity patterns

B. Focused Reforms: NCD End-Organ Diseases, Dialysis and Cancer Treatment

To effectively combat the rising burden of Non-Communicable Diseases (NCDs) and long-term illnesses, the Government of Tamil Nadu will introduce critical structural updates and financial package revisions under the Chief Minister's Comprehensive Health Insurance Scheme (CMCHIS)

- Inclusion of New Packages for NCD-Related End-Organ Diseases
- Expanded Cancer Treatment (Oncology) Framework
- New package to cover growing number of old age population

C. Regulatory and Macro Reforms

- Rolling out stringent enforcement updates to establish unconditional "Zero-Billing" across empanelled entities.
- Capping the maximum price of essential medicines, medical consumables, and implants via centralized rate contracts.

INNUYIR KAAPPOM – NAMMAI KAAKKUM 48 SCHEME

9.2. The Innuyir Kaappom – Nammai Kaakkum 48 Scheme is a pioneering emergency health programme launched by the Government of Tamil Nadu in December 2021 with an objective to provide immediate, free medical care to road accident victims during the "Golden Hour" (the first 60 minutes after an injury) and thereby ensure no human life is lost/ caused permanent disability due to a lack of money or delays in getting medical help.

9.2.1. Key Features & Eligibility

Free Initial Treatment: The scheme gives cashless emergency treatment for all individuals, irrespective of any eligibility criteria, at 773 empanelled hospitals, up to ₹2 lakh per person for the first 48 hours after a road accident within the State of Tamil Nadu.

9.2.2. Current Performance

Since its launch, the scheme has become a national model for emergency trauma financing. Free emergency treatment has been provided to

5,84,785 accident victims across the state amounting to Rs.482.42 Crore.

9.2.3. Road Safety Context & The Accident Burden

Official data from the National Crime Records Bureau (NCRB) shows that, "As per NCRB ADSI 2024, the State recorded 67,526 road traffic accident cases and 18,449 deaths. Provisional SCRB data show 70,196 cases in 2025 and 18,355 cases up to March 2026, underscoring that this post-crash intervention remains necessary". The '108' Emergency Ambulance Service works as the vital first line of defence, rapidly moving victims from accident sites straight to empanelled trauma hubs.

9.2.4. The Way Forward

To move the programme forward, the department will take the following reforms:

- Rework the package as per the existing market rate.
- Mission- Zero Preventable Death due to accident
- Centralized Command Network and data sharing

- Highway Trauma Centres: Upgrading public hospitals along highways
- AI-Based Hotspot Mapping for positioning ambulances
- Universal Awareness & Good Samaritan Laws.

9.3. AYUSHMAN BHARAT DIGITAL MISSION (ABDM)

9.3.1. Introduction and Objectives

The Ayushman Bharat Digital Mission (ABDM), launched by the Government of India in September 2021, establishes an integrated, secure, and interoperable digital health ecosystem across Tamil Nadu. Implemented by the Tamil Nadu Health Systems Project, with the Project Director acting as the State Mission Director, the mission bridges critical structural gaps between patients, healthcare professionals, and medical institutions.

9.3.2. The Core Strategic Goals of ABDM include

- Creating a unified, state-wide national digital health infrastructure.
- Issuing a unique Ayushman Bharat Health Account (ABHA) to every citizen.
- Providing seamless, consent-based access to lifelong longitudinal health records.
- Promoting technical interoperability across diverse public and private health software.
- Enhancing healthcare delivery efficiency, clinical outcomes, and data-driven policymaking.

9.3.3. Current Implementation Performance (Status as of June 10, 2026)

ABHA Generation: A total of 2,16,38,289 citizens in Tamil Nadu have successfully generated their unique ABHA identities.

Health Facility Registry (HFR): 20,823 health facilities have been verified and integrated into the ecosystem. This network includes a balanced mix of 15,189 government facilities and 5,634 private facilities.

Healthcare Professionals Registry (HPR): A verified workforce of 30,443 medical professionals have secured their HPR IDs, comprising 15,551 doctors and 14,892 nurses.

Digital Health Record Linkage: The state has linked 56,25,816 health records directly to ABHA accounts, ensuring secure medical history portability.

9.3.4. Current Structural Challenges

- Awareness and Adoption
- Infrastructure Gaps
- System Interoperability
- Workforce Capacity
- Security and Quality
- Private Sector Deficit

9.3.5. Way Forward and Strategic Roadmap

The Department is executing a multi-pronged strategy to transform Tamil Nadu into a fully digitized health state:

- Saturation Campaigns
- Infrastructure Upgrades
- Network Expansion by Incentivizing private provider

- Interoperable Systems by Standardizing health data exchange protocols
- Data Security Fortification through rigorous encryption
- Grassroots Awareness

9.4. EMERGENCY RESPONSE (108), TELE-HEALTH (104), AND FREE HEARSE SERVICES

9.4.1. Emergency Management Services (EMS) - '108'

Launched on 15.09.2008, the free, 24x7 '108' Emergency Ambulance Service delivers vital pre-hospital trauma and medical care across all 38 districts. To maintain operational efficiency, 87 new ambulances were inducted into the network on 01.11.2025.

9.4.2. Fleet Strength and Operations:

The State actively operates a robust fleet of 1,353 ambulances, consisting of

- 977 Basic Life Support (BLS)
- 307 Advanced Life Support (ALS)
- 65 Neonatal Life Support (NLS)
- 4 VVIP Convoy vehicles.

9.4.3. Performance Milestones:

- Between April 2025 and March 2026, the service successfully benefited 18,46,254 individuals.
- Notably, the average emergency response time for Road Traffic Accidents (RTAs) was optimized to 9 minutes and 35 seconds.

District wise 108 ambulance Performances FY 2025 - 2026

S. No.	District	Number of Ambulances	Beneficiaries
1.	Ariyalur	20	30,386
2.	Chengalpattu	68	76,700
3.	Chennai	70	96,014
4.	Coimbatore	66	84,023
5.	Cuddalore	56	87,909
6.	Dharmapuri	29	44,280
7.	Dindigul	38	45,803
8.	Erode	47	60,865
9.	Kallakurichi	31	41,745
10.	Kancheepuram	36	40,006
11.	Kanniyakumari	18	25,816
12.	Karur	19	27,346
13.	Krishnagiri	39	55,010
14.	Madurai	46	64,648

S. No.	District	Number of Ambulances	Beneficiaries
15.	Mayiladuthurai	16	23,085
16.	Nagapattinam	18	29,646
17.	Namakkal	28	40,328
18.	The Nilgiris	41	28,728
19.	Perambalur	17	22,606
20.	Pudukottai	38	57,260
21.	Ramanathapuram	31	45,664
22.	Ranipet	20	24,857
23.	Salem	52	75,074
24.	Sivaganga	32	46,860
25.	Tenkasi	21	26,194
26.	Thanjavur	37	56,943
27.	Theni	26	37,532
28.	Thiruvallur	74	89,866
29.	Thiruvannamalai	50	67,437
30.	Thiruvarur	22	31,262
31.	Thoothukudi	26	35,450
32.	Tiruchirappalli	46	65,933
33.	Tirunelveli	22	40,409
34.	Thirupattur	29	30,297
35.	Tirupur	33	62,576
36.	Vellore	26	37,904
37.	Villupuram	39	54,850
38.	Virudhunagar	26	34,942
Total		**1,353**	**18,46,254**

9.4.4. Specialized Neonatal Transport

The 65 dedicated NLS units provide safe transit for critically ill newborns under 28 days of age to tertiary NICUs. This specialized service successfully transported 22,450 infants during 2025-2026.

9.4.5. First Responder Bike Ambulances

A fleet of 41 rapid-response bike ambulances successfully treated 36,790 individuals in heavily congested urban zones.

9.4.6. Systemic Challenges

Rising Road Traffic Accidents are escalating the demand for ALS units, while urban traffic congestion and frequent inter-facility transfers strain existing resources.

9.4.7. Future Strategy

The Government will augment the ALS and Neonatal fleets in high-density districts. It will also deploy bike ambulances with advanced equipment (AEDs, ECG units) at mass gatherings and implement Intelligent Traffic Management Systems (ITMS) to enable automatic green corridors for faster transit.

9.4.8. Health Help Line Services- '104'

Operating round-the-clock, the '104' helpline provides accessible medical counselling, health advice, nutritional advice to pregnant women, public grievance redressal, and information on flagship schemes like CMCHIS and the Dr.Muthulakshmi Reddy Maternity Benefit Scheme.

Performance

During the April 2025–March 2026 period, 2,65,876 beneficiaries utilized the helpline.

9.4.9. "NATPUDAN UNGALODU – MANA NALA SEVAI" – (14416)

Tele-Manas (Tele-Mental Health Assistance and Networking Across States) was launched on 27th October 2022. The 24/7 toll free mental health helpline offers comprehensive mental health services which provide

- Counselling for NEET aspirants.
- Counselling for school students on publishing of public exam results.
- Counselling for persons with suicidal tendencies.

- Screening and Identification for mental health issues, emergencies immediate referral to DMHP.
- Integrated counselling services for Self-harm prevention are provided for self-harm attempters by collecting their details along with written consent from TAEI ward.

Since inception up to March 2026 the total beneficiaries are

NEET Students	–	2,35,440
Twelfth Students	–	57,735
Tenth Students	–	54,827

9.4.10. Future Strategy

The State will integrate advanced call analytics into the platform to act as a public health surveillance tool for early disease outbreak warnings. Tele-counselling frameworks will expand to address mental health, adolescent health, and lifestyle-related non-communicable diseases.

9.4.11. Free Drop-Back Service (JSSK) - '102'

Aligned with the Janani Shishu Suraksha Karyakaram (JSSK) guidelines, this service ensures

free, safe return transport from Government hospitals for postpartum mothers and sick infants under one year of age.

- **Performance**: The current fleet of 130 vehicles provided essential drop-back transport to 3,39,966 mothers and infants between April 2025 and March 2026.
- **Future Strategy**: To meet growing demand, the fleet will be systematically expanded, focusing on high-load districts and improving access within remote, tribal, and hilly terrains.

9.4.12. Free Hearse Services '155377'

This 24x7 dignified response service ensures free transportation of deceased persons from Government medical institutions to their homes or places of disposal. To replace aging assets, 37 new hearse vehicles were inducted on November 1, 2025.

- **Performance**: Operating a total network of 194 vehicles, the service assisted 1,62,811 grieving families during the April 2025–March 2026 window, providing vital support during major accidents and disasters.

- **Future Strategy**: In response to rising public awareness and utilization, the Government will increase fleet strength in high-demand zones and enhance coverage across remote and geographically challenging corridors.

9.5. TAMIL NADU HEALTH FOUNDATION (TNHF)

9.5.1. Introduction & Institutional Framework

The Government of Tamil Nadu has established the Tamil Nadu Health Foundation (TNHF) as a centralized platform to mobilize and channel Corporate Social Responsibility (CSR) funds, institutional donations, and individual contributions directed towards the development of the State's public healthcare infrastructure. Initiated under the Tamil Nadu Health Systems Project (TNHSP), TNHF replaces fragmented donation methods with a transparent and structured framework.

TNHF is Registered as a section 8 Company under the Companies Act, 2013 following issuance of Government Order. Income tax exemptions

under sections 12A and 80G are under process to ease corporate onboarding. A dedicated Project Management Unit (PMU) will be housed at the Office of the TNHSP.

The TNHF runs on a user-friendly digital portal designed to maximize the financial accountability and donor choice. The official portal (tnhealthfoundation.org) has a dashboard where donors can select a specific district or hospital for making their contributions. The donor dashboard supports instant online transactions using integrated QR code.

9.5.2. Strategic 5-Year Vision

Channelled CSR contributions through TNHF focus on achieving four structural public health milestones:

- Building world-class patient infrastructure across primary, secondary, and tertiary public healthcare institutions.
- Placing state-of-the-art diagnostic equipment and surgical technologies in rural zones.
- Setting up long-term maintenance systems to ensure medical asset quality.
- Expanding public healthcare delivery to remote and tribal areas.

9.6. TAMIL NADU ACCIDENT AND EMERGENCY CARE INITIATIVE (TAEI)

9.6.1. Summary & Vision 2030

Injuries are a major public health challenge globally and constitute one of the leading causes of death and disability. According to the National Crime Records Bureau (NCRB) – Accidental Deaths and Suicides in India (ADSI) 2024 report, Tamil Nadu reported 33,747 accidental deaths, accounting for 7.2% of all accidental deaths in India. Road Traffic Accidents (RTAs) remain the predominant cause, contributing 18,449 deaths (55%) among all accidental deaths. The State recorded 67,526 road traffic accident cases, 71,493 injuries, and 18,449 road traffic deaths, accounting for 14.4% of all road accident cases and 10.5% of all road traffic deaths reported nationally.

Tamil Nadu continues to report the highest number of road traffic accident cases in India, while recording the second highest number of road traffic deaths, next only to Uttar Pradesh. Further, the State has the highest road traffic death rate in the country at 24 deaths per lakh population, reflecting the enormous burden of road traffic injuries on the healthcare system and society.

Despite this burden, Tamil Nadu's Accident Severity Index (27 deaths per 100 accidents) is among one of the lowest in the country (24th of 36 States / UTs), indicating comparatively stronger emergency medical services, trauma care infrastructure, and post-crash response mechanisms than many other states. However, the sheer volume of crashes continues to result in an unacceptably high number of deaths and disabilities. The estimated Case Fatality Rate of 27.3% underscores the need for further strengthening prevention, pre-hospital care, trauma systems, and rehabilitation services.

The injury burden in Tamil Nadu extends beyond road traffic crashes, Sudden deaths (4,669), drowning (2,362), falls (1,924), poisoning (1,092), electrocution (964), and accidental fires (527) together account for more than one-fourth of all accidental deaths in the State. Poisoning-related deaths are largely attributable to snake bites and pesticide exposure, while drowning and accidental fires continue to cause substantial preventable mortality, particularly among vulnerable populations.

A significant proportion of injury-related deaths occur among individuals aged 18–45 years (42%), representing the most productive segment of the population. The resulting loss of life, disability, productivity, and economic output imposes a considerable burden on families, communities, and the State economy.

In view of the growing burden of *Injuries and* accidental deaths, the Government of Tamil Nadu has implemented the Tamil Nadu Accident and Emergency Care Initiative (TAEI) since 2017 to systematically reconstruct emergency medical responses across all four tiers of healthcare (Primary, Secondary, Tertiary, and Quaternary). This policy note outlines the operational milestones, progress made, challenges, and upcoming strategies designed to replace the outdated causality model in every institution and establish a universal net of immediate, zero-delay trauma care.

The State is committed to achieving the following time-bound clinical milestones by the year 2030:

1. **Trauma Care:** Cut road trauma-related mortality and long-term morbidity by 50%.

2. **Cardiovascular Emergencies:** Reduce Myocardial Infarction (Heart Attack) deaths by 33%.
3. **Cerebrovascular Care:** Reduce Acute Ischemic Stroke mortality and disability by 33%.
4. **Burns Management:** Reduce burn injury-related mortality and complications by 33%.
5. **Toxicological Emergencies:** Reduce self-harm, poison, and envenomation deaths by 66%.
6. **Paediatric Emergencies:** Cut acute paediatric emergency mortality and morbidity by 50%.

To drive this mission, the TAEI structural framework operates on Six Clinical Pillars:

Pillar 1:

Trauma Care

TAEI clinical protocols are actively deployed across 113 dedicated Trauma and Emergency Care centres state-wide. The obsolete "casualty" model has been replaced with dedicated Emergency Departments led by specialized faculty.

To sustain this pipeline, 89 MD Emergency Medicine postgraduate seats have been approved across 23 Government Medical College Hospitals. A massive clinical training campaign has successfully upskilled 15,678 doctors, 19,182 nurses, and 8,482 paramedical personnel in advanced trauma resuscitation.

The state has deployed TAEI Registry 2.0, a live mobile and web platform that seamlessly bridges pre-hospital data from the 108 Emergency Service with in-hospital emergency surgery monitoring and the Electronic Detailed Accident Report (e-DAR).

The Operational Challenges includes maintaining of data entry consistency and audit-grade quality across all 113 centres via Registry 2.0.

Vision:

- **Regional Excellence & Staffing:** To set up advanced Trauma centres of Excellence with simulation labs and rehabilitation across 7 geographic regions, while stabilizing emergency room staffing by reorganizing Emergency Medical Officers.

- **Clinical Standards & Training:** Upgraded Standard Operating Protocols (SOPs) will match international trauma guidelines, supported by international student exchange partnerships to expand postgraduate training and handle rising patient volumes.
- **Emergency Life-Saving Net:** The state intends to work toward a definitive "No More Casualty Concept in Tamil Nadu".

Pillar 2:

Paediatric Resuscitation and Emergency Medicine (PREM)

A grid of 116 specialized PREM centres is fully functional across Tamil Nadu, operating alongside Emergency Departments. The Institute of Child Health (ICH) and Hospital for Children in Egmore serves as the Nodal training centre, delivering structured hands-on resuscitation training. Structured modules have upskilled 569 doctors and 1,315 nurses in advanced pediatric life support since 2021. Decentralized training at district-level Medical College Hospitals has benefited an additional 480 doctors and 480 nurses since 2024.

Vision

- **Institutional Upgrades:** Top-performing district units will become recognized Centres of Excellence, while the apex ICH Egmore facility will expand into a Regional Collaborative Centre for Training and Research.
- **National Integration:** The State is partnering with the NHSRC to merge PREM guidelines into national MusQan quality standards and is advocating to include pediatric emergency medicine within the NMC undergraduate curriculum.

Pillar 3:

Tamil Nadu Heart Attack Management Programme (TNHAMP)

The State has mapped a life-saving cardiac network comprising 21 Hub Hospitals and 195 Spoke Hospitals, guaranteeing that any citizen experiencing a cardiac event can access a hub facility within a two-hour travel window. Tamil Nadu has compressed the critical Door-to-Device time down to 40 minutes, significantly outperforming the international benchmark of 90

minutes. The Government has increased active Cath Labs from 24 to 39 fully functional facilities, expanding advanced interventional access from 15 to 21 districts. This includes newly completed installations at Tiruvallur, Cuddalore, Tiruvannamalai, The Nilgiris, Ramanathapuram and Omandurar Government Multi Speciality Hospital, Chennai.

Since 2021, the programme has successfully conducted 15,042 Primary PCIs,, 1,17,682 coronary angiograms and other procedures of 30,177 totaling upto 1,62,901 interventions in all-shielding these patients from catastrophic out-of-pocket expenditures.

Vision

- Close monitoring of Telecardiology platform for real-time expert support from Hub cardiologists to Spoke emergency rooms.
- Eliminating stockouts of life-saving thrombolytic agents by mandating a permanent stock of 5 vials of Tenecteplase at every District Headquarters Hospital and 3 vials at every Sub-District Hospital.

- ❖ Scaling up Cardio-Thoracic and Vascular Surgery (CTVS) backup capabilities to handle complex multi-vessel complications.

Pillar 4:

Tamil Nadu Stroke Care Management Programme

Since 2019, the initiative has completed 1,35,941 emergency CT scans, accurately diagnosing 1,20,087 stroke cases. Of these, 2,750 critical patients received immediate thrombolysis therapy within 4.5 hours, successfully preventing permanent long-term neurological disability.

Operating through a structured framework of 20 Hub Hospitals and 77 Spoke Hospitals, the network ensures acute stroke patients receive specialized definitive care within the strict 4.5-hour therapeutic window. Hub institutions conduct mandatory, periodic stroke care training cycles covering early recognition, imaging, and thrombolysis for doctors, nurses, and radiologic technicians at mapped Spoke facilities.

Further, to augment the emergency Neurosurgery services in the State, Government of Tamil Nadu has implemented statewide Tele-

Radiology services for Head Injury and other Neurosurgical emergencies for timely interpretation of CT Scans, which are shared by hospitals without Neurosurgeons since 27.05.2026. The objective of this initiative is to facilitate prompt decision making regarding referrals and definitive management through a Hub and Spoke model by categorizing:

- Hospitals with availability of Neurosurgeons round the clock (24/7) as Hub Hospitals

- Hospitals with CT Scan facility but without Neurosurgeons / Radiologists as Spoke Hospitals.

For operational convenience, the State has been divided into North, South, East, West and Central zone. Robust communication and data sharing protocols are in place between the stakeholders in the region. From 27.05.2026 up to 10.06.2026, a total of 192 CT scans were shared of which 156 were due to trauma, 30 due to non trauma / stroke/ spontaneous intracranial bleed, 1-tumor and 5 other neurosurgical emergencies.

Vision:

- Equipping all designated Hub Hospitals to assist in timely thrombolysis of patients at spoke hospitals & perform advanced Mechanical Thrombectomy procedures for patients presented in extended time window.
- Ensuring 24/7 emergency CT scan availability across all District Headquarters and Sub-District Hospitals. Maintaining an uninterrupted supply of the life-saving thrombolytic drug Tenecteplase at all Hub and Spoke facilities.
- Launching comprehensive stroke care units staffed by Neurologists, Physiotherapists, Speech Therapists, and Rehabilitation specialists in a phased manner across Tamil Nadu.

Pillar 5:

Burns Care Management

A total of 21,072 burn patients were treated across state facilities between January 2022 and April 2026. A specialized Hyperbaric Oxygen Therapy (HBOT) unit has been installed at the

Sivakasi Government Hospital Burns Centre at a cost of Rs.1.40 Crore. The apex training centre at Kilpauk Medical College (KMC), Chennai, conducted multi-batch refresher training, successfully upskilling 109 Doctors, 114 Staff Nurses, and 81 Physiotherapists.

Vision

- Strengthening Regional Centres of Excellence for Burn Care at Tiruchirappalli, Thanjavur, Vellore, Salem, Coimbatore, Madurai (GRH), and Kilpauk (KMC) Medical Colleges.
- Provision of protein supplementation to burn patients to support nutritional recovery and promote wound healing.
- Establishment of fully staffed Plastic Surgery Departments at Karur and Tiruvannamalai Medical College Hospitals,
- Bridge Course training in Burns Management for General Surgeons.

Pillar 6:

Poisoning, Bites and Stings Management

A total of 3,24,144 poisoning patients were successfully treated across Tamil Nadu between January 2022 and April 2026. Plasma Exchange

Therapy (PLEX) for severe rodenticide poisoning is operational across 22 hospitals. Out of 18,909 rodenticide cases treated, 2,509 patients underwent PLEX, yielding an exceptional 64.9% survival rate for high-toxicity ingestions. A One day refresher training has been conducted on Intensive Medical Care and Poison Management at Rajiv Gandhi Government General Hospital, for 142 emergency physicians.

Vision

- Establishing 5 Regional Centres of Excellence in Toxicology equipped with Antidote Banks and training facilities at RGGGH (Chennai), Coimbatore, Madurai, Thanjavur, and Tirunelveli.
- Developing a dedicated emergency application and helpline for on-duty doctors to instantly clear doubts, track antidote/Anti-Snake Venom (ASV) availability.
- Implementing strict state-level restrictions and advocating for a ban on the highly toxic herbicide Paraquat.
- Enhancing Hub-and-Spoke medical coordination to streamline poisoning and envenomation referrals.

9.7 May I Help You Desk

"May I Help You Desks" (உதவி மையம்) are established across major public hospitals with high foot-falls to provide seamless navigation, scheme assistance, and compassionate care to patients and visitors.

The Desks are funded from CMCHIS IEC funds – up to Rs.5.00 lakh per institution in the 36 Government Medical College Hospitals and 5 District Headquarters Hospitals, and up to Rs.2,36,000 per institution in the remaining 14 DME&R institutions in Chennai.

9.7.1. The core objectives of the initiative:

- **Centralized Information:** Serve as a visible, accessible hub to eliminate patient confusion.
- **Scheme Facilitation:** Drive enrolment and resolve queries for CMCHIS, PMJAY, and ABHA registrations.
- **Operational Efficiency:** Reduce patient wait times and streamline hospital department coordination.

- **Dignified Experience:** Elevate overall patient satisfaction as a core metric of hospital performance.

It is established in a highly visible, main entrance lobbies or registration areas by following a uniform prototype design. The Head of the institutions / Joint Director of Health Services are given permission to engage the dedicated hospital workers exclusively for the Help Desk.

Key services

- Providing hospital directions and department guiding.
- Assisting with patient admissions and scheme enrolment.
- Registering and escalating patient grievances.
- Maintaining a digital or physical Visitor Assistance Register.

CHAPTER - 10

FOOD SAFETY AND DRUG ADMINISTRATION

10.1. Vision and Introduction

The Tamil Nadu Food Safety Department is committed to ensure safe and wholesome food for all consumers by implementing the Food Safety and Standards Act, 2006 effectively in a transparent and accountable manner, without affecting the business friendly environment of the State.

The Food Safety Department has following key objectives to achieve safe and wholesome food for the consumers.

- To eliminate the availability of 'unsafe' and 'unhygienically prepared' food among the Food Business Operators (FBOs), thereby ensuring safe food to the consumers and consequently reduce the Food Safety Incidents from existing quantum in this State within next 5 years.
- To restrict the food products high in saturated fat or trans-fat or added sugar or sodium at schools, and to enhance Eat Right

promotions to have nutritious and balanced food for the consumers thereby assisting the health sector to reduce the burden of lifestyle diseases such as obesity, diabetes, hypertension etc.

10.2. Department and Mandate

The Tamil Nadu Food Safety Department has role of implementation of the Food Safety and Standards Act, 2006, a Central Act, with an objectives of ensuring safe and whole some food to the consumers by setting up of science-based standards and focusing more on regulatory regime than enforcement and shifting to single reference point for the food business so as to create environment for ease of doing business. It will facilitate interested persons to change in to entrepreneurs in the food sector very easily. The Food Safety and Standards Authority India (FSSAI) is the apex body for implementation of the above said Act.

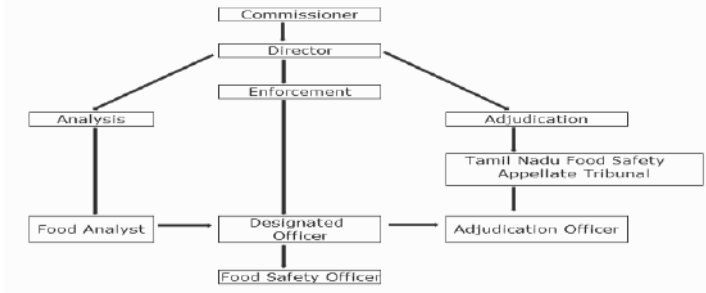
The Food Safety Department is enforcing the Food Safety and Standards Act, 2006 by issuing License and Registration Certificate for the Food Business Operators, Inspecting the Food Business Operators, lifting and analysis of the Food Samples,

launching of Adjudication and Criminal Cases based on Food Analyst Report, Compounding the Offences committed by the petty Food Business Operators and with transparent redressal mechanism for consumer complaints. Also, the Food Safety Department is carrying out promotions to increase capacity of the consumers to identify the unsafe food, misbranded food, and also increase the professional skills by imparting trainings to the food handlers/Food Business Operators, and most importantly the Food Safety Department is trying to change the unhealthy diet habit to healthy one among the consumers right from school level.

10.3. The Commissionerate of Food Safety:

The Food Safety Department has been formed as vertical type of administration as laid down in the Food Safety and Standards Act, 2006.The department has three internal divisions.

The Structure as per the said Act:



10.3.1. Enforcement

The Commissioner – A Secretary Cadre Officer is placed as the Commissioner of Food Safety as per Food Safety and Standards Rules, 2011.

The Commissioner is overall administrative head of the department and he is responsible for prohibiting the contravened food products for the period of one year, framing food safety awareness programmes and sanctioning the Prosecution for the offences in which imprisonment is prescribed in the Act.

The Designated Officer is placed at each district and they are the Administrative in-charge of the Food Safety Department at District Level.

The Designated Officer is leading the department at district level, working under the supervision of the District Collector and functioning as licensing authority, sanctioning authority for the offences in which penalty alone is prescribed, compounding authority for the offences committed by the petty Food Business Operators, and prohibits the sale of contravened food products (unsafe, substandard and misbranded) within their jurisdiction.

The Food Safety Officers (FSO) are placed at each Municipality and block level under the administrative control of the Designated Officers.

The Food Safety Officer is implementing the Food Safety and Standards Act, 2006 at field level by issuing the Registration Certificate for the Food Business Operators in the FSSAI's Food Safety and Compliance System (FoSCoS) Web Portal, who has an annual turnover of less than Rs.1.5 Crore, lifting the food samples from the food Business Operators and sending it to food Analysis Laboratories for analysis and launching the cases before the Adjudication Officer, where only penalty prescribed, or the Judicial Magistrate for the cases, where imprisonment is prescribed in the Act.

10.4. Food Analysis

10.4.1. The Food Analysis Laboratory at Regional level

The Government Food Analysis Laboratories are functioning at Coimbatore, Guindy, Madurai, Palayamkottai, Salem and Thanjavur. All Laboratories have been accredited by National Accreditation Board for Testing and Calibration for Laboratories (NABL) Accreditation/Integrated

Assessment and notified by the FSSAI. All Food Analysis Laboratories are having regular Food Analyst and supported with Microbiologists, Senior Analysts and Junior Analysts. Moreover, the Regional Laboratories are sanctioned with one post of Assistant Public Prosecutor Grade-II to assist the Designated Officers and Food Safety Officers of the respective region on the Adjudication and Court cases.

10.5. Adjudication and Appeal

10.5.1. The Food Safety Appellate Tribunal

The Tamil Nadu State has established the Appellate Tribunal as part of the Food Safety and Standards Act, 2006 and appointed a Presiding Officer in the rank of District Magistrate and Assistant Public Prosecutor. The Tribunal is handling the appeals against the orders of the Adjudication Officers.

10.5.2. The Adjudication Officer

The District Revenue Officers/Additional District Magistrates are notified as the Adjudication Officers at each Revenue Districts to adjudicate the civil cases such as Substandard, Misbranded and Misleading as per the Food Safety and Standards Act, 2006.

Table 10.1: Sanctioned Posts for the Department

S. No.	Offices	Posts Sanctioned	In-position	Vacant
1.	Office of the Commissioner	45	34	11
2.	Food Safety Appellate Tribunal	11	9	2
3.	Office of the Designated Officer	615	497	118
4.	Office of the Food Analysis Laboratory	227	166	61
5.	Assistant Public Prosecutor Grade - II (at the office of the Food Analysis Laboratory)	6	1	5

Table 10.2: Key Functions of the Department:

Enforcement	Promotional
Licensing and Registration of Food Businesses	Conduct regular Awareness programmes for the Food Business Operators
Food Sample lifting	Conduct regular Awareness programmes for the Consumers
Food Sample Analysis	Facilitating the Food Business Operators to get license and Label norms by organizing the special camps

Adjudication, Prosecution	Eat Right Initiatives
Compounding of offence	Releasing awareness short films
Inspection and issuing of Improvement Notice	Social Media Campaigns

10.6. Mandate

The Food Safety Department is delivering its services as prescribed under the Food Safety and Standards Act, 2006 and moving towards positive outcome thereby ensuring safe food to the consumer.

Action	Method	Outcome
Licensing of Food Business	Bring the Food Business Operators under FSSAI by using FoSCoS Web Portal.	All Food Business Operators will be roped under the department and they will be under surveillance.
Inspection of food business and Sampling of Food	FSO inspect the Food Business Operators and lift the Food Sample and send it to Laboratory and upload it in the FoSCoS.	The Designated Officer will issue Improvement Notice and there by the Food Business Operators will rectify the Hygienic and Sanitary lapses, which helps to ensure safe food to the consumers.

Food Analysis	Food Analyst analyse the samples and declare the sample either confirms to the standards, or contravened such as Unsafe/ Substandard / Misbranded/ Misleading.	The Designated Officer getting status of the food quality within their jurisdictions and they are taking administrative action like suspension of license and thereby contravened food products curtailed in the market.
Adjudication and Prosecution	Based on the Food Analyst Report, the FSO will file case before Adjudication Officer/Judicial Magistrate after getting approval from the Designated Officer / Commissioner respectively.	The defaulters will be penalized for their violations and ensuring violations are not repeated in future thereby providing safe food to the consumers.
Eat Right Promotions	Under planning and guidance of the Commissioner and Designated Officer, the FSOs coordinates with the recognised Third-Party Agencies to implement the Eat Right Initiatives. Also, to impart training and to conduct audit among the FBOs through empanelled Third-Party Audit Agencies.	The consumers and the Food Business Operators are enlightened on the safe &wholesome food thereby changing their food and business practices respectively.

Because of ease of doing business policy, the number of Food Business Operators, who have valid FSSAI License and Registration Certificate, are increasing day by day. As the number of Food Safety Officers are less, the workload has increased manyfold.

In the year 2012-2013, there were 400 Food Business Operators per Food Safety Officer, during the year 2025-2026 the number of Food Business Operators has increased to 2,500 per Food Safety Officer. Hence, the following challenges needs to be addressed to avoid probable issues in the future in-order to ensure safe food.

Sector	Challenges	Possible solutions
Food Safety Department	Limited resources and infrastructure, like Human Resources, Civil Infra Structure.	Upgrade the department with additional manpower proportionate to the number of Food Business Operators and by providing necessary infrastructure.

Sector	Challenges	Possible solutions
Food Business Operators	Limited accessibility of technical advice for licensing and other business activities, limited awareness on standards and not standardizing the native food products.	Using existing Tamil Nadu e-governance system for submitting applications, collaboration with recognized Third-Party Agencies and periodical recommendations to the FSSAI.
Consumers	Customary beliefs, myths and restricted facilities for informed choices.	Continuous efforts to implement Eat Right Initiatives by using various modes including social media and enlightening the consumers about the provision to get food sample analyzed at their own cost.

10.7. Flagship schemes and programmes

10.7.1. WhatsApp Complaints

If any of consumers aggrieved by the quality of the food, service of the Food Business Operators,

they can send the complaint through as text/recorded voice/picture to **"94440 42322"**. The complaints will be redressed within 72 hours and action taken report will be sent to the complainant. During the year 2025-26 the Food Safety Department has received 6083 complaints and resolved those complaints.

10.7.2. Tamil Nadu Food Safety Consumer App

It is an application developed by the Food Safety Department in addition to the WhatsApp Complaint redressal mechanism, the Food Safety Department has received 2597 complaints in this App. and resolved those complaints during the period 2025-2026.

10.7.3. Food Safety on Wheels

It is a Mobile Food Testing Laboratory. There is one Mobile Laboratory for each Food Analysis Laboratory. This has been introduced to enable consumers to test their food sample at their doorstep, enlighten the consumers on safe food and to reach the unreached population. Moreover, these Food Safety on Wheels are helpful during the festival gathering to do on the spot analysis to identify the adulteration and unsafe food. Also,

these vehicles are used in the Check-Posts to screen the food transported via Check-Posts. During the year 2025-2026, the Food Safety on Wheels conducted 1,114 camps, tested 22,235 samples and conducted 849 training sessions.

10.7.4. Eat Right Initiatives

This scheme is implemented with the funding assistance of the FSSAI. It is introduced with an objective of promoting safe food practices among the Food Business Operators, improve awareness level among the consumers, thereby bringing behavioural changes among the consumers towards safe and nutritious food.

10.7.5. Food Safety and Training Certification (FoSTaC)

In order to make the food safe to the consumers, it is essential to understand the concepts of food safety norms and regulations. Hence, the Food Safety Department is conducting regular Food Safety and Training Certification for the Food Handlers including Anganwadi Workers, Chief Minister's Breakfast scheme employees, Street Food Vendors, private eateries, manufacturers etc. This certificate is valid for

2 years. During the year 2025-2026, the Department has conducted 5,231 training sessions and 2,01,785 food handlers were trained.

10.7.6. Repurpose Used Cooking Oil (RUCO)

The edible oil must not be reheated again and again in order to avoid exponential increase of the harmful trans-fat in the oil, which causes atherosclerosis. This scheme has been introduced to protect the health of the public and also with a careful mind of minimising the loss to the Food Business Operators. The one-time used oil has been purchased by the FSSAI approved aggregators at the rate decided by the aggregators based on the Total Polar Compounds in the used oil and converting into Bio-Diesel and other non-food products. During the year 2025-2026, the department has collected 5,122 Kilolitres of the used oil and utilized it for Bio-Diesel production and other non-food products such as soap etc.

10.7.7. Eat Right Campus

It is introduced with the objective to supply safe and nutritious food to the incumbents of any campus including colleges, industries and improve their awareness about safe and nutritious food.

This certificate will be issued by the FSSAI after evaluation by the recognized Third-Party Audit agency and it is valid for 2 years. During the year 2025-2026, this department has received 408 certificates.

10.7.8. Eat Right Place of Worships

"Blissful and Hygienic Offering to the God" was renamed as Eat Right Place of Worships. It is introduced to supply safe and nutritious Prasadhams and Annathanams to the pilgrims of the temples and other pilgrimage places. This certificate will also be issued by the FSSAI after evaluation by the recognized Third-Party Audit agency and it is also valid for 2 years. During the year 2025-2026, this department has received 196 certificates.

10.7.9. Eat Right School

It is an academic year programme and it is being conducted at schools with in-class room teaching to the students on weekly basis about safe and nutritious food. This certificate will be issued by the FSSAI after evaluation by the recognized Third-Party Audit agency and it is valid for 2 years. During the year 2025-26, this department had assisted 330 schools to receive this certificate.

10.7.10. Eat Right Station

This scheme is introduced to ensure safe and nutritious food at Railway Stations. This certificate will also be issued by the FSSAI after evaluation by the recognized Third-Party Audit agency and it is valid for 2 years. During the year 2025-26,this department has received 19 certificates.

10.7.11. Clean Street Food Hub

This scheme has been introduced to improve hygiene standards in the street food hubs thereby ensuring safe food to the public. This certificate will be issued by the FSSAI after evaluation by the recognized Third-Party Audit agency and it is valid for 2 years. During the year 2025-2026, this department has received 27 certificates.

10.7.12. Clean and Fresh Fruit and Vegetable Market

This scheme has been introduced to improve hygiene standards at the Markets. This certificate will be issued by the FSSAI after evaluation by the recognized Third-Party Audit agency and it is valid for 2 years. During the year 2025-2026, this department has received 44 certificates.

10.7.13. Hygiene Rating Certificate

It is introduced to ensure safe and hygienic food at eateries including Restaurants, Sweet Stalls by issuing Hygiene Rating Certificates with five smileys. The Food Business Operators certified with 5 smiley Hygiene Rating Certificate has best hygienic standards and those certified with 1 smiley has lesser hygiene standards. Hence, it enables the consumers to choose best hygiene eateries and it is a method of informed choices. This certificate will be issued by the FSSAI after evaluation by the recognized Third-Party Audit agency and it is valid for 2 years. During the year 2025-2026, this department has received 11,237 certificates.

10.7.14. Prohibition of the Food Products containing the Tobacco/Nicotine as the ingredients

As per Food Safety and Standards Act, 2006 provisions, the food materials, which has Tobacco/Nicotine as the ingredients, were prohibited by the Commissioner based on the policy of the Government. Those who have been caught/identified as first time offender, they will be imposed with Rs.25,000/- as penalty and shop will remain closed for not less than 14 days, those

caught/identified as second time offender, will be imposed with Rs.50,000/- as penalty and shop will remain closed for not less than 30 days, those caught/identified as third time offender, will be imposed with Rs.1,00,000/- as penalty and shop will remain closed for not less than 90 days with cancellation of FSSAI License/Registration Certificate. Further, subsequent offences will be dealt with Adjudication proceedings. During the year 2025-2026, the department has seized 68,868 Kgs of prohibited food items, 14,568 shops have been closed and Rs.44.12 crore have been imposed as penalty.

10.8. Achievements

This department has achieved its objectives with limited resources with respect to Human Resources, Finance and Infrastructure. Further, this department has been placed continuously in top 3 ranks in the State Food Safety Index of the FSSAI.

Financial Years	Rank
2020-2021	3
2021-2022	1
2022-2023	3
2023-2024	2

Moreover, all districts of Tamil Nadu participated in the Eat Right Challenge, which is conducted as part of 75th Independence Day celebration, and achieved remarkably.

Financial Year	Tamil Nadu District placed as First at National Level	Number of Tamil Nadu Districts within top 75 districts at National level
2021-2022	-	11
2022-2023	Coimbatore	13
2023-2024	-	26
2024-2025	Thoothukudi Madurai Kancheepuram Karur Erode Kanniyakumari Thanjavur Namakkal Villupuram Tiruvallur Chennai Salem Tiruvannamalai	32

Table 10.3: Key achievements during the year 2025-2026

Variables	Achievement up to 31.03.2026	Cumulative Achievement from inception of the department or programme
Enforcement		
No. of Food Business Operators having valid License and Registration Certificate	8,24,159	-
Amount of License Fees collected (Rs.)	50,09,49,836	222,36,57,575 (for past 5 years)
No. of Enforcement samples lifted	42,539	1,72,319
No. of Samples analysed by the Government and Private Food Analysis Laboratories	44,603	1,64,711
No. of samples identified as conforms to standard	40,619	1,29,996
No. of samples identified as Unsafe	1,849	11,319
No. of samples identified as Substandard, Misbranded, Misleading, etc	2,135	23,390
Surveillance Samples		
No. of Surveillance Samples lifted	70,024	2,41,419
No. of Surveillance Samples not conform to the standards	4,778	11,256
Adjudication, Compounding and Judicial cases		
No. of Adjudication cases launched	1,615	20,119
No. of Adjudication cases judgement delivered by the Adjudication Officer	1,393	17,754

Variables	Achievement up to 31.03.2026	Cumulative Achievement from inception of the department or programme
Amount of penalty imposed by the Adjudication Officer (Rs.)	1,50,12,150	23,76,56,699
No. of Criminal cases launched	819	7,764
No. of Criminal cases judgement delivered by the Judicial Magistrate	900	4,368
Amount of penalty imposed by the Judicial Magistrate (Rs.)	2,34,29,000	10,66,83,000
No. of compounding offences identified by the FSO	23,669	86,231
Amount of compounding penalty imposed by the Designated Officers (Rs.)	44,12,38,000	123,42,99,900
No. of Compounding offences identified in banned food products containing tobacco and nicotine as ingredient	15,295	56,709
Amount of penalty imposed by the Designated Officer (Rs.)	44,12,38,000	123,42,99,900
Quantity of prohibited food items seized (in Kgs)	68,868	12,89,371
No. of shops closed for selling prohibited food items	14,568	47,070
Eat Right Initiatives and Consumer Awareness		
No. of FoSTaC training programmes conducted	5,231	13,045
No. of Food Business Operators trained	2,01,785	5,33,084
No. of events conducted on Information, Education and Communication	10,171	38,778

Variables	Achievement up to 31.03.2026	Cumulative Achievement from inception of the department or programme
No. of Clean Street Food Hubs Certified	27	150
No. of Clean and Fresh Fruit and Vegetable Market Certified	44	221
No. of Temples certified with Eat Right Place of Worship Certificate	196	1,319
No. of eateries including Anganwadi's with Hygiene Rating Certificate	11,237	22,221
No. of Campuses with Eat Right Campuses Certificate	408	1,835
No. of Schools with Eat Right Schools Certificate	330	625
Repurpose used cooking oil collected volume in Kilolitres	5122.817	19,206

10.9. New Initiatives for the Department

10.9.1. Upgradation and Renovation of Complete Infrastructure of Food Testing Laboratories in favour to increase the Testing sample capacity from 36,000 samples per year to 50,000 samples per year

The FSSAI has fixed a target of lifting 10 Statutory Samples and 15 Surveillance Samples per FSO per month and it should be analysed in the Food Testing Laboratories. Since the samples has

to be analysed within 14 days as per Food Safety and Standards Act, 2006, the testing capacity with the available building infrastructure, equipment and manpower was achieved as 1000 samples/month for Food Analysis Laboratory, Guindy and 500 samples/month for other 5 Food Analysis Laboratories. Moreover, it is necessary to upgrade the infrastructure as per Good Food Laboratory Practices (GFLP) and guidance document for regulatory food testing laboratory as per FSSAI Regulations and to meet the International Organization for Standardization (ISO) 17025, which is mandatory for accreditation of NABL Accreditation for all parameters to be tested as per FSSAI Regulations. And also, the Food Analysis Laboratories are to be strengthened for testing export samples as per Agricultural Processed Export Development Authority (APEDA) and Export Inspection Council (EIC) regulations which requires more sophisticated equipment and further upgradation of laboratories in the Microbiology, Pesticide residue testing, Antibiotic residue testing, Naturally occurring Toxins etc. Hence, in order to increase the testing capacity of all the laboratories, enhancement of building infrastructure, equipment and manpower should be upgraded accordingly.

10.9.2. Establishing Microbiological Laboratory wing at the Food Analyst Laboratory of Salem and Palayamkottai

Among Six Food Analysis Laboratories, three Food Analysis Laboratories (Guindy, Coimbatore and Madurai) has been provided with Microbiology laboratory wing. Hence, remaining three Food Analysis Laboratories should be provided with Microbiology Laboratory wing. But, existing building at Food Analysis Laboratory, Thanjavur is not adequate for strengthening of the Laboratory. Therefore, action has been taken to establish Microbiological section at Food Analysis Laboratory, Palayamkottai and Salem at the cost of Rs.10 crore with 60% funding assistance from the FSSAI.

10.10. New Initiatives for the Food Business Operators

10.10.1. E-Seva Centres of Tamil Nadu State will be designated as Centres for filling Application for FSSAI Registration and License at the fees applicable

The Food Business Operators are facing difficulties while applying for FSSAI License and Registration in the website due to lack of computer

knowledge. It is affecting the Food Business Operators financially and mentally. Hence, this department has proposed to include FSSAI License and Registration Application procedure in the E-Sevai Facility under Tamil Nadu e-Governance Agency.

Taluk Headquarters and District Headquarters Hospitals will be designated as a screening centre for Food Handlers

As per provisions of the Food Safety and Standards (Licensing and Registration of Food Businesses) Regulations, 2011, every food handler, working in the Food Business Operators premises, should be certified as 'free from contagious and enteric diseases' after proper screening. Since the infected human is the primary vector to transmit communicable infectious diseases, this department has proposed to designate the Taluk Headquarters and District Headquarters Hospitals as Screening Centres to certify the food handlers in the prescribed Medical Proforma as 'free from contagious and enteric diseases' after screening, in order to ensure safe and hygienic food to the consumers.

10.11. Initiatives for all Stakeholders

10.11.1. Mandatory Microbiological sampling and Testing of the "Prepared Food" such as Briyani, Non-Vegetarian Food Item for the Restaurants, Eateries

It is evident that most of the Food Poisoning incidents occurred due to microbiological contamination in the prepared food and in particularly Non-Vegetarian foods. Hence, the Food Business Operator and the Food Safety authorities have to ensure that the food is free from contamination. In order to ensure this among the annual target of 57,000 surveillance samples, 5,700 surveillance samples (10%) should be lifted for the purpose of testing microbiological parameters. This can be implemented by using funds already allocated by the FSSAI and the Government under Memorandum of Understanding as 60:40 proportion.

10.11.2. Improve the Hygiene and Sanitary Standards of the Street Food Vendors and thereby ensuring safe food to the consumers

Socio-Economically weaker sections and part of middle-income families are depending on the

street foods for their daily needs or more frequently. Hence, this sector should be properly guided, trained and supported by the department to ensure safe food to the public. Therefore, it is decided to uplift the Street Food Vendors by conducting regular trainings on Food Safety and Hygienic Practices. It is proposed to conduct 200 training camps for Street Food Vendors, aiming to cover 20,000 street food vendors. This scheme is going to be implemented by using funds already allocated by the FSSAI and the Government under Memorandum of Understanding as 60:40 proportion.

10.12. Conclusion

In order to ensure safe and wholesome food, change food habits among the consumers, to reduce the Food Safety Incidents, to restrict the food products with high saturated fat, trans-fat, added sugar and high sodium and assisting the health sector reduce the burden of lifestyle diseases, therefore effort has been made to empower, upgrade, strengthen the Food Safety Department with inclusive development by using technology to the international standards.

CHAPTER - 11

TAMIL NADU MEDICAL SERVICES CORPORATION LIMITED

11.1. Introduction

Tamil Nadu Medical Services Corporation Limited (TNMSC) is a Government of Tamil Nadu incorporated on 01.07.1994 under the Companies Act, 1956 as a wholly owned Government company. This Corporation was established with the objective of ensuring efficient, transparent and cost-effective procurement and distribution of medicines and medical supplies through a centralized procurement system. Tamil Nadu Medical Services Corporation Limited has played a pivotal role in strengthening the public healthcare system in the State.

The centralized procurement model enables significant cost savings to the Government, promotes efficiency in public expenditure and ensures uninterrupted availability of essential healthcare commodities. More importantly, the free supply of medicines and access to advanced diagnostic and therapeutic services in Government

hospitals substantially reduce out-of-pocket expenditure for patients, particularly economically weaker sections.

The Corporation's procurement and logistics model has emerged as a widely recognised benchmark in public healthcare supply chain management. TNMSC is certified under ISO 9001:2015 for its Quality Management System, reflecting its commitment to excellence, transparency and continuous improvement in service delivery.

11.2. Administrative Structure

Tamil Nadu Medical Services Corporation Limited (TNMSC) functions under the administrative control of the Health and Family Welfare Department, Government of Tamil Nadu. The governance of the Corporation is vested with its Board of Directors, chaired by the Secretary to Government, Health and Family Welfare Department. The Corporation is headed by the Managing Director, who is responsible for the overall administration and management of its operations.

The Board comprises of Directors representing key stakeholder Directorates, namely:

- Managing Director, TNMSC
- Representative of the Finance Department
- Mission Director, National Health Mission (NHM)
- Director of Medical Education and Research
- Director of Medical and Rural Health Services
- Director of Public Health and Preventive Medicine
- Director of Drugs Control
- Chief Engineer, Public Works Department (Buildings)

The Board of Directors of TNMSC functions as the Tender Accepting Authority for all tenders with a value exceeding Rs.1.00 crore. Tenders valued at Rs.1.00 crore and below are finalized by the Managing Director, TNMSC.

11.3. Major activities of TNMSC

To strengthen healthcare service delivery across the State, TNMSC undertakes the key activities as summarized below:

Functional Area	Activities
Drug Procurement and Supply Chain Management	Procurement, quality testing, storage and distribution of generic and specialty drugs, medicines, surgical consumables and suture materials to Government Medical Institutions and medicines to Veterinary institutions.
Medical Equipment Management	Procurement, installation and maintenance of medical equipment for Government health facilities across the State.
Diagnostic Services	Operation and maintenance of advanced diagnostic equipment such as Computed Tomography (CT) Scanners, Magnetic Resonance Imaging (MRI) Scanners and Lithotripsy units through highly subsidized user-charge based models.
Therapeutic Services	Operation and maintenance of high-end therapeutic equipment including Linear Accelerators, Cobalt Therapy Units and Brachytherapy Systems under the Chief Minister's Comprehensive Health Insurance Scheme (CMCHIS).

Functional Area	Activities
Support to Flagship Government Programmes	Procurement and supply of materials required for schemes such as the Mother and Child Health Nutrition Kit, Mother and Baby Care Kit, Kannoli Kappom Thittam and Menstrual Hygiene Programme.
Hospital Support Services	Finalization of rate contracts for housekeeping, security, diet supply, biomedical waste disposal, oxygen supply and laboratory services for Government health facilities.

11.4. Drug Procurement Process

TNMSC's centralized procurement model enables the State to purchase quality-assured medicines at competitive rates through transparent tendering and supply them free of cost in Government hospitals. In addition, TNMSC procures and supplies medicines required for various veterinary healthcare institutions functioning under the Department of Animal Husbandry.

11.4.1. Drug Committee

The Drug Committee is responsible for finalizing the annual list of Essential Drugs, Specialty Drugs, Surgical items, Suture Consumables, and Anti-Haemophilic Drugs to be procured by TNMSC.

The Committee comprises:

- Managing Director, TNMSC
- Director of Medical Education and Research
- Director of Medical and Rural Health Services
- Director of Public Health and Preventive Medicine
- Director of Family Welfare

Specialists from Government Medical Institutions are invited to participate in the Committee meetings, whenever required, to provide expert opinion and recommendation.

11.4.2. Drug Procurement Framework:

- Procurement of drugs, medicines, surgical consumables, sutures and laboratory reagents are carried out through open e-tendering on the National Informatics Centre (NIC) platform.

- Tendering procedures are governed by the provisions of the Tamil Nadu Transparency in Tenders Act, 1998 and the Tamil Nadu Transparency in Tenders Rules, 2000.
- The procurement process involves detailed scrutiny of statutory and regulatory documents submitted by bidders, assessment of manufacturing capabilities, and inspection of manufacturing facilities to ensure compliance with prescribed quality standards and regulatory requirements.
- Annual and biannual rate contracts are finalized at competitive rates with multiple suppliers to ensure uninterrupted supply and mitigate procurement risks during emergencies.
- Quality assurance measures, including pre-dispatch by the suppliers and post-supply testing through empanelled laboratories of TNMSC, are adopted to ensure the supply of quality medicines and consumables to Government healthcare institutions.

- Based on consumption patterns, drugs are procured on a regular monthly basis to maintain adequate inventory levels to meet the requirement of Government healthcare institutions for a period of up to six months.

Table 11.1. Procurement of various category of Drugs

Category	Number of items procured
Essential Drugs	408
Specialty Drugs	400
Anti-Hemophilic drugs	7
Surgical and Suture Consumables	307
Laboratory Reagents	99
Veterinary Drugs (Animal Husbandry Department)	128

11.4.3. Financial Outlay for the Drug procurement during 2025–2026

TNMSC procured drugs, medicines and other healthcare products valued at Rs.1,193.14 crore for various Directorates as detailed below:

Directorate	Amount (Rs.in Crore)
Director of Medical Education and Research	356.11
Director of Medical and Rural Health Services	120.51
Director of Public Health and Preventive Medicine	361.51
National Health Mission	305.48
Director of Family Welfare	1.99
Directorate of Animal Husbandry	47.54
Total	**1,193.14**

11.4.4. Drug Distribution Management System (DDMS)

The Drug Distribution Management System (DDMS) is an integrated software platform developed by TNMSC for efficient management of drug procurement, warehousing, distribution and inventory across the District Drug warehouses.

Key Features

- Real-time monitoring of procurement, stock availability, distribution and consumption of drugs in the District Drug warehouses.

- Online indent processing, inter-warehouse transfers and supply chain tracking.
- Monitoring of drug expiry and inventory management.
- Tracking of supplier payments and quality control failed batches.
- Retrieval monitoring of drugs from health institutions in case of complaints.
- Generation of MIS reports for administrative review and decision-making.

11.4.5. Drug Warehousing facilities

A robust warehousing and distribution network forms the backbone of TNMSC's supply chain management system, ensuring the timely availability of quality medicines and medical consumables across Government healthcare institutions in the State.

TNMSC currently operates 37 drug warehouses across 36 districts, enabling efficient storage, inventory management and distribution of drugs. Leveraging consumption-based forecasting through the Drug Distribution Management System (DDMS), the Corporation maintains

adequate inventory levels to ensure availability of essential drugs for a minimum period of six months.

The warehousing network supports a streamlined distribution mechanism through the passbook system and facilitates equitable allocation of medicines based on the requirements and budgetary provisions of the respective Directorates. As part of its ongoing efforts to strengthen healthcare logistics infrastructure and enhance supply chain resilience, TNMSC is expanding its warehousing capacity through the establishment of additional warehouses at Anna Nagar (Chennai) and Coimbatore for which construction works are currently in progress.



New District Drug Warehouse, Kallakurichi



Storage area in the Warehouse

11.5. Mudhalvar Marunthagam Scheme

With the objective of making quality generic medicines available to the public at affordable prices through retail pharmacy outlets, the Mudhalvar Marunthagam Scheme was launched under the Cooperation, Food and Consumer Protection Department. Under the Scheme, 1,000 Mudhalvar Marunthagam retail pharmacy outlets are currently functioning across the State. Tamil Nadu Medical Services Corporation is responsible for the procurement and supply of generic medicines required for the implementation of that scheme.

For this scheme, TNMSC has finalized the rates for 374 generic medicines through an open tendering process and based on the requirement furnished by the Registrar of Cooperative Societies, medicines valued at Rs.43.25 crore was procured so far. After undergoing the prescribed quality testing procedures, the medicines are supplied to the district drug warehouses managed by the Registrar of Co-operative Societies and are subsequently distributed to the retail pharmacy outlets based on demand. To facilitate the storage and distribution of medicines under the Scheme, TNMSC has established four warehouses at Chennai, Chengalpattu, Thanjavur and Madurai.

11.6. Important ongoing Government Flagship Schemes

TNMSC plays a key role in the implementation of the following Government flagship programmes aimed at improving maternal health, child health and menstrual hygiene through timely procurement and supply of quality commodities across the State.

11.6.1. Maternal and Child Health Nutrition Kit

Under the Dr.Muthulakshmi Reddy Maternity Benefit Scheme, TNMSC supplied 12.10 lakh Maternal and Child Health Nutrition Kits during 2025–26 at a total outlay of Rs.275.55 crore, supporting improved nutritional outcomes for pregnant women.

11.6.2. Mother and Baby Care Kits

To strengthen post-natal care and newborn well-being, TNMSC supplied 4.09 lakh Mother and Baby Care Kits, comprising 16 essential items, at a total outlay of Rs.59.00 crore during 2025–26.

11.6.3. Menstrual Hygiene Programme

As part of the Menstrual Hygiene Programme, TNMSC supplied 7.53 crore beltless sanitary napkin packs to adolescent girls in rural and urban areas during 2025–26 at a total outlay of Rs.95.70 crore. Further, Belt-type sanitary napkins manufactured by Self-Help Groups were supplied through the Tamil Nadu Corporation for Development of Women for post-natal mothers, women prisoners and women in-patients in

Government health institutions. During the year 2025–26, 35.11 lakh Belt-type sanitary napkin packs were supplied at a total outlay of Rs.7.37 crore.

11.7. Procurement of Medical Equipment

Tamil Nadu Medical Services Corporation (TNMSC) is the nodal agency responsible for centralized procurement of medical equipment, healthcare goods and facility management services like Housekeeping, Security, Dietary services etc., through open tender procedures for the Government health Institutions in the State.

TNMSC procures a wide range of medical equipment with a 3-year warranty and a 7-year Comprehensive Annual Maintenance Contract (CAMC) to ensure sustained performance, reliability and service continuity.

11.7.1. Equipment Procurement Process Flow

The procurement process is carried out through a systematic and structured workflow to ensure transparency, accountability, and alignment with clinical requirements as follows:-



Table 11.2: Financial outlay for the Equipment procurement during Financial Year 2025-26

Directorate	Amount (Rs. in Crore)
National Health Mission	502.31
Directorate of Medical Education and Research	270.02
Directorate of Medical and Rural Health Services	244.65
Directorate of Public Health and Preventive Medicine	10.20
Tamil Nadu Health Systems Project	31.29
Other Departments	12.20
Total	**1,070.67**



Single Plane Cath Lab installed at Government Medical College Hospital, Tiruvallur.



Liquid Medical Oxygen tank installed at Government Mahathma Gandhi Memorial Hospital, Tiruchirappalli

11.8. Equipment Maintenance and Services

Tamil Nadu Medical Services Corporation Limited is responsible for the operation, maintenance and monitoring of advanced diagnostic and therapeutic medical equipment installed across Government health institutions in Tamil Nadu.

11.8.1. The major functions are

- Operation and maintenance of Computed Tomography (CT) Scanners, Magnetic Resonance Imaging (MRI) Scanners and Lithotripsy units through collection of highly subsidized user charges.

- Operation and maintenance of high-end therapeutic equipment such as Linear Accelerators, Telecobalt Therapy Units and Brachytherapy Units under the Chief Minister's Comprehensive Health Insurance Scheme (CMCHIS).

- Maintenance of medical equipment through Biomedical Engineers deployed across Government health institutions.

- Monitoring of equipment inventory, maintenance activities and lifecycle management through the dedicated online module namely, Equipment Maintenance and Management System (EMMS).

11.8.2. Operation and Maintenance of Medical Equipment

To ensure optimal performance and uninterrupted operation of medical equipment in Government healthcare institutions, the Biomedical Engineers of TNMSC coordinate with equipment suppliers and service providers for preventive maintenance and breakdown maintenance. They also attend to minor faults at the institutional level, facilitating prompt restoration of services and minimizing equipment downtime.

11.8.3. Equipment Maintenance Management System (EMMS)

TNMSC has implemented the Equipment Maintenance Management System (EMMS), a web-based platform for effective monitoring and maintenance of medical equipment installed in Government healthcare institutions across the State.

11.8.4. The EMMS facilitates

- Overall management of the equipment lifecycle, including indent updation, procurement, installation, maintenance, performance monitoring and vendor bill payment processing.
- Monitoring of the functionality status of medical equipment.
- Tracking of preventive maintenance schedules and service compliance.
- Management of breakdown maintenance.
- Monitoring of equipment lifecycle, including condemnation status.
- Unique identification and tracking of equipment through a 15-digit identification number and QR code-based system.

11.8.5. Management of Diagnostic Equipment

Access to advanced diagnostic investigations is critical for early diagnosis, timely treatment and improved patient outcomes. TNMSC operates and maintains advanced diagnostic equipment in Government healthcare institutions to ensure affordable and equitable access to these services.

TNMSC currently operates and maintains 140 CT Scanners in 105 Government health institutions and 51 MRI Scanners in 49 Government health institutions across the State. These services are provided at highly subsidized user charges, substantially reducing the financial burden on patients. Beneficiaries covered under the Chief Minister's Comprehensive Health Insurance Scheme (CMCHIS) are provided CT and MRI scan services on a cashless basis. 7 PET-CT Scanners are also being operated through the Public Private Partnership (PPP) mode within the Government healthcare system.

Table 11.3: The performance of CT and MRI Scanners during the financial year 2025-2026

Equipment Name	Number of scan machines in operation	Number of scans taken (In lakh)
CT scan	140	22.49
MRI scan	51	2.55



CT Scan equipment at Institute of Mental Health, Chennai



MRI Scan equipment at Government Medical College Hospital, Kanniyakumari.

Table 11.4: The details of User Charges for the diagnostic Services

Investigation Type	User charges
CT Scan – Plain Study	Rs.500/-
CT Scan – Contrast Study	Rs.800/- (Rs.500 + Rs.300 (contrast charge))
CT Angiogram	Rs.3,000/-
MRI – Plain Study	Rs.2,500/-
MRI – Contrast Study	Rs.4,000/- (Rs.2,500+Rs.1,500 (contrast charge))

Table 11.5: The performance of therapeutic equipment during the financial year 2025-2026

Equipment Name	Number of Machines in Operation	Number of Patients Treated
Linear Accelerators	12	7,924
Brachytherapy Units	09	2,790
Telecobalt Therapy Units	14	1,893
Lithotripsy Units	04	1,625

11.8.6. Management of various Therapeutic Equipment

To ensure access to advanced cancer care and other specialized therapeutic services, TNMSC

install, operates and maintains high-end therapeutic equipment in Government Medical College Hospitals under the Chief Minister's Comprehensive Health Insurance Scheme (CMCHIS).



Linear Accelerator equipment at Rajiv Gandhi Government General Hospital, Chennai.

11.9. Other Facility Management Services for Hospitals

11.9.1. Providing Housekeeping and Security Services

TNMSC has providing housekeeping and security services through a transparent tendering process for the health institutions under the Directorate of Medical Education and Research and Directorate of Medical and Rural Health Services across Tamil Nadu.

11.9.2. Providing Dietary Services to the in-patients of Government Hospitals

To ensure the provision of quality and nutritious food to in-patients in Government healthcare institutions, TNMSC has established a rate contract with the private vendors through an open tendering process for outsourcing dietary services covering six categories of prescribed diets. These services are currently provided to certain health institutions namely, Tamil Nadu Omandurar Government Multi Super Specialty Hospital, Chennai, Kalaignar Centenary Super Specialty Hospital, Chennai and the Government Peripheral Hospital, Periyar Nagar, Chennai.

11.10. Institutional Recognition and Knowledge Sharing

TNMSC's procurement and supply chain management framework has emerged as a role model in the country for ensuring transparency, quality assurance, and cost efficiency in the public healthcare management system.

During 2025–26, Senior officials and technical teams from the following organizations have visited TNMSC to study its procurement, quality assurance, warehousing, inventory management and logistics systems.

- Andhra Pradesh Medical Services and Infrastructure Development Corporation
- Maharashtra Medical Goods Procurement Authority
- Chhattisgarh Medical Services Corporation Limited
- Employees State Insurance Corporation (ESIC), Government of India
- Uttar Pradesh Medical Supplies Corporation Limited and
- Odisha State Medical Corporation Limited

These study visits facilitated knowledge sharing and exchange of best practices among the States.

11.11. Conclusion

Through the above interventions, TNMSC contributes to affordable, equitable, high-standard and easily accessible healthcare for the people of Tamil Nadu. Further, Tamil Nadu Medical Services Corporation Limited (TNMSC) emerging as a leading organization in public healthcare procurement and supply chain management in the Country.

CHAPTER - 12

TAMIL NADU STATE AIDS CONTROL SOCIETY

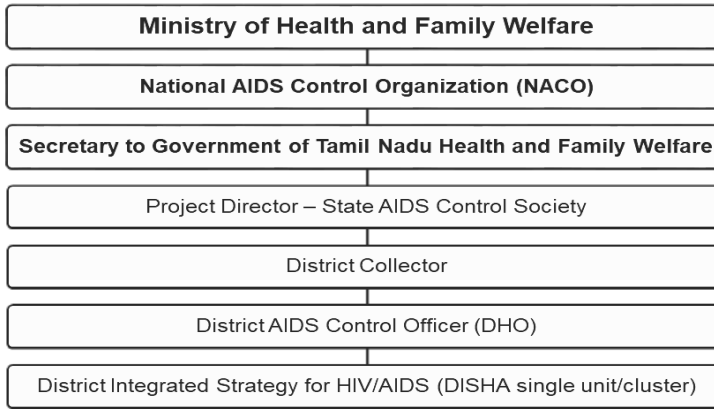
12.1. Introduction

Tamil Nadu State AIDS Control Society (TANSACS) was established on 22nd April 1994 and registered as a Society under the Tamil Nadu Societies Act, 1975, with the mandate to prevent and control the spread of Human Immuno-Deficiency Virus (HIV) and to provide comprehensive care and support to persons affected by Acquired Immuno-Deficiency Syndrome (AIDS). TANSACS implements the HIV/AIDS Control Programme across the State in accordance with the guidelines of the National AIDS Control Programme (NACP) under the National AIDS Control Organization (NACO).

12.1.1. Vision

TANSACS remains steadfastly committed to achieving the Sustainable Development Goal 3.3 – Ending AIDS as a public health threat by 2030, and to the elimination of parent-to-child transmission of HIV, Syphilis and Hepatitis B (EVTHS), in alignment with the UNAIDS 2030 Goals.

12.2. Administrative Structure



12.2.1. Headquarters

The Secretary to Government, Health and Family Welfare Department, serves as the Chairperson of Tamil Nadu State AIDS Control Society (TANSACS). The organisation is headed by the Project Director and Member-Secretary, who is assisted by Programme Officers in the ranks of Joint Directors, Deputy Directors, and Assistant Directors.

12.2.2. District Level Administrative Structure – DISHA

At the district level, TANSACS operates through the District Integrated Strategy for HIV/AIDS (DISHA) – formerly the District AIDS

Prevention and Control Unit (DAPCU). Each DISHA Unit is managed by the District Health Officer (DHO), who concurrently serves as the District AIDS Control Officer (DACO), under the chairpersonship of the District Collector. The DACO/DHO is supported by:

- District Programme Manager / Cluster Programme Manager
- District ICTC Supervisor / Clinical Services Officer
- District Assistant (Programme, Finance, M&E) / Data Monitoring and Documentation Officer

Key structural developments in district-level operations are as follows:

- DAPCUs have been revamped as DISHA Cluster/Unit (District Integrated Strategy for HIV/AIDS).
- From 2024-25 onwards, the 38 districts have been reorganised into 17 Clusters for the effective implementation of NACP Phase-V activities, based on epidemiological profile,

geographic homogeneity, and ease of operational management.

- ❖ Currently, 2,677 personnel are engaged at the district and facility levels as Contractual / Outsourcing staff.

12.3. TANSACS District Facilities

TANSACS manages the HIV/AIDS prevention-to-care continuum through an extensive network of HIV Counselling and Testing Services and Anti-Retroviral Therapy (ART) Centres across the State.

Table 12.1 TANSACS District Facilities

Sl. No.	Details of TANSACS District Facilities	Nos.
1.	Integrated Counselling and Testing Centres (ICTCs)	3,002
	a. NACO Supported Confirmatory Facilities – ICTC including 4 Sampoorna Suraksha Kendra	295
	b. Mobile ICTC (State Owned)	16
	c. Mobile ICTC – Outsourced Vehicles	16

Sl. No.	Details of TANSACS District Facilities	Nos.
	d. PPP Model ICTCs	19
	e. Screening Facility ICTCs (FICTCs)	2,253
	f. NHM Supported Confirmatory Facilities – ICTCs	403
2.	Designated STI/RTI Clinics (DSRCs)	106
3.	Anti-Retroviral Therapy (ART) Centres	86
4.	Link ART Centres (Drug Dispensing Units)	172
5.	Prevention Services (Targeted Intervention – NGOs/CBOs)	87
6.	Link Workers Scheme (LWS – NGO)	17
7.	Opioid Substitution Therapy (OST) Centre	1

12.4. National AIDS Control Programme – Phase V (NACP-V)

NACP-V came into force with effect from April 2021 up to March 2026 and continues to be

implemented. The overarching goal of NACP-V is the achievement of the globally recognised "95-95-95" targets by March 2026, which stipulate that:

- 95% of all people living with HIV should know their HIV status.
- 95% of those who know their HIV status should be on Anti-Retroviral Treatment (ART).
- 95% of those on ART should achieve viral load suppression.
- 95% of pregnant and breastfeeding women living with HIV should have suppressed viral load, towards the elimination of vertical transmission of HIV.
- <10% of PLHIV and key populations should be free from stigma and discrimination.
- Provision of comprehensive services for Sexually Transmitted Infection (STI) / Reproductive Tract Infection (RTI).

12.4.1. Performance of Tamil Nadu

The progress achieved by Tamil Nadu under the 95-95-95 framework, in comparison with national performance, as on March 2026 (SOCH Data), is as follows:

Table 12.2 Performance of Tamil Nadu in achieving 95-95-95

Target	Description	Tamil Nadu
First 95	% of PLHIV who know their HIV status	94.6%
Second 95	% of known PLHIV currently on ART	89.9%
Third 95	% of PLHIV on ART with suppressed viral load	95.9%

Tamil Nadu's performance consistently surpasses the national average across all three targets, reflecting the State's sustained commitment to HIV care and treatment.

12.4.2. AIDS Suraksha – Mobilisation for AIDS Suraksha (MAS) Strategy

To fast-track progress towards the goals of NACP-V, the National AIDS Control Organisation (NACO) introduced the Mobilisation for AIDS Suraksha (MAS) strategy in January 2026. Under

MAS, the targets have been further scaled up from 95-95-95 to 95-95-99, along with the triple elimination of HIV, Syphilis and Hepatitis B.

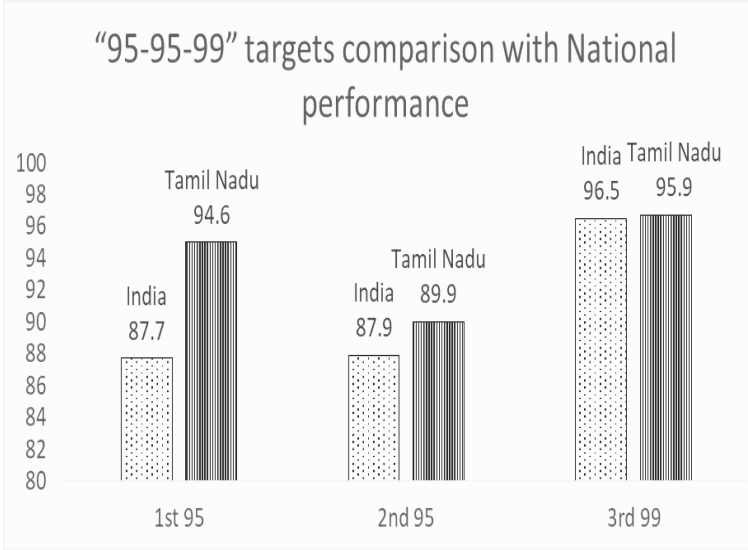
The MAS Strategy aims to achieve the 95-95-99 and triple elimination targets by 1st December 2026, and to sustain this status until World AIDS Day, 1st December 2027, thereby effectively bringing the HIV epidemic under control. All districts have been identified as key implementation units.

Based on target achievement, districts are classified into four categories under MAS:

S. No.	MAS Category	Target Achievement Criteria
1.	Surakshit+ (More Protective)	Districts achieving all three 95-95-99 targets and zero transmission of HIV, Syphilis, and Hepatitis B from mother to child (Triple Elimination).
2.	Surakshit (Protective)	Districts that have achieved all three 95-95-99 targets.
3.	Saksham (Achievable)	Districts that have achieved any two of the 95-95-99 targets.
4.	Sangharshsheel (Backward)	Districts that have achieved only one or none of the 95-95-99 targets.

The goal of the MAS Strategy is to ensure that all districts are declared as Surakshit+ Districts by 1st December 2026.

Fig 12.1 "95-95-99" targets-comparison with National performance



[**Source**: from SOCH Portal (April 2025-March 2026)]

12.5. HIV Prevalence in Tamil Nadu

HIV Sentinel Surveillance (HSS) is conducted once in two years among pregnant women, High Risk Groups (HRGs), Prisoners, and People Living

with HIV/AIDS (PLHIVs), to determine the prevalence rate of HIV across the State.

12.5.1. HIV Prevalence among Ante Natal (AN) Mothers

Tamil Nadu has successfully brought down the HIV prevalence rate among Ante Natal mothers from 1.13% in 2001-02 to 0.16% in 2022-23, through the effective participation and commitment of all stakeholders. The prevalence among Ante Natal Mothers is taken as the proxy indicator for HIV prevalence in the general population.

Tamil Nadu's HIV prevalence among ANC mothers (0.16%) is significantly below the national average (0.23%), evidencing the State's effective HIV prevention strategies.

12.5.2. Adult HIV Prevalence (Age Group 15–49 Years)

As per Sankalak 7th Edition (2025), the adult HIV prevalence in Tamil Nadu stands at 0.19%, which is lower than the national adult HIV prevalence of 0.20%.

12.5.3 HIV Incidence per 1000 Uninfected Population

HIV incidence refers to the number of new HIV infections occurring in a given year. Tamil Nadu has one of the lowest HIV incidence rates in the country.

Year	Tamil Nadu	India
2025	0.02 per 1000	0.05 per 1000

[**Source:** Sankalak – 7th Edition & HIV Estimation Technical Report 2025, NACO]

12.5.4 Mother to Child Transmission (MTCT) Rate of HIV

The MTCT Rate measures the proportion of children born to HIV-positive mothers who acquire HIV infection. Triple Elimination benchmarks a MTCT rate of below 5%.

Year	Tamil Nadu	India
2025	4.82%	11.75%

[**Source:** Sankalak – 7th Edition & HIV Estimation Technical Report 2025, NACO]

Tamil Nadu has achieved the MTCT rate below the 5% threshold – meeting the Triple Elimination benchmark – and records the lowest MTCT rate in the country.

12.5.5. AIDS-Related Deaths per 1,00,000 Population

With the provision of free Anti-Retroviral Therapy (ART) under NACP, a sustained declining trend in AIDS-related deaths has been observed across Tamil Nadu.

Year	Tamil Nadu	India
2025	1.69 per 1,00,000	2.39 per 1,00,000

[**Source**: Sankalak – 7th Edition & HIV Estimation Technical Report 2025, NACO]

12.6. Programmes Implemented

The key programme activities of TANSACS are organised under the following divisions:

12.6.1. Prevention Services – Targeted Intervention (TI)

Targeted Intervention (TI) is implemented through Non-Governmental Organisations (NGOs) and Community Based Organisations (CBOs), with

the objective of bringing about sustainable behavioural changes among High Risk Groups (HRGs) – namely Female Sex Workers (FSW), Men who have Sex with Men (MSM), People who Inject Drugs (PWID), Transgender persons (TG), and bridge populations (Migrants and Transport Workers) – who are at elevated risk of HIV infection.

There are 87 TI-NGOs/CBOs, 17 Link Workers Scheme (LWS) NGOs, and 1 Opioid Substitution Therapy (OST) Centre operational in the State.

Coverage of Registered Urban-based Population under 87 Targeted Intervention NGOs/CBOs (as on March 2026)

Table 12.3: Targeted Intervention Coverage

S.No.	Description	Population
1	High Risk Group (HRG) Populations Registered	93,467
	a. Female Sex Workers (FSW)	48,681
	b. Men having Sex with Men (MSM)	35,392

S.No.	Description	Population
	c. Transgender (TG)	8,630
	d. People who Inject Drugs (PWID)	764
2	Bridge Population Registered	1,31,328
	a. Migrants	67,404
	b. Truckers	63,924
Total		**2,24,795**

12.6.2 Link Workers Scheme (LWS)

The Link Workers Scheme (LWS) is implemented through NGOs across 100 villages each in 17 high-priority districts, to prevent STI, HIV/AIDS, and TB. The Scheme aims at providing a prevention-to-care continuum of services to rural-based HRGs, other Vulnerable populations, and Bridge populations (Migrants and Truckers).

Coverage of Registered / Identified Rural-based Population under LWS NGOs (Financial Year 2025-26, up to March 2026):

Table 12.4: HRG Coverage through LWS

S.No.	Description	Population
1.	High Risk Group Individuals	6,604
	a. Female Sex Workers (FSW)	5,352
	b. Men having Sex with Men (MSM)	1,160
	c. Transgender (TG)	92
2.	Migrants	1,23,648
3.	Truckers	67,849
4.	Other Vulnerable Population (OVP)	1,64,723
Total		**3,62,824**

Among the total High-Risk Group population covered during 2025-26, 3,96,742 individuals were tested for HIV, of whom 660 were found to be HIV positive.

12.6.3. Condom Promotion

As condoms remain the most effective means of preventing HIV and STI transmission, TANSACS provides free condoms to the population through DSRCs, ICTCs, ART Centres, and outreach programmes implemented by NGOs/CBOs under TI and LWS.

12.6.4. HIV Counselling and Testing Services (HCTS)

In Tamil Nadu, HIV Counselling and Testing Services are provided through 3,002 centres located across various healthcare facilities, all branded as Nambikkai Mayyam. Of these, 2,656 centres function as Screening Facility ICTCs (FICTCs) and 346 as Confirmatory Centres.

HIV Testing Coverage (April 2025 to March 2026):

- 23.59 lakh General Clients were screened for HIV.
- 9.52 lakh Antenatal Mothers were screened under HCTS.

12.6.5. Mobile Integrated Counselling and Testing Centres (Mobile ICTCs)

To extend counselling and testing services to remote and hard-to-reach areas, 32 Mobile ICTC Vans have been deployed by TANSACS. Each mobile van is manned by a trained Counsellor and Laboratory Technician. These vehicles are operated by the respective DISHA Units and Clusters.

- State-Owned Mobile ICTC Vans: 16
- Outsourced Mobile ICTC Vans: 16

12.6.6 Sampoorna Suraksha Kendra (SSK)

During 2025-26, 4 Sampoorna Suraksha Kendra Centres have been established. These centres provide continuous surveillance and comprehensive services to HIV-negative individuals who remain at risk of infection.

12.6.7 Triple Elimination – Elimination of Vertical Transmission of HIV, Syphilis and Hepatitis B (EVTHS)

The Triple Elimination Programme aims to eliminate HIV, Syphilis, and Hepatitis B infections among infants through the universal screening of pregnant women for all three conditions, as an essential component of the Antenatal Care (ANC) service package, pursuant to the commitment of the Government of Tamil Nadu.

Key interventions under EVTHS include

- ❖ All identified HIV-positive mothers are provided with lifelong Anti-Retroviral Therapy (ART) and follow-up counselling services.
- ❖ ARV Prophylaxis is administered to all babies born to HIV-positive mothers to prevent vertical transmission.

- ❖ Universal screening for Syphilis and Hepatitis B is integrated into the ANC package.

12.6.8. Sexually Transmitted Infection (STI) / Reproductive Tract Infection (RTI) Services

STI/RTI services are rendered through 106 NACO-supported Designated STI/RTI Clinics (DSRCs), established at Government Medical College Hospitals, Government District Headquarters Hospitals, and Government Hospitals across Tamil Nadu. These DSRCs are branded as Suga Vazhvu Maiyam. Trained Counsellors and Medical Officers provide quality STI services to both General Clients and High-Risk Groups. Syndromic Case Management is followed at all DSRCs to ensure prompt and effective treatment of all STIs among clients and their partners.

12.6.9. Care, Support and Treatment (CST)

All persons diagnosed as HIV-positive at an ICTC are enrolled at Anti-Retroviral Therapy (ART) Centres for lifelong, comprehensive HIV care, support, and treatment, provided free of cost. ART services are delivered through ART Centres attached to Government Hospitals, Government

Medical College Hospitals, and Private Medical College Hospitals.

- ❖ Routine investigations, CD4 tests, and Viral Load testing are performed free of cost.
- ❖ ARV drugs, Opportunistic Infection drugs, Counselling services, and Referral and Linkage services are provided.
- ❖ ART Centres and Viral Load Laboratories have been equipped with Barcode Machines and Scanners for enhanced service delivery.

Table 12.5: Distribution of ART Centres

Category	Number
Government Medical College Hospitals (GMCH) including attached Institutions	38
Private Medical College Hospitals	28
Government District Headquarters Hospitals (GHQH)	9
Government Hospitals (GH)	11
Total ART Centres	86
Link ART Centres (Drug Dispensing Units)	172

People Living with HIV/AIDS (PLHIV) on Anti-Retroviral Therapy as on March 2026 (as per SOCH Report):

Male	Female	Trans Sexual / Transgender (TS/TG)	Children (Male)	Children (Female)	Total
66,501	69,362	509	490	400	1,37,262

31 Care and Support Centres (CSCs), run by NGOs/CBOs, are linked to ART Centres to carry out functions such as tracking of treatment defaulters, following up clients newly initiated on ART, monitoring individuals overdue for viral load testing, linking clients to various social benefit schemes, and providing psychosocial support.

12.6.10. Information, Education, Communication and Mainstreaming (IEC & MS)

Under NACP-V, TANSACS disseminates targeted and consistent messaging on HIV and STI prevention, risk reduction, prevention of mother-to-child transmission, condom promotion, and reduction of stigma and discrimination. These awareness activities effectively reach the general

population, adolescents and youth, and targeted populations, contributing steadily to the decline in HIV prevalence in the State.

12.6.10.1. Red Ribbon Clubs (RRC)

Red Ribbon Clubs have been formed in 2,379 colleges among student communities, including Arts and Science, Polytechnic, Engineering, Medical, B.Ed. Colleges, and Teacher Training Institutions, to promote HIV/AIDS and STI awareness, risk perception, and behavioural change.

12.6.10.2. Life Skill Education Programme (LSEP) in Schools

The Life Skill Education Programme for adolescents in schools is conducted through the State Council of Educational Research and Training (SCERT) and currently covers 9,930 schools across Tamil Nadu.

12.6.11. Strategic Information – Monitoring and Evaluation (M&E)

The Strategic Information Division of TANSACS monitors programme progress through evidence-based data collection and analysis. The

Strengthening Overall Care for HIV Beneficiaries (SOCH) platform enables real-time online reporting and monitoring of programme progress.

HIV Sentinel Surveillance (HSS) is conducted once in two years among pregnant women, HRGs, Prisoners, and PLHIVs. It facilitates an understanding of the dynamics of the HIV epidemic, monitors trends across population groups and geographies, and strengthens prevention and control activities. HSS is conducted as a sample-based survey at 82 Ante Natal Care sites, 4 prison sites, 46 HRG sites, and 8 ART centres.

12.7. State-Sponsored Schemes

The Government of Tamil Nadu extends welfare support to People Living with HIV/AIDS (PLHIVs) through two dedicated beneficiary schemes:

12.7.1. Tamil Nadu Trust for Children Affected with HIV/AIDS (TNTCAA)

Orphans and Vulnerable Children (OVC) affected by HIV/AIDS often face barriers to education and are at heightened risk of nutritional

and health deficiencies. Recognising this, the Government of Tamil Nadu established the Tamil Nadu Trust for Children Affected by HIV/AIDS (TNTCAA) during 2009-10 with an initial corpus fund of Rs. 5 Crore, subsequently enhanced to Rs.25 Crore, deposited with the Tamil Nadu Power Finance Corporation.

The interest accrued on this corpus fund is utilised to provide annual financial assistance to children infected and affected by HIV/AIDS, from 3 years of age up to 18 years, for their educational, medical, and nutritional needs.

From 2025-26, the Government of Tamil Nadu has sanctioned monthly financial assistance of Rs. 1,000/- to OVC children who have registered on the OVC Portal. During 2025-26, an amount of Rs.7.75 Crore has been sanctioned as monthly financial assistance to eligible OVC beneficiaries (June 2025 to March 2026).

12.7.2. Drop-in Centres for People Living with HIV/AIDS (DICs)

The Government of Tamil Nadu has sanctioned Rs. 2.96 Crore for the operation of 34

Drop-in Centres (DICs) through HIV Positive Networks. These Centres provide a range of psychosocial and support services to PLHIVs.

Table 12.6: Services rendered at Drop-in Centres (April 2025 to March 2026)

Sl. No.	Services Rendered	No. of Persons Benefitted
1.	Psychosocial Support to PLHIVs	9,024
2.	Sustainable Counselling and Support Services	66,024
3.	Linkage to Government Welfare Schemes	7,333

12.8. Achievements

- ❖ TANSACS was recognized as the 1st Place among the biggest States across India by the National AIDS Control Organisation (NACO), Ministry of Health and Family Welfare, Government of India, for effective implementation of NACP activities from April to September 2024.
- ❖ Tamil Nadu is recognized for maintaining 96% viral load suppression rate, surpassing the national levels.

- ❖ Tamil Nadu's MTCT Rate of 4.82% is below the Triple Elimination threshold of 5%, making it the lowest in the country.
- ❖ 3 National Reference Laboratories (NRLs) and 12 State Reference Laboratories (SRLs) function in the State to monitor the quality of HIV testing, under NACO. All laboratories are accredited by the National Accreditation Board for Testing and Calibration Laboratories (NABL).
- ❖ Tamil Nadu has achieved an HIV incidence rate of 0.02 per 1,000 population – the lowest in the country – reflecting the effectiveness of the State's prevention strategies.

12.9. Financial Outlay 2026-27

Table 12.7: Budget under Government of India (NACO) Funding

Description	Amount (Rs.in Crore)
Total Approved Budget for 2025-26	120.00
Total Allotted Budget for 2025-26	110.54
Total Expenditure for 2025-26	110.52 (99.9%)

The NACP-V programme will continue into the Financial Year 2026-27, with the continuation of all existing initiatives and the scaling up of the MAS Strategy towards achieving the 95-95-99 and Triple Elimination targets.

12.10. Tamil Nadu State Blood Transfusion Council (TNSBTC) – Blood Transfusion Services

12.10.1. Introduction

The National Blood Transfusion Council (NBTC) is the apex policy-formulating body for all matters related to the operation of blood centres across India. The Blood Transfusion Services (BTS) and NBTC were transitioned from the National AIDS Control Organization (NACO) to the Directorate General of Health Services (DGHS) on 31st March 2021.

The Tamil Nadu State Blood Transfusion Council (TNSBTC), established and registered under the Societies Registration Act (Registration No. 79 of 1997), serves as the State Nodal Agency for all matters relating to Blood Transfusion Services in Tamil Nadu. TNSBTC formally links all

blood centres (banks) in the State and monitors their functioning to ensure the availability of adequate, safe, and high-quality blood and blood components to meet public health needs.

12.10.2. Vision

- ❖ Achieve 100% blood collection from voluntary, non-remunerated donors.
- ❖ Promote the rational use of blood and blood components to enhance patient care, by providing safe blood products to patients in need, free of cost.

12.10.3. Blood Transfusion Services

Tamil Nadu is funded by the Directorate General of Health Services (DGHS), Ministry of Health and Family Welfare, Government of India, New Delhi. TNSBTC supports 103 Blood Centres operating in Government Medical College Hospitals and Government District Headquarters Hospitals across Tamil Nadu.

Table 12.8: Details of Blood Centres and Blood Component Separation Units in the State

Category	Number of Blood Centres	Centres with Component Separation Unit
Government Blood Centres	112*	49
Private Blood Centres	248	173
Total	**360**	**222**

* Including central Institutions

12.10.4. Achievement of Blood Unit Collection

The World Health Organization (WHO) recommends that at least 1% of a country's population should donate blood to meet minimum clinical requirements. With a Mid-Year Population Estimates of 7.75 crore, Tamil Nadu requires a proportionate blood collection target of about 7.75 lakh units; the 9.54 lakh units collected in 2025-2026 exceed the WHO norm. During 2025-26, TNSBTC has achieved a total blood collection of 9.54 lakh units (Government – 4,51,324 units; Private – 5,02,314 units), representing 118% of the assigned target – significantly exceeding the State target.

In addition, a target of conducting 4,400 Voluntary Blood Donation Camps for 2025-26 was set by the DGHS, New Delhi. TNSBTC has conducted 4,874 camps, achieving 111% of the target, reflecting the enthusiastic community participation and robust administrative coordination.

12.11 Conclusion

Tamil Nadu State AIDS Control Society (TANSACS) has made remarkable strides in combating HIV/AIDS in recent years, driven by robust community engagement, targeted interventions, and sustained public health efforts in fast-track mode. The State's performance across all NACP-V indicators – HIV prevalence, ART coverage, viral load suppression, MTCT rates, and AIDS-related deaths – consistently surpasses the national average, underscoring the effectiveness and efficiency of Tamil Nadu's HIV response.

With the introduction of the MAS Strategy and the State's unwavering commitment to the 95-95-99 targets and Triple Elimination, Tamil Nadu is well-positioned to achieve the

Sustainable Development Goal 3.3 – ending AIDS as a public health threat by 2030.

Tamil Nadu State Blood Transfusion Council achievements in voluntary blood collection and the establishment of quality-accredited laboratories further reinforce the State's comprehensive public health infrastructure.

The Government of Tamil Nadu remains resolute in its commitment to protecting the health, dignity, and well-being of all its citizens, particularly those most vulnerable to HIV infection, and to building a healthier, more equitable Tamil Nadu.

CHAPTER - 13

MEDICAL SERVICES RECRUITMENT BOARD

The Medical Services Recruitment Board (MRB) was formed in January 2012 as per G.O.Ms.No.1, Health and Family Welfare (C2) Department, dated 02.01.2012.

The role of the Board is to conduct all direct recruitments in order to fill up vacancies in a speedy manner to various categories of Medical and Para Medical posts in the Health and Family Welfare Department based on the Indents received from the Head of the Departments for ensuring timely provision of health care services to the needy public. The Medical Services Recruitment Board commenced functioning with effect from 06.02.2012.

The Board has been sanctioned with a total of 23 posts and is functioning at DMS Complex, 7th Floor, DMS Building, Teynampet, Chennai 6.

13.1. Recruitments for Various Medical and Para Medical Posts under Health and Family Welfare Department

The Medical Services Recruitment Board conducts recruitments transparently by

advertisement in the Newspapers, collecting the applications on-line, conducting the Computer Based Test (CBT) Examinations and publishing the results in the Medical Services Recruitment Board's Website.

13.2. Method of Recruitment

1. Computer Based Test (Online Mode – in Single Session)
2. Weightage Method (based on Marks scored by the Candidates in their academic performance and technical qualifications)

- ❖ The Selection of Candidates are purely based on marks scored and no oral test is conducted for any of the Recruitments.

- ❖ Reservation Policy and the Service Rules are adhered to with utmost care and transparency at every stage of the recruitment process.

13.3. Achievements

The Medical Services Recruitment Board has so far recruited 48,083 candidates for 51 categories of posts which includes 19,673 Assistant Surgeons, 13,294 Nurses and 15,116 Allied & Health Care Staff since its inception till 31.03.2026.

S. No.	Name of the post	Total
1.	Anaesthesia Technician	77
2.	Assistant Medical officer (Ayurveda)	16
3.	Assistant Medical officer (Homoeopathy)	19
4.	Assistant Medical officer (Siddha)	297
5.	Assistant Medical officer (Unani)	9
6.	Assistant Medical Officer / Lecturer Grade II (Yoga & Naturopathy)	126
7.	Assistant Surgeon (Dental - General)	138
8.	Assistant Surgeon (Dental - Speciality)	36
9.	Assistant Surgeon (General)	14,684
10.	Assistant Surgeon (General - Special Qualifying Examination)	1,984
11.	Assistant Surgeon (Speciality)	2,292
12.	Audiometrician	27
13.	Dark Room Assistant	432
14.	Dental Hygienist	38
15.	Dialysis Technician Grade - II	160
16.	ECG Technician	130
17.	EEG / EMG Technician	22
18.	Field Assistant	203
19.	Food Safety Officer	128
20.	Health Inspector Grade-II	1,401
21.	Heart Lung Hypothermia Machine Technician	7

S. No.	Name of the post	Total
22.	Junior Analyst	40
23.	Lab. Technician Grade II (Food Safety Department)	22
24.	Laboratory Technician - Grade II	528
25.	Laboratory Technician - Grade III	2,735
26.	Medical Physicist	34
27.	Nurses	12,779
28.	Nurses (Special New Born Care Unit)	515
29.	Occupational Therapist	25
30.	Ophthalmic Assistant	83
31.	Personnel for Tamil Nadu Government Multi Super Speciality Hospital	72
32.	Pharmacist	1,985
33.	Pharmacist (Ayurveda)	44
34.	Pharmacist (Homoeopathy)	25
35.	Pharmacist (Siddha)	218
36.	Pharmacist (Unani)	22
37.	Physician Assistant	12
38.	Physiotherapist Grade-II	174
39.	Plaster Technician Grade II	87
40.	Prosthetic craftsman	63

S. No.	Name of the post	Total
41.	Radiographer	287
42.	Radiotherapy Technician	25
43.	Senior Analyst	15
44.	Senior Lecturer in Optometry	2
45.	Skilled Assistant (Electrician Grade II)	3
46.	Skilled Assistant (Fitter Grade II)	159
47.	Skilled Assistant (Welder Grade II)	4
48.	Technician Grade - I	3
49.	Theatre Assistant	334
50.	Therapeutic Assistant (Male and Female)	221
51.	Village Health Nurse	5,341
Total		**48,083**

13.4. Achievements During 2025-2026

During 2025-2026, a total of 6,050 posts under 23 categories were recruited by this Board and allotted to the various Heads of the departments.

Sl. No.	Name of the post	No. of candidates Selected
1.	Assistant Surgeon (General)	1,071
2.	Assistant Surgeon (General) with Speciality Qualification	250
3.	Assistant Surgeon (Special Qualifying Examination)	116
4.	Assistant Medical Officer (Siddha)	84
5.	Assistant Medical Officer (Ayurveda)	10
6.	Assistant Medical Officer (Homoeopathy)	2
7.	Assistant Medical Officer (Unani)	1
8.	Assistant Medical Officer/Lecturer Grade-II (Yoga and Naturopathy)	53
9.	Assistant Surgeon (Dental)	48
10.	Prosthetic Craftsman	1
11.	Dark Room Assistant	5
12.	Senior Analyst	15
13.	Laboratory Technician Grade-III	12
14.	Laboratory Technician Grade-II (FSO)	3
15.	Skilled Assistant Grade-II (Welder Grade-II)	1
16.	Skilled Assistant Grade-II (Fitter Grade-II)	20

Sl. No.	Name of the post	No. of candidates Selected
17.	Health Inspector Grade-II	1,401
18.	Dental Hygienist	37
19.	Nurses (Special New Born Care Unit)	7
20.	Ophthalmic Assistant	83
21.	Field Assistant	40
22.	Audiometrician	9
23.	Village Health Nurse	2,781
Total		**6,050**

13.5. On-Going Recruitments

For the year 2026 – 2027, the Board has proposed to recruit 2,126 posts under 16 categories.

Sl. No.	Name of the post	Tentative vacancies
1.	Pharmacist (Siddha)	131
2.	Pharmacist (Ayurveda)	3
3.	Pharmacist (Unani)	6
4.	Pharmacist (Homoeopathy)	1
5.	Pharmacist	630
6.	Laboratory Technician Grade-II	60

Sl. No.	Name of the post	Tentative vacancies
7.	Dark Room Assistant	22
8.	Radiographer	67
9.	Dental Mechanic	43
10.	Nursing Assistant Grade-II	999
11.	Therapeutic Assistant (Male & Female)	6
12.	Assistant Surgeon (Dental with Speciality Qualification)	31
13.	Radiotherapy Technician	15
14.	Ophthalmic Assistant	102
15.	Refractionist	6
16.	Grief Counsellor	4
Total		**2,126**

13.6. Initiatives taken to strengthen the system

- ❖ 100% Computer Based Test for the conduct of Examination in SINGLE session.
- ❖ Migration of Website from National Informatics Centre to State Data Centre for instant updates on Notification, Results, Syllabus, and G.O.'s.,

- ❖ Introduced the upload of important Hon'ble Court Judgments, Mock test for Computer Based Test, Instruction to candidates, etc., in the official website.

- ❖ Integrated MRB website in the Recruitment related Mobile app "Thervukkalam''

- ❖ The long-time request of the candidates for editing the submitted application has been implemented for the first time by providing the Edit option in the submitted applications.

- ❖ Introduced Software based Communal Roster fitment for arriving foolproof Certificate Verification List and Provisional Selection List in compliance with Tamil Nadu Conditions of Service Act, 2016.

13.7. New Proposals

Upon the receipt of Indents from the Head of the Departments, additional categories of posts will be notified in the Board's website and Annual Planner for 2026-2027 will be updated accordingly.

CHAPTER – 14

TAMIL NADU STATE HEALTH TRANSPORT

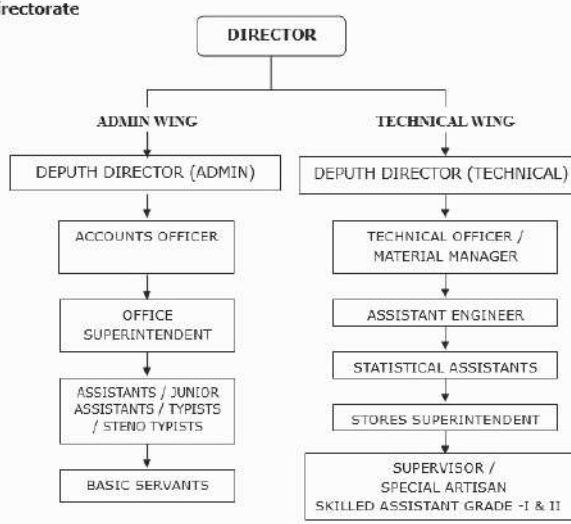
14.1. Introduction

The Tamil Nadu State Health Transport Department (TNSHTD) functions as the technical and logistical backbone of the transport system of the Health and Family Welfare Department. It maintains and manages a fleet of over 3,000 vehicles deployed across Government Medical Colleges, District Headquarters Hospitals, Primary Health Centres and other Public Health Institutions throughout Tamil Nadu.

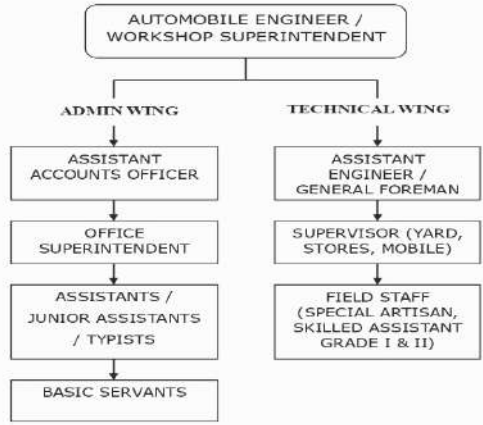
The Department plays a critical role in ensuring uninterrupted healthcare delivery by maintaining operational readiness, safety and roadworthiness of all departmental vehicles through a decentralized network of Regional, District, Mobile and Mini Workshops.

14.2. Organogram:

14.2.1 Directorate



14.2.2 Regional Workshops & District Workshops



14.3. Need for a Comprehensive Vehicle Maintenance Framework

Tamil Nadu has one of the largest public health service delivery networks in India. The increasing utilization of healthcare services, expansion of outreach programmes, emergency medical response systems and public health interventions have substantially increased dependence on vehicle-based service delivery.

More than 3,000 vehicles in the department's maintenance fleet are presently utilized for:

- ❖ Patient transportation services.
- ❖ Public health field activities.
- ❖ Disease surveillance programmes.
- ❖ Mobile healthcare services.
- ❖ Outreach and special health camps.
- ❖ Administrative and supervisory activities.

The continuous expansion of healthcare services necessitates a robust vehicle maintenance ecosystem capable of ensuring maximum fleet availability and operational efficiency.

Fleet Strength (As on 31.03.2026)

Sl. No.	Directorate	Effective Fleet	Grounded Fleet	Total Fleet
1	Public Health and Preventive Medicine	2,081	103	2,184
2	Medical Education & Research	242	42	284
3	Medical and Rural Health Services	230	37	267
4	Family Welfare	173	49	222
5	Tamil Nadu State Health Transport	55	2	57
6	Food Safety & Drug Administration	41	1	42
7	Tamil Nadu State Blindness Control Society	36	3	39
8	Tamil Nadu State Aids Control Society (Blood Bank Vehicles)	34	0	34
9	Indian Medicine and Homoeopathy	18	0	18

10	State Health Society	5	0	5
11	Health and Family Welfare Secretariat	4	0	4
12	Tamil Nadu Health Systems Project	3	0	3
13	Drugs Control	2	0	2
14	Medical Services Recruitment Board	2	0	2
Total		**2,926**	**237**	**3,163**

14.4. Administrative and Workshop service network

The State Health Transport Department is functioning with one Directorate at Chennai, Seven Regional Workshops, Nine District Workshops, Twenty-nine dedicated Mobile Workshops and Nine Mini Workshops spread across Tamil Nadu. The in-position staff strength comprises of 274 - technical staff, 80 - administrative staff and 28 - Basic service staff.

14.5. Services Offered in the Workshops:

- ❖ Major mechanical repairs.
- ❖ Engine and drive-line overhauls.
- ❖ Accident repair.
- ❖ Tinkering and painting.
- ❖ Air-conditioning repairs.
- ❖ Periodical servicing.
- ❖ Fitness certification works.
- ❖ Tyre and battery replacement.
- ❖ Diesel Exhaust Fluid distribution.
- ❖ Repair of special purpose vehicles.

14.6. User specific Innovative Initiatives

14.6.1. Health Department Vehicles Information and Complaint Redressal Programme (HICORP)

HICORP provides a single-window platform for lodging complaints and obtaining information through telephone, WhatsApp and messaging services. The concerned vehicle using officers and drivers, when required utilize this platform to register and resolve all the vehicle related problems and this facility is a significant initiative

taken by the department for maximum and effective utilization of vehicles.

14.6.2. Feedback Monitoring System

Weekly feedback is obtained from vehicle users and drivers to assess service quality and identify areas requiring improvement. Under this programme, inputs are collected every week from each and every driver who has availed the services from the departmental workshops in the preceding week. Apart from immediate rectification of shortcomings, if any, pointed out by them, periodical reviews based on feedback to improve the services offered are also conducted.

14.7. Other Specific activities

14.7.1. Functions other than repairs and maintenance undertaken by the department

- Providing Technical Assistance during the design and fabrication of special purpose vehicles and for Tender Evaluation of purchase / fabrication of new vehicles.

- Identifying the aged vehicles that are uneconomical for further retention and taking

due action for its condemnation and disposal through e-auction.

- ❖ Acting as a repository of all data related to vehicles maintained.

- ❖ Imparting apprenticeship training to the eligible ITI / Diploma / Graduate candidates.

- ❖ Maintaining a Common Pool of un-utilized vehicles and distribute to needy institutions for better fleet utilization.

14.7.2. Training and Skill Development

- ❖ Refresher training along with eye testing is provided to the Drivers of Health and Family Welfare Department to reinforce and upgrade their existing knowledge, skills and also to keep updated on latest changes in Motor Vehicles Act and Rules.

- ❖ Technical Training is regularly provided to the maintenance staff of the department to enhance their skills and keep them updated on new technologies and maintenance techniques.

14.7.3. Special Camps

Special Workshop Camps are organized by this directorate to upkeep the running condition of the vehicles involved in various health care special programmes, natural calamity camps and to provide logistical support to health care providers.

Performance Measures

Sl. No.	Measures	Actual Outcome 2025-2026
1.	Repair jobs undertaken	2,783
2.	Periodical service jobs attended	18,964
3.	Engine Oil changes made	2,827
4.	New Tyres replaced	1,553
5.	New Batteries replaced	730
6.	Vehicle-user feedback calls made	2,632
7.	HICORP complaints resolved	42
8.	Condemned vehicles disposed through e-auction	525
9.	Apprentices trained	17

14.8. Key Issues, Commitments and Goals

14.8.1. Vehicle repair downtime and maintenance cost

The average age of the fleet maintained is 12.8 years and a significant proportion of departmental vehicles have completed substantial operational life, resulting in increased downtime, maintenance requirements and higher repair risks.

The average expenditure on spare parts replacement per vehicle incurred in the year 2025-2026 in the ageing maintenance fleet was Rs.3,967/- while the average repair time for downtime vehicles was 8 calendar days.

Through digital monitoring of departmental workshop activities using an inhouse developed customized database programme, it is proposed to further reduce vehicle repair downtime.

Timely preventive maintenance measures are being undertaken to enhance operational efficiency and extend the service life of vehicle components.

14.8.2 Staff rationalisation

In view of rapid advancement in automobile technology, there is an increasing requirement for more Auto Electricians, and the ageing fleet also demands more bodyshop workers like Tinkers and Painters.

Staff rationalisation would be undertaken to align manpower availability with these requirements and to ensure presence of suitable technicians in the required fields.

14.9. Way forward

The Tamil Nadu State Health Transport Department remains committed to ensuring uninterrupted healthcare delivery by maintaining operational readiness, safety and efficiency of the Government health fleet. Through modernization, digital transformation and citizen-centric initiatives, the Department aims to further strengthen its contribution towards achieving the objectives of the Health and Family Welfare Department.

CHAPTER - 15

THE TAMIL NADU Dr. M.G.R. MEDICAL UNIVERSITY

15.1. Vision & Introduction

The Tamil Nadu Dr. M.G.R. Medical University was established under the Tamil Nadu Dr. M.G.R. Medical University Act, 1987 (Tamil Nadu Act 37 of 1987) and became functional since July 1988. It is the only state owned Medical University in Tamil Nadu and one of the largest Health Sciences Universities in India.

The University is committed to providing quality education, promoting research and innovation, strengthening healthcare services, and developing competent healthcare professionals in alignment with the vision of "Health for All" and "Viksit Bharat 2047".

At present, the University has 844 affiliated institutions with nearly 2,00,000 students pursuing programmes across Medical, Dental, AYUSH, Pharmacy, Nursing, Physiotherapy, Occupational Therapy and Allied Health Sciences.

15.2. Department & Mandate

The University functions under the Chancellor, the Hon'ble Governor of Tamil Nadu, with the Hon'ble Minister for Health and Family Welfare as Pro-Chancellor. The Vice-Chancellor is the principal academic and executive officer.

15.2.1. The mandates of the University is to

- ❖ Provide quality education and training in Health Sciences.
- ❖ Promote research, innovation and dissemination of knowledge.
- ❖ Award degrees, diplomas and academic distinctions.
- ❖ Regulate and monitor standards of affiliated institutions.
- ❖ Support healthcare delivery through academic, research and clinical services.

The University offers undergraduate, postgraduate, super-speciality, fellowship and doctoral programmes and serves as the affiliating authority for all Medical and Health Sciences institutions in Tamil Nadu.

15.3. Key Roles of the Tamil Nadu Dr.M.G.R Medical University

(i) Academic Education and Curriculum Development

The University introduced new academic programmes including D.M. Cardiac Anaesthesia, D.M. Clinical Pharmacology, D.M. Geriatric Mental Health, M.Sc. Psycho-Oncology, M.Sc. Neuroscience and various Post-Doctoral Fellowship Programmes.

Online Certificate Courses on Digital Health and Plasma Exchange therapy for Rodenticide Poisoning were also launched.

The University also conducts postgraduate degree programmes through the Departments of Transfusion Medicine and Epidemiology, namely M.D. (Immuno-Haematology and Blood Transfusion), M.Sc. (Epidemiology), M.Sc. (Public Health), and M.Sc. (Biostatistics)**,** with an approved intake of 5, 4, 16, and 4 seats**,** respectively.

(ii) Research support to students is extended through

- ❖ Short-Term Studentship (STS) Programme for Undergraduate students.
- ❖ Ph.D. Fellowship Programme
- ❖ University Research Grants

The University presently has 104 recognized Ph.D. Centres and 418 recognized Ph.D. Guides and Co-Guides.

(iii) Clinical *and Public Health Services*

The University provides:

- ❖ Siddha Outpatient Services: 7500 new patients have been enrolled since 2014.
- ❖ Blood Component Services: Out of 3,131 units of blood collected**,** 2,034 blood components were provided to patients.
- ❖ HIV National Reference Laboratory Services
- ❖ Leptospirosis Testing Services
- ❖ Autoimmune and Hepatitis Diagnostic Services
- ❖ Research and Bio statistical Support Services

(iv) Collaborations *and International Engagement*

The University entered into collaborations with:

- University of Jaffna
- Christian Medical College, Vellore
- Society of Emergency Medicine India
- Apex Skill Lab
- ECHO India
- Telemedicine Society of India

These collaborations promote Research, Telemedicine, Digital Health and Academic exchange avenues.

15.4. Flagship Schemes & Programmes

(i) Theme-Based Research (TBR) Initiative

The University continues to implement Theme-Based Research Projects under the themes of Artificial Intelligence, Personalized Healthcare and Technology-enabled Public Health. Pilot grants

ranging from Rs.2 lakh to Rs.10 lakh are being sanctioned.

(ii) Ph*.D. Fellowship Programme*

Provision has been made for extending financial assistance to full-time Ph.D. scholars at the rate of:

- Rs.30,000 per month (First Year)
- Rs.40,000 per month (Second Year)
- Rs.50,000 per month (Third Year)

(iii) Short-Term Studentship (STS) Programme

The programme promotes research aptitude among undergraduate students and encourages evidence-based Health Research.

As part of the National Medical Commission's Medical Soft Skills (NATCOM) Programme, orientation training on Medical Soft Skills is being implemented through 15 centres across Tamil Nadu for undergraduate medical students.

(iv) Emergency Care Skills and ethical practice Programme

Since 2025, training in Advanced Life Support (ALS), Basic Life Support (BLS), Good Clinical Practice (GCP), and Good Laboratory Practice (GLP) has been conducted for more than 9,600 Postgraduate Medical students across the State.

(v) Continuing *Educational Programme* (CEP)

Continuing Medical Education (CME) programmes which is a priority area of the University are regularly conducted by the various Departments of the University.

Financial assistance is also provided to affiliated institutions for holding Continuing Educational Programmes.

(vi) Sports Grant Scheme

As part of encouraging sports and physical excellence, selected institutions are provided Sports Grants to conduct Inter-Disciplinary Sports Meets.

15.5. Achievements of the Year (2025–2026)

(i) Research and Innovation

- Nearly 75 Research publications made.
- Five patents are under process.
- The University faculty members and researchers as per mandate, regularly publish research articles in reputed PubMed-indexed and Scopus-indexed journals.
- Department of Epidemiology has organized 38 workshops for Medical and Dental postgraduates, AYUSH postgraduates, Ph.D. scholars, and Research Guides, benefiting 5,102 participants. In addition, Six (6) online workshops on Research Methodology have been held for 4,564 MBBS students.
- Observance of University Research Day on 26th September saw receipt of 1,164 Research abstracts.
- Shine Healthcare Hackathon 2025 was organized in collaboration with StartupTN.

- *Inspiring Young Minds* – A Research Awareness Programme for School Students held on 09 October 2025 for students from across 30 schools in Chennai.
- An one-day Workshop on Artificial Intelligence in Healthcare was organized on 11th September 2025 in collaboration with Anna University, Indian Institute of Technology Madras, and Vellore Institute of Technology.
- An Intellectual Property Rights (IPR) Workshop was organized on 13th March 2025 in collaboration with the Tamil Nadu State Council for Science and Technology.
- An one-day workshop on Research Methods and Artificial Intelligence in Healthcare was held on 12th March 2026 in collaboration with Elsevier.
- A Virtual Conference on Emergency and Austere Medicine was conducted on 07th February 2025, featuring an expert speaker from the United States.

- ❖ The International Homoeopathic Conference – 2025 was held on 14th February 2025.
- ❖ The International Conference on the Role of Siddha Medicine in Public Health was conducted on 21st February 2025.
- ❖ The International Conference on "Naturopathy in the Digital Era: Balancing Technology and Natural Healing" was conducted on 17th April 2025.
- ❖ Tamil Vara Vizha was celebrated on 29th April 2025 with great enthusiasm, dignity and wide reception.
- ❖ The International Conference on the Future of Physiotherapy was conducted on 29th April 2025.
- ❖ International Yoga Day was observed on 21st June 2025.

(ii) Research Infrastructure

- ❖ The Governing council of the Tamil Nadu Dr. M.G.R. Medical University has accorded

approval for the establishment of the Paediatric Post Graduate Institute of Medical Education and Research (PPGIMER) within the campus of the King Institute of Preventive Medicine (KIPM), Guindy, Chennai at an estimated cost of Rs.338.70 crore. Hence, the Budget Estimate for the year 2026–2027 has been appropriately enhanced to accommodate the cost towards the construction of the new Paediatric Hospital.

(iii) Academic Achievements

- 95 new Ph.D. Guides and Co-Guides recognized.
- 121 Ph.D. scholars awarded degrees.
- 88 new Ph.D. scholars enrolled.

(iv) Digital Infrastructure

- The digital infrastructure of affiliated Government Medical Colleges for evaluation of answer scripts in controlled Environment have been strengthened with 250 computers and 25 printers.

(v) Awards and Recognition

- ❖ 59 Best Researcher Awards.
- ❖ 25 Best Outgoing Student Awards.
- ❖ 47 Institution Excellence Awards.

(vi) Student Welfare

- ❖ Thirty (30) Continuing Educational Programmes have been supported with grants amounting to Rs.7.23 lakh.

15.6. New Initiatives (2026–2027)

15.6.1. The University took the following new initiatives

1. Construction of three additional floors in the University Guest House at an estimated cost of Rs.21 crore.
2. Establishment of a High-End Simulation Laboratory for advanced Clinical training and research.
3. Expansion of Artificial Intelligence-enabled healthcare research.
4. Strengthening Telemedicine and Digital health education.

5. Promotion of translational and community-based healthcare research.
6. Expansion of interdisciplinary academic and research collaborations.
7. Rs.1.27 crore released for Multi-Disciplinary Research Units.
8. Rs.23.73 crore released for five University Research Centres.
9. Upgradation of lecture halls with provision of smart class room facilities has been supported in 25 Government Medical Colleges.

15.7. Conclusion

The Tamil Nadu Dr. M.G.R. Medical University continues to play a pivotal and crucial role in the area of Medical education, Research, Innovation and Public Health services. Through its extensive network of affiliated institutions, research initiatives, digital transformation programmes and community-oriented healthcare projects, the University remains committed to producing competent healthcare professionals and advancing the vision towards "Health for All".

CHAPTER - 16

DIRECTORATE OF DRUGS CONTROL

16.1. Vision

The Directorate of Drugs Control Administration has its vision, to ensure access to safe, effective, quality-assured and affordable medicines across Tamil Nadu by strengthening inspection and enforcement systems, modernizing laboratories and achieving a high-maturity, risk-based, globally aligned regulatory ecosystem that supports growth and innovation.

The Directorate is moving from a primarily "license–inspection–prosecution" model to a mature, data-driven, internationally aligned regulatory system that guarantees safe, effective, affordable medicines while supporting a globally competitive Pharma industry.

16.2. Pillars of the mission

Public Health Protection	Regulatory Excellence
❖ Ensure quality, safety, and efficacy of drugs, medical devices, and cosmetics ❖ Safe supply of blood and blood components ❖ Curb misleading advertisements of drugs ❖ Monitor dispensing of prescription-only drugs	❖ Risk-based inspection and digital compliance tools ❖ Strengthen laboratory capacity to global testing standards ❖ Transparent, industry-engaged and public-aware regulation ❖ Build professional, globally aligned and well-trained workforce

16.3. Introduction

Tamil Nadu is one of India's important Pharmaceutical Hubs with a strong presence of formulation manufacturing, Active Pharmaceutical Ingredients (APIs), medical devices, a robust distribution network, and a large retail Pharmacy ecosystem. The State has a high volume of medicine availability, growing exports, and an increasingly competitive domestic market.

However, the sector faces challenges related to upgradation of Good Manufacturing Practices (GMP), digitization**,** pharmacy oversight**,** fragmented supply chains, and emerging threats like Misuse of Habit-forming drugs, growing anti-microbial resistance (AMR) and online-Pharmacy.

16.3.1. Strong Manufacturing Base

- Large number of licensed formulation / Medical devices manufacturers across Chennai, Chengalpet, Kancheepuram, Thiruvallur, Hosur, Vellore, Villupuram, Coimbatore, and Madurai clusters.
- Presence of both small-scale units and medium/large GMP-compliant units.
- Production includes:

- Tablets, capsules, liquids and injectables
- Active Pharmaceutical Ingredients and intermediates
- Medical Oxygen plants
- Medical devices
- Cosmetics

16.3.2. Skilled Workforce

- Strong pool of Pharmacists, Chemists, QA/QC staff and formulation scientists.
- Proximity to academic institutions facilitates talent generation.

16.3.3. Robust Retail & Wholesale Ecosystem

- Dense network of Pharmacies across Urban and Rural areas.
- Availability of chain of pharmacies ensures monitoring standardization.
- Widespread use of generics keeps costs affordable.

Directorate of Drugs Control Administration regulates manufacture, sale and distribution of drugs, medical devices, in-vitro diagnostics and cosmetics, operates statutory laboratories, licenses blood centres and storage centres, and explicitly

focuses on combating spurious/Not of Standard Quality drugs (NSQ), excess pricing and misleading advertisements.

The department is moving towards more digital, risk-based regulation and online licensing system via TNSWP/ONDLS, publication of NSQ lists and lab modernisation, but faces challenges in meeting the demands and growing expectation.

16.4. Issues and Evidences:

16.4.1. Compliance Gaps Among Small / Medium-Scale Manufacturers

- Many small/Medium scale manufacturers struggle with upgrading to revised Schedule M/WHO-GMP standards.
- Risk based inspections (RBI) have been conducted as detailed below:

Sl. No.	Risk based inspections (RBI)	No. of inspections	Non-Compliances observed
1.	Drug Manufacturing units	27	27
2.	Private Testing Laboratories	14	13
3.	Cough Syrup Manufacturing Units	96	60
Total		**137**	**100**

16.4.2. Retail & Distribution Challenges

- Misuse of Habit-forming drugs
- Online sale of drugs
- Sale of Prescription Drugs without prescription by a Registered Medical Practitioners (RMP) which concentrates on Habit forming drugs, antibiotics, Abortion pills and steroids.
- Supply of drugs formulations from other States.
- High fragmentation among stockists and sub-stockists.
- Need for robust track-and-trace system for preventing counterfeit entry.
- Out of 61,701 inspections conducted at sales units, legal actions have been initiated against 443 Sales concerns for violation of Drugs Rules, 1945.

16.4.3. Regulatory Capacity Limitations

- Fragmented data and limited risk-based working: Licensing, inspection, lab and legal data are dispersed across systems; risk-based inspection and market

surveillance are nascent for both manufacturers and sales units.

- Market challenges: The Continued threats from spurious/adulterated/NSQdrugs, excess pricing, misleading advertisements, and weak community-level Pharmacovigilance.
- Enforcement & Compliance Gaps: Rising market complexity due online pharmacies and counterfeit risks pose challenges such as tracking supply chain violations and Limitation in analytics for identifying high-risk manufacturers or distributors.
- Laboratory & Quality System Gaps: Insufficient capacity for advanced testing of biologics and novel dosage forms calls for modernization and accreditation of drugs testing laboratories.

16.4.4. Public Awareness & Education Gaps

- Lack/low awareness among the public about drugs which are Not of Standard quality / spurious/adulterated drugs.
- Inadequate awareness on reporting about Adverse events/complaints.

16.5. Directorate's Commitment And Deliverables

16.5.1. Licences

- All licences and approvals are issued entirely through online portals with zero physical interface. All Drug Sales and Manufacturing Licences are processed via Tamil Nadu Single Window Portal (TNSWP), ONDLS Portal and CDSCO MD Online Portal (Government of India)

16.5.2. Digital and data backbone

- Software for regulatory data warehouse and dashboards: The real-time views of licences, inspections, lab results, prosecutions, NSQ trends and district-wise Key performance indicators (KPI) to drive risk-based planning and public reporting.

16.5.3. Partnerships and transparency

- This Directorate works closely with Public hospitals, Chemist associations, Industry bodies and Academic centres to promote

rational use, Pharmacovigilance, and self-regulation, especially for high-risk areas like antimicrobials and Habit forming drugs.

16.5.4. State drug testing laboratories

- Two Drugs testing laboratories of this State analyse 15,000 samples/year for early detection of Not of standard quality drugs (NSQ) and spurious drugs and to prevent circulation of ineffective or harmful medicines

16.5.5. Habit-Forming Drug Enforcement

- The Directorate maintains stringent surveillance over Schedule H1 drugs, including Tapentadol, Tramadol, Codeine Phosphate, and Nitrazepam, which are prone for abuse.

16.6. Financials

Table 16.1: Revenue Receipts

Licence Fees (Sales & Mfg.)	Court Penalties (Prosecutions)	Total Revenue
Rs.9.28 Cr	Rs.1.06 Cr	Rs.10.34 Cr

Table 16.2: Major Ongoing Schemes — Budget Outlay

Project	Central: State Share	Total Cost (Rs.in crore)
Medical Devices Testing Lab, Coimbatore	60:40	29.67
Strengthening Quality Control Lab - King Institute, Guindy	60:40	20.37
Zone Office Building (Tirunelveli)	60:40	0.75
Zone Office Building (Tiruchirappalli)	60:40	0.75
Total	**51.54 crore**	

16.7. Key performance indicators

16.7.1. Licensing, Inspection and compliance

- 100 % of manufacturing and sales licence applications disposed off within notified timelines.
- 100% coverage of all manufacturers and sales units inspected annually.
- ≥ 80% first-inspection compliance rate for manufacturing sites.
- Declining violation rates year-on-year.

- 100% Digital licensing and inspection workflows.

16.7.2. Product quality

- Establish at least 2 new advanced-quality testing laboratories.
- Year-on-year reduction in NSQ detection rate;
- 100% regulatory action on NSQ/spurious detections within defined timelines.

16.7.3. Public Awareness & Service Delivery

- Launch statewide Safe Medicine Use Campaign reaching 75% population in 2026-2027.

16.8. Annexure

Table 16.3: Performance Highlights

Total Inspections	Samples Drawn	Samples Tested	NSQ Declared
64,402	14,281	16,059	338
Legal Actions on Sales	**Licences Suspended**	**Licences Cancelled**	**Prosecutions Launched**
443	264	43	160

** includes carry-over samples of the previous year*

16.8.1. Inspection and Legal action

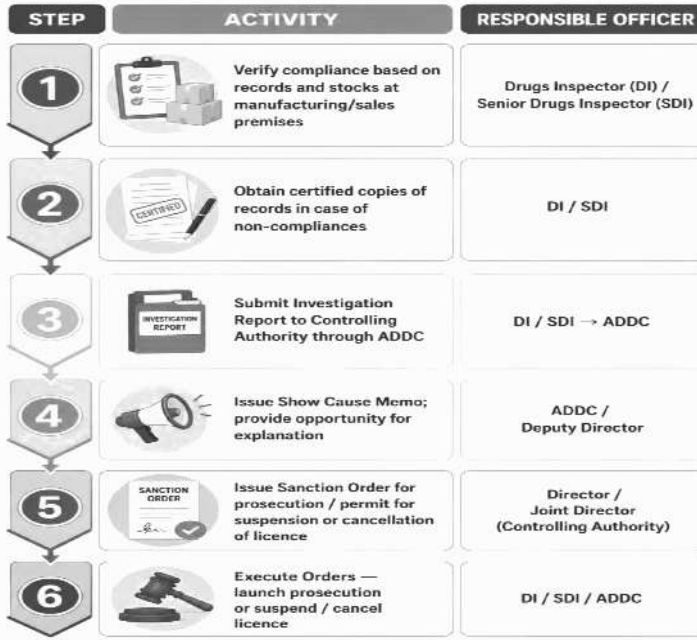


Table 16.4: Inspection Details (2025-26)

Category	Total Number of Units/sales concerns/Blood Centres	Number of Inspections	Inspection Coverage (%)
Manufacturing Concerns	1030	2,083	202
Sales Concerns (Retail / Wholesale) & Hospitals and Other Medical Stores	61,482	61,701	100

Blood Centres/Blood storage centres	360	618	171.6

16.8.2. Drugs Sampling and Testing

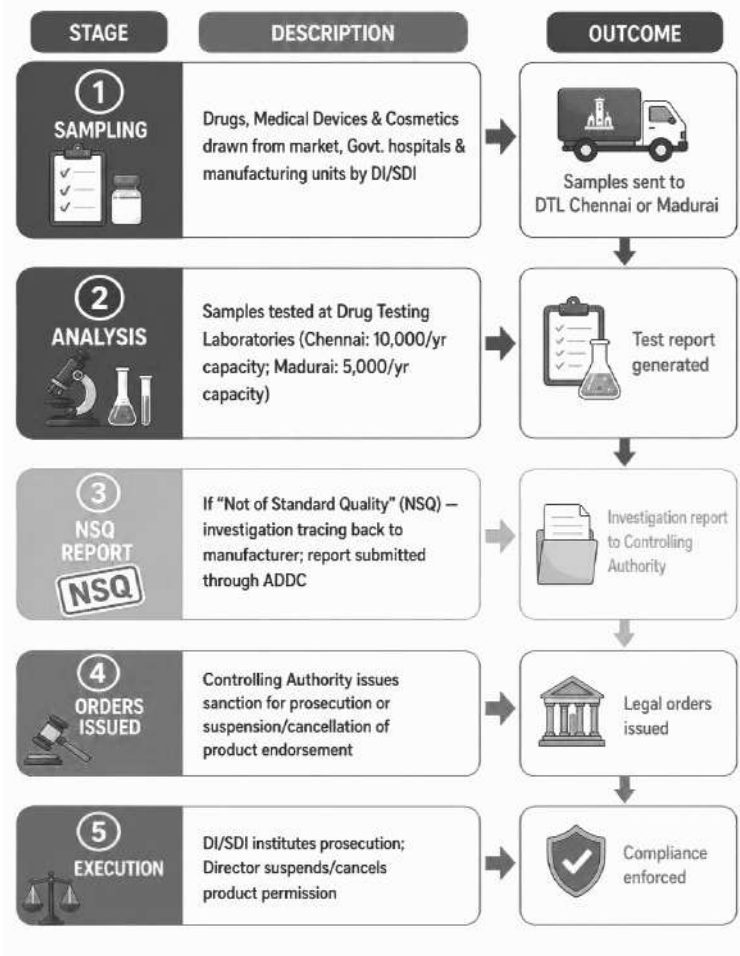


Table 16.5: Sampling & Quality Testing

Year	Samples Drawn	Samples Tested	NSQ Declared	NSQ Rate (%)
2025-2026	14,281	16,059	338	2.10

Table 16.6: Enforcement Actions taken based on inspection including misuse of habit-forming drugs

Action	2025-2026
Legal actions vs. Sales concerns	443
Sales licences Suspended	264
Sales licences Cancelled	43
Prosecutions vs. Sales concerns	160

Table 16.7: Enforcement Actions taken against Substandard and Spurious Drugs

Action	2025-2026
Criminal cases for NSQ / Spurious drugs	125
Product endorsements Suspended (NSQ)	38
Product endorsements Cancelled (NSQ)	6

16.9. Licences in Force (as on 31.03.2026)

16.9.1. Licensing Process

Step	Process	Timeline / Platform
1	Application submitted online by applicant	Tamil Nadu Single Window Portal (TNSWP)
2	Inspection by Drugs Inspector / Senior Drugs Inspector	Within prescribed timeline
3	Verification	By ADDC / DI
4	Approval by Licensing Authority	Review and approval
5	Licence issued through TNSWP portal	Timeline specified in Act & Rule

Table 16.8: Sales Licences

Type	Count
Retail Licences	42,678
Retail with Wholesale	4,795
Wholesale Licences	13,201
Restricted Licences	424
Homoeopathic Licences	384

Table 16.9: Manufacturing Licences

Type	Count
Allopathic Regular	310
Allopathic Loan	80
Cosmetic	338
Cosmetic Loan	10
Medical Devices	283
Homoeopathic	9

Table 16.10: Licences issued for Blood Centres and Blood Storage Centres

Category	Count
State Government Blood Centres	104
Central Government (incl. ESI, Military)	08
Private Blood Centres	248
Umbilical Cord Blood Stem Cell Banks	3
Government Blood Storage Centres	440
Private Blood Storage Centres	200

16.10. Work Force

Table 16.11: Technical Staff

Level	Post	Count	Key Responsibility
Head of Department	Director of Drugs Control	1	Overall administration & review
Joint Directors	Joint Director of Drugs Control	2	State Licensing Authority; Manufacturing licences;
Deputy Directors	Deputy Director of Drugs Control	2	Blood Centres, Blood Storage Centres; Cosmetics licences
Zonal Level	Assistant Director of Drugs Control (ADDC)	26	Sales licensing; Surprise raids; Enforcement oversight
Field Level	Senior Drugs Inspector	14	Inspection, sampling, prosecution, investigations
	Drugs Inspector	146	Inspection, sampling, prosecution, investigations

Level	Post	Count	Key Responsibility
Lab Wing	Govt. Analyst	02	Testing of Drug, Cosmetics & Medical device samples
	Deputy Govt. Analyst	03	Testing of Drug, Cosmetics & Medical device samples
	Senior Analyst	20	Testing of Drug, Cosmetics & Medical device samples
	Junior Analyst	44	Testing of Drug, Cosmetics & Medical device samples
	Technician Grade I	06	Assisting Analyst

Table 16.12: Ministerial Staff

Sl. No.	Name of the Post	No. of Posts
1.	Assistant Director of Drugs Control (Admin)	1
2.	Assistant Accounts Officer	1
3.	Legal Adviser	1
4.	Junior Administrative Officer	1

Sl. No.	Name of the Post	No. of Posts
5.	Lab Technician	4
6.	Laboratory Attendant	7
7.	Office Superintendent	18
8.	Assistant	44
9.	Steno Typist Grade I	1
10.	Steno Typist Grade III	2
11.	Junior Assistant	51
12.	Typist	17
13.	Other Categories	97

Table 16.13: Enforcement of Legislative Framework

Sl. No.	Act / Rule	Year	Scope
1.	Drugs and Cosmetics Act	1940	Core regulation of drugs, cosmetics manufacture, distribution & sale
2.	Drugs Rules	1945	Detailed provisions for licensing, inspection, and sampling
3.	Drugs & Magic Remedies (Objectionable Advertisements) Act & Rules	1954 / 1955	Prohibition of misleading drug advertisements

Sl. No.	Act / Rule	Year	Scope
4.	Drugs (Prices Control) Order	2013	Price ceiling for essential medicines under Essential Commodities Act
5.	Medical Devices Rules	2017	Regulation of Medical devices
6.	New Drugs and Clinical Trial Rules	2019	Oversight of new drugs and clinical investigations
7.	Cosmetics Rules	2020	Manufacture, import, and sale of cosmetics
8.	Drugs & Cosmetics (Compounding of Offences) Rules	2025	Compounding mechanism for offences under D&C Act

16.11. Conclusion

The Directorate of Drugs Control Administration, Tamil Nadu is committed to working towards a year-on-year reduction in Not-of-Standard-Quality (NSQ) drug detection rates by strengthening regulatory capacity, which is being achieved with a robust regulatory framework, combined with substantial up gradation of drug testing laboratory capacity, that guarantees safe, effective, affordable medicines for all citizens.

CHAPTER - 17

CONCLUSION

The Department of Health and Family Welfare has been a growth driver for the State and the Nation. Through remarkable achievements in health infrastructure, technology and the delivery of quality healthcare services, the State has already attained key health indicators and is well-positioned to achieve its planned targets ahead of the stipulated timelines.

Tamil Nadu ranks among the foremost States on every major health indicator - the product of a robust public health system, a strong public health cadre, skilled human resources and a complete network of Primary, Secondary and Tertiary Institutions. A proven drug procurement and distribution system, the Chief Minister's Comprehensive Health Insurance Scheme (CMCHIS) for the underprivileged, a transparent recruitment system and the largest network of

Government Medical Colleges and Hosptials in the country have placed the State firmly on the road to excellence.

In 2026-2027, the Government will carry this legacy forward-deepening non-communicable disease control, strengthening trauma and emergency care, expanding financial protection and building the digital foundations of a health system that reaches every household. Tamil Nadu's pioneering schemes serve as a model for the country; the Government is committed to ensuring they serve, above all, the people of Tamil Nadu.

Dr. K. G. Arunraj

Minister for Health,
Medical Education and Family Welfare



The Hon'ble Chief Minister of Tamil Nadu, Thiru. C. Joseph Vijay, inaugurated Medical Infrastructures constructed under the National Health Mission through video conferencing.



The Hon'ble Chief Minister of Tamil Nadu, Thiru. C. Joseph Vijay, conducted inspection at Government Maternal and Child Health Hospital, Egmore.



The Hon'ble Chief Minister of Tamil Nadu, Thiru. C. Joseph Vijay, handed over the keys to the drivers of 40 New Vehicles procured for the use of the Directorate of Medical and Rural Health Services.



The Hon'ble Chief Minister of Tamil Nadu, Thiru. C. Joseph Vijay, driving the New Vehicle that was provided for the use of the Directorate of Medical and Rural Health Services.



The Hon'ble Chief Minister of Tamil Nadu, Thiru. C. Joseph Vijay, administered Polio drops to children at the National Pulse Polio Immunization Special Camp.



The Hon'ble Chief Minister of Tamil Nadu, Thiru. C. Joseph Vijay, Presented gifts to children at the National Pulse Polio Immunization Special Camp.



The Hon'ble Minister for Health, Medical Education and Family Welfare, Dr. K.G. Arunraj, presented appreciation certificates and prizes to the girl students who won the World Population Day Awareness Competitions.



The Hon'ble Chief Minister of Tamil Nadu, Thiru. C. Joseph Vijay, inaugurated Medical Infrastructures constructed under the Tamil Nadu Health Systems Project through Video Conferencing.



The Hon'ble Chief Minister of Tamil Nadu, Thiru. C. Joseph Vijay, inaugurated the **"Nalam TN"** web portal, developed under the Tamil Nadu Health Systems Project.



The Hon'ble Chief Minister of Tamil Nadu, Thiru. C. Joseph Vijay, distributed appointment orders to 751 Assistant Surgeons, he stepped down from the stage and presented appointment orders to Dr. B. Prashanthi, a differently abled Assistant Surgeon.



The Hon'ble Chief Minister of Tamil Nadu, Thiru. C. Joseph Vijay, presented appointment orders to the candidates selected for appointment to 1,393 post of Health Inspectors (Grade-II).



The Hon'ble Chief Minister of Tamil Nadu, Thiru. C. Joseph Vijay, presented appointment orders to the selected Assistant Surgeons and Health Inspectors (Grade-II), took Group Photo with them.

நலம் TN







